UPMC Health Plan
POLICY AND PROCEDURE MANUAL

SUBJECT: **Commercial, Exchange Prior Authorization Policies**
INDEX TITLE: **Clinical Pharmacy Services**
ORIGINAL DATE: **December 2019**

This policy applies to the following lines of business: (Check those that apply.)

COMMERCIAL	CMS-MA	DHS-MA	ANCILLARY
HMO ( )	PA ( )	Health Choices/PH ( )	Dental ( )
PPO ( )	Ohio ( )	Health Choices/BH ( )	Vision ( )
Fully Insured ( )	All ( )	All ( )	COBRA ( )
Self Funded ( )			All ( )
Marketplace HMO ( )	HMO ( )	**CHIP**	
Marketplace PPO ( )	PPO ( )	CHIP ( )	**WORKPARTNERS**
Marketplace EPO ( )	DSNP ( )		Commercial WC ( )
Indiv. Off Exchange ( )	Part D ( )	**COMMUNITY HEALTHCHOICES**	WCTPA ( )
All (X)	All ( )		Disability Svcs. ( )
		CHC/MLTSS ( )	
CDHP	**GOVT**		Life Solutions/Health Promo ( )
HSA ( )	FEHBP (X)	**COMMUNITY CARE**	Life Solutions/EAP ( )
HRA ( )	PSHBP (X)	Community Care/BH ( )	HCMS Group ( )
HIA ( )			eBenefits Solutions ( )
FSA ( )			On Site Centers ( )
All ( )			All ( )

Reference State Addendums for:
Delaware ( ) Maryland ( ) New Jersey ( ) Ohio ( ) Virginia ( ) West Virginia ( ) Wisconsin ( )

I. POLICY

It is the policy of UPMC Insurance Services Division to maintain a prior authorization process that promotes appropriate utilization of specific drugs with potential for misuse or limited indications. This process involves a review jarousing Food and Drug Administration (FDA) criteria to make a determination of Medical Necessity, and approval by the Pharmacy & Therapeutics Committee of the criteria for prior authorization, as described in RX.003.2 Prior Authorization Process – Commercial, Exchange and RX.008.2 Exceptions Due to Medical Necessity – Commercial, Exchange.

The included medications are subject to the prior authorization process.

II. DEFINITIONS

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2013 ACR/EULAR criteria for the classification of systemic sclerosis (SSc)

Item	Sub-item(s)	Weight/ score¶
Skin thickening of the fingers of both hands extending proximal to the metacarpophalangeal joints *(sufficient criterion)*	-	9
Skin thickening of the fingers *(only count the higher score)*	Puffy fingers	2
	Sclerodactyly of the fingers (distal to the metacarpophalangeal joints but proximal to the proximal interphalangeal joints)	4
Fingertip lesions *(only count the higher score)*	Digital tip ulcers	2
	Fingertip pitting scars	3
Telangiectasia	-	2
Abnormal nailfold capillaries	-	2
Pulmonary arterial hypertension and/or interstitial lung disease *(maximum score is 2)*	Pulmonary arterial hypertension	2
	Interstitial lung disease	2
Raynaud phenomenon	-	3
SSc-related autoantibodies (anticentromere, anti-topoisomerase I [anti-Scl-70], anti-RNA polymerase III) *(maximum score is 3)*	Anticentromere Anti-topoisomerase I Anti-RNA polymerase III	3

¶ The total score is determined by adding the maximum weight (score) in each category. Patients with a total score of ≥ 9 are classified as having definite SSc.

5-alpha-reductase inhibitor – type of medication which reduces the size of the prostate gland by preventing the conversion of testosterone to dihydrotestosterone in the prostate, hair follicles, and other androgen-sensitive tissues. Studies have shown that men with lower concentrations of dihydrotestosterone have smaller prostate glands and do not develop benign prostatic hyperplasia (BPH).

$5HT_3$ – subtype of serotonin receptor

Abnormal Involuntary Movement Scale (AIMS) – a 12-item clinician-rated scale which uses items 1 through 7 to assess the severity of involuntary movements across different body regions. Each of the 7 items is scored on a 0 to 4 scale and is rated as follows:

- 0 = no dyskinesia

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- 1 = low amplitude, present during some but not most of the exam
- 2 = low amplitude and present during most of the exam (or moderate amplitude and present during some of the exam)
- 3 = moderate amplitude and present during most of exam
- 4 = maximal amplitude and present during most of exam

The AIMS dyskinesia total score (sum of items 1 to 7) could range from 0 to 28, with a decrease in score indicating improvement.

Absolute Neutrophil Count (ANC) – total white blood cell count (cells/µL) x % (neutrophils + bands). Bands represent immature neutrophils.

ACE-I – angiotensin converting enzyme inhibitor

Achondroplasia - the most common form of skeletal dysplasia and is caused by pathogenic variants in the fibroblast growth factor receptor 3 (*FGFR3*) gene. Patients with achondroplasia have gain-of-function variant in the *FGFR3* gene. Two specific pathogenic variants in the *FGFR3* gene account for almost all cases of achondroplasia. These variants occur at the same nucleotide in the *FGFR3* gene, 1138G>A (98%) and 1138G>C (1%), in both cases resulting in a glycine-to-arginine substitution in amino acid 380 (p.Gly380Arg) in the transmembrane domain of the *FGFR3* gene. This variant permanently activates the FGFR3 receptor, inhibiting chondrocyte proliferation, which ultimately leads to impaired endochondral bone formation, growth restriction, bone shortening, and other skeletal anomalies.

Acquired thrombotic thrombocytopenic purpura (aTTP) – a rare thrombotic microangiopathy characterized by thrombocytopenia and hemolytic anemia.

Acromegaly – a disease characterized by hypersecretion of growth hormone (GH) which is most commonly caused by a pituitary somatroph adenoma. Increased circulating levels of GH stimulate increased hepatic production of insulin-like growth factor-1 (IGF-1). Excess IGF-1 causes increased growth of bones and soft-tissues while excess GH can cause co-morbid conditions such as diabetes mellitus, hypertension, and increased cardiovascular risk. Lower circulating GH levels can cause a reversal in the co-morbid diseases and a decrease in IGF-1.

Acroparesthesias – intermittent episodes of burning pain in the extremities; neuropathic or limb pain.

ACTH – adrenocorticotropic hormone

Activated phosphoinositide 3-kinase delta syndrome (APDS) – ultra-rare, inherited primary immune disease caused by a genetic phosphoinositide 3-kinase delta (PI3Kδ) mutation with a pathologic variant in PIK3CD or PIK3R1 genes

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Activated prothrombin complex concentrate (aPCC) – bypassing agent that activates the coagulation pathway independently of factor VIII

Acute Chest Syndrome – an acute pulmonary complication of sickle cell disease characterized by lower respiratory tract symptoms (e.g. cough, shortness of breath, retractions, and rales) and a new pulmonary infiltrate on chest radiograph.

Acyanotic Heart Disease – hemodynamically non-significant heart defect present at birth that does NOT result in a low blood oxygen level (e.g., patent ductus arteriosus [PDA], atrial septal defect [ASD], atrioventricular septal defect [ASVD], ventricular septal defect [VSD], pulmonary stenosis, aortic stenosis.)

ADAMTS13 (a disintegrin and metalloproteinase with a thrombospondin type 1 motif, member 13) – an enzyme that cleaves von Willebrand factor, a large protein involved in blood clotting.

Adenosine deaminase (ADA) – an enzyme that catalyzes the conversion of adenosine and deoxyadenosine to inosine and deoxyinosine

Adolescent – at least 12 years of age

Adult body mass index (BMI) – BMI is interpreted using standard weight status categories. These categories are the same for men and women of all body types and ages. The standard weight status categories associated with BMI ranges for adults are shown in the following table.

BMI	Weight Status
Below 18.5	Underweight
18.5 – 24.9	Normal or Healthy Weight
25.0 – 29.9	Overweight
30.0 and Above	Obese

AIDS – Acquired Immunodeficiency Syndrome

Alagille syndrome (ALGS) – a rare, inherited, autosomal dominant disorder, caused by mutations in the genes encoding *JAG1* or *NOTCH2,* both involved in the Notch signaling pathway. It is a multisystem disease that presents in childhood with a broad range of clinical manifestations, but primarily affects the liver, skin, heart, eyes, face, and skeleton. ALGS can cause a range of debilitating complications, including cholestasis, pruritus, progressive liver disease, failure to thrive, and xanthomas, all of which can lead to liver transplantation.

ALK – anaplastic lymphoma kinase

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Allogeneic stem cell transplant – the process of receiving stem cells from a genetically similar, but not identical, donor. The donor may be related or un-related.

Alopecia areata – chronic autoimmune skin disorder that causes hair loss, usually on the scalp and face, but can also affect hair on other parts of the body.

Alpha-1 Antagonist – type of medication which prevents bladder obstruction by relaxing smooth muscle in the bladder neck, prostate capsule, and prostatic urethra. Alpha-1 receptors are plentiful in the prostate and base of the bladder and the density of these receptors is increased in hyperplasic prostate tissue.

Alpha$_1$-Antitrypsin Deficiency – a rare genetic condition characterized by low levels of serum alpha$_1$-antitrypsin (AAT). AAT is a serine protease inhibitor that inhibits neutrophil elastase (NE). Neutrophil elastase degrades elastin and other extracellular matrix components. The imbalance between the AAT and NE increases the risk of emphysema. AAT deficiency also increases the risk of liver disease and several other conditions.

ALT – alanine aminotransferase

Amyotrophic Lateral Sclerosis (ALS) – a progressive and fatal neurodegenerative disorder characterized by loss of motor neurons in the spinal cord, brainstem, and motor cortex resulting in signs and symptoms of muscle wasting and weakness, spasticity, fasciculation, cramps and respiratory failure leading to death

Angiokeratomas – a dermatologic feature of Fabry disease; pin-point red/purple skin lesions raised above the surrounding skin and scattered over the trunk, limbs, and genitals, and less commonly on areas such as the head, face, and oral mucosa.

Angiomyolipoma – kidney neoplasm made up of tissues resembling blood vessels, smooth muscle, and adipose. These lesions are typically benign and may occur sporadically or in association with TSC and/or TSC-LAM.

Ankle/Brachial Index – an assessment of peripheral arterial disease risk that compares blood pressure measured at the ankle with the blood pressure measured at the arm. A low index value indicates narrowing in the vessels in the leg, which can increase the risk of circulatory problems, heart disease, and stroke.

Appendix A. ORAL AND TOPICAL ONCOLOGY PRODUCTS REQUIRING PRIOR AUTHORIZATION (See Oral and Topical Oncology Agents for general criteria)

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Representative Drug Name	Additional Criteria
Abemaciclib (Verzenio™)	N/A
Abiraterone 125mg (Yonsa®)	N/A
Abiraterone 250mg, 500mg (Zytiga®)	○ For abiraterone (Zytiga) 500mg tablet: ▪ Must provide clinical rationale for use of 500mg tablet instead of 250mg tablet. Clinical rationale must include why 250mg tablet would not be expected to produce the same clinical results as 500mg tablet given it contains the same chemical entity.
Acalabrutinib (Calquence®)	N/A
Adagrasib (Krazati™)	○ For members previously taking sotorasib (Lumakras), must have chart documentation showing disease progression did not occur while on sotograsib (Lumakras)
Afatinib (Gilotrif®)	N/A
Alectinib (Alecensa®)	N/A
Alpelisib (Piqray®)	N/A
Apalutamide (Erleada™)	N/A
Asciminib (Scemblix®)	○ For adults patients with newly diagnosed Ph+ CML in chronic phase, must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to a generic BCR-ABL inhibitor (e.g. imatinib, dasatinib, or nilotinib)
Avapritinib (Ayvakit™)	N/A
Avutometinib, defactinib (Avmapki Fakzynja Co-Pack)	N/A
Axitinib (Inlyta®)	N/A
Azacytidine (Onureg®)	N/A
Belzutifan (Welireg®)	N/A
Bexarotene (Targretin®)	N/A
Binimetinib (Mektovi®)	N/A
Bosutinib (Bosulif®)	○ For adult and pediatric patients 1 year of age and older with chronic phase Ph+ CML, newly diagnosed, must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to a generic BCR-ABL inhibitor (e.g. imatinib, dasatinib, or nilotinib)
Brigatinib (Alunbrig™)	N/A
Cabozantinib capsule (Cometriq®)	N/A
Cabozantinib tablet (Cabometyx™)	N/A
Capecitabine tablet (Xeloda®)	N/A

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Representative Drug Name	Additional Criteria
Capivasertib (Truqap™)	N/A
Capmatinib (Tabrecta™)	N/A
Ceritinib (Zykadia®)	N/A
Cobimetinib (Cotellic™)	N/A
Crizotinib (Xalkori®)	N/A
Dabrafenib (Tafinlar®)	N/A
Dacomitinib (Vizimpro®)	N/A
Darolutamid (Nubeqa®)	N/A
Dasatinib (Sprycel®)	N/A
Dasatinib (Phyrago™)	○ Must provide clinical rationale for use of dasatinib (Phyrago) instead of dasatinib (Sprycel). Clinical rationale must include why dasatinib (Phyrago) would not be expected to produce the same clinical results as dasatinib (Sprycel) given that they are the same chemical entity.
Decitabine/cedazuridine (Inqovi®)	N/A
Dordaviprone (Modeyso™)	N/A
Duvelisib (Copiktra®)	N/A
Eflornithine (Iwilfin™)	N/A
Elacestrant (Orserdu™)	N/A
Enasidenib (Idhifa®)	N/A
Encorafenib (Braftovi™)	N/A
Ensartinib (Ensacove™)	N/A
Entrectinib (Rozlytrek™)	○ For entrectinib (Rozlytrek) oral pellets: ▪ Must provide clinical rationale for use of the oral pellets instead of the oral capsule. Clinical rationale must include why the oral capsule would not be expected to produce the same clinical results as the oral pellets given it contains the same chemical entity.
Enzalutamide (Xtandi®)	N/A
Erdafitinib (Balversa™)	N/A
Erlotinib (Tarceva®)	N/A
Estramustine (Emcyt®)	N/A
Etoposide	N/A
Everolimus tablet (Afinitor®, Torpenz™)	N/A
Everolimus tablet soluble (Afinitor Disperz®)	○ For adjunctive treatment of tuberous sclerosis complex (TSC)-associated partial-onset seizures, must have a trial of ALL the following with an inadequate response or

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Representative Drug Name	Additional Criteria
	significant side effect/toxicity or have a contraindication to these therapies: ▪ 2 generic antiepileptic medications ▪ Cannabidiol (Epidiolex) ▪ Generic everolimus tablet unless documentation of the inability to swallow an oral solid dosage form due to age (<7 years) or medical condition (e.g., nasogastric tube)
Fedratinib (Inrebic®)	N/A
Fruquintinib (Fruzaqla™)	N/A
Futibatinib (Lytgobi™)	N/A
Gefitinib (Iressa®)	N/A
Gilteritinib (Xospata®)	N/A
Glasdegib (Daurismo)	○ For AML, must have an assessment of appropriateness of intensive induction chemotherapy based on factors such as age, performance status, functional status, and/or comorbid conditions
Ibrutinib (Imbruvica®)	○ *For Value Choice and Open Choice formularies only*: ○ For ibrutinib (Imbruvica) 140mg and 280mg tablet • Must provide an attestation from the provider indicating why ibrutinib (Imbruvica) 140mg or 280mg tablet would not produce the same clinical results as would be expected with the use of ibrutinib (Imbruvica) 140mg capsule as they are the same chemical entity, just different dosage forms
Idelalisib (Zydelig®)	N/A
Imatinib (Gleevec®)	N/A
Imatinib (Imkeldi)	N/A
Imlunestrant (Inluriyo™)	N/A
Inavolisib (Itovebi™)	N/A
Ivosidenib (Tibsovo®)	○ For AML, must have an assessment of appropriateness of intensive induction chemotherapy based on factors such as age, performance status, functional status, and/or comorbid conditions
Ixazomib (Ninlaro®)	N/A
Lapatinib (Tykerb®)	N/A
Larotrectinib (Vitrakvi®)	N/A
Lazertinib (Lazcluze™)	○ Must have a previous inadequate response or intolerance to osimertinib (Tagrisso) or a regimen containing osimertinib (Tagrisso), unless osimertinib (Tagrisso) or a

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Representative Drug Name	Additional Criteria
	regimen containing osimertinib (Tagrisso) is not FDA-approved for the member's diagnosis OR must have an attestation from the provider that the member is not a candidate for therapy with osimertinib (Tagrisso) or with a regimen containing osimertinib (Tagrisso)
Lenalidomide (Revlimid®)	N/A
Lenvatinib (Lenvima™)	N/A
Lomustine (Gleostine™)	N/A
Lorlatinib (Lorbrena®)	N/A
Mechlorethamine gel (Valchlor®)	o Prior skin-directed therapy could include but is not limited to: topical corticosteroids, phototherapy, bexarotene gel (Targetin), and topical nitrogen mustard
Midostaurin (Rydapt®)	N/A
Mirdametinib (Gomekli™)	o Must have an adequate trial of selumetinib (Koselugo) capsules with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy. o NOTE: an exception to the requirement for a trial of selumetinib (Koselugo) capsules can be made if documentation of the inability to swallow an oral solid dosage form due to age (<7 years) or medical condition (e.g., nasogastric tube)
Mitotane (Lysodren®)	N/A
Momelotinib (Ojjaara™)	o Must have a previous inadequate response or intolerance to ruxolitinib (Jakafi), unless ruxolitinib (Jakafi) is not FDA-approved for the member's diagnosis OR must have an attestation from the provider that the member is not a candidate for therapy with ruxolitinib (Jakafi)
Neratinib (Nerlynx™)	N/A
Nilotinib (Danziten™)	o Must provide clinical rationale for use of nilotinib (Danziten) instead of nilotinib (Tasigna). Clinical rationale must include why nilotinib (Danziten) would not be expected to produce the same clinical results as nilotinib (Tasigna) given that they are the same chemical entity.
Nilotinib (Tasigna®)	N/A
Nilotinib D-tartrate [Cipla Ltd]	o Must provide clinical rationale for use of nilotinib D-tartrate instead of nilotinib (Tasigna). Clinical rationale must include why nilotinib D-tartrate would not be expected to produce the same clinical results as nilotinib (Tasigna) given that they are the same chemical entity.
Nilutamide (Nilandron®)	N/A
Nirogacestat (Ogsiveo™)	N/A

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Representative Drug Name	Additional Criteria
Niraparib (Zejula™)	N/A
Niraparib and abiraterone acetate (Akeega™)	N/A
Olaparib (Lynparza™)	N/A
Olutasidenib (Rezlidhia™)	N/A
Osimertinib (Tagrisso™)	N/A
Pacritinib (Vonjo™)	○ Must have a previous inadequate response or intolerance to ruxolitinib (Jakafi), unless ruxolitinib (Jakafi) is not FDA-approved for the member's diagnosis OR must have an attestation from the provider that the member is not a candidate for therapy with ruxolitinib (Jakafi)
Palbociclib (Ibrance®)	○ Must have a previous inadequate response or intolerance to ribociclib (Kisqali) or abemaciclib (Verzenio), unless those products are not FDA-approved for the member's diagnosis OR must have an attestation from the provider that the member is not a candidate for therapy with ribociclib (Kisqali) or abemaciclib (Verzenio)
Pazopanib (Votrient™)	N/A
Pemigatinib (Pemazyre™)	N/A
Pexidartinib (Turalio™)	N/A
Pirtobrutinib (Jaypirca™)	○ For requests for doses that exceed 200mg orally once daily, must have BOTH of the following (on initial request and reauthorization): ▪ Documentation of a drug interaction with a strong or moderate CYP3A inducer (e.g., rifampin, phenytoin, carbamazepine) ▪ An attestation from the provider that continued treatment with the interacting medication in combination with pirtobrutinib (Jaypirca) is medically necessary
Pomalidomide (Pomalyst®)	N/A
Ponatinib (Iclusig®)	N/A
Pralsetinib (Gavreto™)	N/A
Quizartinib (Vanflyta®)	N/A
Regorafenib (Stivarga®)	N/A
Repotrectinib (Augtyro™)	N/A
Revumenib (Revuforj®)	N/A
Ribociclib (Kisqali®)	N/A
Ribociclib and letrozole (Kisqali® and Femara® Co-Pack)	N/A

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Representative Drug Name	Additional Criteria
Ripretinib (Qinlock™)	N/A
Rucaparib (Rubraca™)	N/A
Selinexor (Xpovio™)	N/A
Selpercatinib (Retevmo™)	N/A
Selumetinib (Koselugo®)	○ For selumetinib (Koselugo) granules, must have documentation of the inability to swallow an oral solid dosage form due to age (< 7 years), medical condition (e.g. nasogastric tube), or other clinical ratrionale for why selumetinib (Koselugo) capsules cannot be used
Sevabertinib (Hyrnuo®)	N/A
Sonidegib (Odomzo®)	N/A
Sorafenib (Nexavar®)	N/A
Sotorasib (Lumakras™)	○ For members previously taking adagrasib (Krazati), must have chart documentation showing disease progression did not occur while on adagrasib (Krazati)
Sunitinib (Sutent®)	N/A
Sunvozertinib (Zegfrovy™)	N/A
Talazoparib (Talzenna™)	N/A
Taletrectinib (Ibtrozi™)	N/A
Tazemetostat (Tazverik™)	N/A
Temozolomide capsule (Temodar®)	N/A
Tepotinib (Tepmetko™)	○ Must have a previous inadequate response or intolerance to capmatinib (Tabrecta), unless capmatinib (Tabrecta) is not FDA-approved for the member's diagnosis OR must have an attestation from the provider that the member is not a candidate for therapy with capmatinib (Tabrecta).
Thalidomide (Thalomid®)	N/A
Thioguanine (Tabloid®)	N/A
Tivozanib (Fotivda®)	N/A
Topotecan capsule (Hycamtin®)	○ First line chemotherapy would include combination of any of the following: Carboplatin, Cisplatin, Cyclophosphamide, Docetaxel (Taxotere), Doxorubicin, Etoposide, Gemcitabine (Gemzar), Ifosfamide (Ifex), Irinotecan, Paclitaxel, Vincristine, Vinorelbine
Toremifene citrate (Fareston®)	○ Must have an adequate trial of tamoxifen with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
Tovorafenib (Ojemda™)	○ For tovorafenib (Ojemda) suspension, must have documentation of the inability to swallow an oral solid dosage form due to age (< 7 years), medical condition

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Representative Drug Name	Additional Criteria
	(e.g. nasogastric tube), or other clinical rationale for why tovorafenib (Ojemda) tablet cannot be used
Trametinib (Mekinist®)	N/A
Trifluridine/tipiracil (Lonsurf®)	N/A
Tucatinib (Tukysa™)	N/A
Vandetanib (Caprelsa®)	N/A
Vemurafenib (Zelboraf™)	N/A
Venetoclax (Venclexta™)	○ For AML, must have an assessment of appropriateness of intensive induction chemotherapy based on factors such as age, performance status, functional status, and/or comorbid conditions
Vimseltinib (Romvimza™)	N/A
Vismodegib (Erivedge®)	N/A
Vorasidenib (Voranigo)	N/A
Vorinostat (Zolinza®)	N/A
Zanubrutinib (Brukinsa™)	N/A
Ziftomenib (Komzifti™)	N/A
Zongertinib (Hernexeos®)	N/A

ARB – angiotensin receptor blocker

Arcus Cornealis – white deposit of lipids in the outer rim of the iris due to high LDL-C levels.

Aromastase Inhibitor – e.g., letrozole (Femara), exemestane (Aromasin), anastrozole (Arimidex)

AST – aspartate aminotransferase

***ATP7B* Gene –** encodes a metal-transporting P-type ATPase, which is mainly expressed in hepatocyte and functions in the transmembrane transport of copper. Absent or reduced function of *ATP7B* protein leads to decreased hepatocellular excretion of copper into bile.

Atypical Hemolytic Uremic Syndrome (aHUS) – a rare autoimmune disorder that results in low red blood cell counts, low platelet counts, and acute renal failure

Autoimmune Hemolytic Anemia (AIHA) – rare and heterogeneous disorders characterized by destruction of red blood cells through autoantibodies. Examples of autoimmune hemolytic anemia include warm autoimmune hemolytic anemia (wAIHA), cold agglutinin disease (CAD), and mixed type.

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Autonomic Neuropathy – a nerve disorder that affects autonomic (involuntary) functions of the body, such as heart rate, blood pressure, bowel and bladder emptying, sweating and digestion. It is an auto-regulation disorder that occurs when damage to the autonomic nerves disrupt signals from the brain to various parts of the autonomic nervous system (ANS) such as the heart and the blood vessels. Autonomic neuropathy is a widespread condition which affects a variety of individuals. The exact incidence is unknown as it is not a disease but rather a complication of a pre-existing disease or injury.

Autosomal dominant polycystic kidney disease (ADPKD) – a rare, hereditary cystic kidney disease characterized by progressive development and enlargement of fluid-filled cysts in the kidneys that compress and destroy the surrounding normal renal tissue. Loss of renal function, eventually resulting in end-stage renal disease (ESRD), occurs in a majority of patients.

Benign Prostatic Hyperplasia (BPH) – enlargement of the peri-urethral part of the prostate gland that occurs especially in men over 50 years of age and tends to obstruct urination by constricting the urethra.

Beta-glucuronidase – enzyme involved in the breakdown of glycosaminoglycans

Bile acid synthesis disorders due to single enzyme defects (SEDs)

List of Single Enzyme Defects (from Prescribing Information)
3β-HSD
AKR1D1
CTX
AMACR
CYP7A1
Smith-Lemli-Opitz

Biologic Disease Modifying Antirheumatic Drugs (DMARDs) by Indication

Biologic Drug	Rheumatoid Conditions					Derma -tology	Gastrointestinal Conditions	
	RA	AS	nr-axSpA	JIA	PsA	PsO	CD	UC
Actemra® (tocilizumab SC, biosimilars)	X			X				
Actemra® (tocilizumab IV, biosimilars)	X			X				
Bimzelx® (bimekizumab-bkzx SC)		X	X		X	X		
Cimzia® (certolizumab pegol SC syringe, lyophilized powder vial SC)	X	X	X	X	X	X	X	
Cosentyx™ (secukinumab SC)		X	X	X (ERA)	X	X		
Cosentyx™ (secukinumab IV)		X	X		X			
Enbrel® (etanercept SC, biosimilars)	X	X		X	X	X		

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Entyvio® (vedolizumab SC)							X	X
Entyvio® (vedolizumab IV)							X	X
Humira® (adalimumab SC, biosimilars)	X	X		X	X	X	X	X
Ilumya™ (tildrakizumab-asmn SC)						X		
Kevzara® (sarilumab SC)	X			X				
Kineret^® (anakinra SC)	X			X				
Omvoh® (mirikizumab-mrkz IV)							X	X
Omvoh® (mirikizumab-mrkz SC)							X	X
Orencia® (abatacept SC)	X			X	X			
Orencia® (abatacept IV)	X			X	X			
Remicade® (infliximab IV, biosimilars)	X	X			X	X	X	X
Rituxan® (rituximab IV, biosimilars)	X							
Siliq™ (brodalumab SC)						X		
Simponi® (golimumab SC)	X	X			X			X
Simponi® Aria (golimumab IV)	X	X		X	X			
Skyrizi™ (risankizumab-rzaa SC)					X	X		
Skyrizi™ (risankizumab-rzaa IV)							X	X
Skyrizi™ (risankizumab-rzaa OBI)							X	X
Stelara® (ustekinumab SC, biosimilars)					X	X	X	X
Stelara® (ustekinumab IV, biosimilars)							X	X
Taltz® (ixekizumab SC)		X	X		X	X		
Tremfya® (guselkumab IV)							X	X
Tremfya® (guselkumab SC)					X	X	X	X
Zymfentra® (infliximab-dyyb SC)							X	X

* Not an all-inclusive list of indication. Refer to the prescribing information for the respective agent for FDA-approved indications; SC – Subcutaneous; AS – Ankylosing spondylitis; CD – Crohn's disease; JIA – Juvenile idiopathic arthritis; ERA -enthesitis related arthritis; PsO – Plaque psoriasis; PsA – Psoriatic arthritis; RA – Rheumatoid arthritis; UC – Ulcerative colitis; nr-axSpA – Non-radiographic axial spondyloarthritis; IV – Intravenous; OBI – On body injector; PJIA – Polyarticular juvenile idiopathic arthritis; ^ Off-label use of Kineret in JIA supported in guidelines

Blepharoptosis – abnormal drooping of the upper eyelid

BMI – body mass index

BMI percentile for adolescents – calculated using age- and sex-specific CDC growth charts or severe obesity growth charts for all children 2-18 years. CDC BMI percentile calculator can be accessed at the following link: BMI Calculator Child and Teen | Healthy Weight | CDC

Brooke Upper Extremity Functional Scale – measurement of motor function. It scores upper extremity function from 1 (can elevate arms full range to the head), 2 (can elevate arms but needs to flex elbow or use accessory muscles), 3 and 4 (unable to elevate the

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shoulders but can raise hands to the mouth with or without weight respectively, 5 (unable to raise hands to the mouth and only some hand movement exists), to 6 (no useful function of hands).

c.2014C>A – missense point mutation of SCN4A changes the coding DNA at nucleotide number 2014 which causes a change in the protein amino acid sequence from the arginine amino acid at residue 672 to a serine

c.2014C>G – missense point mutation of SCN4A changes the coding DNA at nucleotide number 2014 which causes a change in the protein amino acid sequence from the arginine amino acid at residue 672 to a glycine

CAG – the neucleotide sequence that codes for the amino acid glutamine

Carbamoyl phosphate synthase 1 (CPS 1) – the first enzyme of the urea cycle, which converts ammonia into urea.

Carbon monoxide diffusing capacity (DLCO) – A test of the lungs capability to expel carbon monoxide

Carcinoid Syndrome – a group of symptoms that are associated with carcinoid tumors (the most common type of neuroendocrine tumors) which are rare, slow-growing tumors that occur in the gastrointestinal tract or lungs. Patients may experience flushing, abdominal pain, diarrhea, difficulty breathing, rapid heart rate, low blood pressure, skin lesions on the face, and wheezing. It occurs when the carcinoid tumor secretes serotonin or other chemicals into the bloodstream. Only 10% of patients with carcinoid tumors develop carcinoid syndrome, and most have advanced stage carcinoid tumors. Treatment generally involves treating the underlying carcinoid tumor and medications to alleviate symptoms.

Cataplexy – more than one episode of generally brief (< 2 min), usually bilaterally symmetrical, sudden loss of muscle tone with retained consciousness

CCR5 – C-C chemokine receptor type 5, a protein on the surface of white blood cells that is involved in the immune system as it acts as a receptor for chemokines.

CDI – *Clostridium difficile* infection

CDKL5 deficiency disorder – a rare X-linked genetic disorder caused by mutations in the cyclin-dependent kinase-like 5 gene, which is essential for normal brain development. A mutation causes early onset seizures and severe neurodevelopmental delay.

Ceruloplasmin – protein synthesized in the liver which is responsible for copper transport and storage

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CGRP – calcitonin-gene receptor peptide

Children's Hospital of Philadelphia Infant Test for Neuromuscular Disorders (CHOP-INTEND) – measurement of motor function in infants. It scores motor function (0: worst to 4: best) via 16 different items, which capture neck, trunk, proximal, and distal limbs

CHOP – a chemotherapy regimen consisting of the following agents: cyclophosphamide, doxorubicin, vincristine, and prednisone.

CHOP-R – a chemotherapy regimen consisting of the following agents: cyclophosphamide, doxorubicin, vincristine, prednisone, and ritixumab

Chorea – a rapid, involuntary, nonrepetitive movement involving the face, trunk, and limbs

Chronic fibrosing interstitial lung diseases with a progressive phenotype – chronic fibrosing interstitial lung diseases (ILDs) comprise a large, diverse group of rare parenchymal lung diseases that may be associated with other diseases, associated with environmental exposures, or have an unknown cause. A proportion of patients with certain types of chronic fibrosing ILDs will develop a progressive phenotype that demonstrates similar pathogenesis and clinical manifestations as IPF. Although chronic fibrosing ILDs may have different causes and triggers, many patients with a progressive phenotype experience a similar clinical course regardless of the ILD etiology. Examples of fibrosing ILDs associated with a risk of developing a progressive phenotype include hypersensitivity pneumonitis (HP), autoimmune ILDs (such as rheumatoid arthritis ILD, mixed connective tissue disease ILD), sarcoidosis, idiopathic non-specific interstitial pneumonia, and unclassifiable idiopathic interstitial pneumonia.
Chronic hand eczema (CHE) – an inflammatory disorder involving the skin of the hands. CHE is associated with genetic factors as well as environmental factors such as occupational risks (e.g., water exposure) and irritants/allergens. Multiple chronic hand eczema subtypes exist, including irritant contact, allergic contact, hyperkeratotic, atopic, and several other variants. CHE can also be a manifestation of atopic dermatitis.

Chronic kidney disease (CKD) stages

Stage	Description	GFR (mL/min/1.73m^2)
1	Kidney damage with normal or elevated GFR	≥ 90
2	Kidney damage with mildly reduced **kidney function**	60-89
3a	Mildy to moderately reduced **kidney function**	45-59
3b	Moderately to severely reduced kidney function	30-44
4	Severely reduced kidney function	15-29
5	Kidney failure	< 15 (for dialysis)

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Chronic thromboembolic pulmonary hypertension (CTEPH) – a mean pulmonary artery pressure greater than 20mmHg that persists 6 months after pulmonary embolism is diagnosed.

CIAS1 gene – cold induced autoinflammatory syndrome 1 gene

Class Ia Antiarrhythmic Medications – slow conduction velocity and prolong action potential duration; examples include disopyramide, procainamide, and quinidine

Class III Antiarrhythmic Medications – prolong the action potential durations but have no effect on conduction; examples include amiodarone, dofetilide, ibutilide, sotalol, n-acetyl procainamide (NAPA), and dronedarone

CMV – Cytomegalovirus

Cobalamin cofactor metabolism (cbl) defect - inherited metabolic disorder resulting in deficiency in cobalamin, or inability to absorb cobalamin normally. It can result in accumulation of methylmalonic acid and homocysteine in blood and urine.

Compounded medication – a customized medication formulated by combining, mixing, or altering pharmaceutical ingredients. Compounded medications are prepared for an individual member in response to a licensed **practitioner's prescription.**

COPD – Chronic Obstructive Pulmonary Disease

Corneal verticillata – whorl-shaped lesions on the cornea caused by deposition of globotriaosylceramide (GL-3)

CrCl – creatinine clearance

***CTNS* –** gene that encodes the protein cystinosin in humans. Mutations in *CTNS* are responsible for cystinosis, an autosomal recessive lysosomal storage disease

Cushing's disease – overproduction of cortisol due to pituitary hypersecretion of ACTH by pituitary corticotroph tumors; the most common cause of endogenous Cushing's syndrome

Cushing's syndrome – condition of pathological hypercortisolism that can be caused by ectopic secretion of adrenocorticotropic hormone (ACTH) by non-pituitary tumors, pituitary hypersecretion of ACTH (known as Cushing's disease), or hypersecretion of cortisol from adrenal gland tumors

CV – cardiovascular

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CVP Chemotherapy – a chemotherapy regimen consisting of the following agents: cyclophosphamide, vincristine, and prednisone

Cyanotic Heart Disease – hemodynamically significant heart defect present at birth that results in a low blood oxygen level, such as heart valve defects (e.g., tricuspid valve, pulmonary valve, and aortic valve), coarctation or complete interruption of the aorta, Epstein anomaly, hypoplastic left heart syndrome, tetralogy of Fallot, total anomalous pulmonary venous return, transposition of the great arteries, and truncus arteriosus.

Cystathionine Beta-Synthase (CBS) deficiency - a rare metabolic condition caused by mutations in the CBS gene. It is the most common inborn error of sulfur metabolism characterized by very high levels of plasma total homocysteine and methionine.

Cystine Capacity – directly measures the ability of a patient's urine to solubilize or precipitate cystine when additional cystine crystals are added to it. A positive value means the urine is undersaturated and a negative value means the urine is supersaturated.

Cystinuria – rare autosomal recessive defect of the transepithelial transport of cysteine and dibasic amino acids in the kidney and intestine causing profoundly decreased cysteine resorption in the proximal renal tubule. The increased urinary excretion of cysteine results in cysteine crystallization and stone formation.

Cystic fibrosis conductance regulator (*CFTR*) gene mutation – cystic fibrosis is caused by mutations to the *CFTR* gene. Various nomenclature may be used when referring to genetics in practice (i.e., cDNA name, protein name, legacy name). If the *CFTR* mutation is not listed within the prescribing information, resources to check alternative nomenclature and treatment eligibility include the CFTR2 website (https://cftr2.org/), developed by the Cystic Fibrosis Foundation, Johns Hopkins University and the Hospital for Sick Children, or the Vertex treatment eligibility tool (https://www.vertextreatmentshcp.com/eligibility-tool).

Dactylitis – a painful inflammation of the fingers or toes; a sausage shaped digit associated with psoriatic arthritis

Dawn Phenomenon – A physiologic increase in blood sugar levels, especially in the early morning before breakfast that is observed in people with diabetes due to insufficient insulin production despite fasting throughout the night.

DBS – dried blood spot

Demodex blepharitis – a condition caused by an infestation of Demodex (mites) in the eyelash follicles and the meibomian glands in the eyelids

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Deoxyadenine nucleotides (dAXP) – measureable purine nucleotides that build up in patients with ADA deficiency

Diencephalic Syndrome – a rare neurological disorder caused by a tumor located in the diencephalon, an area of the brain that contains the thalamus and hypothalamus. It is characterized by a failure to thrive or the inability to gain or maintain fat tissue (hence leading to emaciation). The complications related to the syndrome can be severe and even life-threatening

Distal ulcerative colitis (UC) – limited to below the descending colon and thus within reach of topical therapy

Dravet syndrome – a refractory childhood-onset epilepsy that typically appears between one and eighteen months of age and is characterized initially by prolonged, often hemiclonic seizures triggered by hyperthermia and light that frequently lead to status epilepticus and later evolve into multiple seizure types; the syndrome is associated with developmental delay, behavioral issues, and gait abnormalities; most patients have a mutation of the sodium channel subunit gene (SCN1A)

Duchenne Muscular Dystrophy (DMD) – a rare, X-linked, recessive, life-threatening, degenerative neuromuscular disease affecting males. It is attributed to mutations in the DMD gene (chromosome Xp21), which is responsible for producing the protein dystrophin. Dystrophin is needed for proper muscle functioning and provides mechanical stability to muscle fibers during muscle contraction. The absence of or defect in this protein, leads to progressive muscle degeneration with loss of independent ambulation, as well as respiratory and cardiac complications.

Duchenne Muscular Dystrophy Gene Mutations Amenable to Exon 51 Skipping – Please refer to the following website – Cure Duchenne. Duchenne population potentially amenable to exon skipping, Exon 51 skip-amenable.
http://support.cureduchenne.org/site/PageServer?pagename=DuchennePopulationPotentiallyAmenabletoExonSkipping

Dutch Lipid Clinic Network/WHO Criteria for Familial Hypercholesterolemia – scoring system developed to calculate a numeric score that predicts the probability of the diagnosis of familial hypercholesterolemia. Diagnosis of familial hypercholesterolemia is definite when the score is > 8. Diagnosis is probable when the score is 6-8. Diagnosis is possible when the score is 3-5. Diagnosis is unlikely when the score is <3.

	Clinical Signs	**Score**
Family History	First degree relative known with premature (men <55 years; women <60 years) coronary and vascular diseases	1
	First degree relative known the LDL-cholesterol >95th percentile	1

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	First degree relative with tendon xanthomas and/or arcus cornealis	2
	Children below 18 years with LDL-C >95th percentile	2
Clinical History	Patient has premature (natal males <55 years; natal females <60 years) coronary artery disease	2
	Patient has premature (natal males <55 years; natal females <60 years) cerebral or peripheral vascular disease	1
Physical Examination	Tendon xanthomas	6
	Arcus cornealis below the age of 45 years	4
Laboratory Analysis	LDL-C >330 mg/dL (8.5 mmol/L)	8
	LDL-C = 250-329 mg/dL (6.5-8.4 mmol/L)	5
	LDL-C = 190-249 mg/dL (5.0-6.4 mmol/L)	3
	LDL-C = 155-189 mg/dL (4.0-4.9 mmol/L)	1
DNA Analysis	Functional mutation in the LDLR, apo B, or PCSK9 gene	8

DVT – deep vein thrombosis

DXA – Dual-energy x-ray absorptiometry

EBV Serology – laboratory tests which detect the presence of EBV antibodies

ED – erectile dysfunction

EGFR – epidermal growth factor receptor

Electrophysiological Testing – nerve stimulation test that measures a muscle's response to electric impulses

- EMG: electromyography
- CMAP: compound muscle action potential
- RNS: repetitive nerve stimulation

Encapsulated Bacteria – bacteria with a polysaccharide capsule membrane. Examples include Streptococcus pnemoniae, Klebsiella pneumonia, group B streptococci, Escherichia coli, Neisseria meningitides and Haemophilus influenzae

Enthesitis – inflammation of sites where tendons or ligaments insert into the bone. It is also called enthesopathy, or any pathologic condition involving the entheses. The entheses are any point of attachment of skeletal muscles to the bone, where recurring stress or inflammatory autoimmune disease can cause inflammation or occasionally fibrosis and calcification.

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Eosinophilic esophagitis (EoE) – a chronic, immune/antigen-mediated, inflammatory disorder of the esophagus characterized by symptoms of esophageal dysfunction and eosinophil-predominant inflammation

Eosinophilic granulomatosis with polyangiitis (EGPA) – is characterized by asthma, sinusitis, pulmonary infiltrates, neuropathy, and eosinophilic vasculitis of one or more end-organs. The pathogenic effects of eosinophils are thought to be by means of tissue and vascular infiltration.

Epstein Bar Virus (EBV) – a member of the herpes virus family and one of the most common human viruses with as many as 95% of adults between the ages of 35-40 in the United States having been infected at some point during their lives, most often during childhood. Symptoms of the virus in children may be absent or indistinguishable from other mild, brief illnesses. Infections during adolescence will present as mononucleosis 35-50% of the time. Patients undergoing organ transplants who have not been exposed to EBV, and therefore do not have immunity, are at increased risk of post-transplant lymphoproliferative disorder (PTLD, found to be the most common malignancy in transplant recipients) due to chronic immunosuppression since EBV can infect B cells and cause them to proliferate.

Epworth Sleepiness Scale (ESS) – an assessment used to evaluate the extent of sleepiness in everyday situations. Patients are asked to rate how likely they are to fall asleep during each of 8 activities on a scale from 0-3 (0=never; 1=slight; 2=moderate; 3=high). Total scores range from 0-24 with higher scores correlating with increasing degrees of sleepiness. A score > 10 is consistent with excessive sleepiness.

ESRD – end stage renal disease

Evanescent rash – a macular, salmon-colored rash on the trunk and extremities that is transient

Examples of Vitamin D/Vitamin D Analogues Dosing

Generic Name	Dosage/day
Calcitriol	0.5-1.0mcg
Ergocalciferol (D2)	25,000 to 100,000 IU
Cholecalciferol (D3)	25,000 to 100,000 IU

Fabry Disease – a rare genetic disorder caused by a defect in the gene for the lysosomal enzyme alpha-galactosidase resulting in inability or diminished ability to catabolize certain lipids. These lipids then accumulate in many cell-types throughout the body.

Facial angiofibroma – a benign tumor comprised of blood vessels and fibrous tissue appearing on the face, especially the malar region. Facial angiofibromas appear as small pink or red papules and are common in patients with tuberous sclerosis complex.

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Familial Amyloidotic Polyneuropathy (FAP) Stage

Stage	Description
0	No symptoms
1	Unimpaired ambulation; mostly mild sensory, motor, and autonomic neuropathy in the lower limbs
2	Assistance with ambulation required**,** mostly moderate impairment progression to the lower limbs, upper limbs, and trunk
3	Wheelchair-bound or bedridden, severe sensory, motor, and autonomic involvement of all limbs

Familial chylomicronemia syndrome (FCS) – a rare autosomal recessive disorder characterized by extremely high triglyceride levels. It is caused by a deficiency in lipoprotein lipase (LPL) enzyme activity due to mutations in the *LPL* gene or genes encoding its co-factors. Signs and symptoms include nausea, vomiting, eruptive xanthomas, difficulty concentrating, forgetfulness, lipemia retinalis, hepatosplenomegaly, recurrent episodes of mild to incapacitating abdominal pain, and failure to thrive. The most serious complication is severe and recurrent episodes of acute pancreatitis, which can be life-threatening.

Familial Cold Autoinflammatory Syndrome (FCAS) – an autosomal dominant inherited inflammatory disease characterized by episodes of rash, fever, and joint pain, following exposure to cold with attacks occurring 1-2 hours after exposure to cold and lasting less than 24 hours

Familial Mediterranean Fever (FMF) – an autosomal recessive inherited disorder with a childhood onset and >90% of patients having the first attack of periodic fever by the age of 20 years. The attacks may be triggered by certain events (cold exposure, infections, stress, menstrual cycle, exercise, fat-rich meals) and usually lasts 1-3 days with fever and severe pain. Symptoms can include: peritonitis, arthritis, pleuritis, arthritis, and erysipelas-like erythema.

FDA – Food and Drug Administration

Febrile Neutropenia (FN) – a single temperature of >38.3 degrees C orally or >38 degrees C over 1 hour, neutrophils <500/µL or <1000/µL and predicted decline <500 neutrophils over the next 48 hours

The severity of neutropenia relates to the relative risk of infection and is classified as follows:

- Mild (ANC = 1000 – 1500/µL)
- Moderate (ANC = 500 – 1000/µL)
- Severe (ANC < 500/µL)

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FEV_1 – Forced expiratory volume in 1 second

Fibrodysplasia ossificans progressiva (FOP) – an ultra-rare, genetic connective tissue disorder characterized by severe, progressive development of bone in areas outside of the skeleton (heterotopic ossification; HO), such as the ligaments, tendons, and muscles. FOP is caused by mutations in the activin A receptor type 1 gene (ACVR1), which encodes a bone morphogenetic protein (BMP) type I receptor that is important during the formation of the skeleton in the embryo and repair of the skeleton following birth. The mutation in the ACVR1 gene increases BMP signaling, resulting in the formation of heterotopic bone.

FODMAP – Fermentable oligosaccharides, disaccharides, monosaccharides and polyols, which include fructo-oligosaccharides (FOS), galacto-oligosaccharides (GOS), disaccharides (e.g., lactose), monosaccharides (e.g., fructose) and polyols (e.g., sorbitol). FODMAPs are consumed across a range of foods such as wheat, rye, vegetables, fruits, and legumes. These are poorly absorbed and rapidly fermentable in the gut by bacteria and thought to cause gas production as well as osmotic effects resulting in gastrointestinal distention.

Forced vital capacity (FVC) – the maximal volume of gas that can be exhaled from full inhalation by exhaling as forcefully and rapidly as possible.

Fragility fracture – low-trauma fracture in the absence of other metabolic bone disease, independent of bone mineral density score

FRAX – Fracture Risk Assessment Tool

FVIII – Factor VIII

Gaucher Disease – a rare, autosomal recessive disorder caused by mutations in the GBA (acid β-glucosidase) gene that leads to a deficiency of the lysosomal enzyme beta-glucocerebrosidase. Glucocerebrosidase is responsible for converting sphingolipid glucocerebroside into glucose and ceramide. The enzymatic deficiency leads to a build-up of glucocerebroside, causing an accumulation of "Gaucher cells" in the liver, spleen, bone marrow, and other organs. Extra Gaucher cells in the liver and spleen cause organomegaly whereas excess in the bone marrow and spleen causes anemia and thrombocytopenia. Type I disease is distinguished from type 2 and type 3 diseases by the lack of the characteristic involvement in the central nervous system.

Gender Dysphoria (GD)/Gender Incongruence – refers to the distress or unease experienced if gender identity and designated gender are not completely congruent.

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In the United States, the American Psychiatric Association (APA) permits a diagnosis of gender dysphoria if the diagnostic criteria in the Diagnostic and Statistical Manual of Mental Disorders, 5th Edition, (DSM-5) are met. The criteria are:

A. A marked incongruence between one's experienced/expressed gender and natal gender, of at least six month's duration, as manifested by at least two of the following:
    - A marked incongruence between one's experienced/expressed gender and primary and/or secondary sex characteristics (or in young adolescents, the anticipated secondary sex characteristics); or
    - A strong desire to be rid of one's primary and/or secondary sex characteristics because of a marked incongruence with one's experienced/expressed gender (or in young adolescents, a desire to prevent the development of the anticipated secondary sex characteristics); or
    - A strong desire for the primary and/or secondary sex characteristics of the other gender; or
    - A strong desire to be of the other gender (or some alternative gender different from one's assigned gender); or
    - A strong desire to be treated as the other gender (or some alternative gender different from one's assigned gender); or
    - A strong conviction that one has the typical feelings and reactions of the gender (or some alternative gender different from one's assigned gender); and
B. The condition is associated with clinically significant distress or impairment in social, occupational, or other important areas of functioning.

GFR – glomerular filtration rate

GI – gastrointestinal

Globotriasylceramide (GL-3) – a type of glycolipid compound that accumulates in blood vessel walls of people with Fabry disease

Glucose-6-phosphate dehydrogenase (G6PD) deficiency – X-linked recessive genetic metabolism error that predisposes a person to hemolysis (spontaneous destruction of red blood cells), jaundice, and hemolytic anemia. Hemolytic episodes can be triggered by certain foods, fever from illness, or medication in people with this deficiency.

GOLD – Global Initiative for Chronic Obstructive Lung Disease

Gottron's papules – erythematous to violaceous papules over the extensor surfaces of joints, which are sometimes scaly. May occur over the finger joints, elbows, knees, malleoli and toes

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Gottron's sign – erythematous to violaceous macules over the extensor surfaces of joints, which are not palpable

Granulomatosis with polyangiitis (GPA) – previously called Wegener's granulomatosis, this vasculitis primarily affects capillaries, venules, arterioles, arteries, and veins. Patients with GPA experience inflammation in the upper and lower respiratory tract as well as a pauci-immune glomerulonephritis, which is a rapidly progressive condition leading to renal failure within days to weeks. Approximately 80% of GPA patients have ANCA autoantibodies present.

HAE specialist – an allergist/immunologist who demonstrates clinical expertise in HAE through research, publication, referrals/consults

Hamartoma – a benign tumorlike lesion made up of an abnormal mixture of glandular, adipose, and/or fibrous tissue. Hamartomas may also be known as fibroadenolipomas, lipofibroadenomas, or adenolipomas.

Hammersmith Infant Neurological Exam, Section 2 (HINE-2) – measurement of functional ability and achievement of motor milestones in infants. It scores seven different areas of motor milestone development, with a maximum score between 2-4 points for each, depending on the milestone, and a total maximum score of 26.

HCV RNA (Hepatitis C Virus Ribonucleic Acid) – fragments of the replicating hepatitis C virus which can be detected by testing to determine levels of hepatitis C virus in the body

Heliotrope rash – purple, lilac-colored or erythematous patches over the eyelids or in a periorbital distribution, often associated with periorbital edema

Hemophagocytic lymphohistiocytosis (HLH) – an aggressive and life-threatening condition characterized by lymphocyte dysfunction and hypercytokinemia resulting in coagulopathy, end organ damage and severe cytopenias.

Hemophilia A (classic hemophilia) – an X-linked hereditary disorder caused by mutations in the coagulation FVIII gene, which result in a deficiency in circulation levels of FVIII. Inadequate levels of FVIII lead to disruption of the coagulation cascade, resulting in impaired closing and excessive bleeding episodes.

Hereditary Angioedema (HAE) – a rare disorder characterized by recurrent attacks of swelling that may involve the peripheral extremities, abdomen, genitalia, face, oropharynx, or larynx due to low levels of endogenous or functional C1 inhibitor.

Hereditary Angioedema Specialist – an allergist/immunologist who demonstrates clinical expertise in HAE through research, publication, referrals/consults.

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Hereditary orotic aciduria – an extremely rare, genetic disorder of pyrimidine metabolism caused by a defect in uridine monophosphate synthase (UMPS). The defect in UMP synthase results in (1) a systemic deficiency of pyrimidine nucleotides which is the cause of most clinical consequences and (2) excretion of orotic acid in the urine.

Hereditary transthyretin-mediated amyloidosis (hATTR) – in hATTR amyloidosis, there is misfolding of the TTR protein resulting in accumulation of amyloid in organs and tissue causing symptoms based on the organ(s) involved. Commonly affected sites for deposition and accumulation of amyloid include the peripheral and autonomic nervous systems and the heart. Onset typically begins in the second decade of life or later, with progressive symptoms over the next two decades.

Hereditary Tyrosinemia Type 1 (HT-1) – a rare metabolic disease in children. In normal, unaffected individuals, excess amounts of the amino acid tyrosine are degraded in several steps. In HT-1, however, one of the enzymes in this degradation, fumarylacetoacetase hydrolase (FAH), is deficient. Tyrosine and its toxic metabolites [fumarylacetoacetate, maleylacetoacetate, succinylacetone (SA), and succinylacetoacetate (SAA)] thus build up in the body and cause serious medical problems such as liver failure and hepatocellular carcinoma. Kidney dysfunction, skeletal changes, and neurological

Heterotopic bone – bone that has grown in an abnormal place, sometimes referred to as ectopic bone

Heterotopic ossification (HO) - abnormal formation of bone in soft tissue such as muscle, tendons, and ligaments

Heterozygous Familial Hypercholesterolemia (HeFH) – a common genetic disorder that is characterized by an elevated LDL-C level, which, if left untreated, can cause premature coronary heart disease.

HgbA1c – Glycated hemoglobin

High fracture risk – advanced age, frailty, concomitant glucocorticoid use, or increased fall risk

High-resolution computed tomography (HRCT) – a scan of the lungs that is essential component of the diagnostic pathway in IPF

HIV – Human Immunodeficiency Virus

HLA – human leukocyte antigen

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HMG CoA Reductase – 3-hydroxy-3-methyl-glutaryl-CoA reductase, the rate-controlling enzyme of the mevalonate pathway (the metabolic pathway that produces cholesterol and other isoprenoids)

Homocystinuria – an inherited disorder of the metabolism of the amino acid methionine, which is broken down into to homocysteine, which is in turn converted back to methionine. Homocystinuria results in severely elevated levels of homocysteine which manifests as detrimental effects in organ systems such as the eyes, bones, vascular and neurological systems.

Honeycombing – is manifested on HRCT as clustered airspaces that resemble a honeycomb structure.

HSCT – hematopoietic stem cell transplant

Huntington's Disease – an inherited progressive neurodegenerative disorder characterized by chorea, psychiatric problems, and dementia.

Hutchinson-Gilford Progeria Syndrome (HGPS) – also called "Progeria"; a rare, fatal, autosomal dominant heterozygous genetic disorder that leads to accumulation of an abnormal protein (progerin) in the cell nuclear membrane and disruption of cellular structure and tissue repair functions. This causes a syndrome of premature aging and accelerated cardiovascular disease. Affected children have a mean life expectancy of 14.5 years. Death occurs due to complications from heart failure, myocardial infarction, and stroke. The genetic mutation in HGPS is a single base mutation, c.1824C > T (p.Gly608Gly), in exon 11 of the *LMNA* gene.

Hyperimmunoglobulin D Syndrome (HIDS)/Mevalonate Kinase Deficiency (MKD) – an autosomal recessive disorder with a childhood onset, and characterized by recurring episodes of systemic inflammation and high spiking fevers. The attack can be spontaneous or may be triggered by certain events (vaccination, minor trauma, stress, surgery). The attacks last 3-7 days and the patient can experience prodromal symptoms. Symptoms can include: painful lymph nodes, abdominal pain, vomiting, diarrhea, skin-rash, join pain headache, hepatosplenomegaly.

Hyperphagia - a chronic feeling of intense, persistent hunger leading to severe consequences such as stomach rupture, obesity, diabetes, and cardiovascular disease. Hyperphagia is a hallmark symptom of Prader Willi Syndrome.

Hyponatremia – an excess of water in relation to the sodium in the extracellular fluid

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Hypophosphatasia (HPP) – a rare, genetic disorder caused by a defect in the gene for the tissue-nonspecific isozyme of alkaline phosphatase (TNSALP) resulting in the accumulation of pyrophosphate which leads to rickets or osteomalacia.

Ichthyosis – a group of disorders of keratinization characterized by generalized scaling of the skin. Most ichthyoses are inherited, but the condition can develop as a result of malignancy, autoimmune disease, infectious disease, and/or nutritional deficiency. Multiple subtypes exist, including ichthyosis vulgaris (the most common form), recessive X-linked ichthyosis, autosomal recessive congenital ichthyosis, lamellar ichthyosis.

Idiopathic pulmonary fibrosis (IPF) – a specific form of chronic, progressive fibrosing interstitial pneumonia of unknown cause, occurring primarily in older adults, and limited to the lungs.

Interstitial lung disease (ILD) – a term broadly used to describe a heterogeneous group of disorders that are classified together because of similar clinical, radiographic, physiologic, or pathologic manifestations

Immune tolerance induction (ITI) – a treatment strategy that aims to eradicate inhibitors, enabling the effective use of FVIII replacement therapy. ITI involves administering large doses of FVIII concentrates on a regular basis (e.g., daily or every other day) for a period of months to years.

Immunoglobulin light-chain amyloidosis (AL Amyloidosis) – is the most common type of systemic amyloidosis. AL amyloidosis results from a dyscrasia of a type of white blood cell called plasma cells in the bone marrow, and is closely related to multiple myeloma.

Infantile-onset Pompe disease – a genetic absence or deficiency of acid alpha-glucosidase (GAA) resulting in build-up of glycogen in the cardiac and skeletal muscles, and in hepatic tissue. This results in the development of cardiomyopathy, progressive muscle weakness, and impairment of respiratory function. Infantile-onset Pompe disease is diagnosed within the first year of life and is characterized by cardiac manifestations and complications. The cause of death in patients with infantile-onset disease is often cardiac or respiratory failure.

Inflammatory Neuropathy Cause and Treatment Scale (INCAT) – used to assess functional disability of both upper and lower extremities in chronic inflammatory demyelinating polyradiculoneuropathy (CIDP)

Grade	Arm Disability
0	No upper limb problems

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1	Symptoms, in one or both arms, not affecting the ability to perform any of the following functions: doing all zips and buttons; washing or brushing hair; using a knife and fork together; and handling small coins
2	Symptoms, in one arm or both arms, affecting but not preventing any of the above-mentioned functions
3	Symptoms, in one arm or both arms, preventing one or two of the above-mentioned functions
4	Symptoms, in one arm or both arms, preventing three or all of the functions listed, but some purposeful movements still possible
5	Inability to use either arm for any purposeful movement
Grade	Leg Disability
0	Walking not affected
1	Walking affected, but walks independently outdoors
2	Usually uses unilateral support (stick, single crutch, one arm) to walk outdoors
3	Usually uses bilateral support (sticks, crutches, frame, two arms) to walk outdoors
4	Usually uses wheelchair to travel outdoors, but able to stand and walk a few steps with help
5	Restricted to wheelchair, unable to stand and walk a few steps with help

Informed consent – refers to a person's consent to a proposed medical intervention after receiving relevant information. The information that is legally required includes: diagnosis, nature and purpose of the proposed intervention, risks and consequences of the proposed treatment, probability that the treatment will be successful, feasible treatment alternatives and prognosis if the treatment is not given.

Insulin-like Growth Factor-1 (IGF-1) – a hormone that functions as the major mediator of growth hormone (GH)-stimulated somatic growth, as well as a mediator of GH-independent anabolic responses in many cells and tissues

Intertriginous (inverse) psoriasis – a type of psoriasis with presentation in skin folds (e.g., inguinal, perineal, genital, intergluteal, axillary, or inframammary regions).

Kayser-Fleischer Ring – deposition of copper in the periphery of the cornea

Kurtzke Expanded Disability Status Scale (EDSS) – a method of quantifying disability in multiple sclerosis. EDSS steps 1.0 to 4.5 refer to Multiple Sclerosis (MS) patients who are fully ambulatory; EDSS steps 5.0 to 9.5 are defined by the impairment in ambulation.

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Lactate Dehydrogenase (LDH) – a catalytic enzyme that is highly concentrated in red blood cells. Increased serum levels of LDH correlate with increase hemolysis of red blood cells

Lambert-Eaton Myasthenic Syndrome (LEMS) – LEMS is a rare, autoimmune disorder affecting the neuromuscular junction and causing proximal muscle weakness, autonomic dysfunction, and areflexia. Presenting symptoms include leg weakness (60%), generalized weakness (18%), muscle pain or stiffness (5%), dry mouth (5%), arm weakness (4%), diplopia (4%), and dysarthria (2%). Weakness normally spreads proximally (most common in the upper arms and legs) to distally (involving feet and hands) and from the posterior toward the head, finally reaching the oculo-bulbar region.

Late-onset Pompe Disease – a genetic deficiency of acid alpha-glucosidase (GAA) resulting in build-up of glycogen in the cardiac and skeletal muscles, and in hepatic tissue. This results in the development of progressive muscle weakness, and impairment of respiratory function. Cardiomyopathy is a rare finding in late-onset disease. Age of diagnosis with late-onset disease is highly variable and can occur at any time. Patients can present at an age less than 12 months without cardiomyopathy or at any time after 12 months. The most common presenting symptoms include myopathy, skeletal muscle weakness, and respiratory dysfunction. The cause of death in patients with late-onset disease is often respiratory failure.

Left ventricular outflow tract (LVOT) – the region of the left ventricle that lies between the mitral valve and ventricular septum. An LVOT gradient greater than or equal to 30mmHg is typically diagnostic of obstructive hypertrophic cardiomyopathy.

Lennox-Gastaut syndrome – a difficult to treat form of childhood-onset epilepsy that most often appears between the second and sixth year of life, and is characterized by frequent seizures and different seizure types; it is often accompanied by mental retardation and behavior problems

Leptin – a hormone secreted by adipocytes in quantities associated with fat cell mass. Leptin exerts its effects via binding to leptin receptors located in the central nervous system and several peripheral organs. Through interactions with these receptors, leptin plays a role in regulation of energy homeostasis and neuroendocrine function.

Levator function – is an assessment of the functional status of the levator muscle and is measured by placing the thumb firmly against the patient's eyebrow, with eyes in downward gaze. Then patient looks upward, and the amount of excursion is measured with a scale which can be graded as normal (15 mm), excellent (over 12 mm), good (9 to 11 mm), fair (5 to 9 mm) or poor (less than 4 mm).

Levodopa equivalents – calculated using the total daily dose of levodopa and COMT inhibitors (e.g., entacapone, opicapone)

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Levodopa source	Multiplication factor
IR levodopa (including enteral suspension)	No adjustment (multiply by 1)
SR/ER levodopa	Multiply by 0.75
Any COMT inhibitor (e.g., entacapone, opicapone)	Multiply sum of daily levodopa total by 1.33

Example: member is taking levodopa/carbidopa ER 200 mg/50 mg, 1 tablet twice daily with entacapone 200mg, 1 tablet twice daily
ER Levodopa: 400 mg x 0.75 = 300 mg
For COMT component: 300 mg x 1.33 = 399 mg total daily levodopa equivalents

Lipodystrophy – an acquired or congenital disorder characterized by selective loss of subcutaneous adipose tissue from the face, arms, and legs. Affected patients are predisposed to insulin resistance, dyslipidemia, and hepatic steatosis.

Long-chain fatty acid oxidation disorders (LC-FAOD) – a group of rare, life-threatening autosomal recessive disorders characterized by metabolic deficiencies in which the body is unable to convert long-chain fatty acids into energy. LC-FAOD are a heterogeneous group of diseases, with clinical manifestations varying among subtypes and differing by age of onset and evolution of the clinical phenotype over time. Clinical manifestations associated with the disease include cardiomyopathy, hypoglycemia, skeletal myopathy, rhabdomyolysis, and encephalopathy.

Long exercise test (LET) – recording evoked compound motor action potentials (CMAP) over 30-45 minutes, every one to two minutes and then every five minutes, after long effort (2-5 minutes, with brief 3 to 4 second rest periods every 15-45 seconds).

Interpretation of LET:
CMAP amplitude decrement = [(maximum CMAP amplitude prior to exercise – minimum CMAP amplitude post exercise)/ maximum CMAP amplitude prior to exercise] x 100%

CMAP area decrement = [(maximum CMAP area prior to exercise – minimum CMAP area post exercise)/ maximum CMAP area prior to exercise] x 100%

Positive LET for periodic paralysis is identified by one of the following:
- Minimum 40% CMAP amplitude decrement post exercise, OR
- Minimum 50% CMAP area decrement post exercise

Lupus Nephritis – inflammatory kidney disease that results as a complication of SLE
- **Class III (focal):** active, chronic, at high risk of developing end stage renal disease

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- **Class IV (diffuse):** active, chronic at high risk of developing end stage renal disease
- **Class V (membranous):** presents similar to nephrotic syndrome with subendothelial deposits

Luteinizing Hormone Releasing Hormone (LHRH) Agents - utilized to suppress puberty for relief of gender dysphoria, as well as to improve psychological and physical outcomes. LHRH agents may be used with cross-sex hormones, up to the time of sex confirmation surgery.

Lymphangioleiomyomatosis (LAM) – an uncommon multisystem disease that affects primarily natal females and is characterized by proliferation of abnormal smooth muscle-like cells (LAM cells) that lead to cystic lung destruction, chylous pleural effusions, lymphatic masses (lymphangioleiomyomas), and abdominal angiomyolipomas.

Lymphangiomyomatosis – a condition affecting the lungs, kidneys, and lymphatic system that may occur sporadically or in association with tuberous sclerosis complex (TSC-LAM). Overgrowth of atypical smooth muscle-like cells (LAM cells) in the lungs result in the formation of lung cysts and destruction of normal lung tissue.

Lymphoproliferative disorder – condition where lymphocytes are produced in excessive quantities. This disorder usually occurs in patients who have compromised immune systems (e.g., leukemia, lymphoma).

Lysosomal Acid Lipase Deficiency (LAL-D) – is an under-recognized, rare, autosomal recessive disease that is caused by mutations in the *LIPA* gene. Lysosomal acid lipase is a lipid-metabolizing enzyme that breaks down lipid particles and regulates lipid metabolism. In infants, known as Wolman Disease, the disease has a rapidly progressive course, with death typically by six-months of age. In older children and adults, known as Cholesteryl Ester Storage Disease (CESD), the disease leads to cirrhosis and other complications.

MAC-directed antibiotic therapy – treatment with multiple (typically 3) antibiotics which include a macrolide (clarithromycin or azithromycin), rifabutin or rifampin, and ethambutol; regimens may also include a systemic aminoglycoside; alternative antibiotics for macrolide resistance include isoniazid and fluoroquinolones

MAOI – a drug in the Monoamine Oxidase Inhibitor class of medications that is FDA-approved to treat major depressive disorder

Marginal reflex distance of the upper lid (MRD_1) – a measurement used for ptosis diagnosis in which the patient fixates on a light held by the examiner and the distance between the corneal reflection and the upper lid margin is measured. A value of 4-5mm is considered as normal MRD_1

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Mayo imaging classification – a validated tool that uses height-adjusted total kidney volume (htTKV) and age to identify ADPKD patients at the highest risk for progression independent of renal function. This classification system categorizes patients into five prognostic classes from the lowest to the highest risk for disease progression (classes 1A, 1B, 1C, 1D and 1E). Classes 1C, 1D and 1E are defined as high risk for progression to end-state kidney disease (ESRD).

Metastatic carcinoid tumor – tumors of the gastrointestinal tract formed from the endocrine (argentaffin) cells of the mucosal lining of a variety of organs including the stomach and intestine that have metastasized, or spread, to nearby muscle tissue and lymph nodes, or distant organs, such as the liver, and bones. Symptoms of these tumors include facial flushing, diarrhea, abdominal pain, asthma, rash, heart disease, and intestinal bleeding.

Methylenetetrahydrofolate reductase (MTHFR) deficiency – a genetic mutation that may lead to high levels of homocysteine in the blood and low levels of folate and other vitamins.

MI – myocardial infarction

Microscopic polyangiitis (MPA) – a necrotizing vasculitis predominantly affecting the capillaries, venules, or arterioles. Almost all patients with MPA have kidney manifestations with or without some pulmonary involvement. Approximately 90% of MPA patients have ANCA autoantibodies present.

Missense point mutation – a form of point mutation resulting in a codon that codes for a different amino acid, and thus, causes the synthesis of a protein with an altered amino acid sequence during translation.

Modified Medical Research Council Scale – an ordinal scale of muscle strength that ranges from 0-5 with lower scores associated with lower muscle contraction (zero, trace, poor, fair, good, normal).

Modified–release – dosage formulation that gradually releases active drug components over several hours. Metformin ER modified-release (Glumetza) 500 mg tablet utilizes Acuform® gastric retention technology and 1000 mg tablet utilizes Smartcoat® gastric retention technology. Acuform is a drug delivery technology that uses a unique swelling polymer to allow the tablet to be retained in the stomach and release active ingredient over a longer period of time. Smartcoat is a drug delivery technology that uses a strong molecular diffusion membrane in the tablet to control the release of the active ingredient.

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Muckle-Wells Syndrome (MWS) – an autosomal dominant inherited inflammatory disease characterized by chronic recurrent urticaria, periodic arthritis, sensorineural deafness, general inflammation, and secondary amyloidosis

Mucopolysaccharidosis I – a rare, autosomal recessive genetic disease caused by a defect in the gene coding for the lysosomal enzyme alpha-L-iduronidase resulting in inability to produce sufficient amounts of the enzyme

Mucopolysaccharidosis type IVA Morquio A syndrome – autosomal recessive lysosomal storage disorder result from a deficiency in GALNS activity, which results in an accumulation of keratin sulfate and chondroitin-6-sulfate in the lysosome. This accumulations leads to impaired cellular function, which causes short stature, skeletal dysplasia, bone deformity, as well as reduced visual, auditory, digestive, cardiovascular and respiratory function.

Mucopolysaccharidosis VI – a progressive lysosomal storage disorder caused by a deficiency in the arylsulfatase B enzyme causing retention of glycosaminoglycans leading to multisystemic organ damage

Mucopolysaccharidosis VII – autosomal recessive lysosomal storage disorder characterized by the deficiency of beta-glucuronidase, which results in an accumulation of glycosaminoglycans in cells throughout the body leading to multisystem tissue and organ damage.

Multi-luminance mobility testing (MLMT) score – the MLMT was designed to measure changes in function vision as assessed by the ability of a subject to navigate a course accurately and at a reasonable pace at different levels of environment illumination. The MLMT assesses both eyes and each eye separately at one or more of 7 levels of illumination ranging from 400 hix (corresponding to a brightly lit office) to 1 hix (corresponding to a moonless summer night). Each light level was assigned a score code ranging from 0 to 6. A higher score indicated that, a subject, was able to pass the MLMT at a lower light level.

Multiple Sleep Latency Test (MSLT) – generally performed 1.5 to 3 hours after the PSG if there is a clinical suspicion of narcolepsy. The patient is placed in a sleep-inducing environment and instructed to try to sleep every 2 hours for a total of 4-5 naps. Each nap continues for 15 minutes after sleep onset to detect rapid eye movement (REM) sleep, called sleep onset rapid eye movement periods (SOREMPs). In addition, sleep latency is documented for each nap. Normal results indicate a mean sleep latency of ~10-15 min with no SOREMPs. Stimulants should be stopped 1 week before testing and antidepressants should be stopped 2-4 weeks before testing as these medications can suppress REM sleep and prevent the appearance of SOREMPs. In addition, withdrawal from these drugs can produce rebound SOREMPs for several weeks after discontinuation.

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Myometry – measurement of muscle strength with an apparatus.

Myelofibrosis – a myeloproliferative disorder (of the bone marrow cells) in which collagen accumulates fibrous scar tissue in the marrow cavity, leading to anemia, neutropenia, thrombocytopenia, weakness, fatigue, and often, an enlarged spleen and liver. A diagnosis of myelofibrosis can be confirmed by (1) having megakaryocyte proliferation and atypia with fibrosis, (2) indicating a clonal marker (e.g. JAK2V617F or other markers), (3) not meeting World Health Organization criteria for polycythemia vera, myelodysplastic syndrome, CML, or other myeloid neoplasms, and (4) having the presence of at least two of the following: leukoerythroblastosis, increased serum lactate dehydrogenase (LDH) levels, anemia, palpable splenomegaly. These characteristics can be confirmed through a variety of procedures, such as a bone marrow biopsy, CT/MRI, CBC and other blood tests, or clinical findings.

Myelofibrosis risk stratification – based on the International Prognostic Scoring System (IPSS), which estimates the survival for newly diagnosed myelofibrosis patients by assigning a point to each of the following five prognostic variables: age > 65 years, hemoglobin < 10 g/dL, white blood cell count > 25 x 109/L, circulating blood blasts ≥ 1%, and constitutional symptoms (e.g. weight loss >10%, unexplained fever, excessive sweating). Patients are classified as low-risk with 0 points, intermediate-1-risk (INT-1) with 1 point, intermediate-2-risk (INT-2) with 2 points, and high risk with ≥ 3 points.

Myelokathexis – a congenital disorder of the white blood cells that causes severe, chronic leukopenia (a reduction of circulating white blood cells) and neutropenia (a reduction of neutrophil granulocytes).

N-acetylgalactosamine-6-sulfate sulfatase (GALNS) – lysosomal enzyme responsible for degrading glycosaminoglycans keratin sulfate and chondroitin-6-sulfate.

N-acetylglutamate (NAG) – an essential allosteric activator of carbamoyl phosphate synthetase 1 (CPS 1) in liver mitochondria. NAG is the product of N-acetylglutamate synthase (NAGS), a mitochondrial enzyme. Carglumic acid acts as a replacement for NAG in NAGS deficiency patients by activating CPS 1.

N-acetylglutamate synthase (NAGS) – an enzyme which converts acetylCoA and glutamate into N-acetylglutamate (NAG).

Narcolepsy – a central disorder of hypersomnolence that is characterized by excessive daytime sleepiness with or without cataplexy. Diagnosis is made when an overnight PSG and a subsequent MSLT show (1) a mean sleep latency of < 8 minutes and (2) at least 2 SOREMPs during the MSLT OR a SOREMP during overnight PSG and at least 1 SOREMP during the subsequent MSLT

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Nasal Polyps Score (NPS) – the score is the sum of the right and left nostril scores, as evaluated by means of nasal endoscopy. NPS is graded based on polyp size as described in the following table.

Nasal Polyps Score (NPS)	
Polyp Score	Polyp Size
0	No polyps
1	Small polyps in the middle meatus not reaching below the inferior border of the middle turbinate
2	Polyps reaching below the lower border of the middle turbinate
3	Large polyps reaching the lower border of the inferior turbinate or polyps medial to the middle turbinate
4	Large polyps causing complete obstruction of the inferior nasal cavity

Neutropenia – a reduction in the blood neutrophil count. Neutrophils represent 40% - 70% of the total white blood cell count and serve as the primary defense against infection.

Niemann-Pick disease type C (NPC) – an ultrarare genetic disorder caused by NPC1 or NPC2 gene changes. It is characterized by an inability of the body to transport cholesterol and other fatty substances, such as lipids, inside of cells. NPC is a neurodegenerative disease with symptoms such as loss of previously achieved gross and fine motor skills, ataxia, seizures, cataplexy, hearing loss, and cognitive decline. Patients with NPC commonly struggle with speech, swallowing, and eating.

NLRP3 gene – nucleotide-binding domain, leucine-rich family, pyrin domain containing 3 gene. This gene encodes cryopyrin, a protein contained in intracellular inflammasomes. Cryopyrin regulates protease caspase-1 and controls interleukin-1 beta.

NMDA – N-methyl-D-aspartate

NMOSD – neuromyelitis optica spectrum disorder – rare, relapsing, autoimmune, inflammatory disorder of the central nervous system (CNS) including the opic nerves and spinal cord. Historically it was considered a clinical variant of multiple sclerosis (MS), but is no distinguished from MS as an independent disease

Nocturnal polyuria – an excessive volume of urine production during sleeping hours (>33% of total urine production) while 24 hour urine output remains <40 mL/kg

Non-acral – not occurring on the hands, feet, toes, or fingers

Non-Diabetic Autonomic Neuropathy (NDAN) – can occur as a result of injury or secondary to various diseases such as: amyloidosis, autoimmune disease, certain infections (such as syphilis, Lyme, HIV/AIDS), Multiple Sclerosis, renal failure, vitamin

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B-12 deficiency, alcohol abuse and Parkinson's Disease. NDAN may also occur as a side effect of medications, such as certain cytotoxic chemotherapeutic agents and amiodarone.

Non-tuberculous mycobacteria (NTM) – mycobacterial species other than the *Mycobacterium tuberculosis* complex (*M. bovis, M. africanum, M microti, M. canetti, M. caprae, M. pinnipedii, M. suricattae,* and *M. mungi*) and those organisms causing leprosy (*M. leprae* and *M. lepromatosis*). NTM are ubiquitous environmental organisms mostly found in soil and water and cause lung, sinus, lymph node, joint, CNS, and catheter-related and disseminated infections in susceptible individuals.

Nonsegmental vitiligo – a subtype of vitiligo. Vitiligo is a chronic autoimmune disorder characterized by the development of depigmented, white macules on the skin. Nonsegmental vitiligo is the most common type and involves bilateral and symmetrical distribution of depigmented macules.

North Star Ambulatory Assessment/Timed Functional Test – measures motor function. It tests various items ranging from standing to running, each item can be scored from 2 (normal, achieves goal without any assistance, 1 (modified method, but achieves goal independent of physical assistance from another person), to 0 (unable to achieve independently), certain items can also be timed.

NSAID – Non-steroidal anti-inflammatory drug

NTM pulmonary disease – progressive inflammatory lung damage caused by chronic infection with NTM; can be classified as fibro-cavitary disease and or as nodular bronchiectatic disease; the most common species causing NTM pulmonary disease are *M. avium* complex (MAC) (consisting of *M. avium, M. intracellulare,* and *M. chimaera*), *M. kansasii, M. malmoense, M. xenopi, M. abscessus, M. chelonae,* and *M. fortuitum*.

NYHA – New York Heart Association

NYHA Functional Classification for heart failure

- NYHA I: no limitation of physical activity, ordinary physical activity does not cause undue fatigue, palpitation, dyspnea
- NYHA II: slight limitation of physical activity, comfortable at rest, ordinary physical activity results in fatigue, palpitation, dyspnea (shortness of breath).
- NYHA III: marked limitation of physical activity, comfortable at rest, less than ordinary activity causes fatigue, palpitation, or dyspnea
- NYHA IV: unable to carry on any physical activity without discomfort, symptoms of heart failure at rest, if any physical activity is undertaken, discomfort increases

OAT4 – Organic anion transporter 4, a transporter associated with diuretic-induced hyperuricemia.

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Obstructive Sleep Apnea (OSA) – a sleep-related breathing disorder characterized by daytime sleepiness, loud snoring, gasping, choking, snorting, or interruptions in breathing while sleeping. Diagnosis is made with (1) signs/symptoms (eg, associated sleepiness, fatigue, insomnia, snoring, subjective nocturnal respiratory disturbance, or observed apnea) OR associated medical or psychiatric disorder (ie, hypertension, coronary artery disease, atrial fibrillation, congestive heart failure, stroke, diabetes, cognitive dysfunction, or mood disorder) and (2) 5 or more predominantly obstructive respiratory events (obstructive and mixed apneas, hypopneas, or respiratory effort-related arousals) per hour of sleep during polysomnography. Alternatively, the diagnosis can be made with ≥ 15 obstructive respiratory events per hour even in the absence of associated symptoms or disorders.

ODT – orally disintegrating tablet

OFF episode – a return of Parkinson's symptoms despite medication use and occurs due to an inadequate or inconsistent delivery of dopamine to the brain.

"Off" state – patient experiences worsening parkinsonian symptoms

"On" state – patient enjoys good response to a dose of levodopa

Opioid Tolerant – receiving one of the following opioid regimens for 1 week or longer:

- 60mg/day of oral morphine
- 25mcg/hr of transdermal fentanyl
- 30mg/day of oral oxycodone
- 8mg/day of oral hydromorphone
- 25mg/day of oral oxymorphone
- 60mg/day of oral hydrocodone
- Equianalgesic dose of another opioid

Oral targeted anti-inflammatory agent – oral drug designed to block specific molecules, receptors, or pathways involved in the development of autoimmune diseases. These include but are not limited to: phosphodiesterase-4 (PDE4) inhibitors (e.g., apremilast [Otezla]), Janus kinase (JAK) inhibitors (e.g., tofacitinib [Xeljanz, Xeljanz XR]), sphingosine 1-phosphate (S1P) receptor modulators (e.g., ozanimod [Zeposia]), tyrosine kinase 2 (TYK2) inhibitors (e.g., deucravacitinib [Sotyktu]). Drugs NOT included in this category are corticosteroids, NSAIDs and conventional synthetic DMARDs (e.g., methotrexate, leflunomide, hydroxychloroquine, sulfasalazine).

Orilissa Recommended Dose and Duration

Dosing Regimen	Maximum Treatment Duration	Coexisting Condition

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Initiate treatment with Orilissa 150 mg once daily	24 months	None
Consider initiating treatment with Orilissa 200 mg twice daily	6 months	Dyspareunia
Initiate treatment with Orilissa 150 mg once daily. Use of 200 mg twice daily is not recommended.	6 months	Moderate hepatic impairment (Child-Pugh Class B)

Osmotic–release – Dosage formulation that uses osmotic pressure to release drug at a constant rate. Metformin ER osmotic-release (Fortamet) tablet utilizes SCOT™ technology. SCOT is a drug delivery technology that uses an osmotically active core formulation surrounded by a semipermeable membrane with two laser-drilled holes on either side of the tablet that enables the drug to exit the tablet at a constant rate.

Osteomalacia – characterized by the softening of the bones in adults

OTC – Over the counter

Papilledema – bilateral optic disc swelling due to raised intracranial pressure

Paroxysmal Nocturnal Hemoglobinuria (PNH) – a rare disorder where the immune system attacks red blood cells, resulting in anemia and thrombosis

PDC – Proportion of Days Covered

PDGFR – platelet-derived growth factor receptor

PE – pulmonary embolism

Pediatric body mass index (BMI) – for children and teens, BMI is age- and sex-specific and is often referred to as BMI-for-age. After BMI is calculated for children and teens, it is expressed as a percentile obtained from either a graph or a percentile calculator. These percentiles express a child's BMI relative to US children who participated in national surveys. Weight and height change during growth and development, as does their relation to body fatness. Consequently, a child's BMI must be interpreted relative to other children of the same sex and age. The BMI-for-age percentile growth charts are the most commonly used indicator to measure the size and growth patterns of children and teens in the US. BMI-for-age weight status categories and the corresponding percentiles are shown in the following table.

Weight Status Category	Percentile Range
Underweight	Less than the 5th percentile

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Normal or Healthy Weight	5th percentile to less than the 85th percentile
Overweight	85th to less than the 95th percentile
Obese	Equal to or greater than the 95th percentile

Percent predicted FEV_1 – percent of the predicted FEV_1 value for persons of the same age, sex, and height

Perseveration – the repetition of a particular response regardless of the absence or cessation of a stimulus.

Phenylketonuria – an inherited genetic disorder caused by a deficiency in the enzyme phenylalanine hydroxylase resulting in elevated blood concentrations of the amino acid phenylalanine. Untreated excessive blood phenylalanine levels lead to intellectual disabilities, delayed development, seizures, and behavioral problems.

Pheochromocytoma – a rare tumor that develops in the central part of the adrenal gland the adrenal medulla. The tumor develops in chromaffin cells, which are situated in the center of the adrenal glands. Chromaffin cells are also found in the heart, head, neck, bladder, back wall of the abdomen, and along the spine; tumors in these chromaffin cells are called paragangliomas. Most of the cases of pheochromocytomas and paragangliomas are benign, but about 10% to 15% of pheochromocytomas and 20% to 50% of paragangliomas can be malignant. Pheochromocytomas and paragangliomas release hormones, primarily epinephrine and norepinephrine, that cause either episodic or persistent high blood pressure. Hypertensive crisis can lead to cardiac arrhythmias (e.g., atrial and ventricular fibrillation), myocardial infarction, and even death. If the severe hypertension is left untreated, it can result in severe or life-threatening conditions (e.g., heart disease, stroke, kidney disease, acute respiratory distress, damage to the nerves of the eye). Other clinical signs and symptoms include perspiration, headache, tachycardia/forceful palpitations, tremors, dyspnea.

***PHEX* gene mutation –** phosphate-regulating gene located on the X-chromosome that encoding for phosphate-regulating endopeptidase homolog X-linked, a mutation results in excess circulating fibroblast growth factor 23 (FGF-23).

Phosphodiesterase-5 Enzyme Inhibitor (PDE5) – type of medication that prevents the action of phosphodiesterase-5, an enzyme that breaks phosphodiester bonds in cyclic guanosine monophosphate (cGMP). Inhibition of PDE5 results in increased cGMP concentration which leads to smooth muscle relaxation and inflow of blood to the corpus cavernosum.

Phlebotomy – the removal of blood and a procedure recommended for all patients with polycythemia vera to maintain the hematocrit at less than 45%.

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***PIK3CA*-Related Overgrowth Spectrum (PROS) -** a group of overgrowth syndromes caused by sporadic activating mutations in the PIK3CA gene. Examples include: KTS (Klippel-Trenaunay Syndrome), CLOVES syndrome (congenital lipomatous overgrowth, vascular malformations, epidermal nevi, scoliosis/skeletal and spinal), ILM (isolated lymphatic malformation), MCAP or MCM (megalencephaly-capillary malformation), HME (hemimegalencephaly)/DMEG (dysplastic megalencephaly)/focal cortical dysplasia type II, HHML (hemihyperplasia-multiple lipomatosis), FIL (facial infiltrating lipomatosis), FAVA (fibroadipose vascular anomaly), macrodactyly, muscular hemihyperplasia, FAO (fibroadipose hyperplasia or overgrowth), CLAPO syndrome (capillary malformation of the lower lip, lymphatic malformation of the face and neck, asymmetry of the face and limbs, and partial or generalized overgrowth), epidermal nevus, benign lichenoid keratosis, or seborrheic keratosis

Polycythemia vera – a myeloproliferative disorder characterized by clonal proliferation of the erythroid, myeloid, and megakaryocyte lineages. Increase erythrocyte production results in increase red blood cell mass, and white blood cell and platelet counts may also be elevated. Symptoms may include fatigue, pruritis, night sweats, and enlarge spleen.

Polyneuropathy Disability (PND) Score

Stage	Description
0	No symptoms
I	Sensory disturbances but preserved walking capability
II	Impaired walking capacity but ability to walk without a stick or crutches
IIIA IIIB	Walking with the help of one stick or crutch Walking with the help of two sticks or crutches
IV	Confined to a wheelchair or bedridden

Polysomnography (PSG) – diagnostic test for sleep disorders that uses electroencephalography (EEG) in addition to several other monitoring devices to evaluate sleep architecture, sleep quality, and other physiologic parameters. PSG is typically performed overnight during a full night of sleep, however split-night protocols can be used when there is a high clinical suspicion for obstructive sleep apnea to allow for the diagnosis to be established in the first portion of the study and for the titration of continuous positive airway pressure (CPAP) during the second portion of the study, rather than performing CPAP titration on a second full night.

Postrema Syndrome – an episode of otherwise unexplained hiccups or nausea and vomiting due to a lesion in the dorsal medulla

Presbyopia – a vision condition in which the shape of the crystalline lens of the eye changes, which makes it difficult to focus on close objects.

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Primary biliary cholangitis (PBC) – a chronic, progressive autoimmune cholestatic liver disease that is characterized by lymphocytic cholangitis and intralobular bile duct destruction leading to development of fibrosis, cirrhosis, and liver failure.

Primary hyperoxalurias – are a group of rare genetic metabolic disorders that are characterized by the accumulation of a substance known as oxalate in the kidneys and other organ systems of the body (extrarenal systemic oxalosis - e.g., thyroid, heart, blood vessels, skin, bone, bone marrow, joints, central and peripheral nervous system, and retina). Patients with this disease lack functional levels of a specific enzyme that normally prevents the accumulation of oxalate. There are three types of PH differentiated by the specific enzyme that is deficient (type 1, 2, 3). PH type 1 is caused by mutations in the AGXT gene, and is the most severe, and common form (70-80%). PH type 2 is caused by mutations in the GRHPR gene. PH type 3 is caused by mutations in the HOGA gene (formerly known as the DHDPSL gene). The genetic mutations that cause PH are inherited as autosomal recessive traits. The prevalence of PH type 1 has been estimated to be 1-3 cases per 1 million. In the kidneys, excess oxalate binds with calcium to form calcium oxalate that is the main component of kidney and urinary stones. Common symptoms include the formation of stones throughout the urinary tract (urolithiasis) and kidneys (nephrolithiasis) and progressively increased levels of calcium in the kidneys (nephrocalcinosis). Recurrent stone formation and the accumulation of calcium oxalate in kidney tissue can cause chronic kidney disease, which can ultimately progress to end stage renal disease.

Primary progressive multiple sclerosis (PPMS) – characterized by progressive accumulation of disability from onset with no distinct relapses

Processing-deficient Progeroid Laminopathy (PL) – rare, fatal genetic disorders that lead to accumulation of abnormal protein (progerin or progerin-like proteins) in the cell nuclear membrane and disruption of cellular structure and tissue repair functions. These disorders produce clinical features that overlap with HGPS. Many processing-deficient genetic mutations have been identified in the *ZMPSTE24* and *LMNA* (other than c.1824C > T [p.Gly608Gly] seen with HGPS) genes in these patients.

Progressive familial intrahepatic cholestasis (PFIC) – a spectrum of rare, heterogeneous, genetic liver disorders, characterized by early onset cholestasis in infancy which leads to rapidly progressive liver failure and early mortality

Propionic acidemia (PA) and Methylmalonic Acidemia (MMA) – accumulation of propionic acid and/or methylmalonic acid caused by inborn errors of metabolism due to deficiency of propionyl-CoA carboxylase (for PA) or methylmalonyl-CoA mutase (for MMA). This can lead to metabolic acidosis and hyperammonemia.

Proportional Assist Ventilation (PAV) – spontaneous breathing mode created to improve ventilator synchrony by creating more natural breathing. PAV is a form of

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synchronized partial ventilator assistance where support is adaptive to keep the work of breathing constant.

Pruriginous lesions – excoriated, scaling and/or crusted papules and/or nodules and/or plaques, often with a whitish or pink center and hyperpigmented border.

Pseudobulbar Affect (PBA) – condition that occurs secondary to a variety of otherwise unrelated neurological conditions, and is characterized by involuntary, sudden, and frequent episodes of laughing and/or crying. PBA episodes typically occur out of proportion or incongruent to the underlying emotional state.

Pulmonary Airway Clearance Therapy – airway clearance therapies improve the efficacy of the mucociliary escalator and elicit a cough, this therapy is necessary to maintain lung health in individuals affected by cystic fibrosis. Therapies include postural drainage/percussion, positive expiratory pressure (device), active cycle of breathing technique, autogenic drainage, oscillatory positive expiratory pressure (device), high-frequency chest wall oscillation (vest), and exercise

Pulmonary Arterial Hypertension (PAH) – PAH is a syndrome resulting from restricted flow through the pulmonary arterial circulation resulting in increased pulmonary vascular resistance and subsequent right heart failure. The diagnosis of PAH requires confirmation with a complete right heart catheterization. The hemodynamic definition of PH-ILD includes a mean pulmonary artery pressure (mPAP) greater than or equal to 25 mmHg in the setting of a normal pulmonary capillary wedge pressure (PCWP) of 15 mmHg or less with a pulmonary vascular resistance (PVR) greater than 3 Wood units.

Pulmonary endarterectomy (PEA) – surgical procedure to remove obstructive, hardened, thromboembolic materials from the pulmonary arteries to improve hemodynamics in patients with CTEPH.

Pulmonary hypertension due to interstitial lung disease (PH-ILD) – Pulmonary hypertension due to interstitial lung disease can include idiopathic pulmonary fibrosis, chronic hypersensitivity pneumonitis, and nonspecific interstitial pneumonia, combined pulmonary fibrosis and emphysema. The diagnosis of PAH requires confirmation with a complete right heart catheterization. The hemodynamic definition of PAH includes a mean pulmonary artery pressure (mPAP) greater than or equal to 25 mmHg in the setting of a normal pulmonary capillary wedge pressure (PCWP) of 15 mmHg or less with a pulmonary vascular resistance (PVR) greater than 3 Wood units.

Pulmonary Hypertension Specialist – a cardiologist or pulmonologist who is a member of the UPMC Presbyterian Comprehensive Pulmonary Hypertension Program OR who is a member of the Pulmonary Hypertension Association

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Pulmonary Vascular Resistance (PVR) – PVR (wood units) = (mPAP – PCWP)/ Cardiac Output. PVR (wood units) = PVR (dynes-sec-cm^{-5})/80

Pyruvate kinase (PK) deficiency – a rare, inherited genetic disorder characterized by premature destruction of red blood cells (hemolytic anemia). It is caused by genetic mutations in the pyruvate kinase liver and red blood cell (*PKLR*) gene, which leads to a deficiency of the enzyme pyruvate kinase. Patients with PK deficiency have a range in severity of symptoms, which may include fatigue, jaundice, and shortness of breath. Complications associated with PK deficiency include gallstones, pulmonary hypertension, extramedullary hematopoiesis, osteoporosis, and iron overload.

QT interval – a measure of the time between the start of the Q wave and the end of the T wave in the heart's electrical cycle. A prolonged QT interval is a risk factor for ventricular tachyarrhythmia and sudden death.

Recommended palovarotene (Sohonos) dosage regimens

	Weight	Daily dosage	Week 1 to 4 Flare-up Dosage	Week 5 to 12 Flare-up Dosage
Pediatric patients ≤13 years	10 kg to 19.9 kg	2.5 mg	10 mg	5 mg
	20 kg to 39.9 kg	3 mg	12.5 mg	6 mg
	40 kg to 59.9 kg	4 mg	15 mg	7.5 mg
	≥ 60 kg	5 mg	20 mg	10 mg
Adults and pediatric patients ≥ 14 years	All weights	5 mg	20 mg	10 mg

Recurrence – a new decrease in the platelet count that necessitates the reinitiation of plasma exchange after normalization of the platelet count (≥ 150 x 10^9/L), has occurred.

Refractory Myasthenia Gravis Disease – unchanged or worse disease after corticosteroids and at least 2 other immunosuppressants, used in adequate doses for an adequate duration, with persistent symptoms or side effects that limit functioning, as defined by patient and physician

Relapsing forms of Multiple Sclerosis (RMS) – include secondary progressive MS characterized as active (with relapses) and relapsing remitting multiple sclerosis (RRMS) – the most common type of MS which is characterized by clearly defined relapses (clinical attacks) with full or partial recovery

REMS – Risk Evaluation and Mitigation Strategy

Ribavirin ineligibility – defined as one of the following: hemoglobin level < 10 g/dL, known hypersensitivity reaction (e.g., Stevens-Johnson syndrome, toxic epidermal

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necrolysis, and erythema multiforme) to ribavirin, autoimmune hepatitis, hemoglobinopathies (e.g. thalassemia major, sickle-cell anemia), creatinine clearance < 50 mL/min, concomitant administration with didanosine

Rickets – characterized by imperfect calcification, softening, and distortion of the bones in children, typically resulting in bowed legs and short stature

Risk stratification – based on the Dynamic International Prognostic Scoring System (DIPSS), which estimates the survival for myelofibrosis patients using the following five prognostic variables: age > 65 years, hemoglobin < 10 g/dL, white blood cell count > 25 x 10^9/L, circulating blood blasts ≥ 1%, and constitutional symptoms (e.g. weight loss >10%, unexplained fever, excessive sweating). Patients are classified as low-risk with 0 factors, intermediate-risk with 1-2 factors, and high risk with ≥ 3 factors.

***RPE65* mutation-associated retinal dystrophy –** *RPE65* mutation-associated retinal dystrophy is one of many different types of inherited retinal dystrophies (IRDs). These IRDs are clinically heterogeneous and vary widely in their pathogenesis, progression, and mutation inheritance. Biallelic *RPE65* mutation-associated retinal dystrophy is an orphan disease, with an estimated 1,000-2,000 patients affected by this disease in the US. For patients with *RPE65* mutation-associated retinal dystrophy, common clinical diagnoses include Leber Congenital Amaurosis (LCA) and Retinitis Pigmentosa (RP). RP is characterized by reduced ability to perceive light and progressive loss of visual field. A less common but more severe inherited retinal dystrophy, LCA, a retinitis pigmentosa subtype, is further characterized by earlier onset, more rapid progression, and nystagmus. *RPE65* mutations are found in approximately 8-16% of patients with LCA and 2% of patients with RP. In the presence of biallelic mutations in the *RPE65* gene, there is a lack of functional RPE65 protein and the visual cycle is broken, resulting in progressive vision loss, and ultimately nearly all patients progress to complete blindness.

Severe Combined Immunodeficiency Disease (SCID) – a rare primary immune deficiency usually characterized by a severe defect in both the T and B lymphocyte systems resulting in serious infections

SCNA4 – sodium channel protein type 4 subunit alpha

Severe Primary IGF-1 Deficiency – includes classical and other forms of growth hormone insensitivity, including those with mutations in the GH receptor (GHR), post-GHR signaling pathway, and IGF-1 gene defects. These patients are not GH deficient and therefore would not be expected to respond to GH treatment.

Shagreen patch – an irregularly shaped, elevated skin patch comprised of excess fibrous tissue. These lesions typically appear on the lower back.

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Shiga-like Toxin-Producing Escherichia Coli (STEC-HUS) – disorder caused by infection with E. Coli O157:H7. Most often occurs when an infection in the digestive system produces toxic substances which destroy red blood cells and cause kidney injury.

Sialorrhea - excessive salivation or hypersalivation or drooling

Singleton Pregnancy – a pregnancy involving only one fetus

Singleton spontaneous preterm birth – a pregnancy involving only one fetus in which birth is not induced and occurs before 37 weeks of pregnancy

SLIT – Sublingual Immunotherapy

Slow Vital Capacity (SVC) – the slow vital capacity spirometry test is a functional test of the lungs, similar to the forced vital capacity test where patients exhale as hard and fast as possible and results are compared to others within age, gender, and weight range. The difference with this test, as its name might imply, is that patient exhale slowly.

SNRI – a drug in the Serotonin-Norepinephrine Reuptake Inhibitor class of medications that is FDA-approved to treat major depressive disorder

Soluble membrane attack complex (SMAC) or elevated plasma C5b9 – soluble C5b-9 levels are elevated in the majority of patients with active atypical hemolytic uremic syndrome but less frequently when the disease is in remission. Soluble C5b-9 may be uselful as a biomarker to monitor activity of terminal pathway of complement

Spinal Muscular Atrophy (SMA) – an autosomal recessive neuromuscular disease characterized by degeneration of the motor neurons in the anterior horn of the spinal cord, resulting in atrophy of the voluntary muscles of the limbs and trunk. Despite being a rare disease, SMA is a leading genetic cause of infant mortality and a major cause of childhood morbidity. It is attributed to deletions or mutations in the *SMN1* gene (chromosome 5q13), causing insufficient expression of survival motor neuron (SMN) protein. The lack of SMN protein appears to result in dysfunction and eventual death of motor neurons. SMA can present clinically at any time from in utero to adulthood with gross motor function deficits, muscle weakness, and pulmonary disease due to neuromuscular weakness. Common complications include: difficulty feeding, swallowing, failure to thrive, loss of ambulation, scoliosis, joint contracture, pulmonary disease, and death**.**

Spinal Muscular Atrophy Types I, II, III – Type I manifests around or before the patient is 6 months of age. The presentation may include: hypotonia, unable to control head movement, unable to sit without assistance. Type II has an onset between 6 to 18 months. Patients are generally able to sit independently, the ability to walk is usually not

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achieved without assistance. Type III manifests after the patient is 18 months of age or older. Patients may be able to walk without assistance or lose the ability to walk.

SSRI – a drug in the Selective Serotonin Reuptake Inhibitor class of medications that is FDA-approved to treat major depressive disorder

Syndrome of Inappropriate Antidiuresis (SIAD) – a disorder of sodium and water balance characterized by urinary dilution impairment and hypotonic hyponatremia, in the absence of renal disease or any identifiable non-osmotic stimulus able to induce antidiuretic hormone release.

Systemic Lupus Erythematosus (SLE) – a chronic inflammatory autoimmune condition that can cause disease of the skin, heart, lungs, kidneys, joints, and/or nervous system

Systemic sclerosis (SSc) – a rare, multisystem, connective tissue disease involving the skin, underlying tissues, blood vessels, and major organs. It is characterized by microvascular damage and fibrosis of the skin and of various internal organs, including the lung, heart, kidneys and the gastrointestinal tract. There is significant variability in the severity of the disease and the specific organ involvement, ranging from limited skin involvement to systemic multisystem disease. SSc is traditionally classified based on the extent of skin involvement and the accompanying pattern of internal organ involvement. The different disease subtypes are also associated with different patterns of involvement and disease evolution. The major subsets of SSc include limited cutaneous SSc (lcSSc), diffuse cutaneous SSc (dcSSc), SSc sine scleroderma, and SSc overlap syndrome.

Systemic Sclerosis-associated Interstitial lung disease (SSc-ILD) – a common complication of systemic sclerosis. It is a chronic lung disease in which scar tissue (fibrosis) and/or inflammation builds up in the walls of the air sacs of the lungs in a person with a diagnosis of SSc.

TCA – a drug in the Tricyclic class of medications that is FDA-approved to treat major depressive disorder

T Lympocyte (T-cell) – activated T-cells are the predominant mediators of immunologic rejection

Tanner Stages - staging system consisting of systematized descriptions of the development of secondary sexual characteristics, including breast changes in females, genital changes in males, and pubic hair in both males and females.

Tanner Stage	**Boys – Development of External Genitalia**	**Girls – Breast Development**	**Boys and Girls – Pubic Hair**
Stage 1	Prepubertal		

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Stage 2	Enlargement of scrotum and testes; scrotal skin reddens and changes in texture	Breast bud stage with elevation of breast and papilla; enlargement of areola	Sparse growth of long, slightly pigmented hair, straight or curled, at base of penis or along labia
Stage 3	Enlargement of penis (length at first); further growth of testes	Further enlargement of breast and areola; no separation of their contour	Darker, coarser, and more curled hair, spreading sparsely over junction of pubes
Stage 4	Enlargement of penis (length at first); further growth of testes	Areola and papilla form a secondary mound above level of breast	Hair adult in type, but covering smaller area than in adult; no spread to medial surface of thighs
Stage 5	Adult genitalia	Mature stage: projection of papilla only, related to recession of areola	Adult female in type and quantity, with horizontal upper border

Tardive dyskinesia – a neurological syndrome caused by the long-term use of dopamine receptor blocking drugs and is characterized by repetitive, involuntary, purposeless movements, features of the disorder may include: grimacing, tongue protrusion, lip smacking, puckering and pursing, rapid eye blinking, rapid movements of the arms, legs, and trunk, involuntary movements of the fingers

Tendon Xanthoma – cholesterol deposit formation on the Achilles tendon and/or extensor tendons of hands and feet, or on the knees and elbows, caused by increased LDL-C levels.

Thrombotic Microangiopathy (TMA) – a pathology that results in thrombosis in capillaries and arterioles. It may be seen in association with thrombocytopenia, anemia, purpura and renal failure. The classic TMAs are hemolytic uremic syndrome and thrombotic thrombocytopenic purpura

TNFi – tumor necrosis factor inhibitor

Transfusional iron overload – a complication of multiple or chronic blood transfusion where excess iron accumulates in the body and is deposited into multiple organ systems, leading to organ dysfunction and/or organ failure. Iron chelation therapy is necessary to remove excess iron to decrease associated morbidity and mortality. Examples of various anemia syndromes where patients may require regular red blood cell (RBC) transfusions and as a result require iron chelation therapy include, but are not limited to, the

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following: thalassemia, sickle cell disease, myelodysplastic syndromes (MDS), Diamond Blackfan anemia, aplastic anemia, and other congenital and acquired forms of refractory anemia.

Transthyretin-mediated amyloidosis (ATTR) – can be hereditary (hATTR, ATTRm) or wild-type (ATTRwt) genotype. The TTR protein is primarily produced in the liver and transports vitamin A and thyroxine throughout the body. In ATTR, there is misfolding of the TTR protein resulting in accumulation of amyloid in organs and tissue causing symptoms based on the organ(s) involved.

Transthyretin-mediated amyloid cardiomyopathy (ATTR-CM) – is caused by the deposition of transthyretin amyloid fibrils in the myocardium. The deposition occurs when wild-type or variant transthyretin becomes unstable and misfolds. It has been estimated that ~100,000 people in the US are living ATTR-CM and that only 1–2 % of patients have been diagnosed. Symptoms of heart failure are seen, including dyspnea, fatigue, orthostatic hypotension, and syncope. Death is mostly from cardiac causes, including sudden death and heart failure.

Treatment experienced – patients who have been previously treated with a combination of antiretroviral therapy

Treatment naïve – patients who have not been treated with any antiretroviral therapy

Tuberous Sclerosis Complex – a genetic disorder characterized by the formation of nonmalignant tumors (tubers) in multiple organ systems, primarily in the brain, eyes, heart, kidney, skin, and lungs; one of the leading genetic causes of epilepsy; about 85% of tuberous sclerosis complex patients present with seizures that typically begin during the first year of life, are often focal seizures or infantile spasms, and are refractory to treatment in about 60% of patients

Tumor-Induced Osteomalaia (TIO) – is also known as oncogenic osteomalacia, is a very rare paraneoplastic syndrome characterized by hypophosphatemia resulting from decreased tubular phosphate reabsorption, with a low or inappropriately normal level of active vitamin D. Tumors associated with TIO (e.g., phosphaturic mesenchymal tumor (PMT), which are usually benign) can produce fibroblast growth factor 23. Increased FGF-23 activity impairs renal phosphate reabsorption, which leads to hypophosphatemia, and decreases the synthesis of the active metabolite of vitamin D, 1,25 dihydroxyvitamin D (1,25[OH]2D). Signs and symptoms include fatigue, bone pain, muscle weakness, difficulty walking, height loss, fractures, pseudo-fractures, and decreased physical function. Biochemical features include: hypophosphatemia, elevated serum alkaline phosphatase, renal tubular phosphate wasting by calculation of tubular reabsorption of phosphate (TRP) and/or tubular maximum reabsorption of phos-phate to glomerular filtration rate (TmP/GFR), and elevated serum fibroblast growth factor 23 (FGF23).

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Tumor Necrosis Factor Receptor Associated Periodic Syndrome (TRAPS) – an autosomal dominant disorder with a childhood onset and characterized by periodic episodes of high spiking fevers. The attacks may be triggered by certain events (local injury, minor infection, exercise, stress, hormonal change) and last 1-3 weeks. The patient can experience myalgia as a prodromal symptom. Symptoms can include: rash, muscle cramps, headache abdominal pain, pleuritis, pericarditis, painful lymph nodes, conjunctivitis, periorbital edema.

Type I Gaucher Disease – an inherited disease that is caused by a functional deficiency of glucocerebrosidase, the enzyme that mediates degradation of the glycosphingolipid glucosylceramide. The failure to degrade glucosylceramide results in the lysosomal storage of this lipid material within tissue macrophages leading to widespread pathology. Macrophages containing stored glucosylceramide are typically found in the liver, spleen, and bone marrow and occasionally in the lung, kidney, and intestine. Secondary hematologic consequences include severe anemia and thrombocytopenia in addition to the characteristic progressive hepatosplenomegaly. Skeletal complications include osteonecrosis and osteopenia with secondary pathological features. Enzyme replacement therapy is the standard of care for most patients who require treatment of type I Gaucher disease. Type I disease is distinguished from type 2 and type 3 diseases by the lack of the characteristic involvement in the central nervous system.

Types of Peroxisomal Disorders (from Prescribing Information)

Neonatal Adrenoleukodystrophy (NALD)
Generalized Peroxisomal Disorder
Refsum Disease
Zellweger Syndrome (ZWS)
Peroxisomal Disorder, Type Unknown

URAT1 – Uric acid transporter 1, a transporter responsible for uric acid reabsorption from the renal tubular lumen.

Usual interstitial pneumonia (UIP) – the hallmark pathologic and/or radiologic pattern seen in idiopathic pulmonary fibrosis (IPF)

UVAR® – a photophoresis system used for treatment of cutaneous T-cell lymphoma

Vasoactive intestinal peptide tumor (VIPoma) – rare cancers in which tumor cells arise from certain hormone-producing cells called islet cells most often located in the pancreas. VIPomas produce excessive amounts of hormones, particularly vasoactive intestinal peptide, which disrupts water transport in the intestines causing the symptom of watery diarrhea, which is the most prominent symptom.

Vaso-occlusive crisis (VOC) – a hallmark acute complication for people with sickle cell disease and manifests as acute severe pain

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Venous thromboembolic event (VTE) – condition that occurs when a blood clot forms in a vein. VTE includes deep vein thrombosis and pulmonary embolism.

Vernal keratoconjunctivitis – a seasonally recurring, severe form of allergic inflammation that affects the ocular surface and can cause visual impairment, itching, burning, and photophobia.

Weight-for-age growth charts – growth charts are available for weight-for-age for those up to 20 years of age. These charts indicate the percentile weight falls under. CDC clinical growth charts are available at the following link: https://www.cdc.gov/growthcharts/clinical_charts.htm

Wernicke's encephalopathy – a neurologic emergency resulting from thiamine (Vitamin B1) deficiency. Signs and symptoms of Wernicke's encephalopathy may include ataxia, mental status changes, and ophthalmoplegia (e.g., nystagmus, diplopia).

WHIM syndrome (Warts, Hypogammaglobulinemia, Infections, and Myelokathexis) – a rare, autosomal-dominant immunodeficiency and congenital neutropenic disorder caused by a mutation in the CXCR4 gene in which the body's immune system does not function properly and has trouble fighting infections.

Wilson's Disease (or hepatolenticular degeneration) – a rare autosomal recessive genetic disorder of copper metabolism, which is characterized by hepatic and neurological disease.

X-linked hypophosphatemia (XLH) – is a hereditary, lifelong, and progressive disorder resulting from chronic hypophosphatemia based on the loss of function of the phosphate-regulating gene located on the X-chromosome *(PHEX)* which results in elevated blood levels of FGF-23. XLH is rare with X-linked dominant inheritance. XLH impacts children and adults throughout their lives and results in limitations in physical function and mobility. In children this manifests in rickets, lower extremity deformity, growth impairment, pain and diminished mobility. Adults suffer from fractures, pain and physical function limitations. Overall, rickets and osteomalacia due to chronic hypophosphatemia associated with XLH result in poor skeletal, muscular, and dental health.

World Health Organization (WHO) Etiologic Classification of Pulmonary Hypertension

Group 1	Pulmonary arterial hypertension
Group 2	Pulmonary hypertension with left heart disease
Group 3	Pulmonary hypertension associated with lung disease and/or hypoxemia
Group 4	Pulmonary hypertension due to chronic thrombotic and/or embolic disease
Group 5	Miscellaneous

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World Health Organization (WHO) Functional Classification of PAH

Class I	Members with no symptoms and for whom ordinary physical activity does not cause dyspnea or fatigue, chest pain or near syncope.
Class II	Members who are comfortable at rest but who have symptoms** with ordinary physical activity.
Class III	Members who are comfortable at rest but have symptoms** with less-than-ordinary effort.
Class IV	Members who have symptoms** at rest. Unable to carry out any physical activity without symptoms, discomfort is increased by any physical activity

**Key symptoms of PH include dyspnea or fatigue, chest pain, or near syncope (fainting)

Zar Score – scoring system developed to calculate a numeric score that determines the severity of the *Clostridium difficile* infection. Diagnosis of severe CDI is certain when the score is ≥ 2.

Clinical Signs	Score
Age ≥ 60 years	1
Temperature >38.3º C (100.9º F)	1
Albumin level <2.5 mg/dL	1
Peripheral white blood cell count >15,000 cells/mm^3	1
Endoscopic evidence of pseudomembranous colitis	2
Intensive Care Unit (ICU) status	2

III. PURPOSE

The purpose of this policy is to define the prior authorization process for all included medications.

IV. SCOPE

This policy applies to the Pharmacy Services Department.

V. PROCEDURE

Limitations:
If the established criteria below are not met for the requested medication, the request is referred to a UPMC Insurance Services Division Medical Director for review.

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Criteria for individual drug or class of drugs for the Advantage Choice, Your Choice, Value Choice and Open Choice Formularies:

- **Aceclidine (Vizz™):** [1, 2, 3]

Initial Authorization Criteria:
- Must be prescribed by an optometrist or ophthalmologist
- Must be between the ages of 45 to 75 at the initiation of treatment
- Must have chart documentation of a diagnosis of presbyopia confirmed by a refraction assessment
- Must have an adequate trial of pilocarpine ophthalmic solution (Vuity) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
- Must have a documented history of a failure of corrective eyeglasses and contact lenses to solve symptoms of presbyopia
  - Must provide clinical rationale to support why corrective eyeglasses and contact lenses are not able to treat symptoms of presbyopia despite having an up-to-date prescription and an assessment completed to determine whether a different type of contact lenses or eyeglasses would be an appropriate alternative
- Must provide chart documentation that the member has been counseled on the risk of using aceclidine (Vizz) in conditions with poor illumination (e.g., night driving)
- Must be prescribed a dose that is consistent with FDA-approved package labeling, nationally-recognized compendia, or peer-reviewed medical literature

If the above criteria are met, aceclidine (Vizz) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved based upon the prescriber's assessment while on therapy or the member continues to benefit from therapy.

- **Acute Hereditary Angioedema Products** [4, 5, 6, 7, 8, 9, 10, 11, 12]
  - Applies to: Berinert®, Firazyr®, Sajazir™, Ruconest®, Ekterly®
  - Requests for ecallantide (Kalbitor) please refer to policy RX.PA.406 Commercial, Exchange Medical Prior Authorization Policies

Initial Authorization Criteria:
- Must be prescribed by or in consultation with a HAE specialist
- Must meet the following age requirements:

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        - Ruconest: 13 years or older
        - Ekterly: 12 years or older
        - Note: Age restrictions are not required for icatibant (Firazyr, Sajazir) or Berinert
    - Must be prescribed for the treatment of acute HAE attacks
    - Must have a diagnosis of HAE confirmed by ALL of the following laboratory values (copy of laboratory reports required, must include reference ranges):
        - Low C4 complement level (mg/dL)
        - Normal C1q complement component level (mg/dL)
            - C1q level is not required for members with a documented family history of HAE OR symptom onset before age 30
        - Either of the following:
            - Low C1 esterase inhibitor antigenic level (mg/dL)
            - Low C1 esterase inhibitor functional level (percent)
    - For members age 18 years or older:
        - Must have an adequate trial of icatibant (Firazyr, Sajazir) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
    - Must NOT be using the requested product in combination with another HAE agent used for acute treatment (e.g., Berinert, Ekterly, Firazyr, icatibant, Kalbitor, Ruconest, Sajazir)

If the above criteria are met, the requested acute HAE product is approved for 1 fill.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one-year intervals based upon the following:
- Must have documentation from the provider that the member's condition has improved while on therapy
- Must NOT be using the requested product in combination with another HAE agent used for acute treatment (e.g., Berinert, Ekterly, Firazyr, icatibant, Kalbitor, Ruconest, Sajazir)

• Acoramidis (Attruby®) [13, 14]

Initial Authorization Criteria:
- Must be prescribed by or in consultation with a cardiologist who specialized in the treatment of amyloidosis
- Must be age 18 years or older
- Must have chart documentation of a diagnosis of transthyretin-mediated amyloid cardiomyopathy (ATTR-CM), including ALL of the following:
    - NYHA Functional Class I or II heart failure

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        - Evidence of TTR amyloid in the myocardium obtained by ONE of the following:
            - Biopsy and amyloid typing (immunohistochemistry or mass spectrometry) OR
            - Radionuclide imaging demonstrating cardiac uptake of one of the following radiopharmaceutical agents (a copy of the imaging report must be provided)
                - Technetium-99m pyrophosphate (Tc 99m PYP) OR
                - Technetium-99m hydroxymethylene diphosphonate (HMDP), aka technetium-99m oxidronate
        - Confirmation of the absence of immunoglobulin light-chain amyloidosis by serum studies (immunofixation without free light-chains) or hematology/oncology consultation
    - Must not be used in combination with a TTR-lowering agent (patisiran [Onpattro], vutrisiran [Amvuttra], eplontersen [Wainua]), or a TTR-stabilizing agent (tafamidis [Vyndaqel, Vyndamax])

If the above criteria are met, acoramidis (Attruby) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for the continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or stabilized based upon the prescriber's assessment while on therapy or the member continues to benefit from therapy.

• Alitretinoin (Panretin®) [15, 16]

Initial Authorization Criteria:
- Must be prescribed by or in consultation with a dermatologist or an HIV specialist
- Must be age 18 years or older
- Must have a diagnosis of cutaneous lesions in patients with AIDS-related Kaposi's sarcoma
- Must be on an antiretroviral regimen

If the above criteria are met, alitretinoin (Panretin) is approved for 3 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of treatment. Authorization may be extended at one-year intervals based upon documentation from the provider that the member's condition has stabilized or improved based upon the prescriber's assessment while on therapy.

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- **Alosetron (Lotronex®)** [17, 18, 19, 20]

Initial Authorization Criteria:

- Must be prescribed by a gastroenterologist
- Must be a female member
- Must be age 18 years or older
- Must have a diagnosis of diarrhea-predominant IBS with chronic IBS symptoms
- Must provide documentation showing that nutritional interventions (such as low FODMAP diet, dietary counseling, nutritional assessment, etc.) did not result in a decreased occurrence of symptoms
- Must provide an assessment from the physician for the need of behavioral modifications (such as cognitive behavioral therapy or psychotherapy). If behavioral modifications recommended, must have documentation showing that the modifications did not result in a decreased occurrence of symptoms.
- Must have an adequate trial and failure of loperamide or antispasmodics (such as dicyclomine or hyoscyamine) with inadequate response or significant side effect/toxicity or have a contraindication to these therapies

If the above criteria are met, alosetron (Lotronex) is approved for 3 months.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved while on therapy.

- **Alpelisib (Vijoice®)** [21, 22]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with a medical geneticist, an oncologist, a hematologist, or a vascular anomaly specialist
- Must be age 2 years and older
- Must have a diagnosis of *PIK3CA*-Related Overgrowth Spectrum (PROS) with severe manifestations requiring systemic therapy with chart documentation of ALL of the following:
  - Overgrowth is sporadic (no family history) and mosaic (patchy, irregular distribution)
  - Documentation of a *PIK3CA* mutation in affected tissue, peripheral blood, or saliva. A copy of the laboratory report confirming the mutation is required.
  - Congenital or early childhood onset of symptoms
- Must have chart documentation of at least ONE of the following:

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- Overgrowth in adipose, muscle, nerve, or skeletal tissue
- Vascular malformations (e.g. capillary, venous, arteriovenous malformation, lymphatic)
- Isolated macrodactyly (enlarged fingers or toes), overgrown splayed feet/hands, overgrown limbs
- Megalencephaly, hemimegalencephaly, dysplastic megalencephaly, focal cortical dysplasia

If the above criteria are met, alpelisib (Vijoice) is approved for 6 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon chart documentation from the provider of improvement in the member's baseline manifestations of PROS based upon the prescriber's assessment while on therapy.

• Ambrisentan (Letairis®) [23, 24, 25, 26]

Initial Authorization Criteria:

- Must be prescribed by a cardiologist or pulmonologist who specializes in pulmonary hypertension
- Must have a diagnosis of PAH (WHO Group 1) with WHO functional class II or III symptoms confirmed by right heart catheterization; must provide a copy of the RHC showing ALL of the following:
  - Mean pulmonary arterial pressure (mPAP) ≥25 mmHg
  - Pulmonary capillary wedge pressure (PCWP), left atrial pressure, or left ventricular end-diastolic pressure (LVEDP) ≤15 mmHg
  - Pulmonary vascular resistance >3 Wood units

If the above criteria are met, ambrisentan (Letairis) is approved for 3 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or the member continues to benefit from therapy.

• Amifampridine (Firdapse®) [27, , 28, 29, 30, 31, 32, 33]

Initial Authorization Criteria:

- Must be prescribed by a neurologist
- Must be age 6 years or older
- Must have a diagnosis of Lambert-Eaton Myasthenic Syndrome confirmed by ONE of the following:

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        - Electrophysiological testing (e.g., EMG, CMAP, RNS) [copy of test result required]
        - Positive anti-P/Q type voltage-gated calcium channel (VGCC) antibody testing (copy of test result required)
    - Must have chart documentation of proximal muscle weakness indicated by muscle strength testing that impacts physical function or quality of life
    - Must not have a history of seizures

If the above criteria are met, amifampridine (Firdapse) is approved for 3 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for the continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has stabilized or improved based upon the prescriber's assessment while on therapy.

- **Amikacin liposomal inhalation (Arikayce®)** [34, 35, 36, 37, 38, 39, 40]

Initial Authorization Criteria:
- Must be prescribed by a pulmonologist or infectious diseases specialist
- Must be 18 years of age or older
- Must have a diagnosis of MAC pulmonary disease (either fibrocavitary or nodular bronchiectatic disease)
- Must have failed to convert to respiratory culture negative after 6 months of multidrug MAC-directed antibiotic therapy (typically includes the following 3 antibiotics: clarithromycin or azithromycin, rifabutin or rifampin, and ethambutol; may also include intravenous aminoglycoside; may substitute isoniazid or fluoroquinolones for macrolide resistance)
- Must be using in combination with multidrug MAC-directed antibiotic therapy
- Must not have infection with amikacin-resistant MAC

If the above criteria are met, amikacin liposomal inhalation (Arikayce) is approved for a duration of 6 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed after the initial 6 month authorization period to determine the Medical Necessity for continuation of therapy. Authorizations may be extended based upon chart documentation showing ONE of the following criteria is met:

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- If the member has converted to respiratory culture negative for MAC, authorization is extended for 12 additional months of therapy after the first negative respiratory culture
- If the member has not converted to respiratory culture negative for MAC, authorization is extended for an additional 6 months of therapy, up to a maximum of 2 reauthorizations (18 total months of therapy)

• Apomorphine (Apokyn®) [41, 42, 43, 44]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with a neurologist
- Must have a diagnosis of Parkinson's disease with motor fluctuations and "off" episodes
- Must have continued "off" symptoms despite adherence to optimized medication therapy regimen based on the prescriber's assessment
- Must be on concomitant therapy with carbidopa/levodopa
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to TWO of the following, taken in combination with carbidopa/levodopa for the adjunctive treatment of "off" episodes:
  - Entacapone
  - A generic MAO-B inhibitor (e.g. selegiline or rasagiline)
  - A generic dopamine agonist (e.g. ropinirole, pramipexole)
  - Istradefylline (Nourianz)
- Must have an adequate trial of inhaled levodopa (Inbrija) with an inadequate response or significant side effects/toxicity or have a contraindication to this therapy
- Must not be on concomitant therapy with 5HT3 antagonists (e.g. ondansetron, granisetron, palonosetron, alosetron)

If the above criteria are met, apomorphine (Apokyn) is approved for 1 year.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved while on therapy or the member continues to benefit from therapy.

• Apomorphine subcutaneous infusion (Onapgo™) [45, 46, 47, 48]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with a neurologist
- Must have a diagnosis of advanced Parkinson's disease

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- Must have inadequately controlled motor fluctuations with average daily "off" time OR "on" time with troublesome dyskinesia of ≥ 2.5 hours per day despite adherence to optimized medication therapy regimen based on the prescriber's assessment. Chart documentation must be provided.
- Must have an attestation from the provider that the member's and/or primary caregiver's ability to use the subcutaneous infusion device has been assessed
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to TWO of the following, taken in combination with carbidopa/levodopa for the adjunctive treatment of "off" episodes:
  - Entacapone or opicapone (Ongentys)
  - A generic MAO-B inhibitor (e.g., selegiline, rasagiline)
  - A generic dopamine agonist (e.g., ropinirole, pramipexole)
  - Istradefylline (Nourianz)
- Must not use in combination with ANY of the following:
  - Other apomorphine containing products
  - 5HT3 antagonists, including antiemetics (e.g., ondansetron, granisetron, dolasetron, palonosetron) and alosetron
- Must be using a dose and duration of therapy consistent with the FDA-approved package labeling

If the above criteria are met, apomorphine subcutaneous infusion (Onapgo) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon the following:
- Documentation from the provider that the member's condition has improved or stabilized while on therapy.
- Must be using a dose and duration of therapy consistent with the FDA-approved package labeling

• Aprocitentan (Tryvio™) [49, 50, 51, 52, 53]

Initial Authorization Criteria:
- Must be prescribed by a cardiologist, nephrologist, or endocrinologist
- Must be age 18 years or older
- Must have a diagnosis of resistant hypertension confirmed by documentation of a current blood pressure that is above goal despite combination therapy with at least three antihypertensive drug classes
- Must provide documentation of the current and goal blood pressure level. The date of the blood pressure reading must be provided

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- Must have an adequate trial (at least 4 weeks) of combination therapy at optimized doses using ALL of the following therapeutic classes concurrently with an inadequate response despite adherence (PDC >80%), significant side effect/toxicity or have a contraindication to these therapies:
  - Calcium channel blockers
  - Angiotensin-converting enzyme (ACE) inhibitors or angiotensin receptor blockers (ARB)
  - Diuretics
  - Mineralocorticoid receptor antagonists (i.e., spironolactone)
  - Beta-blockers
  - Vasodilators (i.e., hydralazine and minoxidil)
  - Central alpha agonists (i.e., clonidine)
- Must provide chart documentation of the current antihypertensive regimen that is to be used concurrently with aprocitentan (Tryvio)
- Must not be pregnant

If the above criteria are met, aprocitentan (Tryvio) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon chart documentation showing the following:
- The member's condition has improved while on therapy based on a reduction in blood pressure from baseline. The date of the recent blood pressure reading must be provided
- Aprocitentan (Tryvio) continues to be used concurrently as part of an optimized regimen

• Arimoclomol (Miplyffa®) [54, 55, 56, 57]

Initial Authorization Criteria:
- Must be prescribed by provider who specializes in the treatment of metabolic disorders or a neurologist
- Must be age 2 years or older
- Must have a diagnosis of Niemann-Pick disease type C (NPC) confirmed by ONE of the following:
  - A mutation in both alleles of NPC1 or NPC2 gene (a copy of the genetic test result must be provided)
  - A mutation in one allele (NPC1 or NPC2) and a positive filipin-staining or elevated cholestane triol/oxysterols >2x the upper limit of normal (a copy of the genetic test result and laboratory results must be provided)

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- Must have neurological symptoms of NPC (e.g., cognitive decline, cerebellar ataxia, gaze palsy, dysarthria, dysphagia, dystonia, seizures, psychiatric disorders). Chart documentation of symptoms must be provided.
- Must be ambulatory (e.g., walk independently or walk with assistance)
- Must use in combination with miglustat (Opfolda)
- Must not use in combination with levacetylleucine (Aqneursa)

If the above criteria are met, arimoclomol (Miplyffa) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation showing the following:

- Must have improvement or stabilization in function or neurologic symptoms affected by NPC (e.g., ambulation, fine motor skills, swallowing, or speech)
- Must use in combination with miglustat (Opfolda)
- Must not use in combination with levacetylleucine (Aqneursa)

• Aripiprazole ODT (Abilify Discmelt®) and Aripiprazole Oral Solution (Abilify®) [58, 59]

Initial Authorization Criteria:

- Commercial and Exchange members < 12 years of age, please refer to 'Atypical and Typical Antipsychotics – Children Under 12 years of Age' criteria below
- Must have one of the following diagnoses:
  - Bipolar Disorder
  - Schizophrenia
  - Major Depression with Psychosis
  - Irritability associated with Autistic disorder
  - Tourette's disorder
  - Major Depressive Disorder and meet the following criteria:
    - Must have an adequate trial, of at least 4 weeks each, of monotherapy with TWO different antidepressants with an inadequate response or significant side effect/toxicity or have a contraindication to these therapies
    - Must also be taking a Selective Serotonin Reuptake Inhibitor (SSRI) or Selective Serotonin Norepinephrine Reuptake Inhibitor (SNRI) antidepressant as adjunctive treatment
- Must have documentation of the inability to swallow an oral solid dosage form due to age (< 7 years), medical condition (e.g. nasogastric tube), or other clinical rationale for why aripiprazole (Abilify) tablet cannot be used

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If the above criteria are met, the requested product is approved.

- **Asenapine (Saphris®)**[60]

Criteria:

- For members < 12 years of age, please refer to the 'Atypical and Typical Antipsychotics – Children Under 12 years of Age' criteria below
- Must be prescribed by or in consultation with a psychiatrist
- Must have one of the following diagnoses:
  - Schizophrenia
  - Manic or mixed episodes associated with bipolar I disorder
- Must have an adequate trial, of at least 4 weeks, with an inadequate response or significant side effect/toxicity or have a contraindication to TWO generic atypical antipsychotics

If the above criteria are met, asenapine (Saphris) is approved.

- **Asenapine transdermal system (Secuado®)** [61]

Criteria:

- For members < 12 years of age, please refer to the 'Atypical and Typical Antipsychotics – Children Under 12 years of Age' criteria below
- Must be prescribed by or in consultation with a psychiatrist
- Must have a diagnosis of schizophrenia
- Must have an adequate trial, of at least 4 weeks, with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
  - Asenapine sublingual tablet (Saphris)
    - Must provide clinical rationale for use of asenapine transdermal system (Secuado) instead of asenapine sublingual tablet (Saphris). Clinical rationale must include why asenapine sublingual tablet (Saphris) would not be expected to produce the same clinical results as asenapine transdermal system (Secuado) given that they are the same chemical entity
  - Two additional generic atypical antipsychotics

If the above criteria are met, asenapine (Secuado) is approved.

- **Asfotase alpha (Strensiq™)** [62, 63, 64, 65, 66, 67, 68, 69]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with an endocrinologist or a provider who specializes in the treatment of inherited metabolic disorders

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  - Must have a diagnosis of perinatal/infantile-onset or juvenile-onset hypophosphatasia (HPP) with symptom onset prior to age 18 confirmed by ONE of the following:
    - Genetic test results confirming a mutation in the *ALPL* gene (copy of test result must be provided)
    - Diagnostic laboratory findings confirmed by BOTH of the following:
- Serum alkaline phosphatase (ALP) level below the age-adjusted normal range based on the lab reference range provided by the laboratory. Chart documentation of the lab result is required.
- Elevated plasma pyridoxyl phosphate (also known as PLP, pyridoxine, or vitamin B6) or urine phosphoethanolamine (PEA) based on the upper limit of the reference range provided by the laboratory. Chart documentation of the lab result is required.
  - Must be prescribed a dose and duration of therapy that is consistent with FDA-approved package labeling, nationally-recognized compendia, or peer-reviewed medical literature
  - Must have symptomatic disease confirmed by chart documentation of ONE of the following:
    - Skeletal abnormality (e.g. rickets, premature tooth loss, poorly healing or recurrent fractures, gait disturbance)
    - Respiratory failure or distress
    - Rachitic chest deformity
    - Vitamin B6-dependent seizures

If the above criteria are met, asfotase alpha (Strensiq) is approved for 6 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon chart documentation showing the following:
  - The member's condition has improved or stabilized based on diagnostic laboratory findings or symptoms from baselnie
  - Must be provided a dose and duration of therapy that is consistent with FDA-approved package labeling, nationally-recognized compendia, or peer-reviewed medical literature

- **Atogepant (Qulipta®):** [70, 71, 72, 73, 74]

Initial Authorization Criteria:
  - Must be 18 years or older
  - Must have a diagnosis of migraine
    - Must have at least 4 migraine days per month
  - Must have an adequate trial of at least 2 months each within 2 prophylactic therapy classes including: beta-blockers, anticonvulsants, and antidepressants

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(TCAs, SSRIs, SNRIs) with an inadequate response or significant side effects/intolerance
    - For members in whom one of these therapy classes is not clinically appropriate, additional prophylactic therapy classes to be used are calcium channel blockers or angiotensin converting enzyme inhibitors (ACEIs)
- Must not be using concomitantly with rimegepant (Nurtec ODT), onabotulinumtoxinA (Botox) or an injectable CGRP antagonist [e.g., erenumab (Aimovig), galcanezumab (Emgality), fremanezumab (Ajovy), eptinezumab (Vyepti)] for prevention of migraine

If the above criteria are met, atogepant (Qulipta) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for the continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the prescriber that the member's condition has improved based upon the prescriber's assessment while on therapy.

• Atrasentan (Vanrafia®) [75, 76, 77, 78, 79]

Initial Authorization Criteria:
- Must be prescribed by a nephrologist
- Must be age 18 years or older
- Must have a diagnosis of primary immunoglobulin A nephropathy (IgAN) confirmed by a kidney biopsy. Documentation of the biopsy results is required.
- Must be at risk of rapid disease progression defined by the following (chart documentation is required):
    - Proteinuria ≥ 0.75g/day OR Urinary Protein-to-Creatinine Ratio (UPCR) ≥1.5g/g despite at least 90 days of optimized supportive care treatment with ALL the following:
        - Maximally tolerated dose of an angiotensin converting enzyme inhibitor (ACEi) or an angiotensin receptor blocker (ARB) unless there is a contraindication to these therapies
            - NOTE: for members who have experienced significant side effects/toxicities to one of these classes, an adequate trial of the other class is required (e.g., ACEi-induced cough requires a trial of an ARB)
        - Other supportive care measures as clinically indicated (e.g., lifestyle modifications, blood pressure management, assessment of cardiovascular risk)

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- Must be on a stable dose of maximally-tolerated renin-angiotensin system (RAS) inhibitor therapy (e.g., angiotensin converting enzyme inhibitor or an angiotensin receptor blocker)
- Must have an adequate trial of at least 3 months with an inadequate response (proteinuria > 1 g/day OR UPCR ≥ 1.5 g/g), significant side effect/toxicity or have a contraindication to BOTH of the following:
  - A sodium-glucose co-transporter 2 (SGLT-2) inhibitor [e.g., empagliflozin (Jardiance)]
  - Sparsentan (Filspari)
- Must have an attestation from the prescriber that the member is to remain on optimized supportive care as clinically indicated
- Must not use in combination with the following:
  - Sparsentan (Filspari)
  - Iptacopan (Fabhalta)
  - Sibeprenlimab-szsi (Voyxact)
  - Budesonide DR (Tarpeyo)

If the above criteria are met, atrasentan (Vanrafia) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon chart documentation showing the following:

- Must have a reduction in proteinuria OR urinary protein-to-creatinine ratio from baseline
- Must be on optimized supportive care as clinically indicated
- Must be on a stable dose of maximally-tolerated renin-angiotensin system (RAS) inhibitor therapy (e.g., angiotensin converting enzyme inhibitor or an angiotensin receptor blocker)
- Must not use in combination with the following:
  - Sparsentan (Filspari)
  - Iptacopan (Fabhalta)
  - Sibeprenlimab-szsi (Voyxact)
  - Budesonide DR (Tarpeyo)

• Atypical and Typical Antipsychotics – Children Under 12 Years of Age [80, 81, 82, 83, 84, 85, 86, 87, 88, 89]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with a child and adolescent psychiatrist, pediatric neurologist, or child development pediatrician

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- Must have severe behavioral problems (defined as behaviors that create imminent threat to the safety of the child or others) related to one of the following diagnoses:
  - Bipolar I Disorder
    - Must be age 6 years or older
  - Autism
    - Must be age 5 years or older
  - Schizophrenia
  - Intellectual disability
  - Conduct disorder
  - Tic disorder/Tourette's syndrome
  - Transient encephalopathy
- Must provide chart documented evidence of a comprehensive evaluation and non-pharmacologic therapies such as, but not limited to, evidence-based behavioral, cognitive, and family-based therapies.
- Must provide documentation of baseline monitoring of weight, BMI, glucose, lipids, extrapyramidal symptoms (EPS) using the Abnormal Involuntary Movement Scale (AIMS).
- For asenapine (Saphris, Secuado), brexipexazole (Rexulti), cariprazine (Vraylar), iloperidone (Fanapt), lumateperone (Caplyta), or paliperidone (Invega), must have an adequate trial and failure or an inadequate response or intolerance to TWO generic atypical antipsychotics.
- For aripiprazole ODT (Abilify Discmelt), aripiprazole oral solution (Abilify), and aripiprazole oral film (Opipza):
  - Must have documentation of the inability to swallow an oral solid dosage form due to age (< 7 years), medical condition (e.g., nasogastric tube), or other clinical rationale for why aripiprazole (Abilify) tablet cannot be used
  - For aripiprazole oral film (Opipza), must provide clinical rationale for use of aripiprazole oral film (Opipza) instead of aripiprazole ODT (Abilify Discmelt) or aripiprazole oral solution (Abilify). Clinical rationale must include why aripiprazole oral film (Opipza) would not be expected to produce the same clinical results as aripiprazole ODT (Abilify Discmelt) or aripiprazole oral solution (Abilify) given that they are the same chemical entity
- For olanzapine and samidorphan (Lybalvi):
  - Must have an adequate trial and failure of olanzapine with unacceptable weight gain
    - Must not have failed olanzapine due to lack of efficacy or adverse effects other than weight gain
    - Must provide documentation demonstrating unacceptable weight gain while on olanzapine
  - Must not currently be using opioids or undergoing acute opioid withdrawal

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        - Must delay initiation of olanzapine and samidorphan for at least 7 days after last use of short-acting opioids and at least 14 days after last use of long-acting opioids
    - For multiple antipsychotic prescribing:
        - Must provide documentation of discontinuation/taper plan from one drug to another drug in the same drug class; OR
        - Must meet all of the following criteria:
            - Must provide clinical rationale for multiple drug therapy for treatment of unique symptoms or treatments of medication side effects
            - Must have an adequate trial of a single drug within the same class before treatment with multiple agents in the same class will be considered
            - Must provide literature or guidelines which support multiple antipsychotic prescribing

If the above criteria are met, the requested typical or atypical antipsychotic is approved for 3 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one-year intervals when the following criteria are met:

- Must have chart documentation from the prescriber that indicates the member's condition has improved based upon the prescriber's assessment while on therapy.
    - Must provide an evaluation indicating improvement in target symptoms
    - Must provide documentation of ongoing behavioral health support
    - Must provide rationale for continued use or plan for taper/discontinuation
- Must provide documentation that the member is being followed for adverse effects through monitoring of weight, BMI, glucose, lipids, and extrapyramidal symptoms (EPS) using the Abnormal Involuntary Movement Scale (AIMS) on an annual basis.

• Avacopan (Tavneos®) [90, 91, 92, 93]

Initial Authorization Criteria:

- Must be prescribed by a rheumatologist
- Must be used in combination with standard therapy (including glucocorticoids) for a diagnosis of severe, active anti-neutrophil cytoplasmic autoantibody (ANCA)-associated vasculitis (granulomatosis with polyangiitis [GPA] and microscopic polyangiitis [MPA])

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        - Must be on concomitant glucocorticoids and standard therapy with rituximab or cyclophosphamide
        - Must provide baseline glucocorticoid dose
    - Must confirm diagnosis with documentation of ALL of the following:
        - Positive myeloperoxidase (MPO) or proteinase 3 (PR3) ANCA serology
        - New, persistent, or worsening clinical signs and symptoms attributed to GPA or MPA and not related to prior damage
        - Life threatening or organ threatening manifestations (e.g., alveolar hemorrhage, glomerulonephritis)
    - Must not currently require dialysis
    - Must not have severe hepatic impairment (Child Pugh C) or cirrhosis

If the above criteria are met, avacopan (Tavneos) is approved for 6 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one year intervals based upon documentation from the provider on BOTH of the following:
- Stabilization or improvement in the member's organ function
- Decrease in glucocorticoid dose from baseline

• Avatrombopag (Doptelet®) [94, 95, 96, 97, 98, 99]

Initial Authorization Criteria:

1. **For chronic liver disease (CLD) [Applies to avatrombopag tablets only]**
    - Must be prescribed by a hepatologist, gastroenterologist, hematologist, oncologist, or surgeon
    - Must be age 18 years or older
    - Must have a diagnosis of thrombocytopenia in chronic liver disease (CLD)
    - Must have documentation of a scheduled procedure
    - Must have a baseline platelet count < 50 x 10^9/L

If the above criteria are met, avatrombopag (Doptelet) tablet is approved for 1 month (one 5-day course of treatment).

2. **For immune thrombocytopenia (ITP)**
    - Must be prescribed by a hematologist or oncologist
    - Must be age 1 year or older
    - Must have a diagnosis of immune thrombocytopenia (ITP)
    - Must have an adequate trial with an inadequate response or significant side effect/toxicity to ONE of the following:
        - Corticosteroids
        - Intravenous immunoglobulin (IVIG)

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- Must have an adequate trial of generic eltrombopag (Promacta) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
- Must have a platelet count <30 x 10^9/L prior to initiation
- For avatrombopag (Doptelet) oral granules:
  - Must have documentation of the inability to swallow an oral solid dosage form due to age (< 7 years), medical condition (e.g., nasogastric tube), or other clinical rationale for why avatrombopag (Doptelet) tablets cannot be used

If the above criteria are met, avatrombopag (Doptelet) is approved for 6 months.

Reauthorization Criteria:

1. **For chronic liver disease (CLD)**
   - Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response and clinical rationale for re-treatment.

2. **For immune thrombocytopenia (ITP)**
   - All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation showing improvement in platelet count from baseline.

• Becaplermin (Regranex®) [100, 101, 102, 103, 104]

Initial Authorization Criteria:

Becaplermin (Regranex) will be covered for an initial period of 1 fill. After the initial fill, the following criteria must be met:

- Must be prescribed by or in consultation with a wound care specialist (e.g. podiatrist or surgeon)
- Must be used for lower extremity diabetic neuropathic ulcers that extend into the subcutaneous tissue or beyond and have an adequate blood supply
- Must not be used for the treatment of pressure ulcers, venous stasis ulcers, diabetic neuropathic ulcers that do not extend into subcutaneous tissue, or ischemic diabetic ulcers
- Must be used as an adjunct to, and not a substitute for, good ulcer care practices, including initial sharp debridement, pressure relief, and infection control
- Must not have a known neoplasm at the site(s) of application
- Must have improved healing of the ulcer after the initial period of 1 fill
- Must have documentation that there is a reasonable medical expectation that continued treatment will result in healing and resolution of the wound

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If the above criteria are met, becaplermin (Regranex) is approved for 6 weeks.

Reauthorization Criteria:
All prior authorization renewals are reviewed to determine the Medical Necessity for continuation of therapy. Authorizations will be approved up to a maximum of 20 weeks of therapy when the following criteria are met:

- Must have improved healing of the ulcer
- Must be used as an adjunct to, and not a substitute for, good ulcer care practices, including initial sharp debridement, pressure relief, and infection control
- Must have documentation that there is a reasonable medical expectation that continued treatment will result in healing and resolution of the wound

- **Bedaquiline (Sirturo®)** [105, 106, 107, 108, 109, 110]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with an infectious disease specialist or pulmonologist.
- Must be age 2 years (weighing at least 8kg) or older
- Must have a diagnosis of multi-drug resistant pulmonary tuberculosis (TB)
- Must have previously had an inadequate response to at least one first line TB regimen containing isoniazid and rifampin OR have documentation of susceptibility testing of Mycobacterium tuberculosis isolates demonstrating resistance to isoniazid and rifampin
- Must be used as part of a combination regimen
- Must indicate that the member understands the importance of compliance to the full medication regimen

If the above criteria are met, bedaquiline (Sirturo) is approved for 6 months.

Reauthorization Criteria:
All prior authorizations renewals are reviewed on a case-by-case basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at six-month intervals based upon the following:

- Must have documentation of the member's initial response to therapy and clinical rationale for continuation of treatment or re-treatment
- Must have documentation that the member was compliant with previous course of therapy
- Must have documentation of susceptibility testing of Mycobacterium tuberculosis isolates demonstrating continued susceptibility to bedaquiline (Sirturo)

- **Belimumab Subcutaneous (Benlysta® SC)** [111, 112, 113, 114, 115, 116, 117, 118, 119, 120, 121, 122]

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- Requests for belimumab intravenous (Benlysta® IV) please refer to policy RX.PA.406 Commercial, Exchange Medical Prior Authorization Policies

Initial Authorization Criteria:

1. **For Systemic Lupus Erythematosus (SLE)**
   - Must be prescribed by a rheumatologist
   - Must be age 5 years or older
   - Must have a diagnosis of systemic lupus erythematosus (SLE) confirmed by documentation of ONE of the following:
     - Positive antinuclear antibodies (ANAs) titre ≥ 1:80
     - Anti-double stranded DNA (dsDNA) ≥ 30 IU/mL
   - Must have an adequate trial of at least 3 months with an inadequate response or significant side effect/toxicity or have a contraindication to ONE of the following:
     - Hydroxychloroquine
     - Azathioprine
     - Methotrexate
     - Mycophenolate
   - Must use in combination with ONE of the following:
     - Corticosteroid
     - Antimalarial
     - Non-steroidal anti-inflammatory drug (NSAID)
     - Immunosuppressant
   - Must not have severe active central nervous system lupus
   - Must not use in combination with biologic therapies, obinutuzumab (Gazyva), or voclosporin (Lupkynis)

2. **For Lupus Nephritis (LN)**
   - Must be prescribed by a rheumatologist or a nephrologist
   - Must be age 5 years or older
   - Must have a diagnosis of class III or IV, with or without concomitant class V active lupus nephritis confirmed by kidney biopsy. Documentation of biopsy result with classification must be provided.
   - Must be autoantibody-positive for SLE confirmed by documentation of ONE of the following:
     - Positive antinuclear antibodies (ANAs) titre ≥ 1:80
     - Anti-double stranded DNA (dsDNA) ≥ 30 IU/mL
   - Must use in combination with an immunosuppressive regimen that includes BOTH of the following:
     - Systemic corticosteroids (e.g. prednisone, methylprednisolone)
     - Mycophenolate mofetil [MMF] or cyclophosphamide
   - Must have an adequate trial of obinutuzumab (Gazyva) with an inadequate response, significant side effect/toxicity, or have a contraindication to this therapy
   - Must not have severe active central nervous system lupus

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- Must not use in combination with biologic therapies, obinutuzumab (Gazyva), or voclosporin (Lupkynis)

If the above criteria are met, belimumab (Benlysta) is approved for 6 months.

Reauthorization Criteria:
All prior authorizations renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based on documentation showing:

1. **For SLE:**
   - Must use in combination with a standard SLE regimen (e.g., corticosteroids, antimalarials, non-steroidal anti-inflammatory drugs [NSAIDs], or immunosuppressives)
   - The member's condition has improved or stabilized while on therapy.
2. **For LN:**
   - Must use in combination with an immunosuppressant maintenance therapy (e.g., MMF and corticosteroids)
   - Must show ONE of the following:
     - Stabilization or improvement in renal function from baseline (e.g., decreased proteinuria, improvement in urine creatinine ratio, improved eGFR rate). \
     - Reduction in corticosteroid dose if member has been on a corticosteroid
   - Must not use in combination with biologic therapies, belimumab (Benlysta) or voclosporin (Lupkynis)

• Belumosudil (Rezurock™) [123, 124]

Initial Authorization Criteria:

- Must be prescribed by a hematologist or oncologist
- Must be 12 years of age and older
- Must be status-post an allogeneic hematopoietic stem cell transplantation
- Must have a diagnosis of chronic graft-versus-host disease (chronic GVHD) after failure of at least two prior lines of systemic therapy [e.g., corticosteroids, cyclosporine, tacrolimus, sirolimus, mycophenolate mofetil, methotrexate, imatinib, rituximab, ibrutinib (Imbruvica), Ruxolitinib (Jakafi), extracorporeal photopheresis (ECP)]
- Requests for doses that exceed 200mg daily:
  - Must have documentation of a drug interaction with either a proton pump inhibitor or strong CYP3A inducer (e.g., rifampin, phenytoin, carbamazepine)
  - An attestation from the provider that continued treatment with the interacting medication in combination with belumosudil (Rezurock) is medically necessary

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        - For members taking a proton pump inhibitor, must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to an H2 receptor antagonist

If the above criteria are met, belumosudil (Rezurock) is approved for 12 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon

- Documentation from the provider that the member's condition has improved based upon the prescriber's assessment while on therapy or the member continues to benefit from therapy
- Requests for doses that exceed 200mg daily:
    - Must have documentation of a drug interaction with either a proton pump inhibitor or strong CYP3A inducer (e.g., rifampin, phenytoin, carbamazepine)
    - An attestation from the provider that continued treatment with the interacting medication in combination with belumosudil (Rezurock) is medically necessary
    - For members taking a proton pump inhibitor, must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to an H2 receptor antagonist

- **Benralizumab Pen (Fasenra™)** [125, 126, 127]
    - Requests for benralizumab syringe (Fasenra®) please refer to policy RX.PA.406 Commercial, Exchange Medical Prior Authorization Policies

Initial Authorization Criteria

1. **For severe asthma**
    - Must be prescribed by or in consultation with an allergist, an immunologist, or a pulmonologist
    - Must be age 6 years or older
    - Must have a diagnosis of severe eosinophilic phenotype asthma with a blood eosinophil count of ≥150 cells/µL within the past year. The date the test was performed must be provided.
    - Must have received at least 3 consecutive months of therapy with ONE of the following combinations for the maintenance treatment of asthma:
        - A medium-high dose inhaled corticosteroid AND one additional asthma controller medication (e.g., inhaled long-acting beta-agonist, inhaled long-acting muscarinic antagonist)
        - A maximally tolerated inhaled corticosteroid AND long-acting beta-agonist combination product

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- Must have history of one asthma exacerbation in the previous year despite adherence to optimized medication therapy regimen. Asthma exacerbation is defined by ONE of the following:
  - Hospitalization for asthma
  - Emergency room visit for asthma
  - Requirement for 2 or more courses of treatment with systemic (e.g., oral, parenteral) corticosteroids
- Must be used as an add-on maintenance treatment to asthma controller therapy
- Must not be receiving combination therapy with another biologic agent (e.g., Xolair, Cinqair, Dupixent, Nucala, Tezspire) for this diagnosis

2. For eosinophilic granulomatosis with polyangiitis (EGPA)

- Must be prescribed by or in consultation with an allergist, immunologist, pulmonologist, or rheumatologist
- Must be age 18 years or older
- Must have a diagnosis of relapsed, refractory eosinophilic granulomatosis with polyangiitis. Chart documentation of diagnosis must be provided.
  - Must have a history or presence of asthma
  - Must have a blood eosinophil level of 10% or an absolute eosinophil count of >1000 cells/µL
  - Must have two or more of the following criteria that are typical of eosinophilic granulomatosis with polyangiitis:
    - Histopathological evidence of eosinophilic vasculitis, perivascular eosinophilic infiltration, or eosinophil-rich granulomatous inflammation
    - Neuropathy
    - Pulmonary infiltrates
    - Sino-nasal abnormality
    - Cardiomyopathy
    - Glomerulonephritis
    - Alveolar hemorrhage
    - Palpable purpura
    - Antineutrophil cytoplasmic antibody [ANCA] positivity
- Must have an adequate trial ≥3 months of systemic corticosteroids (prednisone or prednisolone at a dose of ≥7.5 mg/day) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy

If the above criteria are met, benralizumab pen (Fasenra) is approved for 1 year.

Reauthorization Criteria:

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All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon the following:

1. **For asthma**
   - Must have documentation from the provider that the member's condition has improved while on therapy (e.g., reduction in exacerbations, hospitalizations, emergency department visits, or requirement for oral corticosteroid therapy)
2. **For EGPA**
   - Must have documentation from the provider that the member's condition has improved while on therapy (e.g. reduction in requirement for oral corticosteroid therapy)

• Berdazimer gel (Zelsuvmi™) [128, 129, 130]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with a dermatologist
- Must be age 1 year or older
- Must have a diagnosis of molluscum contagiosum with at least ONE of the following:
  - Itching, pain, concomitant bacterial infection, concomitant atopic dermatitis or other chronic inflammatory skin condition, or lesions that cannot be reasonably covered using a bandage
  - Presence of lesions for at least 6 months
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to BOTH the following:
  - Cryotherapy or curettage
  - Cantharidin (Ycanth)

If the above criteria are met, berdazimer gel (Zelsuvmi) is approved for 3 months for 1 course of therapy. Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response and clinical rationale for re-treatment.

• Betaine Anhydrous (Cystadane®) [131, 132, 133, 134, 135, 136]

Initial Authorization Criteria:

- Must be prescribed by a provider who specializes in the treatment of inherited metabolic disorders
- Must have a diagnosis of homocystinuria with elevated homocysteine blood (plasma) concentrations. Chart documentation of laboratory results with reference range is required.
- Must have chart documentation of ONE of the following metabolic disorders confirmed by enzymatic, biochemical or genetic analysis:
  - Cystathionine beta-synthase (CBS) deficiency

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- 5, 10-methylenetetrahydrofolate reductase (MTHFR) deficiency
- Cobalamin cofactor metabolism (cbl) defect

If the above criteria are met, betaine anhydrous (Cystadane) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of treatment. Authorization may be extended at one-year intervals based upon chart documentation showing the following:

- Must have a reduction in homocysteine blood (plasma) concentrations from baseline while on therapy

• Birch triterpene topical gel (Filsuvez®) [137, 138, 139, 140]

Initial Authorization Criteria:

- Must be prescribed by dermatologist or medical geneticist
- Must be age 6 months or older
- Must have a diagnosis of recessive dystrophic epidermolysis bullosa (RDEB) confirmed with a biallelic mutation in the collagen type VII alpha 1 chain (COL7A1) gene (a copy of the genetic test must be provided)
- Must have chart documentation of at least one open partial thickness wound lesion of 10cm^2 to 50 cm^2 in surface area associated with RDEB for ≥ 21 days (documentation of size, time the wound has been open, and number of total wounds must be provided)
- Must have an adequate trial of standard wound therapy with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
- Must not be on concurrent gene therapy for epidermolysis bullosa (e.g., beremagene geperpavec-svdt [Vyjuvek®], prademagene zamikeracel [Zevaskyn®])
- Must not be concurrently applied with other skin products (e.g., creams, ointments, gels, or emollients)
- Must not have history or current malignancy including basal cell carcinomas and squamous cell carcinomas
- Must be using a dose and duration of therapy consistent with the FDA-approved package labeling

If the above criteria are met, birch triterpene topical gel (Filsuvez) is approved for 3 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at 6 month intervals based upon chart documentation showing the following:

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- Must have continued presence of at least one open partial thickness wound (documentation of size and total number of wounds must be provided)
- Must be using a dose and duration of therapy consistent with the FDA-approved package labeling
- Must have a decrease from baseline in ONE of the following:
  - Size of partial thickness wound(s)
  - Number of partial thickness wounds
  - Pain or itch
- Must not have current malignancy including basal cell carcinomas and squamous cell carcinomas
- Must not be concurrently applied with other skin products (e.g., creams, ointments, gels, or emollients)
- Must not be on concurrent gene therapy for epidermolysis bullosa (e.g., beremagene geperpavec-svdt [Vyjuvek], prademagene zamikeracel [Zevaskyn])

• Bosentan (Tracleer®) [141, 142, 143, 144]

Initial Authorization Criteria:

- Must be prescribed by a cardiologist or pulmonologist who specializes in pulmonary hypertension
- Must have a diagnosis of PAH (WHO Group 1) with WHO functional class II-IV symptoms confirmed by right heart catheterization (RHC); must provide a copy of the RHC showing ALL of the following:
  - Mean pulmonary arterial pressure (mPAP) ≥25 mmHg
  - Pulmonary capillary wedge pressure (PCWP), left atrial pressure, or left ventricular end-diastolic pressure (LVEDP) ≤15 mmHg
  - Pulmonary vascular resistance >3 Wood units
- For pediatric members at least three 3 years of age:
  - An exception to the RHC for diagnosis is permitted for congenital heart defect with single ventricle physiology that requires repair or was repaired (e.g., s/p Glenn shunt, s/p Fontan procedure)

If the above criteria are met, bosentan (Tracleer) is approved for 3 months.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or the member continues to benefit from therapy.

• Botulinum Toxins A and B (OnabotulinumtoxinA [Botox®], AbobotulinumtoxinA [Dysport®],

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RimabotulinumtoxinB [Myobloc®], IncobotulinumtoxinA [Xeomin®], daxibotulinumtoxinA-lanm injection [Daxxify®])

145, 146, 147, 148, 149, 150, 151, 152, 153, 154, 155, 156, 157, 158, 159, 160, 161, 162, 163, 164, 165, 166, 167, 168, 169, 170, 171, 172, 173, 174, 175, 176, 177, 178, 179, 180, 181, 182, 183, 184, 185, 186, 187

Initial Authorization Criteria:

- Requests for OnabotulinumtoxinA (Botox), RimabotulinumtoxinB (Myobloc), daxibotulinumtoxinA-lanm (Daxxify):
  - Must meet the indication-specific criteria listed below
  - Must have adequate trials of BOTH of the following with an inadequate response or significant side effect/toxicity or have a contraindication to these therapies:
    - AbobotulinumtoxinA (Dysport)
    - IncobotulinumtoxinA (Xeomin)
    - Note: An exception to these drug trials may be made if the preferred product is not FDA-approved for the member's diagnosis

FDA Approved Indications

- Must have a diagnosis of:
  - Strabismus
  - Blepharospasm
  - Cervical dystonia (spasmodic torticollis)
  - Spasticity in members ≥ 2 years of age
  - Severe primary axillary hyperhidrosis
    - Must be prescribed by a dermatologist
    - Must have tried 10-20% topical aluminum chloride with an inadequate response or adverse effect of a severe rash
  - Chronic migraine prophylaxis
    - Must be prescribed by a neurologist
    - Must be age 18 years or older
    - Must have a diagnosis of migraine
      - Migraine must occur on at least 15 or more days per month lasting 4 hours a day or longer
    - Must have an adequate trial of at least 2 months each within 2 prophylactic therapy classes including: beta-blockers, anticonvulsants, and antidepressants (tricyclic antidepressants, selective serotonin reuptake inhibitors, serotonin and norepinephrine reuptake inhibitors) with an inadequate response or significant side effects/intolerance
      - For members in whom one of these therapy classes is not clinically appropriate, additional prophylactic therapy classes to be used are calcium channel

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blockers, or angiotensin converting enzyme inhibitors (ACEIs).
    - Must not be using in combination with a calcitonin-gene receptor peptide (CGRP) antagonist used for the prevention of migraine
- Urinary incontinence due to detrusor over activity associated with a neurologic condition in adults who meet the following criteria:
    - Must have a previous trial and failure of an anticholinergic medication
- Neurogenic detrusor overactivity (NDO) in pediatric patients 5 years of age and older who meet the following criteria:
    - Must have a previous trial and failure of an anticholinergic medication
- Overactive bladder (OAB) with symptoms of urge urinary incontinence, urgency, and frequency, in adults who meet the following criteria:
    - Must be prescribed by a urologist or a fellowship-trained urogynecologist
    - Must have > 3 urinary urgency incontinence episodes in a 3-day period
    - Must have > 8 micturitions per day
    - Must provide chart documentation showing specific examples of how quality of life is impacted by disease (e.g., sleep disturbances, work disruption, decrease in social interactions, etc.)
    - Must have a trial and failure of behavioral therapy (includes but not limited to weight loss, dietary changes, exercise, etc.)
    - Must have an adequate trial (of at least 4 weeks) at the recommended dose of 2 different drugs from the anticholinergic and/or the beta-3 adrenergic agonist [e.g., mirabegron (Myrbetriq)] drug class with an inadequate response or intolerance or contraindication
- Chronic sialorrhea
    - For adult members age 18 years or older:
        - Must have an adequate trial with an inadequate response or significant side effects/toxicity or have a contraindication to TWO of the following medications:
            - Atropine
            - Amitriptyline
            - Hyoscyamine
            - Benztropine
            - Transdermal scopolamine (Hyoscine)
            - Trihexyphenidyl hydrochloride (Benzhexol)

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        - Glycopyrrolate
    - Must not have history of aspiration pneumonia, amyotrophic lateral sclerosis, salivary gland or duct malformation, or gastroesophageal reflux disease
- For pediatric members age 2 to 17 years:
    - Must have adequate trial with an inadequate response or significant side effects/toxicity or have a contraindication to ONE of the following standard treatments:
        - Hyoscyamine
        - Benztropine
        - Transdermal scopolamine (Hyoscine)
        - Glycopyrrolate

If the above criteria are met, the requested botulinum toxin product is approved for 1 year.

Off Label Uses

1. **Autonomic Disorders**
    - Detrusor sphincter dyssynergia after spinal cord injury
    - Drooling in children with a neurologic disorder (e.g., cerebral palsy, brain injury, spinal cord injury, stroke)

2. **Movement Disorders**
    - Focal upper and lower extremity dystonia
    - Adductor laryngeal dystonia
    - Essential hand tremor in patients after trial and failure of at least one previous agent

3. **Achalasia**
    - Must have a diagnosis of achalasia
    - Must be prescribed by a gastroenterologist
    - Must be ineligible for surgery (e.g., high risk surgical candidate, age > 70 years of age, cardiac or pulmonary comorbidities, renal or hepatic insufficiency)

4. **Chronic Anal Fissure**
    - Must have a diagnosis of chronic anal fissure
    - Must be prescribed by a surgeon
    - Must have an adequate trial and failure of conservative treatment through local wound care and relief of constipation (e.g., sitz bath, high fiber diet and fiber supplements, stool softeners)
    - Must have an adequate trial and failure of a duration of at least 6-8 weeks of compounded topical diltiazem, compounded nifedipine, or nitroglycerin

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ointment with an inadequate response or significant side effect/toxicity or have a contraindication to therapy

All other off-label requests are reviewed on a case-by-case basis for literature-supported indication and previous medication trials.

If the above criteria are met, the requested botulinum toxin product is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of treatment. Authorization may be extended at one-year intervals based upon documentation from the prescriber that the member's condition has improved based upon the prescriber's assessment while on therapy.

• Brensocatib (Brinsupri™) [188, 189]

Initial Authorization Criteria:
- Must be prescribed by or in consultation with a pulmonologist
- Must be age 12 years or older
- Must have a diagnosis of non-cystic fibrosis bronchiectasis based on clinical symptoms (e.g., cough, sputum production, dyspnea, hemoptysis) and confirmed by chest computed tomography demonstrating bronchiectasis affecting one or more lobes. Chart documentation must be provided
- Must not have a diagnosis of cystic fibrosis
- For adults 18 years of age and older: Must have history of two pulmonary exacerbations in the previous year defined by hospitalization and/or a need for IV antibiotic treatment
- For pediatric members age 12 to 17 years:
  - Must have a history of one pulmonary exacerbation in the previous year defined by hospitalization and/or a need for IV antibiotic treatment
- For members with comorbid asthma and/or COPD:
  - Must have an attestation from the provider that all other respiratory comorbidities continue to be treated with standard of care

If the above criteria are met, brensocatib (Brinsupri) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved while on therapy as demonstrated by a reduction in number of bronchiectasis exacerbations per year from baseline.

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- If no documentation of reduction in exacerbations per year from baseline, must provide clinical rationale for continuation of treatment

- **Brexpiprazole (Rexulti®)** [190, 191, 192, 193, 194, 195, 196, 197]

Criteria:

- For members < 12 years of age, please reference 'Atypical and Typical Antipsychotics – Children Under 12 years of Age' criteria above
- Must be prescribed by or in consultation with a psychiatrist

1. **For Schizophrenia:**
   - Must have a diagnosis of schizophrenia
   - Must have an adequate trial and failure or an inadequate response, duration of at least 4 weeks, or intolerance to FOUR generic atypical antipsychotics
2. **For Major Depressive Disorder**
   - Must be age 18 years or older
   - Must have a diagnosis of major depressive disorder
   - Must have an adequate trial and failure or an inadequate response, duration of at least 4 weeks, or intolerance to ALL of the following:
     - Monotherapy with three different antidepressant therapies from three different classes AND
     - Combination therapy consisting of a SSRI or SNRI antidepressant plus aripiprazole as adjunctive treatment AND
     - Combination therapy consisting of a SSRI or SNRI antidepressant plus quetiapine ER as adjunctive treatment AND
   - Must also be taking brexpiprazole (Rexulti) with a SSRI or SNRI antidepressant as adjunctive treatment

If the above criteria are met, brexpiprazole (Rexulti) is approved.

Initial Authorization Criteria:

1. **For Agitation associated with Dementia due to Alzheimer's Disease (AD):**
   - Must be prescribed by or in consultation with a psychiatrist, neurologist, or geriatrician
   - Must be age 18 years or older
   - Must have a diagnosis of agitation associated with dementia due to AD
   - Must be assessed for underlying causes of agitation (e.g., pain, delirium, depression, infection, medication adverse effects, sleep-wake disturbances)
     - If underlying cause(s) are present, must provide an attestation that the member is being treated appropriately for their condition
   - Must have an adequate trial of non-pharmacologic interventions (e.g., redirection, use of structured routines, avoiding environmental triggers) with an inadequate response

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- Must have an adequate trial and failure of THREE generic atypical antipsychotic with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy

If the above criteria are met, brexpiprazole (Rexulti) is approved for 6 months.

Reauthorization criteria (applies to agitation associated with dementia due to AD diagnosis only):
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one-year intervals based upon the following:

- Must have documentation from the provider that the member remains a candidate for treatment based upon the prescriber's assessment and that the member continues to benefit from therapy
- Must have documentation of an evaluation to taper the dose or discontinue therapy with brexpiprazole (Rexulti) or provide clinical rationale for why this would not be appropriate

• Brimonidine Gel (Mirvaso®) & Oxymetazoline Cream (Rhofade™) [198, 199, 200, 201, 202]

Initial Authorization Criteria:

- Must have a diagnosis of persistent erythema associated with rosacea
- Must have an adequate trial of at least two other generic medications that are used for the treatment of rosacea (e.g. metronidazole, sodium sulfacetamide/sulfur, tretinoin, adapalene, doxycycline)

If the above criteria are met, brimonidine topical gel (Mirvaso) or oxymetazoline topical cream (Rhofade) is approved.

• Brivaracetam (Briviact®) [203]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with a neurologist
- Must be 1 month of age and older
- Must have a diagnosis of partial-onset seizures
- Must not be given with levetiracetam (Keppra)
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to at least 1 generic antiepileptic medications

If the above criteria are met, brivaracetam (Briviact), is approved.

• Budesonide DR Capsules (Tarpeyo™) [204, 205, 206]

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Initial Authorization Criteria:

- Must be prescribed by a nephrologist
- Must be age 18 years or older
- Must have a diagnosis of primary immunoglobulin A nephropathy (IgAN) confirmed by a kidney biopsy. Documentation of the biopsy results are required.
- Must be at risk of disease progression defined by the following (chart documentation is required):
  - Proteinuria > 0.75 g/day OR Urinary Protein-to-Creatinine Ratio (UPCR) ≥ 0.8 g/g despite at least 90 days of optimized supportive care treatment with ALL of the following:
    - Maximally tolerated dose of an angiotensin converting enzyme inhibitor (ACEi) or an angiotensin receptor blocker (ARB) unless there is a contraindication to these therapies
      - NOTE: for members who have experienced significant side effects/toxicities to one of these classes, an adequate trial of the other class is required (e.g., ACEi-induced cough requires a trial of an ARB)
    - Other supportive care measures as clinically indicated (e.g., lifestyle modifications, sodium-glucose co-transporter 2 [SGLT-2] inhibitors, blood pressure management, assessment of cardiovascular risk)
- Must have an adequate trial of at least 6 months of a glucocorticoid (e.g., prednisone or methylprednisolone) with an inadequate response or severe side effect/toxicity or have a contraindication to these therapies
  - Must provide clinical rationale to support the use of budesonide DR (Tarpeyo) when a glucocorticoid has not been effective to treat the diagnosis
    - Rationale must include reason why a glucocorticoid (e.g., prednisone or methylprednisolone) will not produce the same clinical results as would be expected with budesonide DR (Tarpeyo)
- Must have eGFR ≥35 mL/min per 1.73 m^2
- Must have an attestation from the prescriber that the member will remain on optimized supportive care as clinically indicated
- Must be using a dose and duration of therapy consistent with the FDA-approved package labeling
- Must not use in combination with the following:
  - Sparsentan (Filspari)
  - Atrasentan (Vanrafia)
  - Iptacopan (Fabhalta)

Sibeprenlimab-szsi (Voyxact)

If the above criteria are met, budesonide DR (Tarpeyo) is approved for 10 months.

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Reauthorization Criteria:
Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response and clinical rationale for re-treatment.

- **Budesonide Extended Release Tablet and Budesonide Rectal Foam (Uceris®)** [207, 208, 209, 210, 211]

Initial Authorization Criteria:

1. **Budesonide ER tablet:**
   - Must be prescribed by or in consultation with a gastroenterologist
   - Must be age 18 years or older
   - Must have a diagnosis of active, mild to moderate ulcerative colitis
   - Must be used to induce remission of active disease
   - Must have documentation of concurrent maintenance therapy for the treatment of ulcerative colitis (e.g. sulfasalazine, balsalazide)
   - Must have an adequate trial of one of the following with an inadequate response or significant side effect/toxicity or have a contraindication to these therapies:
     - Oral sulfasalazine or balsalazide
     - Oral corticosteroid
   - Must not have current evidence of infection

   If the above criteria are met, budesonide ER tablet (Uceris) is approved for 8 weeks.

2. **Budesonide rectal foam:**
   - Must be prescribed by or in consultation with a gastroenterologist
   - Must be age 18 years or older
   - Must have a diagnosis of active, mild to moderate distal ulcerative colitis
   - Must have documentation of concurrent maintenance therapy for the treatment of ulcerative colitis (e.g. sulfasalazine, balsalazide)
   - Must have an adequate trial of one of the following with an inadequate response or significant side effect/toxicity or have a contraindication to these therapies:
     - Topical mesalamine (i.e. Canasa® rectal suppository, mesalamine enema)
     - Topical steroids (i.e. hydrocortisone enema, hydrocortisone foam, hydrocortisone suppositories)
   - Must not have current evidence of infection

If the above criteria are met, budesonide rectal foam (Uceris) is approved for 6 weeks.

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Reauthorization Criteria:
All prior authorization renewals are reviewed on a case-by-case basis to determine the Medical Necessity for continuation of treatment. Authorization may be extended based upon one of the following:

- Must have documentation of concurrent maintenance therapy for the treatment of ulcerative colitis
- Clinical rationale from the prescriber for continuation of treatment beyond:
  - 8 weeks for budesonide ER tablets
  - 6 weeks for budesonide rectal foam
- Documentation that the member is experiencing a subsequent flare-up and experienced improvement in the condition as a result of treatment with budesonide (Uceris) during a previous flare-up

• Budesonide oral suspension (Eohilia™) [212, 213, 214, 215,216]

Initial Authorization Criteria:

- Must be prescribed by an allergist, immunologist, or gastroenterologist
- Must be age 11 years or older
- Must have a diagnosis of eosinophilic esophagitis (EoE) confirmed by ALL of the following:
  - Endoscopy with esophageal biopsy demonstrating ≥ 15 intraepithelial eosinophils per high-power field (eos/hpf). A copy of the report is required
  - Current or history of recurrent episodes of dysphagia or feeding difficulties (e.g., vomiting, regurgitation, or feeding refusal). Chart documentation is required
  - Must have an attestation from the prescriber that the member does not have a secondary cause of eosinophilic esophagitis
- Must have an adequate trial of at least 2 months with an inadequate response or significant side effect/toxicity or have a contraindication to BOTH of the following:
  - Proton pump inhibitor (PPI)
  - Swallowed inhaled respiratory corticosteroid (e.g., fluticasone HFA, budesonide respules)
- Must provide clinical rationale for use of Eohilia (budesonide oral suspension) instead of swallowed inhaled budesonide respules. Clinical rationale must include why swallowed inhaled budesonide respules would not be expected to produce the same clinical results as Eohilia (budesonide oral suspension) given that they are the same chemical entity

If the above criteria are met, budesonide oral suspension (Eohilia) is approved for 12 weeks. Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response and clinical rationale for re-treatment.

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- **Buprenorphine Extended-Release Injection (Brixadi™):** 217, 218, 219

Initial Authorization Criteria:

- Must have a diagnosis of moderate to severe opioid use disorder
- Must have initiated or tried treatment with a buprenorphine-containing product
- Must be prescribed a dose and duration of therapy that is consistent with FDA-approved package labeling, nationally-recognized compendia, or peer-reviewed medical literature

If the above criteria are met, buprenorphine extended-release injection (Brixadi) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at 1-year intervals based upon documentation showing the following:

- The member's condition has improved or stabilized (e.g., no evidence of relapse, no or decreased withdrawal symptoms or cravings) based upon the prescriber's assessment while on therapy
- Must be prescribed a dose and duration of therapy that is consistent with FDA-approved package labeling, nationally-recognized compendia, or peer-reviewed medical literature

- **Buprenorphine Extended-Release Injection (Sublocade™)** 220, 221, 222

Initial Authorization Criteria:

- Must have a diagnosis of moderate to severe opioid use disorder
- Must have initiated or tried treatment with a buprenorphine-containing product

If the above criteria are met, buprenorphine injection (Sublocade) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at 1-year intervals based upon documentation showing the following:

- The member's condition has improved or stabilized (e.g., no evidence of relapse, no or decreased withdrawal symptoms or cravings) based upon the prescriber's assessment while on therapy

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- **Canakinumab (Ilaris®)** [223, 224, 225, 226, 227, 228, 229, 230, 231, 232, 233, 234, 235, 236, 237, 238, 239, 240, 241, 242, 243, 244, 245, 246, 247, 248, 249, 250, 251, 252, 253, 254, 255, 256]

Initial Authorization Criteria:

1. **For Muckle-Wells Syndrome (MWS)**
   - Must have a negative tuberculosis skin test [such as Tuberculin PPD (purified protein derivative) test] or Interferon-Gamma Release Assay (IGRA) whole-blood test [such as QuantiFERON®-TB Gold In-Tube test (QFT-GIT) or T-SPOT®.TB test (T-Spot)]
   - Must be prescribed by a rheumatologist, dermatologist, immunologist, or genetic specialist
   - Must have a diagnosis of Muckle-Wells syndrome (MWS). Chart documentation of a clinical work-up to rule out other diagnoses and clinical rationale for the diagnosis and exclusion of other diagnoses must be provided. The diagnosis must either be confirmed by genetic testing or a clinical diagnosis defined as one of the following scenarios:
     - Must have a mutation in the NLRP3 (formerly CIAS1) gene. Documentation of lab result confirming mutation is required.
     - Must meet 3 of the following diagnostic criteria (chart documentation required):
       - History of intermittent episodes of fever for at least 24 hours duration
       - Autosomal dominant pattern of disease inheritance
       - Presence of severe fatigue
       - Presence of musculoskeletal symptoms (e.g., arthralgia, arthritis, myalgia)
       - Presence of ocular symptoms (e.g., conjunctivitis, anterior uveitis, papilledema)
       - Presence of erythematous rash
       - Presence of amyloidosis
       - Presence of hearing loss
   - Must have an adequate trial of a least 3 months of anakinra (Kineret) with an inadequate response or significant side effects/toxicity or have a contraindication to this therapy
   - Must not be using an oral targeted anti-inflammatory agent or biologic agent in combination with canakinumab (Ilaris)
   - Must have no evidence of infection

2. **For Familial Cold Autoinflammatory Syndrome (FCAS)**

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- Must have a negative tuberculosis skin test [such as Tuberculin PPD (purified protein derivative) test] or Interferon-Gamma Release Assay (IGRA) whole-blood test [such as QuantiFERON®-TB Gold In-Tube test (QFT-GIT) or T-SPOT®.TB test (T-Spot)]
- Must be prescribed by a rheumatologist, dermatologist, immunologist, or genetic specialist
- Must have a diagnosis of Familial Cold Autoinflammatory Syndrome (FCAS). Chart documentation of a clinical work-up to rule out other diagnoses and clinical rationale for the diagnosis and exclusion of other diagnoses must be provided. The diagnosis must either be confirmed by genetic testing or a clinical diagnosis defined as one of the following scenarios:
  - Must have a mutation in the NLRP3 (formerly CIAS1) gene. Documentation of lab result confirming mutation is required.
  - Must meet 3 of the following diagnostic criteria (chart documentation required):
    - History of intermittent episodes of fever and rash that primarily follow natural, experimental, or both types of generalized cold exposures for less than 24 hours duration
    - Autosomal dominant pattern of disease inheritance
    - Age of onset <6 months of age
    - Presence of conjunctivitis associated with attacks
    - Absence of deafness, periorbital edema, lymphadenopathy, and serositis
- Must have an adequate trial of a least 3 months of anakinra (Kineret) with an inadequate response or significant side effects/toxicity or have a contraindication to this therapy
- Must not be using an oral targeted anti-inflammatory agent or biologic agent in combination with canakinumab (Ilaris)
- Must have no evidence of infection

3. **For Familial Mediterranean Fever (FMF)**
   - Must have a negative tuberculosis skin test [such as Tuberculin PPD (purified protein derivative) test] or Interferon-Gamma Release Assay (IGRA) whole-blood test [such as QuantiFERON®-TB Gold In-Tube test (QFT-GIT) or TSPOT ®.TB test (T-Spot)]
   - Must be prescribed by a rheumatologist, dermatologist, immunologist, or genetic specialist
   - Must have a diagnosis of Familial Mediterranean Fever (FMF). Chart documentation of a clinical work-up to rule out other diagnoses and clinical rationale for the diagnosis and exclusion of other diagnoses must be provided. The diagnosis must either be confirmed by genetic testing or a clinical diagnosis defined as one of the following scenarios:
     - Must have a mutation of the MEFV gene. Documentation of lab result confirming mutation is required.

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        - Must meet 3 of the following diagnostic criteria (chart documentation required):
            - History of intermittent episodes of fever and pain for 1-3 days duration
            - Autosomal recessive pattern of disease inheritance
            - Presence of peritonitis
            - Presence of arthritis
            - Presence of pleuritis
            - Presence of erysipelas-like erythema
    - Must have an adequate trial of a least 3 months of colchicine with an inadequate response or significant side effects/toxicity or have a contraindication to this therapy
    - Must have an adequate trial of a least 3 months of anakinra (Kineret) with an inadequate response or significant side effects/toxicity or have a contraindication to this therapy
    - Must not be using an oral targeted anti-inflammatory agent or biologic agent in combination with canakinumab (Ilaris)
    - Must have no evidence of infection

4. **For Hyperimmunoglobulin D Syndrome (HIDS)/Mevalonate Kinase Deficiency (MKD)**
    - Must have a negative tuberculosis skin test [such as Tuberculin PPD (purified protein derivative) test] or Interferon-Gamma Release Assay (IGRA) whole-blood test [such as QuantiFERON®-TB Gold In-Tube test (QFT-GIT) or TSPOT ®.TB test (T-Spot)]
    - Must be prescribed by a rheumatologist, dermatologist, immunologist, or genetic specialist
    - Must have a diagnosis of Hyperimmunoglobulin D Syndrome (HIDS)/Mevalonate Kinase Deficiency (MKD). Chart documentation of a clinical work-up to rule out other diagnoses and clinical rationale for the diagnosis and exclusion of other diagnoses must be provided. The diagnosis must either be confirmed by genetic testing or a clinical diagnosis defined as one of the following scenarios:
        - Must have a mutation in the MVK gene. Documentation of lab result confirming mutation is required.
        - Must meet 3 of the following diagnostic criteria (chart documentation required):
            - History of intermittent episodes of fever and inflammation for at least 3 days duration
            - Autosomal recessive pattern of disease inheritance
            - Presence of painful lymph nodes
            - Presence of joint pain
            - Presence of headache

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        - Presence of hepatosplenomegaly.
        - Presence of abdominal pain, vomiting, diarrhea
        - Presence of a skin rash
    - Must have an adequate trial of a least 3 months of anakinra (Kineret) with an inadequate response or significant side effects/toxicity or have a contraindication to this therapy
    - Must not be using an oral targeted anti-inflammatory agent or biologic agent in combination with canakinumab (Ilaris)
    - Must have no evidence of infection

5. **For Tumor Necrosis Factor Receptor Associated Periodic Syndrome (TRAPS)**
    - Must have a negative tuberculosis skin test [such as Tuberculin PPD (purified protein derivative) test] or Interferon-Gamma Release Assay (IGRA) whole-blood test [such as QuantiFERON®-TB Gold In-Tube test (QFT-GIT) or TSPOT ®.TB test (T-Spot)]
    - Must be prescribed by a rheumatologist, dermatologist, immunologist, or genetic specialist
    - Must have a diagnosis of Tumor Necrosis Factor Receptor Associated Periodic Syndrome (TRAPS). Chart documentation of a clinical work-up to rule out other diagnoses and clinical rationale for the diagnosis and exclusion of other diagnoses must be provided. The diagnosis must either be confirmed by genetic testing or a clinical diagnosis defined as one of the following scenarios:
        - Must have a mutation in the TNFRSF1A gene. Documentation of lab result confirming mutation is required.
        - Must meet 3 of the following diagnostic criteria (chart documentation required):
            - History of intermittent episodes of fever for at least 1 week duration
            - Autosomal dominant pattern of disease inheritance
            - Presence of painful lymph nodes
            - Presence of headache
            - Presence of a skin rash
            - Presence of muscle cramps
            - Presence of abdominal pain
            - Presence of pleuritis
            - Presence of pericarditis
            - Presence of conjunctivitis, periorbital edema
    - Must have an adequate trial of a least 3 months of etanercept (Enbrel) with an inadequate response or significant side effects/toxicity or have a contraindication to this therapy

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- Must not be using an oral targeted anti-inflammatory agent or biologic agent in combination with canakinumab (Ilaris)
- Must have no evidence of infection

6. **For Juvenile Idiopathic Arthritis with systemic symptoms, includes systemic juvenile idiopathic arthritis (SJIA)**
   - Must have a negative tuberculosis skin test [such as Tuberculin PPD (purified protein derivative) test] or Interferon-Gamma Release Assay (IGRA) whole-blood test [such as QuantiFERON®-TB Gold In-Tube test (QFT-GIT) or TSPOT ®.TB test (T-Spot)]
   - Must be prescribed by a rheumatologist
   - Must be age 2 years or older
   - Must have a diagnosis of active juvenile idiopathic arthritis. Chart documentation of a clinical work-up to rule out other diagnoses and clinical rationale for the diagnosis and exclusion of other diagnoses must be provided. Chart documentation must demonstrate all of the following:
     - History of fever for at least 2 week duration
     - History of at least 1 of the following:
       - Evanescent rash
       - History of arthritis in 1 or more joints
       - Generalized lymph node enlargement
       - Hepatomegaly or splenomegaly
       - Pericarditis, pleuritis, or peritonitis
   - Must have an adequate trial of at least 3 months of anakinra (Kineret) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
   - Must not be using an oral targeted anti-inflammatory agent or biologic agent in combination with canakinumab (Ilaris)
   - Must have no evidence of infection

7. **Still's Disease**
   - Must have a negative tuberculosis skin test [such as Tuberculin PPD (purified protein derivative) test] or Interferon-Gamma Release Assay (IGRA) whole-blood test [such as QuantiFERON®-TB Gold In-Tube test (QFT-GIT) or TSPOT TB test (T-Spot)]
   - Must be prescribed by a rheumatologist
   - Must be age 18 years or older
     - If less than 18 years of age refer to criteria for systemic juvenile idiopathic arthritis (SJIA)
   - Must have diagnosis of Still's disease. Chart documentation of a clinical work-up to rule out other diagnoses and clinical rationale for the diagnosis and exclusion of other diagnoses must be provided.
   - Must have a history of at least 5 or more diagnostic criteria below, two or more must be major criteria

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        - Major criteria:
            - Fever for at least 1 week
            - Rash (usually over trunk or extremities while febrile)
            - History of arthralgia or arthritis in 1 or more joints for at least 2 weeks
            - Leukocytosis (10,000/microL or greater)
        - Minor criteria:
            - Sore throat
            - Generalized lymph node enlargement
            - Hepatomegaly or splenomegaly
            - Abnormal liver function tests
            - Negative tests for antinuclear antibody and rheumatoid factor
    - Must have an adequate trial of at least 3 months of methotrexate with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
    - Must have an adequate trial of at least 3 months of anakinra (Kineret) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
    - Must not be using an oral targeted anti-inflammatory agent or biologic agent in combination with canakinumab (Ilaris)
    - Must have no evidence of infection

8. Gout Flares

- Must be prescribed by or in consultation with a rheumatologist
- Must be age 18 years or older
- Must be prescribed for symptomatic treatment of gout flares
- Must have chart documentation of an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to BOTH of the following:
    - Two NSAIDs (e.g., naproxen, indomethacin, sulindac, celecoxib) at a maximum tolerated anti-inflammatory dose
    - Colchicine
- Must meet ONE of the following criteria:
    - Must have chart documentation of an adequate trial with an inadequate response or significant side effect/toxicity to a maximally tolerated corticosteroid
    - Must have a clinical rationale why repeated courses of corticosteroids are not appropriate
- Must have an adequate trial of anakinra (Kineret) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
- Must have documentation of concurrent maintenance therapy for the treatment of gout (e.g. allopurinol, febuxostat, probenecid)

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- Must not be using a Tumor Necrosis Factor (TNF) blocking agent or biologic agent in combination with canakinumab (Ilaris)
- Must have an attestation from the prescriber that the member has not received therapy with canakinumab (Ilaris) at least 12 weeks prior to the requested course of therapy
- Must have no evidence of infection

If the above criteria are met, canakinumab (Ilaris) is approved for a duration based upon diagnosis as follows:
- For gout flares: 1 month
- For all other diagnoses: 4 months

Reauthorization Criteria:
All prior authorization renewals are reviewed to determine the Medical Necessity for continuation of therapy.

1. **For gout flares**
   - Authorization may be extended at 6-month intervals based on the following:
     - The member's condition improved while on therapy
     - Must have attestation from the prescriber that member will not receive additional therapy with canakinumab (Ilaris) within 12 weeks of receiving a previous course of therapy
     - Must have adherence (PDC >80%) to maintenance therapy for the treatment of gout (e.g. allopurinol, febuxostat, probenecid)
2. **For all other diagnoses:**
   - Authorization may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved while on therapy and documentation that there is no evidence of infection.

- **Cannabidiol (Epidiolex®)** [257, 258, 259, 260, 261, 262, 263, 264, 265, 266, 267]

FDA Approved Indications:

1. **For Lennox-Gastaut Syndrome**
   - Must be prescribed by or in consultation with a neurologist
   - Must be age 1 year or older
   - Must have diagnosis of Lennox-Gastaut syndrome
   - Must have an adequate trial of at least 2 generic antiepileptic medications with an inadequate response or significant side effect/toxicity or have a contraindication to these therapies

2. **For Dravet Syndrome**
   - Must be prescribed by or in consultation with a neurologist
   - Must be age 1 year or older

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- Must have diagnosis of Dravet syndrome
- Must have an adequate trial of at least 2 generic antiepileptic medications with an inadequate response or significant side effect/toxicity or have a contraindication to these therapies

3. For Tuberous Sclerosis Complex
- Must be prescribed by or in consultation with a neurologist
- Must be age 1 year or older
- Must have a diagnosis of Tuberous Sclerosis Complex
- Must have an adequate trial of at least 2 generic antiepileptic medications with an inadequate response or significant side effect/toxicity or have a contraindication to these therapies

Off-Label Use:

1. For Intractable or Treatment Refractory Seizures
- Must be prescribed by or in consultation with a neurologist
- Must be age 2 years or older
- Must have an adequate trial of at least 3 generic antiepileptic medications with an inadequate response or significant side effect/toxicity or have a contraindication to these therapies

If the above criteria are met, cannabidiol (Epidiolex) is approved.

• Caplacizumab-yhdp (Cablivi®) [268, 269, 270, 271]

Initial Authorization Criteria:
- Must be prescribed by or in consultation with a hematologist
- Must be age 12 years or older
- Must have a diagnosis of acquired thrombotic thrombocytopenia purpura (aTTP), confirmed by ALL of the following:
  - Thrombocytopenia (platelet count < 100 x 10^9/L)
  - Microangiopathic hemolytic anemia (MAHA) (e.g., hemoglobin and hematocrit below reference range, low haptoglobin, elevated LDH, presence of schistocytes in peripheral blood smear)
  - Laboratory test for ADAMTS13 activity level has been completed or is in progress
- Must have documentation caplacizumab-yhdp (Cablivi) was started in combination with plasma exchange and provide date of last plasma exchange for this aTTP episode
- Must be used in combination with immunosuppressive therapy (e.g., corticosteroids ± rituximab)
- Must not have experienced more than 2 recurrences of aTTP while on caplacizumab-yhdp (Cablivi)

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If the above criteria are met, caplacizumab-yhdp (Cablivi) is approved for 1 month.

Continuation/Discontinuation Criteria
All requests for treatment extension are reviewed to determine the Medical Necessity for continuation of therapy. Authorizations may be extended up to one month based upon ALL of the following:

- Must have a diagnosis of aTTP confirmed by suppressed ADAMTS13 activity level
- Must have chart documentation of sign(s) of persistent underlying disease (e.g., suppressed ADAMTS13 activity levels) after the initial 30-day treatment course
- Must not have received more than 58 days of caplacizumab-yhdp (Cablivi) after completion of plasma exchange therapy for this aTTP episode
- Must not have experienced more than 2 recurrences of aTTP while on caplacizumab-yhdp (Cablivi)

Authorizations for a new aTTP episode are made on a case-by-case basis, are subject to the above initial criteria, and require chart documentation describing the previous response and clinical rationale for re-treatment.

• Carbidopa (Lodosyn®) [272]

Criteria:

- Must be prescribed by or in consultation with a neurologist
- Must have a diagnosis of Parkinson's disease
- Must take in combination with carbidopa/levodopa (Sinemet®)

If the above criteria are met, carbidopa (Lodosyn) is approved.

• Carbidopa/levodopa enteral suspension (Duopa) [273, 274]

Initial Authorization Criteria:

- Must be prescribed by a neurologist
- Must have a diagnosis of advanced Parkinson's Disease with ALL of the following:
  - A clear motor response to levodopa
  - Chart documentation of motor fluctuations despite optimized medication therapy regimen based on the prescriber's assessment, including initial benefit after dose of levodopa followed by return of parkinsonian features before onset of benefit from subsequent dose (e.g., wearing off) or evidence of involuntary movements when therapeutic effect of levodopa is maximal (e.g., peak dose dyskinesia)
- Must have an adequate trial of TWO of the following medications, taken in combination with carbidopa/levodopa for the adjunctive treatment of "off"

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episodes, with an inadequate response or significant side effect/toxicity or have a contraindication to these therapies:
- Entacapone or opicapone (Ongentys)
- A generic MAO-B inhibitor (e.g., selegiline or rasagiline)
- A generic dopamine agonist (e.g., ropinirole tablet, pramipexole tablet)
- Istradefylline (Nourianz)

If the above criteria are met, carbidopa/levodopa enteral suspension (Duopa) is approved for 6 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member continues to benefit from therapy or the member's disease course has stabilized or improved based upon the prescriber's assessment while on therapy

• Carglumic Acid (Carbaglu®) [275, 276, 277, 278, 279]

Initial Authorization Criteria:
- Must be prescribed by or in consultation with a physician who specializes in the treatment of inherited metabolic disorders
- Must have a diagnosis of one of the following deficiencies. Chart documentation describing how diagnosis was confirmed (e.g., genetic testing results, enzyme assays, ammonia levels, progress notes, etc.) is required.
  - N-acetylglutamate synthase (NAGS)
  - N-acetylglutamate (NAG)
  - Carbamoyl phosphate synthetase 1 (CPS 1)
  - Acute hyperammonemia due to propionic acidemia (PA) or methylmalonic acidemia (MMA)
- For acute hyperammonemia due to PA or MMA:
  - Must be used to treat an acute episode of hyperammonemia

If the above criteria are met, carglumic acid (Carbaglu) is approved:
- For NAGS deficiency, NAG deficiency, or CPS 1 deficiency: 3 months
- For acute hyperammonemia due to PA or MMA: 1 week

Reauthorization Criteria:
All prior authorization renewals for NAGS deficiency, NAG deficiency, or CPS 1 deficiency diagnoses are reviewed on an annual basis to determine the Medical Necessity for continuation of treatment. Authorization may be extended at one-year intervals based upon chart documentation from the prescriber that the member's condition has improved based on prescriber's assessment while on therapy.

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Additional authorizations for episodes of acute hyperammonemia due to PA or MMA are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response and clinical rationale for re-treatment.

- **Cariprazine (Vraylar®)** [280, 281, 282, 283, 284, 285, 286]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with a psychiatrist
- For members < 12 years of age, please reference 'Atypical and Typical Antipsychotics – Children Under 12 years of Age' criteria above
- Must have one of the following diagnoses:
  - Schizophrenia or manic or mixed episodes associated with bipolar I disorder and meet the following criteria:
    - Must have an adequate trial, of at least 4 weeks, with an inadequate response or significant side effect/toxicity or have a contraindication to FOUR generic atypical antipsychotics
  - Depressive episodes associated with bipolar I disorder (bipolar depression) and meet the following criteria:
    - Must be age 18 years or older
    - Must have an adequate trial, of at least 4 weeks, with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
      - Lithium or lamotrigine AND
      - Quetiapine or quetiapine ER AND
      - Lurasidone AND
      - Olanzapine or olanzapine/fluoxetine
  - Major depressive disorder and meet the following criteria:
    - Must be age 18 years or older
    - Must have an adequate trial, of at least 4 weeks, with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
      - Monotherapy with three different antidepressant therapies from three different classes AND
      - Combination therapy consisting of a SSRI or SNRI antidepressant plus aripiprazole as adjunctive treatment AND
      - Combination therapy consisting of a SSRI or SNRI antidepressant plus quetiapine ER as adjunctive treatment AND
    - Must also be taking cariprazine (Vraylar) with a SSRI or SNRI antidepressant as adjunctive treatment

If the above criteria are met, cariprazine (Vraylar) is approved.

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- **Cenegermin-bkbj (Oxervate™)** [287, 288, 289, 290, 291, 292, 293, 294]

Initial Authorization Criteria:

- Must be prescribed by an ophthalmologist
- Must be age 2 years or older
- Must indicate if left eye, right eye, or both eyes are affected
- Must have a persistent epithelial defect (stage 2) or corneal ulceration (stage 3) of at least 2 weeks duration, refractory to conventional therapies. Must provide chart documentation of the following to support the diagnosis of neurotrophic keratitis:
  - Corneal sensitivity assessment demonstrating decreased corneal sensitivity
  - Persistent epithelial defect on slit-lamp examination
  - Documentation of a clinical work-up, including clinical rationale for the diagnosis and exclusion of other possible conditions
- Must have an assessment of the underlying cause of neurotrophic keratitis and be receiving appropriate treatment
- Must have concomitant ocular surface diseases other than/associated with neurotrophic keratitis (e.g., blepharitis, exposure keratitis) treated (if present)
- Must have an adequate trial of the following standard of care therapies for treatment of neurotrophic keratitis with an inadequate response or have a contraindication to these therapies
  - Preservative-free artificial tears
  - Autologous serum eye drops, therapeutic contact lenses, or amniotic membrane therapy

If the above criteria are met, cenegermin-bkbj, (Oxervate) is approved for one course of treatment (8 weeks) per affected eye(s).

Reauthorization Criteria:

Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response and clinical rationale for re-treatment.

- **Cenobamate (Xcopri®)** [295, 296, 297, 298, 299, 300, 301, 302, 303, 304, 305]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with a neurologist
- Must be age 18 years or older
- Must have a diagnosis of partial-onset seizures
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to at least 2 generic antiepileptic medications

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If the above criteria are met, cenobamate (Xcopri) is approved.

- **Chenodiol (Chenodal®)** [306, 307, 308]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with a gastroenterologist
- Must be age 18 years or older
- Must have a diagnosis of radiolucent gallstones. Chart documentation must be provided.
- Must not be a candidate for surgery (e.g., due to systemic disease or advanced age). Chart documentation must be provided.
- Must have an adequate trial of ursodiol with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
- Must not have ANY of the following:
  - Gallstone complications or compelling need for surgery (e.g., unremitting acute cholecystitis, cholangitis, biliary obstruction, gallstone pancreatitis, or biliary gastrointestinal fistula)
  - Hepatocyte dysfunction or bile ductal abnormalities (e.g., intrahepatic cholestasis, primary biliary cirrhosis, or sclerosing cholangitis)
  - Nonvisualizing gallbladder after 2 consecutive single doses of dye
  - Pregnancy
  - Radiopaque stones
- Must be prescribed a dose and duration of therapy that is consistent with FDA-approved package labeling, nationally-recognized compendia, or peer-reviewed medical literature

If the above criteria are met, chenodiol (Chenodal) is approved for 2 years. Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response and clinical rationale for re-treatment.

- **Chenodiol (Ctexli™)** [309, 310]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with a provider who specializes in the treatment of inherited metabolic disorders
- Must have a diagnosis of cerebrotendinous xanthomatosis (CTX) confirmed by ONE of the following:
  - Biallelic pathogenic mutation in the *CYP27A1* gene (a copy of the genetic test must be provided)
  - Elevated serum cholestanol level (chart documentation of laboratory result including reference range must be provided)
- Must not use in combination with cholic acid (Cholbam)

If the above criteria are met, chenodiol (Ctexli) is approved for 1 year.

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Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or stabilized while on therapy.

- **Cholic acid (Cholbam®)** [311, 312]
  - Must be prescribed by or in consultation with a provider who specializes in the treatment of inherited metabolic disorders, a gastroenterologist, or hepatologist
  - Must have ONE of the following diagnoses:
    - Bile acid synthesis disorders due to single enzyme defects confirmed by ONE of the following:
      - Genetic testing (a copy of the genetic test must be provided)
      - Abnormal serum or urinary bile acid levels (chart documentation must be provided)
    - Adjunctive treatment of peroxisomal disorders including Zellweger spectrum disorders confirmed by ALL of the following:
      - Must have diagnosis confirmed by ONE of the following:
        - Genetic testing (a copy of the genetic test must be provided)
        - Abnormal serum or urinary bile acid levels (chart documentation must be provided)
      - Must exhibit manifestations of liver disease, steatorrhea or complications from decreased fat-soluble vitamin absorption
  - Must be using a dose and duration of therapy consistent with the FDA-approved package labeling
  - Must not use in combination with chenodiol (Ctexli, Chenodal)

If the above criteria are met, cholic acid (Cholbam) is approved for 3 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation showing improvement in liver function (e.g., aspartate aminotransferase [AST], alanine aminotransferase [ALT]).

- **Cholinesterase Inhibitors and N-Methyl-D-Aspartate Receptor Antagonist** [313, 314, 315, 316, 317, 318, 319, 320, 321, 322, 323, 324]

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- Applies to: donepezil (Aricept®), galantamine (Razadyne®/Razadyne ER®), rivastigmine tartrate oral capsule (Exelon®), rivastigmine patch (Exelon® Patch), memantine (Namenda®), memantine ER (Namenda XR®)
- Applies to: donepezil transdermal system (Adlarity®), memantine/donepezil ER (Namzaric XR®) (Value Choice and Open Choice formularies only)

Criteria:

1. **For Cholinesterase Inhibitors:**
   - Must be age 18 years or older
     - Donepezil (Aricept) tablets are covered without a prior authorization for ages 50 years or older
   - For a diagnosis of mild to moderate Alzheimer's dementia (applies to all cholinesterase inhibitors):
     - Must have at least ONE of the following characteristics:
       - Memory loss
       - Other cognitive changes
       - Mood/behavior changes
     - For rivastigmine patch (Exelon):
       - Must have had an adequate trial of one formulary, oral cholinesterase inhibitor with an inadequate response or significant side effects/toxicity or must have a contraindication to these therapies
   - For a diagnosis of dementia associated with Parkinson's disease (applies to rivastigmine [Exelon] only)
     - Must have at least ONE of the following characteristics:
       - Memory loss
       - Other cognitive changes
       - Mood/behavior changes
     - For rivastigmine patch (Exelon):
       - Must have had an adequate trial of rivastigmine capsule (Exelon) with an inadequate response or significant side effects/toxicity or must have a contraindication to this therapy

2. **For N-Methyl-D-Aspartate Receptor Antagonists:**
   - For memantine tablet (Namenda) and memantine ER (Namenda XR):
     - Must be age 18 years or older
       - Memantine tablet (Namenda) and memantine ER (Namenda XR) are covered without a prior authorization for ages 50 years or older
     - Must have a diagnosis of moderate to severe Alzheimer's dementia
       - Must have difficulty with basic activities of daily living or be fully dependent
       - Must have at least ONE of the following characteristics:
         - Memory loss

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        - Other cognitive changes
        - Mood/behavior changes

- For memantine oral solution:
    - Must meet above criteria for memantine tablet (Namenda)
        - Must have documentation of the inability to swallow an oral solid dosage form due to a medical condition (e.g., nasogastric tube), or other clinical rationale for why memantine tablet (Namenda) cannot be used

If the above criteria are met, the requested medication is approved.

3. **For *Open Choice* and *Value Choice* only:**
    - For benzgalantamine (Zynveyl®):
        - Must meet above criteria for cholinesterase inhibitors
        - Must have an adequate trial of galantamine (Razadyne) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
        - Must provide clinical rationale for use of benzgalantamine (Zynveyl) instead of galantamine (Razadyne). Clinical rationale must include why galantamine (Razadyne) would not be expected to produce the same clinical results as benzgalantamine (Zynveyl)

    - For donepezil transdermal system (Adlarity):
        - Must meet above criteria for cholinesterase inhibitors
        - Must have an adequate trial of donepezil tablet (Aricept) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
        - Must provide clinical rationale for use of donepezil transdermal system (Adlarity) instead of donepezil tablet (Aricept). Clinical rationale must include why donepezil tablet (Aricept) would not be expected to produce the same clinical results as donepezil transdermal system (Adlarity) given that they are the same chemical entity.

    - For memantine/donepezil ER (Namzaric XR):
        - Must meet above criteria for N-Methyl-D-Aspartate Receptor Antagonists
        - Must have an adequate trial of memantine tablet (Namenda) with inadequate response or significant side effects/toxicity or have a contraindication to this therapy
        - Must be stable on donepezil 10 mg tablet (Aricept) with inadequate response or significant side effects/toxicity or have a contraindication to this therapy

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        - Must have an adequate trial of memantine tablet (Namenda) in combination with donepezil tablet (Aricept) with inadequate response or significant side effects/toxicity or have a contraindication to this therapy
        - Must provide clinical rationale for use of memantine/donepezil ER (Namzaric XR) instead of memantine tablet (Namenda) in combination with donepezil tablet (Aricept). Clinical rationale must include why this combination would not be expected to produce the same clinical results as memantine/donepezil ER (Namzaric XR) given that they are the same chemical entities.

If the above criteria are met, the requested medication is approved. (Applies to Value Choice and Open Choice Formularies only)

- **Cladribine (Mavenclad®)** [325, 326, 327, 328, 329, 330]

Initial Authorization Criteria:

- Must be prescribed by a neurologist
- Must be age 18 years or older
- Must have a diagnosis of relapsing remitting, or active secondary progressive disease form of multiple sclerosis
- Must have an adequate trial with an inadequate response or significant side effects/toxicity, or have a contraindication to ONE of the following:
    - Glatiramer acetate/Glatopa® (Copaxone®)
    - Dimethyl fumarate (Tecfidera®)
    - Fingolimod (Gilenya®)
    - Teriflunomide (Aubagio®)
- Must have an adequate trial with an inadequate response or significant side effects/toxicity, or have a contraindication to ONE of the following:
    - Interferon beta-1a (Avonex®, Rebif®)
    - Peginterferon beta-1a (Plegridy®)
    - Interferon beta-1b (Betaseron®)
    - Siponimod (Mayzent®)
    - Ozanimod (Zeposia®)
    - Ofatumumab (Kesimpta®)
- Must have an adequate trial of natalizumab (Tysabri) with an inadequate response or significant side effects/toxicity, or have a contraindication to this therapy if JC virus seronegative
- Must have an adequate trial with an inadequate response or significant side effects/toxicity, or have a contraindication to ONE of the following:
    - Rituximab product
    - Ocrelizumab or ocrelizumab and hyaluronidase-ocsq (Ocrevus, Ocrevus Zunovo)

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- Must not be on concomitant therapy with a disease modifying drug for multiple sclerosis

If the above criteria are met, cladribine (Mavenclad) is approved for 2 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon the following:

- The member's condition has improved or stabilized while on therapy
- The member is not on concomitant therapy with a disease modifying drug for multiple sclerosis
- The member completed their first course of treatment (first two cycles) at least 43 weeks prior to the second course of therapy and has not received more than one course (two cycles) of treatment previously

- **Clobazam (Onfi®, Sympazan®)** [331, 332, 333, 334, 335, 336, 337, 338, 339, 340, 341, 342, 343, 344, 345, 346]

Initial Authorization Criteria:

1. **For clobazam (Onfi) tablet**
   - **For Lennox-Gastaut Syndrome:**
     - Must be prescribed by or in consultation with a neurologist
     - Must be age 2 years or older
     - Must have a diagnosis of Lennox-Gastaut syndrome
     - Must have an adequate trial of at least 2 generic antiepileptic medications with an inadequate response or significant side effect/toxicity or have a contraindication to these therapies
     - Must be on concomitant therapy with at least one other anticonvulsant medication

   - **For Dravet Syndrome:**
     - Must be prescribed by or in consultation with a neurologist
     - Must be age 2 years or older
     - Must have a diagnosis of Dravet syndrome

2. **For clobazam (Onfi) suspension**
   - Must meet the above criteria for clobazam (Onfi)
   - Must have documentation of the inability to swallow an oral solid dosage form due to age (< 7 years) or medical condition (e.g. nasogastric tube)

If the above criteria are met, the requested product is approved.

3. **For *Open Choice* and *Value Choice* only:**

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- **For clobazam (Sympazan) sublingual film**
  - Must meet the above criteria for clobazam (Onfi)
  - Must have documentation of the inability to swallow an oral solid dosage form due to age (< 7 years) or medical condition (e.g. nasogastric tube) and clinical rationale for why clobazam (Onfi) suspension cannot be used

If the above criteria are met, clobazam (Sympazan) is approved. (Applies to Value Choice and Open Choice Formularies only)

• Colchicine (Lodoco®): [347, 348, 349, 350]

Initial Authorization Criteria:

- Must be prescribed by a cardiologist
- Must be age 18 years or older
- Must have a diagnosis of clinical atherosclerotic cardiovascular disease (ASCVD). Documentation confirming the history of at least one of the following is required:
  - Myocardial infarction (MI) or other acute coronary syndromes (ACS) including ST-elevation myocardial infarction (STEMI), non-ST-elevation myocardial infarction (NSTEMI), and unstable angina (UA)
  - Coronary or other revascularization procedure
  - Ischemic stroke or transient ischemic attack (TIA)
  - Atherosclerotic peripheral arterial disease (includes ankle/brachial index of <0.90)
  - Coronary artery calcium (CAC) ≥ 100 Agatston units or ≥ 75th percentile for age, sex, and ethnicity
    - CAC reference values can be found at: http://www.mesa-nhlbi.org/CACReference.aspx
  - Carotid plaque ≥ 50%
  - Coronary atherosclerosis as demonstrated by angiography (cardiac CT angiography or conventional cardiac catheterization)
- Must have an adequate trial of generic colchicine 0.6 mg (Colcrys®, Mitigare®) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
  - Must provide clinical rationale for use of Lodoco instead of generic colchicine 0.6 mg. Clinical rationale must include why colchicine 0.6 mg would not be expected to produce the same clinical results as Lodoco given that they are the same chemical entity
- Must have an attestation from the prescriber that the member is on an optimized clinical regimen for coronary artery disease with at least one other medication (e.g., low-dose aspirin or other antiplatelets, statins, beta-blockers, and/or anticoagulants)

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- Must not have ANY of the following:
  - History of pre-existing blood dyscrasias, renal failure, or severe hepatic impairment
  - Concomitant use of strong CYP3A4 inhibitors (e.g., itraconazole, ketoconazole, clarithromycin) or P-gp inhibitors (e.g., amiodarone, cyclosporine, verapamil)

If the above criteria are met, colchicine (Lodoco) is approved.

• Collagenase Ointment (Santyl®) [351, 352]

Initial Authorization Criteria:
Collagenase ointment (Santyl) will be covered for an initial period of 1 fill. After the initial fill, the following criteria must be met:

- Must be prescribed by or in consultation with a wound care specialist (e.g. podiatrist or surgeon) or a burn specialist
- Must be used for debriding chronic dermal ulcers and severely burned areas
- Must continue to require debridement of necrotic tissue from a wound without well-established granulation tissue
- Must not be using any wound care products simultaneously that could decrease the enzymatic activity of collagenase (e.g. products containing mercury or silver) on the same application site as collagenase ointment (Santyl)
- Must have improved healing of the wound after the initial period of 1 fill
- Must have documentation that there is a reasonable medical expectation that continued treatment will result in healing and resolution of the wound

If the above criteria are met, collagenase ointment (Santyl) is approved for 6 weeks.

Reauthorization Criteria:
All prior authorization renewals are reviewed to determine the Medical Necessity for continuation of therapy. Authorization may be extended for up to 6 week intervals based upon documentation of the following:

- Must have improved healing of the wound
- Must continue to require debridement of necrotic tissue from a wound without well-established granulation tissue
- Must not be using any wound care products simultaneously that could decrease the enzymatic activity of collagenase (e.g. products containing mercury or silver) on the same application site as collagenase ointment (Santyl)
- Must have documentation that there is a reasonable medical expectation that continued treatment will result in healing and resolution of the wound

• Compounded Medications

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Initial Authorization Criteria:
Compounded medications priced over $50.00 are reviewed based on the ingredients and indication for the medication as well as the criteria listed below:

- All ingredients that are used in the compound must be provided by the prescriber or compounding pharmacist
- At least one agent in the compound must be for a prescription medication.
    - Compounds are not approved if all ingredients are available over-the-counter.
- If an A-rated generic is available for any of the ingredients in the compound, the generic must be utilized.
- Must have an adequate trial of commercially-available formulary medications with an inadequate response or significant side effects/toxicity or have a contraindication to these therapies.
- The price of the compound is determined based on all ingredients in the product plus a negotiated compounding fee.

Compounded medications are not approved if any of the following apply:

- The medication is commercially-available.
- One of the ingredients is listed as a benefit exclusion or as a non-covered medication based on the pharmacy benefit rider.
- The compound is for an oral narcotic analgesic or hormone replacement therapy.

If the above criteria are met, the requested compounded medication is approved on a case-by-case basis for a length of time determined to be clinically appropriate for the indication, up to a maximum of 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one-year intervals based upon chart documentation from the prescriber that the member's disease has improved based upon the prescriber's assessment while on therapy.

- **Continuous Glucose Monitoring Systems** [353, 354, 355, 356, 357, 358]

Initial Authorization Criteria:

- Must have a diagnosis of diabetes
- Must currently be using insulin; or has a history of problematic hypoglycemia, defined as a history of recurrent (more than one) level 2 hypoglycemic events (plasma glucose concentration below 54 mg/dL) or at least one level 3 hypoglycemic event (serious event characterized by a change in the mental status or impairment in the patient's physical ability that requires intervention by another person to correct the glucose concentration)

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If the above criteria are met, the Continuous Glucose Monitoring System is approved for 1 year.

Reauthorization Criteria:
All prior authorization requests are reviewed on an annual basis to determine the Medical Necessity for continuation of treatment. Authorizations may be extended at one-year intervals based upon documentation from the prescriber that the member's condition has improved based upon the prescriber's assessment while on therapy or the member continues to benefit from therapy.

• Contraceptives – Religious Group Exemption/ Accomodation [359]

Criteria:

- Must have a medical condition, in which contraceptive coverage would be considered medically necessary, including, but not limited to:
  - Acne
  - Amenorrhea
  - Dysfunctional uterine bleeding (DUB)
  - Dysmenorrhea
  - Endometriosis
  - Headaches/migraines
  - Hypermenorrhea
  - Hypomenorrhea
  - Menorrhagia
  - Metrorrhagia
  - Menometrorrhagia
  - Oligomenorrhea
  - Perimenopausal symptoms
  - Polycystic ovarian syndrome (PCOS)
  - Polymenorrhea
  - Premenstrual dysphoric disorder
  - Treatment or prevention of ovarian cysts
  - Uterine fibroids/cysts

If the established criteria are met, the requested contraceptive medication is approved for one year.

• Crinecerfont (Crenessity®) [360, 361, 362, 363, 364]

Initial Authorization Criteria:

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- Must be prescribed by an endocrinologist or a provider who specializes in the treatment of congenital adrenal hyperplasia inherited metabolic disorders or an endocrinologist
- Must be age 4 years or older
- Must have chart documentation of a diagnosis of 21-hydroxylase deficiency classic congenital adrenal hyperplasia (CAH) confirmed by ONE of the following:
  - Elevated 17-hydroxyprogesterone level
  - Confirmed cytochrome (CYP)21A2 genotype (a copy of the genetic test must be provided)
  - Positive newborn screening with confirmatory second-tier testing (e.g., diagnostic 17-OHP concentration in newborn period, cosyntropin stimulation testing)
  - 21-hydroxylase deficiency with cosyntropin stimulation test supporting diagnosis
- Must use in combination with a glucocorticoid (e.g., hydrocortisone, prednisone, prednisolone, dexamethasone)
- Must have symptoms of classic CAH related to excess androgen production (e.g., growth acceleration, central precocious puberty, hirsutism, irregular menses, advanced bone maturation, testicular adrenal rest tumors, insulin resistance)
- Must be using a dose and duration of therapy consistent with the FDA-approved package labeling
- For crinecerfont (Crenessity) oral solution:
  - Must meet above criteria and ONE of the following:
    - Must weigh less than 20 kg
    - Must have documentation of the inability to swallow an oral solid dosage form due to age (< 7 years), medical condition (e.g., nasogastric tube), or other clinical rationale for why crinecefont (Crenessity) capsule cannot be used

If the above criteria are met, Crinecerfont (Crenessity) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation showing the following:

- Must have a reduced glucocorticoid dose from baseline
- Must have improved or stabilized symptoms of classic congenital adrenal hyperplasia (e.g., improved insulin resistance, reduction of hirsutism, regular menses)
- For crinecerfont (Crenessity) oral solution:
  - Must meet above criteria and ONE of the following:

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- Must weigh less than 20 kg
- Must have documentation of the inability to swallow an oral solid dosage form due to age (< 7 years), medical condition (e.g., nasogastric tube), or other clinical rationale for why crinecefont (Crenessity) capsule cannot be used

- **Crisaborole (Eucrisa™)** [365, 366, 367, 368, 369, 370, 371, 372, 373, 374, 375, 376, 377, 378, 379, 380, 381, 382, 383, 384]

Criteria:
- Must have a diagnosis of mild to moderate atopic dermatitis (eczema)
- Must be age 3 months or older
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to BOTH of the following:
  - A topical corticosteroid
    - NOTE: for members with atopic dermatitis on areas other than the face or intertriginous areas, a moderate to high potency topical corticosteroid is required
  - Pimecrolimus cream (Elidel) or tacrolimus ointment (Protopic)

If the above criteria are met, crisaborole (Eucrisa) is approved.

- **Crofelemer (Mytesi™)** [385]

Initial Authorization Criteria:
- Must be prescribed by an infectious disease specialist or HIV specialist
- Must be age 18 years or older
- Must have a diagnosis of non-infectious diarrhea
- Must be actively utilizing antiretroviral agents to treat HIV/AIDS
- Must have documentation of persistent loose stools despite regular use of two anti-diarrheal medications (e.g., loperamide, diphenoxylate, and bismuth subsalicylate)

If the above criteria are met, crofelemer (Mytesi) is approved for 1 month.

Reauthorization Criteria:
All prior authorization renewals are reviewed on a six month basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at six month intervals based upon the following:
- Documentation of a decrease in number of watery bowel movements and improvement in the condition based upon the prescriber's assessment while on treatment
- Must be continuously utilizing antiretroviral agents to treat HIV/AIDS

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- **Cyclosporine (Verkazia®)** [386, 387, 388]

Initial Authorization Criteria:

- Must be prescribed by an optometrist or ophthalmologist
- Must be at least 4 years of age
- Must have a diagnosis of vernal keratoconjunctivitis including active signs and symptoms of the condition (e.g., photophobia, tearing, burning, pain, or blurred vision)
- Must have an adequate trial to each of the following therapies with an inadequate response or significant side effects/toxicity or have a contraindication to these therapies:
    - Ophthalmic antihistamine (e.g., azelastine, olopatadine)
    - Ophthalmic mast cell stabilizer (e.g., cromolyn, lodoxamide [Alomide®])
- Must be prescribed a dose that is consistent with FDA-approved package labeling, nationally-recognized compendia, or peer-reviewed medical literature

If the above criteria are met, cyclosporine (Verkazia) is approved for 4 months.

Reauthorization Criteria:

All prior authorization renewals are reviewed every 4 months to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at 4-month intervals based upon documentation from the provider that the member continues to have active signs and symptoms of vernal keratoconjunctivitis and that the member's condition has improved based upon the prescriber's assessment while on therapy.

- **Cysteamine ophthalmic solution (Cystaran™, Cystadrops®)** [389, 390, 391, 392, 393, 394, 395]

Initial Authorization Criteria:

- Must be prescribed by a physician who specializes in the treatment of inherited metabolic disorders or in consultation with this specialist
- Must have a diagnosis of corneal cystine crystal accumulation due to cystinosis. Chart documentation of a clinical work-up to rule out other diagnoses and clinical rationale for the diagnosis and exclusion of other diagnoses must be provided. The diagnosis must be confirmed by all of the following scenarios (chart documentation is required):
    - Elevated baseline white blood cell (WBC) cystine levels > 2 nmol/1/2cystine/mg protein
    - Laboratory result confirming CTNS gene mutation
    - Clinical symptoms consistent with corneal cystine crystal accumulation including photophobia, corneal erosions, or keratopathies

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        - Ophthalmologic exam confirming the diagnosis of corneal cystine crystal accumulation due to cystinosis

If the above criteria are met, the requested product is approved for 3 months.

Reauthorization Criteria:
All authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of treatment. Authorization may be extended at one-year intervals based upon the following:
- Chart documentation that the member's condition has improved based upon the prescriber's assessment while on therapy
- Chart documentation of evaluation of compliance with therapy as
    - Cystaran is dosed every waking hour
    - Cystadrops is dosed 4 times a day during waking hours

- **Cystic Fibrosis Products** [396, 397, 398, 399, 400, 401, 402, 403, 404, 405, 406, 407]
    - Applies to: Ivacaftor (Kalydeco®), Lumacaftor/Ivacaftor (Orkambi®), Tezacaftor/Ivacaftor and Ivacaftor (Symdeko®), Elexacaftor/Tezacaftor/Ivacaftor and Ivacaftor (Trikafta®), and Deutivacaftor/Tezacaftor/Vanzacaftor (Alyftrek®)

Initial Authorization Criteria:

1. **For ivacaftor (Kalydeco)**
    - Must be prescribed by a pulmonologist or provider who specializes in the treatment of cystic fibrosis
    - Must be age 1 month or older
    - Must have a diagnosis of cystic fibrosis
    - Must have one cystic fibrosis transmembrane conductance regulator (CFTR) gene mutation responsive to ivacaftor (Kalydeco) per the prescribing information (a copy of the genetic test must be provided)
    - Must not use in combination with another CFTR modulator
    - For ivacaftor (Kalydeco) granule packets:
        - Must have documentation of the inability to swallow an oral solid dosage form due to age (< 7 years), medical condition (e.g. nasogastric tube), or other clinical rationale for why ivacaftor (Kalydeco) tablet cannot be used

2. **For lumacaftor/ivacaftor (Orkambi):**
    - Must be prescribed by a pulmonologist or provider who specializes in the treatment of cystic fibrosis
    - Must be age 1 year or older
    - Must have a diagnosis of cystic fibrosis

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- Must have homozygous F508del CFTR gene mutation (a copy of the genetic test must be provided)
- Must not use in combination with another CFTR modulator
- For lumacaftor/ivacaftor (Orkambi) granule packets:
  - Must have documentation of the inability to swallow an oral solid dosage form due to age (< 7 years), medical condition (e.g. nasogastric tube), or other clinical rationale for why lumacaftor/ivacaftor (Orkambi) tablet cannot be used

3. For tezacaftor/ivacaftor and ivacaftor (Symdeko)
- Must be prescribed by a pulmonologist or provider who specializes in the treatment of cystic fibrosis
- Must be age 6 years or older
- Must have a diagnosis of cystic fibrosis
- Must have ONE of the following:
  - Homozygous F508del CFTR gene mutation (a copy of the genetic test must be provided)
  - CFTR gene mutation responsive to tezacaftor/ivacaftor and ivacaftor (Symdeko) per the prescribing information (a copy of the genetic test must be provided)
- Must not use in combination with another CFTR modulator

4. For elexacaftor/tezacaftor/ivacaftor and ivacaftor (Trikafta)
- Must be prescribed by a pulmonologist or provider who specializes in the treatment of cystic fibrosis
- Must be age 2 years or older
- Must have a diagnosis of cystic fibrosis
- Must have ONE of the following:
  - F508del CTFR gene mutation (a copy of the genetic test must be provided)
  - CFTR gene mutation responsive to elexacaftor/tezacaftor/ivacaftor and ivacaftor (Trikafta) per the prescribing information (a copy of the genetic test must be provided)
- Must not use in combination with another CFTR modulator
- For elexacaftor/tezacaftor/ivacaftor and ivacaftor (Trikafta) granule packets:
  - Must have documentation of the inability to swallow an oral solid dosage form due to age (< 7 years), medical condition (e.g. nasogastric tube), or other clinical rationale for why elexacaftor/tezacaftor/ivacaftor and ivacaftor (Trikafta) tablet cannot be used

5. For deutivacaftor/tezacaftor/vanzacaftor (Alyftrek®)
- Must be prescribed by a pulmonologist or provider who specializes in the treatment of cystic fibrosis

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- Must be age 6 years or older
- Must have a diagnosis of cystic fibrosis
- Must have chart documentation of ONE of the following:
  - F508del CFTR gene mutation (a copy of the genetic test must be provided)
  - CFTR gene mutation responsive to deutivacaftor/tezacaftor/vanzacaftor (Alyftrek) per the prescribing information (a copy of the genetic test must be provided)
- Must have an adequate trial of elexacaftor/tezacaftor/ivacaftor and ivacaftor (Trikafta) with an inadequate response or significant side effect/toxicity to this therapy
  - NOTE: A trial of elexacaftor/tezacaftor/ivacaftor and ivacaftor (Trikafta) is not required if the member has a mutation that is not responsive to elexacaftor/tezacaftor/ivacaftor and ivacaftor (Trikafta) per the prescribing information
- Must not use in combination with another CFTR modulator

If the above criteria are met, the requested drug is approved for 1 year.

Reauthorization Criteria:
All prior authorizations renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or stabilized, or the member continues to benefit from therapy.

• Dabigatran Etexilate (Pradaxa®) Oral Pellet [408]

Initial Authorization Criteria:

1. **For treatment of venous thromboembolic event (VTE)**
   - Must be prescribed by or in consultation with a pediatric cardiologist or pediatric hematologist
   - Must be age 3 months to 12 years of age
   - Must be used for treatment of VTE for members who had been previously treated with a parenteral anticoagulant for at least 5 days
   - Must have documentation of the inability to swallow and oral solid dosage form due to age (< 7 years), medical condition (e.g., nasogastric tube), or other clinical rationale for why dabigatran capsule (Pradaxa) cannot be used

If the above criteria are met, dabigatran etexilate (Pradaxa) oral pellet is approved for 3 months.

2. **For risk reduction of recurrence of VTE**

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- Must be prescribed by or in consultation with a pediatric cardiologist or pediatric hematologist
- Must be age 3 months to 12 years of age
- Must be used for risk reduction of recurrence of VTE in member who have been previously treated
- Must have documentation of the inability to swallow an oral solid dosage form due to age (< 7 years), medical condition (e.g., nasogastric tube), or other clinical rationale for why dabigatran capsule (Pradaxa) cannot be used

If the above criteria are met, dabigatran etexilate (Pradaxa) oral pellet is approved for 1 year.

Reauthorization Criteria:

1. **For treatment of VTE**
   - All prior authorization renewals are reviewed to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at 3-month intervals based upon documentation showing the following:
     - The member's condition has improved or stabilized while on therapy
     - The inability to swallow an oral solid dosage form due to age (< 7 years), medical condition (e.g., nasogastric tube), or other clinical rationale for why dabigatran capsule (Pradaxa) cannot be used
     - Clinical rationale justifying why extended treatment is required (e.g., unprovoked VTE requiring extended treatment)

2. **For risk reduction of recurrence of VTE**
   - All prior authorization renewals are reviewed to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation showing the following:
     - The member's condition has improved or stabilized while on therapy
     - The inability to swallow an oral solid dosage form due to age (< 7 years), medical condition (e.g., nasogastric tube), or other clinical rationale for why dabigatran capsule (Pradaxa) cannot be used

• Dalfampridine (Ampyra™) [409, 410, 411, 412]

Initial Authorization Criteria:

- Must be prescribed by one of the following:
  - Neurologist
  - Physical Medicine and Rehabilitation physician in consultation with the member's treating Neurologist
- Must have a diagnosis of multiple sclerosis
- Must have chart documentation of the member's baseline motor disability/dysfunction (e.g., have an expanded disability status scale (EDSS)

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score ≤ 6, must be able to ambulate with assistance or must not be confined to a wheelchair)
- Must be age 18 years or older
- Must not be on concomitant therapy with any other forms of 4-aminopyridine
- Must not have a history of seizure
- Must not have moderate or severe renal impairment (CrCl ≤50mL/min)

If the above criteria are met, dalfampridine (Ampyra) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of treatment. Authorization may be extended at one-year intervals based upon chart documentation from the provider showing:
- The member's motor function improved while on therapy

• Danicopan (Voydeya®) [413, 414]

Initial Authorization Criteria:
- Must be prescribed by or in consultation with a hematologist, oncologist, immunologist or genetic specialist
- Must be age 18 years or older
- Must have a laboratory confirmed diagnosis of paroxysmal nocturnal hemoglobinuria (PNH) with extravascular hemolysis (EVH) showing BOTH of the following:
  - Must have detectable GPI-deficient hematopoietic clones (Type III PNH red blood cells [RBC]) via Flow Cytometry. Documentation of Flow Cytometry pathology report showing presence of PNH-type RBC must be provided.
  - Must have clinically significant extravascular hemolysis (EVH) defined by hemoglobin ≤ 9.5 g/dL with absolute reticulocyte count ≥ 120×10^9 /L. Chart documentation of laboratory testing must be provided
- Must be on a stable dose of a C5 complement inhibitor (e.g., an eculizumab product [Epysqli] or ravulizumab-cwvz [Ultomiris]) for PNH at an FDA approved dose for at least 6 months
- Must have an attestation from the provider that danicopan (Voydeya) is to be used in combination with an eculizumab product (Epysqli) or ravulizumab-cwvz (Ultomiris)
- Must have signs or symptoms attributed to PNH with EVH while receiving a C5 complement inhibitor (e.g., persistent anemia, fatigue, shortness of breath, history of red blood cell transfusion while on a C5 complement inhibitor). Chart documentation must be provided.
- Must provide documentation that the member is vaccinated for encapsulated bacteria (e.g., Streptococcus pneumoniae, Neisseria meningitidis, and

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Haemophilus influenzae type B [Hib]) according to current immunization guidelines at least 2 weeks prior to the administration of the first dose of danicopan (Voydeya)
    - NOTE: an exception may be made if the risks of delaying treatment outweigh the risks of developing infection per the provider
- Must not use in combination with pegcetocoplan (Empaveli), iptacopan (Fabhalta), or crovalimab-akkz (PiaSky)

If the above criteria are met, danicopan (Voydeya) is approved for 6 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon chart documentation showing the following:
- Must use in combination with an eculizumab product (Epysqli) or ravulizumab-cwvz (Ultomiris)
- Must have an improvement from baseline in hemoglobin and absolute reticulocyte count
- Must have an improvement from baseline in clinical symptoms of EVH
- For members who required blood transfusions at baseline:
    - Must have decreased requirement or no longer require blood transfusions
- Must not use in combination with pegcetocoplan (Empaveli), iptacopan (Fabhalta), or crovalimab-akkz (PiaSky)

• Deferasirox (Exjade®, Jadenu™) and Deferiprone (Ferriprox®) [415, 416, 417, 418, 419, 420, 421, 422, 423, 424, 425]

Initial Authorization Criteria:
For deferasirox (Exjade, Jadenu):
- Must be prescribed by a hematologist
- For treatment of chronic iron overload due to blood transfusions
    - Must be age 2 years or older
- For treatment of chronic iron overload due to non-transfusion dependent thalassemia syndromes
    - Must be age 10 years or older
    - Must have a liver iron (Fe) concentration (LIC) of at least 5mg Fe per gram dry weight
    - Must have a serum ferritin greater than 300mcg/L
- Must not have a serum creatinine greater than 2 times the age-appropriate upper limit of normal or creatinine clearance less than 40mL/min
- Must not have high-risk myelodysplastic syndromes (MDS)
- Must not have advanced malignancies
- Must have a platelet count $\geq 50 \times 10^9$/L

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- For deferasirox granules (Jadenu sprinkle)
    - Must have an adequate trial of deferasirox tablets for suspension (Exjade) or deferasirox tablets (Jadenu) with an inadequate response despite dose titration or significant side effects/toxicity

For deferiprone (Ferriprox):

- Must be prescribed by a hematologist
- Must have transfusional iron overload due to thalassemia syndromes, sickle cell disease or other anemias
- Must not be used for transfusional iron overload due to myelodysplastic syndrome (MDS) or Diamond Blackfan anemia
- Must have an adequate trial of deferasirox tablets for suspension (Exjade) or deferasirox tablets (Jadenu)
- Must have an assessment of ANC prior to starting deferiprone therapy

If the above criteria are met, the requested product is approved for one year.

Reauthorization Criteria:

All prior authorizations renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's disease has improved based upon the prescriber's assessment while on therapy.

- **Deflazacort Products** [426, 427, 428, 429, 430, 431, 432, 433, 434, 435, 436, 437, 438, 439, 440, 441, 442, 443,444,445]

Applies to: Deflazacort tablet (Emflaza®, Jaythari™, Kymbee™) and Deflazacort suspension (Emflaza®, Jaythari, Pyquvi™)

Initial Authorization Criteria:

- Must be prescribed by a neurologist (specifically a neuromuscular specialist) who specializes in the treatment of muscular dystrophy
- Must be age-appropriate according to FDA-approved package labeling, nationally recognized compendia, or peer-reviewed medical literature
- Must have a diagnosis of Duchenne muscular dystrophy (DMD) with confirmatory genetic testing demonstrating an out-of-frame mutation [e.g. frameshift (deletion or duplication) mutation or a premature stop codon mutation] of the DMD gene (a copy of the genetic testing must be provided)
- Must have chart documentation of an adequate trial of prednisone including ONE of the following:
    - Documentation of significant side effect(s)/toxicity impacting quality of life
    - Documentation that target doses of prednisone were ineffective
- Must not be using in combination with prednisone or vamorolone (Agamree)

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If the above criteria are met, the requested product is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or stabilized, or the member continues to benefit from therapy.

- **Delgocitinib cream (Anzupgo®)** [446, 447, 448, 449]

Initial Authorization Criteria:
- Must be prescribed by a dermatologist, an allergist, or an immunologist
- Must be age 18 years or older
- Must have a diagnosis of moderate to severe chronic hand eczema (CHE) with symptoms that persist for more than 3 months or recur 2 or more times within a 12-month period
- Must use only on hands and wrists
- Must have CHE symptoms that persist despite the use of non-pharmacologic measures including ALL of the following:
  - Avoidance of contact with identified allergens or irritants
  - Use of protective gloves when performing work with hand contact to water and/or hazardous substances
  - Use of emollients, barrier creams, and/or moisturizers
- Must use in conjunction with the non-pharmacologic measures outlined above
- Must have an adequate trial with an inadequate response (despite adherence based on prescriber attestation and/or claims history) or significant side effect/toxicity or have a contraindication to ALL of the following:
  - Two moderate-to-high potency topical corticosteroids
  - Tacrolimus 0.1% ointment (Protopic)
- For members with atopic dermatitis on the hands, must have an adequate trial of ruxolitinib cream (Opzelura) with an inadequate response (despite adherence based on prescriber attestation and/or claims history) or significant side effect/toxicity or have a contraindication to this therapy
- Must not use in combination with a JAK inhibitor or a potent immunosuppressant (e.g., azathioprine, cyclosporine)

If the above criteria are met, delgocitinib cream (Anzupgo) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon the following:

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- Improvement in the member's condition while on therapy
- Must use in conjunction with non-pharmacologic measures, including ALL of the following:
  - Avoidance of contact with identified allergens or irritants
  - Use of protective gloves when performing work with hand contact to water and/or hazardous substances
  - Use of emollients, barrier creams, and/or moisturizers

- **Desmopressin Acetate Nasal Spray (Noctiva®) and Desmopressin Acetate Sublingual Tablet (Nocdurna®)** [450, 451, 452, 453]

Initial Authorization Criteria:

- Must have a diagnosis of nocturnal polyuria, defined as night-time urine production exceeding one-third of the 24-hour urine production confirmed with a 24-hour urine frequency/volume chart. Documentation of the frequency/volume chart must be provided.
- Must have a 6 month history of at least 2 nocturia episodes per night on average
- Must have documentation of a clinical work-up, including clinical rationale for the nocturnal polyuria diagnosis and exclusion of other possible conditions that could result in nighttime waking to urinate (e.g., diuretic use, overactive bladder, benign prostatic hyperplasia, low bladder capacity, heart failure, unstable diabetes, sleep disorders)
- Must have chart documentation of an attempt to limit fluid intake prior to bedtime
- Must not have any of the following contraindications:
  - Hyponatremia or history of hyponatremia
  - Polydipsia
  - Concomitant use with loop diuretics or inhaled or systemic glucocorticoids
  - Renal impairment with an estimated glomerular filtration rate (eGFR) <50ml/min/1.73m2
  - Known or suspected syndrome of inappropriate antidiuretic hormone (SIADH) secretion
  - During illnesses that can cause fluid or electrolyte imbalance, such as gastroenteritis, salt-wasting nephropathies, or systemic infection
  - Congestive heart failure (New York Heart Association Class II to IV)
  - Uncontrolled hypertension
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to both of the following:
  - Oral desmopressin tablet

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        - Diuretic (e.g., bumetanide, furosemide)

In addition to the above, the below criteria applies per drug:
Desmopressin acetate nasal spray (Noctiva)
- Must be age 50 years or older
- Must not have the following additional contraindication:
    - Primary nocturnal enuresis

If the above criteria are met, desmopressin acetate nasal spray (Noctiva) is approved for 3 months.

Desmopressin acetate sublingual tablet (Nocdurna)
- Must be age 18 or older

If the above criteria are met, desmopressin acetate sublingual tablet (Nocdurna) is approved for 3 months.
- Desmopressin acetate sublingual tablet 27.7 mcg is approved for females only
- Desmopressin acetate sublingual tablet 55.3 mcg is approved for males only

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of treatment. Authorization may be extended at 1-year intervals based upon chart documentation from the provider indicating a decrease in nocturic episodes based upon the prescriber's assessment.

• Deuruxolitinib (Leqselvi®) [454, 455, 456, 457, 458, 459]

Initial Authorization Criteria:
- Must be prescribed by a dermatologist
- Must be age 18 years or older
- Must have a diagnosis of severe alopecia areata confirmed by BOTH of the following:
    - At least 50% scalp hair loss or complete loss of eyelashes with scalp hair loss
    - Current episode of alopecia areata lasting at least 6 months
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
    - Potent topical corticosteroids, intralesional steroids, or oral corticosteroids (e.g., triamcinolone, oral prednisone)
    - Systemic oral immunosuppressant therapy (e.g., methotrexate, cyclosporine)
    - NOTE: An exception to the requirement for a trial of a conventional systemic therapy (e.g., corticosteroid, oral immunosuppressant) can be made if the member has already had a 3 month trial or previous

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intolerance to an FDA approved therapy for alopecia areata (e.g., ritlecitinib [Litfulo], baricitinib [Olumiant], deuruxolitinib [Leqselvi])
- Must provide documentation to support member does not have hair loss due to androgenetic alopecia (male and female pattern hair loss), chemotherapy-induced hair loss, or other causes of hair loss other than alopecia areata (e.g., trichotillomania, telogen effluvium, and systemic lupus erythematosus)
- Must be using a dose and duration of therapy consistent with the FDA-approved package labeling
- Must not use in combination with an oral targeted anti-inflammatory agent, biologic agent, or potent immunosuppressant (e.g. azathioprine, cyclosporine)

If the above criteria are met, deuruxolitinib (Leqselvi) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended one year intervals based upon documentation showing ONE of the following:
- For members without complete eyelash or hair regrowth of at least 80% scalp coverage:
  - Must have increased hair growth from baseline
  - Must be using a dose and duration of therapy consistent with the FDA-approved package labeling
- For members with complete eyelash or hair regrowth of at least 80% scalp coverage:
  - Must provide clinical rationale to continue treatment, including why topical treatment could not be used to maintain hair growth
  - Must be using a dose and duration of therapy consistent with the FDA-approved package labeling
- For members being re-treated and had complete hair regrowth during a previous episode of severe alopecia areata
  - Must have documentation of prior treatment response
  - Must have documentation of a current episode of alopecia areata, including the percentage of hair loss or eyelash loss
  - Must be using a dose and duration of therapy consistent with the FDA-approved package labeling

- **Deutetrabenazine (Austedo®, Austedo XR®)** [460, 461, 462, 463, 464, 465, 466, 467, 468, 469, 470]

Initial Authorization Criteria:
1. **For Chorea associated with Huntington's disease**

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- Must be prescribed by a neurologist who specializes in the treatment of movement disorders
- Must be age 18 years or older
- Must have a diagnosis of chorea associated with Huntington's disease confirmed by ONE of the following:
  - Huntington Disease Mutation Analysis showing an expanded CAG repeat (≥ 36) in the huntington gene (HTT) (also known as HD gene). Chart documentation must be provided.
  - A positive family history of Huntington's disease
- Must provide chart documentation of a clinical work up for Huntington's disease showing ONE of the following symptoms:
  - Motor (e.g., finger tapping, rigidity)
  - Oculomotor (e.g., slow or unusual eye movements)
  - Bulbar (e.g., dysarthria, dysphagia)
  - Affective (e.g., depression)
  - Cognitive (e.g., lack of impulse control/outbursts, trouble prioritizing or focusing on tasks, slowness of processing thoughts or finding words)
- Must have an evaluation by a neurologist or behavioral health provider prior to the use of deutetrabenazine (Austedo, Austedo XR) if the member has a history of depression or suicidal thoughts or behaviors. These members must also be followed concurrently with a behavioral health provider while on deutetrabenazine (Austedo, Austedo XR). Chart documentation must be provided.
- Must not have ANY of the following:
  - Untreated/inadequately treated depression or suicidal ideations
  - Concomitant use with a monoamine oxidase inhibitor (MAOI), reserpine, tetrabenazine (Xenazine), or valbenazine (Ingrezza)

2. For Tardive Dyskinesia

- Must be prescribed by a neurologist who specializes in the treatment of movement disorders or a psychiatrist
- Must be age 18 years or older
- Must have a diagnosis of moderate to severe tardive dyskinesia confirmed by chart documentation of ALL of the following:
  - History of dopamine receptor blocking drug use (e.g., antipsychotic, metoclopramide, prochlorperazine)
  - Involuntary movements for at least 3 months
  - Must provide chart documentation of ONE of the following:
    - Baseline Abnormal Involuntary Movement Scale (AIMS) score of ≥ 10
    - Tardive dyskinesia that impacts the functional ability to perform essential activities of daily living (e.g., swallowing,

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writing, walking), which leads to a significant decrease in quality of life
- Must have been evaluated for dose reduction, withdrawal, or switch of the dopamine receptor blocking agent causing the tardive dyskinesia if currently taking deutetrabenazine (Austedo, Austedo XR)
- Must not have ANY of the following:
  - Comorbid involuntary movement disorder (e.g., parkinsonism, akathisia, truncal dystonia)
  - Concomitant use with a monoamine oxidase inhibitor (MAOI), reserpine, tetrabenazine (Xenazine), or valbenazine (Ingrezza)

If the above criteria are met, deutetrabenazine (Austedo) is approved for 3 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon the following documentation from the provider:
For chorea associated with Huntington's Disease:
- The member's condition has improved while on therapy or the member continues to benefit from therapy.
- For members with a diagnosis of depression and/or suicidal thoughts or behaviors or for members with a current treatment for depression and/or suicidal thoughts or behaviors, chart documentation must be provided to show current evaluation by a neurologist or behavioral health provider.

For tardive dyskinesia:
- The member remains a candidate for treatment based upon the provider's assessment (e.g., AIMS score, functional assessment) and continues to benefit from therapy.
- An evaluation to taper the dose or to discontinue therapy with deutetrabenazine (Austedo, Austedo XR). Must provide clinical rationale for why unable to taper or discontinue therapy.

- **Dextromethorphan and bupropion (Auvelity™)** [471, 472, 473, 474, 475]

Initial Authorization Criteria:
- Must be prescribed by or in consultation with a psychiatrist
- Must be age 18 years or older
- Must have a diagnosis of major depressive disorder (MDD)
- Must have chart documentation of an adequate trial, of at least 4 weeks, with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
  - Monotherapy medication trials:
    - Four generic antidepressants (e.g., SSRI, SNRI, vilazodone)

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        - One of the following brand antidepressants:
            - Vortioxetine (Trintellix™)
            - Levomilnacipran (Fetzima®)
    - Combination medication trials:
        - One generic SSRI or SNRI antidepressant in combination with bupropion
            - Must not have failed this medication trial due to adverse effects caused by bupropion
        - Two different trials with augmenting therapies (e.g., buspirone, TCA, antipsychotic, lithium or liothyronine) in combination with a generic SSRI or SNRI antidepressant
- Must not have ANY of the following:
    - Seizure disorder
    - Current or prior diagnosis of bulimia or anorexia nervosa
    - Abrupt discontinuation of alcohol, benzodiazepines, barbiturates, and antiepileptic drugs
    - Concomitant therapy with an MAOI or within 14 days of stopping an MAOI (including reversible MAOIs such as linezolid or intravenous methylene blue)

If the above criteria are met, dextromethorphan and bupropion (Auvelity) is approved.

- **Dextromethorphan/Quinidine (Nuedexta®)** [476, 477, 478, 479, 480, 481, 482]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with a neurologist
- Must have a diagnosis of pseudobulbar affect (PBA). Must supply chart documentation for the following to support the diagnosis of PBA:
    - Documentation that patient is experiencing involuntary outbursts of laughing and/or crying that are incongruous or disproportionate to the patient's emotional state
    - Documentation of a clinical work-up, including clinical rationale for the PBA diagnosis and exclusion of other possible conditions that could result in emotional lability (e.g. depression, bipolar disorder, schizophrenia, epilepsy)
- Must have an underlying neurological disorder such as amyotrophic lateral sclerosis (ALS), multiple sclerosis (MS), Alzheimer's and related diseases, Stroke, Traumatic Brain Injury (TBI), or Parkinsonian Syndromes
- Must not be on concomitant therapy with any of the following medications:
    - Quinidine, quinine, or mefloquine
    - Monoamine oxidase inhibitors (MAOIs) within the past 14 days

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        - Drugs that both prolong the QT interval and are metabolized by CYP2D6, such as thioridazine and pimozide
    - Must not have a history of hypersensitivity to quinidine, quinine, mefloquine, or dextromethorphan
    - Must not have a prolonged QT interval, congenital long QT syndrome or a history suggestive of torsades de pointes, heart failure, complete AV (atrioventricular) block without an implanted pacemaker, or be at high risk of complete AV block

If the above criteria are met, dextromethorphan/quinidine (Nuedexta) is approved for 3 months.

Reauthorization Criteria:
All prior authorizations renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon chart documentation from the provider showing decrease in number of laughing and/or crying episodes with dextromethorphan/quinidine (Nuedexta) therapy.

- **Diazoxide choline extended-release (Vykat XR®)** [483, 484, 485, 486, 487, 488]

Initial Authorization Criteria:
- Must be prescribed by an endocrinologist
- Must be age 4 years or older
- Must have a diagnosis of Prader-Willi Syndrome confirmed by abnormal DNA methylation of chromosome 15q11.2Q13 (a copy of the genetic test must be provided)
- Must have hyperphagia related to PWS with behaviors that significantly impact quality of life (e.g., food seeking behavior, food related aggression or manipulation, food perseveration) Chart documentation is required.
- Must be using a dose and duration of therapy consistent with the FDA-approved package labeling. Must provide documentation of weight within 30 days to ensure appropriate dose.
- Must have an attestation from the prescriber that diazoxide choline (Vykat XR) is used in conjunction with healthy lifestyle modifications (e.g., behavior therapy, coping mechanisms, locked food storage, restrictive food environment, diet, physical activity)

If the above criteria are met, diazoxide choline extended- release (Vykat XR) is approved for 6 months.

Reauthorization Criteria:

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All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation showing the following:

- Must have documentation from the provider that the member's hyperphagia behaviors have improved or stabilized (e.g., decreased food related aggression or manipulation, lessened food preoccupation that interferes with daily activities), or the member continues to benefit from therapy based on the prescriber's assessment
- Must have an attestation from the prescriber diazoxide choline (Vykat XR) is used in conjunction with healthy lifestyle modifications (e.g., behavior therapy, coping mechanisms, restrictive food environment diet, physical activity)
- Must be using a dose and duration of therapy consistent with the FDA-approved package labeling. Must provide documentation of weight within 30 days.

• Dichlorphenamide (Keveyis®, Ormalvi™) [489, 490, 491]

Initial Authorization Criteria:

1. **For dichlorphenamide (Ormalvi™) for Open Choice and Value Choice:**
   - Must meet below criteria
   - Must have an adequate trial of dichlorphenamide (Keveyis) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
   - Must provide clinical rationale for use of dichlorphenamide (Ormalvi) instead of dichlorphenamide (Keveyis). Clinical rationale must include why dichlorphenamide (Ormalvi) would not be expected to produce the same clinical results as dichlorphenamide (Keveyis) given that they are the same chemical entity
2. **For dichlorphenamide (Keveyis):**
   - Must be prescribed by a neurologist
   - Must have a diagnosis of primary hyperkalemic periodic paralysis, primary hypokalemic periodic paralysis, or related variants
   - For primary hypokalemic periodic paralysis:
     - Must have documentation confirming diagnosis, defined as one of the following scenarios:
       - Two or more attacks of muscle weakness with documented serum K <3.5mEq/L, OR
       - One attack of muscle weakness in the member with documented serum potassium <3.5mEq/L and one attack of weakness in a relative with a history of the condition, OR
       - Three of the following clinical/laboratory features:
         - Onset of symptoms in the first or second decade of life

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        - Duration of attack (muscle weakness involving one or more limbs) longer than two hours
        - The presence of triggers (previous carbohydrate rich meal, symptom onset during rest after exercise or during stressful situations) for attacks
        - Improvement in symptoms with potassium intake
        - A family history of the condition or genetically confirmed skeletal calcium or sodium channel mutation
        - Positive long exercise test
    - Must have chart documentation excluding other causes of hypokalemia (renal, adrenal, thyroid dysfunction; renal tubular acidosis; diuretic and laxative abuse)
    - Must have an adequate trial and failure or an inadequate response of acetazolamide unless has one of the following mutations in SCNA4:
      - c.2014C>G
      - c.2014C>A
    - Must be using a potassium supplement
    - Must have been counseled on appropriate dietary restrictions (high potassium intake, low sodium intake, and low carbohydrate intake)
- For hyperkalemic periodic paralysis:
    - Must have documentation confirming diagnosis:
      - For diagnosis confirmed by genetic testing, must have both of the following:
        - A family history of the condition or genetically confirmed skeletal sodium channel mutation associated with hyperkalemic periodic paralysis, AND
        - A history of at least two attacks of flaccid limb weakness (which may also include weakness of the muscles of the eyes, throat, and trunk) or 1 attack with a family history of attacks of hyperkalemic periodic paralysis OR
      - For diagnosis made based on clinical presentation, must have all of the following:
        - A history of at least two attacks of flaccid limb weakness (which may also include weakness of the muscles of the eyes, throat, and trunk) or 1 attack with a family history of attacks of hyperkalemic periodic paralysis
        - Serum potassium >5mEq/L or an increase of serum potassium concentration of at least 1.5 mEq/L during an attack of weakness and/or onset/worsening of an attack as a result of oral potassium intake

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        - Presence of myotonia OR any 3 of the following clinical features:
            - Typical attack duration less than 2 hours
            - Onset before 30 years.
            - Positive long exercise test (>40% decrement in CMAP)
            - Typical external triggers (rest after exercise, potassium load, fasting)
    - Must have chart documentation of normal serum potassium concentration and muscle strength between attacks
    - Must have chart documentation of ECG recording for the exclusion of a long QT and ventricular arrhythmias
    - Must not have either of the following:
        - Secondary hyperkalemic periodic paralysis due to ingestion of potassium or of a potassium sparing diuretic
        - Paramyotonia (i.e. muscle stiffness that is worsening after exercise or cold-induced)
    - Must have documentation of exclusion of other hereditary forms of hyperkalemia (i.e., Andersen-Tawil syndrome) and acquired forms of hyperkalemia (drug abuse, renal and adrenal dysfunction)
    - Must have an adequate trial of acetazolamide with an inadequate response
    - Must have been counseled on appropriate dietary restrictions (frequent meals high in carbohydrate)
- Must not have any of the following:
    - Concomitant use with high dose aspirin
    - Severe pulmonary disease, limiting compensation to metabolic acidosis that may be caused by dichlorphenamide (Keveyis)
    - Hepatic encephalopathy

If the above criteria are met, dichlorphenamide (Keveyis) is approved for 2 months.

Reauthorization Criteria:
All prior authorization requests are reviewed on an annual basis to determine the Medical Necessity for continuation of treatment. Authorizations may be extended at one-year intervals based upon documentation from the prescriber that the member's condition has improved based upon the prescriber's assessment while on therapy.

- **Dihydroergotamine Products**[492, 493, 494, 495, 496]
    - Applies to: dihydroergotamine ampule (D.H.E. 45®) and dihydroergotamine nasal spray (Migranal®, Trudhesa™)

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- Applies to dihydroergotamine autoinjector (Brekiya®) (Value choice and Open Choice formularies only)

Criteria:

- Must be 18 years of age or older
- Must have a diagnosis of migraine or cluster headache
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
  - TWO different generic triptans (e.g. naratriptan, rizatriptan, sumatriptan) of at least two different dosage formulations (e.g. injection, nasal spray, oral tablet, orally disintegrating tablet)
  - Rimegepant (Nurtec ODT)
  - Ubrogepant (Ubrelvy)
- Must not have ANY of the following:
  - Diagnosis of hemiplegic or basilar migraine
  - Ischemic heart disease (angina pectoris, history of myocardial infarction, or documented silent ischema) or coronary artery vasospasm (including Prinzmetal's variant angina)
  - Uncontrolled hypertension
  - Use within 24 hours of taking 5-HT_1 agonists (e.g., sumatriptan), ergotamine-containing or ergot-type medications, or methysergide
  - Peripheral arterial disease
  - Severe hepatic impairment
  - Severe renal impairment
  - Sepsis
  - Hypersensitivity to ergot alkaloids
- Must not be on concomitant treatment with any of the following:
  - A strong CYP3A4 inhibitor (e.g., protease inhibitors, macrolide antibiotics)
  - Peripheral and central vasoconstrictors
- Must not use following vascular surgery

If the above criteria are met, the requested drug is approved.

For *Open Choice* and *Value Choice* only:

- **For dihydroergotamine autoinjector (Brekiya®):**
  - Must meet the above criteria for the dihydroergotamine products
  - Must have an adequate trial of dihydroergotamine ampule (D.H.E. 45) and dihydroergotamine nasal spray (Migranal, Trudhesa) with inadequate response or significant side effects/toxicity or have a contraindication to this therapy
  - Must provide clinical rationale for use of the dihydroergotamine autoinjector (Brekiya) instead of dihydroergotamine ampule (D.H.E. 45) or dihydroergotamine nasal spray (Migranal,

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Trudhesa). Clinical rationale must include why dihydroergotamine autoinjector (Brekiya) would not be expected to produce the same clinical results as dihydroergotamine ampule (D.H.E. 45) or dihydroergotamine nasal spray (Migranal, Trudhesa) given that they are the same chemical entity.

If the above criteria are met, dihydroergotamine autoinjector (Brekiya) is approved. (Applies to Value Choice and Open Choice Formularies only)

• Diroximel fumarate (Vumerity®) [497, 498, 499]

Initial Authorization Criteria:

- Must be prescribed by a neurologist
- Must be age 18 years or older
- Must have a diagnosis of relapsing form of multiple sclerosis, to include clinically isolated syndrome, relapsing-remitting disease, and active secondary progressive disease
- Must have an adequate trial with an inadequate response, significant side effect/toxicity or contraindication to ONE of the following:
    - Glatiramer acetate/Glatopa® (Copaxone®)
    - Dimethyl fumarate (Tecfidera®)
    - Fingolimod (Gilenya®)
    - Teriflunomide (Aubagio®)
- Must not be on concomitant therapy with a disease modifying drug for multiple sclerosis

If the above criteria are met, diroximel fumarate (Vumerity) is approved for 1 year.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upondocumentation from the provider that the member's condition has stabilized or improved based while on therapy.

• Dronabinol (Marinol®) [500]

Initial Authorization Criteria:

- Must be age 18 and older
- Must have one of the following diagnoses:
    - Anorexia associated with weight loss with AIDS
    - Nausea and vomiting associated with chemotherapy
        - Must be on concurrent chemotherapy
        - Must have an adequate trial and failure or an inadequate response or intolerance of BOTH of the following:

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        - $5HT_3$ antagonist (e.g., ondansetron, granisetron) AND
        - One conventional antiemetic treatment (e.g., prochlorperazine, promethazine)
    - Must not have a history of a hypersensitivity reaction to dronabinol or sesame oil

If the above criteria are met, dronabinol (Marinol), is approved.

• Dornase Alfa (Pulmozyme®) [501, 502]

Initial Authorization Criteria:

- Must have a diagnosis of cystic fibrosis

If the above criteria are met, dornase alfa (Pulmozyme) is approved for 1 year.

Reauthorization Criteria:

All prior authorizations renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or stabilized, or the member continues to benefit from therapy.

• Doxecitine and doxribtimine (Kygevvi®)[503]

Initial Authorization Criteria:

- Must be prescribed by a neurologist, geneticist, or a provider who specializes in the treatment of mitochondrial myopathies
- Must have chart documentation of a diagnosis of thymidine kinase 2 deficiency (TK2d) confirmed by biallelic pathogenic variants in the *thymidine kinase 2 (TK2)* gene (a copy of the genetic test must be provided)
- Must have chart documentation of TK2d related symptoms (e.g., progressive muscle weakness, respiratory insufficiency, dysarthria, dysphagia) with age of symptom onset on or before 12 years
- Must use a dose and duration of therapy consistent with the FDA-approved package labeling

If the above criteria are met, doxecitine and doxribtimine (Kygevvi) is approved for 1 year.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon chart documentation showing the following:

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- The member's condition has improved or stabilized while on therapy or that the member remains a candidate for treatment with doxecitine and doxribtimine (Kygevvi) based upon the prescriber's assessment while on therapy
- Must use a dose and duration of therapy consistent with the FDA-approved package labeling

- **Droxidopa (Northera™)** 504, 505, 506, 507, 508, 509, 510, 511, 512, 513, 514, 515, 516

Initial Authorization Criteria:

- Must be prescribed by a cardiologist or a neurologist
- Must be age 18 years or older
- Must have a diagnosis of symptomatic neurogenic hypotension (NOH) caused by ONE of the following conditions:
  - Primary autonomic failure (Parkinson's disease, multiple system atrophy or pure autonomic failure)
  - Dopamine beta-hydroxylase deficiency
  - Non-diabetic autonomic neuropathy
- Must provide chart documentation demonstrating how the diagnosis was made, including blood pressure readings showing a systolic blood pressure decrease of at least 20 mmHg or a diastolic blood pressure decrease of at least 10 mmHg within 3 minutes of standing
- Must provide documentation that the member is symptomatic as a result of low-pressure readings, including documentation to support that the member is experiencing at least ONE of the following symptoms:
  - Dizziness
  - Lightheadedness
  - Feeling faint or feeling like member might black out
- Must provide chart documentation indicating an attempt to discontinue or reduce the dose of medications which can cause orthostatic hypotension, such as:
  - Anti-hypertensive agents
  - Nitrates
  - α-1 adrenergic blockers (i.e. terazosin, prazosin)
  - Antiparkinsonian agents (i.e. levodopa, bromocriptine, ropinirole, pramipexole)
  - Diuretics
  - Monoamine oxidase inhibitors
  - Narcotics/tranquilizers/sedatives
  - Medications used for erectile dysfunction
  - Tricyclic antidepressants

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- Must provide chart documentation demonstrating that the member has tried non-pharmacologic interventions to treat their condition such as:
    - Elevation of the bed by 5-20 degrees
    - Use of compression stockings
    - Increase of salt and water intake
    - Avoidance of precipitating factors such as: arising too quickly from supine to sitting or standing, standing motionless, large meals, alcohol consumption, heat, hot baths, hot environment
    - Performance of physical counter-maneuvers such as: crossing legs, stooping, squatting, and tensing muscles
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to BOTH of the following:
    - Midodrine
    - Fludrocortisone
    - NOTE: most patients respond to these medications within 1 week

If the above criteria are met droxidopa (Northera) is approved for 1 month.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one-year intervals based upon the following:

- Must have documentation from the provider that the member's condition has improved while on therapy, as evidenced by symptom improvement (e.g., dizziness, lightheadedness, feeling faint or feeling like member might black out)
- Must have chart documentation from the provider to support that the member continues to be monitored for adverse effects (e.g., supine hypertension) and that additional drug therapies have not been added to the member's medication regimen that would potentiate the risk of supine hypertension

• Edavarone oral suspension (Radicava ORS®) [517]

- Requests for edavarone intravenous solution (Radicava® IV) please refer to policy RX.PA.406 - Commercial, Exchange Medical Prior Authorization Policies

Initial Authorization Criteria:

- Must be prescribed by a neurologist
- Must have a diagnosis of definite or probable amyotrophic lateral sclerosis (ALS)
- Must have disease duration of less than 2 years
- Must have a FVC ≥ 80%. Chart documentation of FVC is required

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- Must not have a tracheostomy, have an artificial respirator with intubation, or be receiving tube feedings
- Must not use in combination with edavarone intravenous (Radicava IV)

If the above criteria are met, edavarone oral suspension (Radicava ORS) is approved for 6 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member is not dependent on invasive ventilation and remains a candidate for treatment based upon the prescriber's assessment while on therapy.

• Efgartigimod-alfa and hyaluronidase prefilled syringe (Vyvgart Hytrulo®) [518, 519, 520, 521]

- Requests for efgartigimod-alfa-fcab (Vyvgart®) and efgartigimod-alfa and hyaluronidase vials (Vyvgart Hytrulo®) please refer to policy RX.PA.406 Commercial, Exchange Prior Authorization Policies

Initial Authorization Criteria:

1. **For Generalized Myasthenia Gravis**
   - Must be prescribed by a neurologist
   - Must be age 18 years or older
   - Must have a diagnosis of anti-AchR positive generalized myasthenia gravis. Chart documentation of a positive serological test of anti-acetylcholine receptor antibodies must be provided
   - Must have a history of myasthenia gravis with symptoms that impair the ability to perform daily functions and essential activities of daily living (e.g., difficulty chewing, swallowing, or breathing). Chart documentation must be provided.
   - Must provide current weight and have a requested dose and frequency that falls within the recommended dosing guidelines from the manufacturer
   - Must have an adequate trial of corticosteroid with adequate response, significant side effects/toxicity or have a contraindication to therapy
   - Must have an adequate trial of at least 3 months with an inadequate response of ONE of the following immunosuppressants, OR have a significant side effects/toxicities or contraindication of at least TWO of the following:
     - Azathioprine
     - Mycofenolate mofetil
     - Cyclosporine
     - Tacrolimus

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- Must have an adequate trial of at least 6 months with efgartigimod-alfa-fcab (Vyvgart) showing clinical benefit. Chart documentation of response to therapy must be provided.
- Must provide clinical rationale for use of efgartigimod alfa and hyaluronidase pre-filled syringes (Vyvgart Hytrulo) instead of efgartigimod alfa-fcab (Vyvgart). Clinical rationale must include why efgartigimod alfa-fcab (Vyvgart) would not be expected to produce the same clinical results as efgartigimod alfa and hyaluronidase pre-filled syringes (Vyvgart Hytrulo)
- Must be on concomitant therapy with conventional therapy (e.g., AChE inhibitor [pyridostigmine], steroids, non-steroidal immunosuppressive therapies)
- Must not use in combination with ANY of the following:
  - An eculizumab product
  - Ravulizumab-cwvz (Ultomiris)
  - Rozanolizumab-nilo (Rystiggo)
  - Zilucoplan (Zilbrysq)
  - Nipocalimab-aahu (Imaavy)
  - Inebilizumab-cdon (Uplizna)

2. For chronic inflammatory demyelinating polyneuropathy (CIDP)
- Must be prescribed by a neurologist
- Must be age 18 years or older
- Must have a diagnosis of chronic inflammatory demyelinating polyneuropathy (CIDP) confirmed by electrodiagnostic testing (e.g., an electromyography [EMG] report). Chart documentation must be provided.
- Must has significant disability in upper and lower limb function as defined as having arm disability of inflammatory neuropathy cause and treatment (INCAT) grade 2 or higher OR leg disability of INCAT grade 1 or higher
- Must have an adequate trial of immune globulin (intravenous or subcutaneous) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy

If the above criteria are met, efgartigimod alfa and hyaluronidase pre-filled syringes (Vyvgart Hytrulo) is approved based on the diagnosis as follows:
- Myasthenia Gravis: 6 months for 3 cycles of treatment
- CIDP: 6 months.

Reauthorization Criteria:
All prior authorizations renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy.
- For Myasthenia Gravis:
  - Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has

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improved while on therapy (e.g., improvement in daily function or activities of daily living)
- For CIDP:
  - Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved while on therapy (e.g., improvement or stability in INCAT disability scores)

• Elafibranor (Iqirvo®) [522, 523]

Initial Authorization Criteria:
- Must be prescribed by or in consultation with a gastroenterologist, hepatologist, or liver transplant specialist
- Must be age 18 years or older
- Must have a diagnosis of primary biliary cholangitis (PBC) confirmed by chart documentation of TWO of the following:
  - Elevated alkaline phosphatase (ALP) above the upper limit of normal (ULN) for at least 6 months based on the reference range provided by the laboratory
  - Positive anti-mitochondrial antibodies (AMA) or other PBC-specific auto-antibodies, if AMA is negative
  - Liver biopsy consistent with PBC
- Must have an adequate trial of at least 12 months with ursodiol at a dose of 13-15 mg/kg/day with an inadequate response defined as ALP 1.5-times the ULN or significant side effect/toxicity or have a contraindication to this therapy
- Must be used in combination with ursodiol unless there is a clinical contraindication or intolerance to ursodiol
- Must not be used in combination with obeticholic acid (Ocaliva) or seladelpar (Livdelzi)
- Must not have evidence of decompensated cirrhosis (e.g., ascites, variceal bleeding, hepatic encephalopathy)

If the above criteria are met, elafibranor (Iqirvo) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon chart documentation showing the following:
- The member's condition has improved while on therapy
- Must not have evidence of decompensated cirrhosis (e.g., ascites, variceal bleeding, hepatic encephalopathy)

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- **Elagolix (Orilissa™)** [524]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with a gynecologist
- Must have a diagnosis of endometriosis
  - Must be confirmed by laparoscopy
  - If the diagnosis is not confirmed by surgery, then documentation of an adequate work-up and the clinical rationale for the diagnosis must be provided.
- Must be using a dose and duration of therapy consistent with the FDA-approved package labeling
- Must have tried hormonal contraceptives and/or progestins
- Must have tried non-steroidal anti-inflammatory drugs (NSAIDs)
- Must have a behavioral health assessment prior to the use of elagolix (Orilissa) if the member has a history of depression and/or suicidal thoughts or behaviors, or if the member currently is undergoing treatment for depression and/or suicidal thoughts or behavior. These members must also be followed while on elagolix (Orilissa).

If the above criteria are met, elagolix (Orilissa) is approved for a duration based upon strength as follows:

- For elagolix (Orilissa) 150mg: 2 years per lifetime
- For elagolix (Orilissa) 200mg: 6 months per lifetime

Additionally, cumulative use of any of the following for more than 2 years will not be approved:

- Relugolix/estradiol/norethindrone acetate (Myfembree)
- Elagolix/estradiol/norethindrone acetate and elagolix (Oriahnn)
- Elagolix (Orilissa) 150mg
  - NOTE: Elagolix (Orilissa) 200mg is only approved for 6 months per lifetime. Treatment with another GnRH antagonist (e.g., Myfembree, Oriahnn, Orilissa) will not be approved.

- **Elagolix/estradiol/norethindrone acetate and elagolix (Oriahnn™)** [525, 526, 527]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with a gynecologist
- Must have a diagnosis of heavy menstrual bleeding associated with uterine leiomyomas (fibroids)
  - Documentation of imaging confirming the diagnosis must be provided (e.g. ultrasound or MRI)
- Must have tried and failed or have a documented contraindication to all of the following:
  - Non-steroidal anti-inflammatory drugs (NSAIDs)

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        - Hormonal contraceptives or progestins
        - Tranexamic acid (Lysteda)
    - Must have a behavioral health assessment prior to the use of elagolix/estradiol/norethindrone acetate and elagolix (Oriahnn) if the member has a history of depression and/or suicidal thoughts or behaviors, or if the member is currently undergoing treatment for depression and/or suicidal thoughts or behaviors. These members must also be followed while on elagolix/estradiol/norethindrone acetate and elagolix (Oriahnn).

If the above criteria are met, elagolix/estradiol/norethindrone acetate and elagolix (Oriahnn) is approved for 2 years per lifetime. No reauthorization is granted after this period of coverage. Additionally, cumulative use of any of the following for more than 2 years will not be approved:

- Relugolix/estradiol/norethindrone acetate (Myfembree)
- Elagolix/estradiol/norethindrone acetate and elagolix (Oriahnn)
- Elagolix (Orilissa) 150mg
    - NOTE: Elagolix (Orilissa) 200mg is only approved for 6 months per lifetime. Treatment with another GnRH antagonist (e.g., Myfembree, Oriahnn, Orilissa) will not be approved.

• Elapegademase-lvlr (Revcovi®) [528, 529, 530]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with an immunologist
- Must have confirmed diagnosis of adenosine deaminase deficiency (ADA) with Severe Combined Immunodeficiency (SCID) with documentation of the following:
    - Absent or very low ADA activity in red blood cells (<1% of normal)
    - Presence of dAXP in red blood cells
- For infants:
    - Must provide documentation of signs and symptoms consistent with the disease (e.g., lymphopenia)
- For children and adults:
    - Must provide documentation of severely impaired immune function (e.g., lymphopenia, extensive dermatitis, persistent diarrhea, recurrent pneumonia, life threatening illnesses caused by opportunistic infections)
- Member meets ONE of the following:
    - Is awaiting a hematopoietic stem cell transplant
    - Has failed a hematopoietic stem cell transplant
    - Is not a candidate for a hematopoietic stem cell transplant or gene therapy

If the above criteria are met, elapegademase-lvlr (Revcovi) is approved for one year.

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Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine Medical Necessity for continuation of treatment. Authorization may be extended at one-year intervals based upon chart documentation from the provider that the member's condition has improved based upon the prescriber's assessment while on therapy and the member is not a candidate for bone marrow transplantation or gene therapy.

• Eltrombopag (Promacta®) [531, 532, 533, 534, 535, 536]

Initial Authorization Criteria:

1. **For immune thrombocytopenia (ITP)**
   - Must be prescribed by a hematologist or oncologist
   - Must be age 1 year or older
   - Must have a diagnosis of immune thrombocytopenia (ITP)
   - Must have an adequate trial with an inadequate response or significant side effect/toxicity to ONE of the following:
     - Corticosteroids
     - Intravenous immunoglobulin (IVIG)
   - Must have a platelet count < 30 x 10^9/L prior to initiation

2. **For thrombocytopenia in chronic hepatitis C infection**
   - Must be prescribed by a gastroenterologist, hematologist, hepatologist, or infectious disease specialist
   - Must be age 18 years or older
   - Must have a diagnosis of chronic hepatitis C
   - Must currently be treated with or anticipating hepatitis C treatment with an interferon agent
   - Must have a platelet count < 75 x 10^9/L prior to initiation

3. **For severe aplastic anemia with insufficient response to immunosuppressive therapy (refractory)**
   - Must be prescribed by a hematologist or oncologist
   - Must be age 18 years or older
   - Must have a diagnosis of severe aplastic anemia
   - Must have an adequate trial with an inadequate response or significant side effect/toxicity to immunosuppressive therapy (e.g., antithymocyte globulin, cyclosporine)
   - Must have a platelet count < 30 x 10^9/L prior to initiation

4. **For first-line treatment in severe aplastic anemia**
   - Must be prescribed by a hematologist or oncologist
   - Must be age 2 years or older

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- Must have a diagnosis of severe aplastic anemia
- Must be used in combination with standard immunosuppressive therapy (e.g., antithymocyte globulin, cyclosporine)
- Must have a platelet count < 30 x 10^9/L prior to initiation

For eltrombopag (Promacta) oral packet for suspension: must have documentation of the inability to swallow an oral solid dosage form due to age (< 7 years), medical condition (e.g., nasogastric tube), or other clinical rationale for why eltrombopag (Promacta) tablets cannot be used

If the above criteria are met, eltrombopag (Promacta) is approved for 6 months.

- Immune thrombocytopenia (ITP), chronic hepatitis C, severe aplastic anemia (refractory): 6 months
- Severe aplastic anemia (first-line): 6 months for one course of treatment

Reauthorization Criteria:
All prior authorization renewals are reviewed to determine the Medical Necessity for continuation of therapy. Authorizations may be extended for a duration depending upon the diagnosis and based upon the following:

- For immune thrombocytopenia (ITP): 1 year
  - Must have documentation showing improvement in platelet count from baseline
- For severe aplastic anemia, refractory: 1 year
  - Must have documentation showing improvement in platelet count from baseline
- For severe aplastic anemia, first-line
  - Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response and clinical rationale for re-treatment.
- For thrombocytopenia in chronic hepatitis C infection:
  - Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response and clinical rationale for re-treatment.

• Eluxadoline (Viberzi™) Tablet [537, 538, 539, 540]

Initial Authorization Criteria:

- Must be prescribed by a gastroenterologist
- Must be age 18 years or older
- Must have a diagnosis of diarrhea-predominant IBS with chronic IBS symptoms
- Must provide documentation showing that nutritional interventions (such as low FODMAP diet, dietary counseling, nutritional assessment, etc.) did not result in a decreased occurrence of symptoms

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- Must provide an assessment from the physician for the need of behavioral modifications (such as cognitive behavioral therapy or psychotherapy). If behavioral modifications recommended, must have documentation showing that the modifications did not result in a decreased occurrence of symptoms.
- Must have an adequate trial and failure of loperamide or antispasmodics (such as dicyclomine or hyoscyamine) with inadequate response or significant side effect/toxicity or have a contraindication to these therapies

If the above criteria are met, eluxadoline (Viberzi) is approved for 3 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of treatment. Authorization may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved as a result of treatment.

• Ensifentrine (Ohtuvayre™) [541, 542, 543, 544, 545]

Initial Authorization Criteria:
- Must be prescribed by or in consultation with a pulmonologist
- Must be age 18 years or older
- Must have a diagnosis of moderate to severe COPD (e.g., GOLD stage II or III) confirmed by BOTH of the following:
  - Baseline spirometry with FEV1/FVC < 0.7
  - Predicted FEV1 ≥ 30% and ≤ 70%
- Must have symptomatic disease confirmed by ONE of the following:
  - Modified Medical Research Council (mMRC) dyspnea scale grade ≥ 2
  - COPD Assessment Test (CAT) score ≥ 10
  - Presence of dyspnea while walking on level ground that requires stopping
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
  - An inhaled long-acting beta-agonist (LABA)
  - An inhaled long-acting anticholinergics (LAMA)
  - Combination therapy with an inhaled LABA and LAMA
  - Combination triple therapy with an inhaled LABA, LAMA, and corticosteroid
- Must be using a dose and duration of therapy consistent with the FDA-approved package labeling

If the above criteria are met, ensifentrine (Ohtuvayre) is approved for 1 year.

Reauthorization Criteria:

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All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or stabilized while on therapy.

- **Entecavir oral solution (Baraclude®)** [546, 547, 548, 549]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with an infectious disease physician, a gastroenterologist, a hepatologist, or a transplant physician
- Must have a diagnosis for which the requested drug is FDA-approved or medically-accepted
- Must have documentation of the inability to swallow an oral solid dosage form due to age (<7 years), medical condition (e.g., nasogastric tube), or other clinical rationale for why entecavir (Baraclude) tablets cannot be used

If the above criteria are met, the requested product is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved based upon the prescriber's assessment while on therapy or the member continues to benefit from therapy.

- **Epoprostenol (Flolan®, Veletri®)** [550, 551, 552, 553, 554]

Initial Authorization Criteria:

- Must be prescribed by a cardiologist or pulmonologist who specializes in pulmonary hypertension
- Must have a diagnosis of PAH (WHO Group 1) with WHO functional class III or IV symptoms confirmed by right heart catheterization (RHC); must provide a copy of the RHC showing ALL of the following:
  - Mean pulmonary arterial pressure (mPAP) ≥25 mmHg
  - Pulmonary capillary wedge pressure (PCWP), left atrial pressure, or left ventricular end-diastolic pressure (LVEDP) ≤15 mmHg
  - Pulmonary vascular resistance >3 Wood units

If the above criteria are met, the requested product is approved for 3 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at

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one-year intervals based upon documentation from the provider that the member's condition has improved or the member continues to benefit from therapy.

- **Erythropoiesis Stimulating Agents (ESAs)** [555, 556, 557, 558, 559, 560, 561, 562]
  - Applies to: darbepoetin alfa (Aranesp®), epoetin alfa (Epogen® or Procrit®), and epoetin alfa-epbx (Retacrit™)

Initial Authorization Criteria:

1. **Darbepoetin alfa (Aranesp)**
   - Must be prescribed by or in consultation with a nephrologist, hematologist/oncologist, gastroenterologist, hepatologist, transplant physician, or an infectious disease physician
   - Must have ONE of the following diagnoses and hemoglobin levels:
     - Anemia (Hgb <10g/dL) of chronic kidney disease and on renal dialysis in adult patients
     - Anemia (Hgb <10g/dL) of chronic kidney disease not requiring dialysis in adult patients
     - Anemia (Hgb <10g/dL) of chronic kidney disease in pediatric patients
     - Anemia (Hgb <10g/dL) due to concomitant myelosuppressive chemotherapy in cancer patients with non-myeloid malignancies when the anticipated outcome is not cure in whom the anemia cannot be managed by transfusion
       - Upon initiation there is a minimum of two additional months of planned chemotherapy
     - Anemia (Hgb <10g/dL) associated with myelodysplastic syndrome
   - Must not have ANY of the following:
     - Uncontrolled hypertension
     - Pure red cell aplasia
     - Serious allergic reactions to darbepoetin alfa (Aranesp)
     - Used as substitute for RBC transfusions in patients who require immediate correction of anemia
   - Must be using a dose and duration of therapy consistent with the FDA-approved package labeling

2. **Epoetin alfa**
   - Must be prescribed by or in consultation with a nephrologist, hematologist/oncologist, gastroenterologist, hepatologist, transplant physician, or an infectious disease physician
   - Must have ONE of the following diagnoses and hemoglobin levels:
     - Anemia (Hgb <10g/dL) of chronic kidney disease and on renal dialysis in adult patients
     - Anemia (Hgb <10g/dL) of chronic kidney disease not requiring dialysis in adult patients

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        - Anemia (Hgb <10g/dL) of chronic kidney disease in pediatric patients
        - Anemia (Hgb <10g/dL) due to concomitant myelosuppressive chemotherapy in cancer patients with non-myeloid malignancies when the anticipated outcome is not cure in whom the anemia cannot be managed by transfusion
            - Upon initiation there is a minimum of two additional months of planned chemotherapy
        - Anemia (Hgb <10g/dL) due to Zidovudine (> 4200 mg/week) in patients with HIV infection with endogenous serum erythropoietin levels of < 500 mUnits/mL
        - To reduce the need for allogeneic RBC transfusions among patients with perioperative hemoglobin >10 to ≤13 g/dL who are at high risk for perioperative blood loss from elective, noncardiac, nonvascular surgery, not for use in patients who are willing to donate autologous blood pre-operatively
        - Anemia (Hgb <10g/dL) associated with myelodysplastic syndrome
    - Must not have ANY of the following:
        - Uncontrolled hypertension
        - Pure red cell aplasia
        - Serious allergic reactions to epoetin alfa
        - Undergoing cardiac or vascular surgery, or used as a substitute for RBC transfusions in patients who require immediate correction of anemia
    - Must be using a dose and duration of therapy consistent with the FDA-approved package labeling

If the above criteria are met, the requested product is approved for 6 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at 6 month intervals based upon documentation of the following:

- Must be using a dose and duration of therapy consistent with the FDA-approved package labeling
- For anemia of chronic kidney disease:
    - Must have improvement in Hgb level from baseline
- For anemia due to concomitant myelosuppressive chemotherapy in cancer patients with non-myeloid malignancies:
    - Must have improvement in Hgb level from baseline
    - Must have at least 2 more months of chemotherapy planned
- For anemia due to Zidovudine in patients with HIV-infection (epoetin alfa only):
    - Must have improvement in Hgb level from baseline
- For anemia associated with myelodysplastic syndrome:

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        - Must have improvement in Hgb level from baseline
    - For elective, noncardiac, and nonvascular surgery to reduce the need for allogeneic RBC transfusions:
        - Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response and clinical rationale for re-treatment

- **Esketamine (Spravato™)** [563, 564, 565, 566, 567, 568, 569, 570, 571, 572, 573]

Initial Authorization Criteria:

- Must be prescribed by a psychiatrist
- Must be age 18 years or older
- Must have one of the following diagnoses:
    - Treatment resistant depression and meet the following criteria:
        - Must have moderate to severe depression confirmed by a depression rating scale. Documentation of baseline depression symptoms measured by an appropriate rating scale (such as MADRS, HAM-D) must be provided.
        - Must have an adequate trial of at least 4 weeks with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
            - Two antidepressants from different pharmacologic classes (e.g., SNRI, SSRI, MAOI, TCA, or others)
            - Combination therapy consisting of an antidepressant plus an augmenting therapy (e.g., buspirone, TCA, antipsychotic, lithium or liothyronine)
    - Major depressive disorder with acute suicidal ideation or behavior and meet the following criteria:
        - Must have moderate to severe depression confirmed by a depression rating scale. Documentation of baseline depression symptoms measured by an appropriate rating scale (such as MADRS, HAM-D) must be provided.
        - Must have acute suicidal ideation or behavior
        - Must be used in combination with an oral antidepressant medication and must show adherence with oral antidepressant medication
- Must not have a history of aneurysmal vascular disease or intracerebral hemorrhage
- Must not have severe hepatic impairment (Child-Pugh class C)

If the above criteria are met, esketamine (Spravato) is approved for a duration based upon diagnosis as follows:

- For treatment resistant depression: 2 months

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- For major depressive disorder with acute suicidal ideation or behavior: 4 weeks

Reauthorization Criteria:

1. For the diagnosis of treatment resistant depression

- All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for the continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved based on the following:
  - Must provide documentation showing improvement in depression per the same rating scale used at baseline
  - Must have a requested dose that falls within the recommended dosing guidelines from the manufacturer

2. For the diagnosis of major depressive disorder with acute suicidal ideation or behavior

- Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing previous response and clinical rationale for re-treatment.
- All requests for reauthorization must be reviewed in consultation with a Community Care Behavioral Health (CCBH) Psychiatric Medical Director.

• Eslicarbazepine Acetate (Aptiom®) [574, 575, 576, 577, 578]

Criteria:

- Must be prescribed by or in consultation with a neurologist
- Must be age 4 years or older
- Must have a diagnosis of partial-onset seizures
- Must not be using with oxcarbazepine as adjunctive therapy
- Must have had inadequate response or intolerance to at least 2 generic antiepileptic medications
- Must have documentation of baseline transaminase and bilirubin levels

If the above criteria are met, eslicarbazepine acetate (Aptiom) is approved.

• Fecal microbiota spores, live-brpk (Vowst™) [579, 580, 581]

Initial Authorization Criteria:

- Must be prescribed by a gastroenterologist or an infectious disease specialist
- Must be age 18 years or older
- Must have a diagnosis of recurrent *Clostridioides difficile* infection (CDI) confirmed by the following:

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        - Documentation of a stool test positive for toxigenic *Clostridioides difficile*
    - Must have documentation of at least 2 recurrent episodes of CDI (≥3 total CDI episodes)
    - Must have an adequate trial of fecal microbiota, live-jslm (Rebyota) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
    - Must have documentation that the member has completed a standard of care antibiotic regimen for the treatment of the current occurrence of CDI (e.g., oral vancomycinor oral fidaxomicin) prior to the administration of Vowst

If the above criteria are met, fecal microbiota spores, live-brpk (Vowst) is approved for 1 month to allow for 1 course of treatment per lifetime. No reauthorization is granted after this period of coverage.

- **Fenfluramine (Fintepla®)** [582, 583, 584, 585, 586, 587, 588]

Initial Authorization Criteria:

1. **For Lennox-Gastaut Syndrome**
    - Must be prescribed by or in consultation with a neurologist
    - Must be age 2 years or older
    - Must have a diagnosis of Lennox-Gastaut Syndrome
    - Must have an adequate trial of at least 4 antiepileptic medications with an inadequate response or significant side effect/toxicity or have a contraindication to these therapies
    - Must have a baseline echocardiogram to evaluate for valvular heart disease and pulmonary arterial hypertension
        - For patients with established valvular heart disease and/or pulmonary arterial hypertension, must have an attestation from the prescriber assessing the risks and benefits of therapy and desire to use fenfluramine (Fintepla)
2. **For Dravet Syndrome**
    - Must be prescribed by or in consultation with a neurologist
    - Must be age 2 years or older
    - Must have a diagnosis of Dravet Syndrome
    - Must have an adequate trial of ALL of the following with an inadequate response or significant side effect/toxicity or have a contraindication to these therapies:
        - 2 generic antiepileptic medications
        - Cannabidiol (Epidiolex)
        - Stiripentol (Diacomit) in combination with clobazam (Onfi)
    - Must have a baseline echocardiogram to evaluate for valvular heart disease and pulmonary arterial hypertension

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        - For patients with established valvular heart disease and/or pulmonary arterial hypertension, must have an attestation from the prescriber assessing the risks and benefits of therapy and desire to use fenfluramine (Fintepla)

If the above criteria are met, fenfluramine (Fintepla) is approved.

• Ferric Citrate (Auryxia®) [589, 590, 591]

Criteria:

- Must have one of the following diagnoses:
    - Hyperphosphatemia with chronic kidney disease (on dialysis)
    - Iron deficiency anemia with chronic kidney disease (not on dialysis)
- For hyperphosphatemia with chronic kidney disease (on dialysis)
    - Must have an adequate trial and failure with an inadequate response, or significant side effects/toxicity, or a contraindication to TWO of the following:
    - Calcium acetate (tablet or capsule)
    - Lanthanum chewable tablet (Fosrenol®)
    - Sevelamer tablet (Renvela®)
- For iron deficiency anemia with chronic kidney disease (not on dialysis)
    - Must have an adequate trial and failure with an inadequate response, or significant side effects/toxicity, or a contraindication to BOTH of the following at a maximum daily dose of 200 mg elemental iron:
    - Ferrous sulfate (20% elemental iron), ferrous fumarate (33% elemental iron) or ferrous gluconate (12% elemental iron) AND
    - IV iron
        - Must provide clinical rationale for use of ferric citrate (Auryxia) instead of ferrous sulfate, ferrous fumarate, ferrous gluconate, or IV iron. Clinical rationale must include why ferrous sulfate, ferrous fumarate, ferrous gluconate, or IV iron would not be expected to produce the same clinical results as ferric citrate (Auryxia)

If the above criteria are met, ferric citrate (Auryxia) is approved.

• Finerenone (Kerendia®) [592, 593, 594, 595, 596, 597, 598]

Initial Authorization Criteria:

1. **For Chronic Kidney Disease associated with Type 2 Diabetes:**
    - Must be age 18 years or older

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- Must have a diagnosis of chronic kidney disease associated with type 2 diabetes
- Must have estimated glomerular filtration rate (eGFR) ≥ 25 mL/min/1.73 m^2
- Must be on concomitant therapy with an angiotensin-converting enzyme (ACE) inhibitor (e.g., benazepril, lisinopril, ramipril) or angiotensin II receptor blocker (ARB) (e.g., losartan, irbesartan, valsartan) at maximally tolerated dose for diabetic nephropathy unless there is a significant side effect/toxicity or a contraindication to these therapies
- Must have documented serum potassium level of ≤ 5 mmol/L at initiation of therapy
- Must not use in combination with a strong CYP3A4 inhibitor (e.g., itraconazole, clarithromycin)
- Must not have adrenal insufficiency

2. **For Heart Failure (HF):**
   - Must be age 18 years or older
   - Must have a diagnosis of heart failure
   - Must have Left Ventricular Ejection Fraction (LVEF) ≥40%
   - Must have estimated glomerular filtration rate (eGFR) ≥ 25 mL/min/1.73 m^2
   - Must have documented serum potassium level of ≤ 5 mmol/L at initiation of therapy
   - Must not use in combination with a strong CYP3A4 inhibitor (e.g., itraconazole, clarithromycin)
   - Must not have adrenal insufficiency

If the above criteria are met, finerenone (Kerendia) is approved for one year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon the following:

- An attestation from the provider that the member's condition has stabilized or improved based upon the prescriber's assessment while on therapy.
- Must provide laboratory documentation and/or an attestation from the prescriber that serum potassium is being monitoring while on therapy
- For HF:
  - Must have Left Ventricular Ejection Fraction (LVEF) ≥40%

- **Fingolimod (Gilenya® 0.25 mg, Tascenso ODT®)** [599, 600, 601, 602, 603, 604, 605]

Initial Authorization Criteria:

- Must be prescribed by a neurologist
- Must be age 10 years or older

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- Must have a diagnosis of relapsing form of multiple sclerosis to include clinically isolated syndrome, relapsing-remitting disease, and active secondary progressive disease
- Must not be on concomitant therapy with a disease modifying drug for multiple sclerosis
- For fingolimod (Gilenya) 0.25 mg:
  - Must provide current weight and have requested dose that falls within the recommended dosing guidelines from the manufacturer
- For fingolimod orally disintegrating tablets (Tascenso ODT):
  - Must provide clinical rationale for use of fingolimod orally disintegrating tablets (Tascenso ODT) instead of fingolimod (Gilenya). Clinical rationale must include why fingolimod (Gilenya) would not be expected to produce the same clinical results as fingolimod orally disintegrating tablets (Tascenso ODT).

If the above criteria are met, the requested product is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or stabilized while on therapy.

• Fluticasone propionate (Xhance™) [606, 607, 608, 609]

Initial Authorization Criteria:
- Must be age 18 years or older
- Must have a diagnosis of chronic rhinosinusitis with nasal polyps (CRSwNP) or without nasal polyps (CRSsNP)
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to the following:
  - For CRSwNP:
    - One intranasal corticosteroid
  - For CRSsNP:
    - Three intranasal corticosteroids

If the above criteria are met, fluticasone propionate (Xhance) is approved.

• Foscarbidopa and foslevodopa (Vyalev™) [610, 611, 612, 613]

Initial Authorization Criteria:
- Must be prescribed by or in consultation with a neurologist
- Must have a diagnosis of advanced Parkinson's disease with BOTH of the following:

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        - A clear motor response to levodopa
        - Inadequately controlled motor fluctuations with average daily "off" time OR "on" time with troublesome dyskinesia of ≥ 2.5 hours per day despite adherence to optimized medication therapy regimen based on the prescriber's assessment. Chart documentation must be provided.
    - Must be established on at least 400 mg/day of levodopa equivalents from levodopa-containing medications and COMT inhibitors
    - Must have an attestation from the provider that the member's and/or primary caregiver's ability to use the subcutaneous pump device has been assessed
    - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to TWO of the following, taken in combination with carbidopa/levodopa for the adjunctive treatment of "off" episodes:
        - Entacapone or opicapone (Ongentys)
        - A generic MAO-B inhibitor (e.g., selegiline or rasagiline)
        - A generic dopamine agonist (e.g., ropinirole tablet, pramipexole tablet)
        - Istradefylline (Nourianz)
    - Must not be on concomitant therapy with a nonselective monoamine oxidase (MAO) inhibitor (e.g., phenelzine, tranylcypromine)
    - Must be using a dose and duration of therapy consistent with the FDA-approved package labeling

If the above criteria are met, foscarbidopa and foslevodopa (Vyalev) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon the following:
    - Documentation from the provider that the member's condition has improved or stabilized while on therapy.
    - Must be using a dose and duration of therapy consistent with the FDA-approved package labeling

- **Fostamatinib disodium hexahydrate (Tavalisse™)** [614, 615, 616]

Initial Authorization Criteria:
    - Must be prescribed by a hematologist or oncologist
    - Must be age 18 years or older
    - Must have a diagnosis of immune thrombocytopenia (ITP)
    - Must have an adequate trial with inadequate response or significant side effect/toxicity to ONE of the following:
        - Corticosteroids

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        - Intravenous immunoglobulin (IVIG)
    - Must have an adequate trial of generic eltrombopag (Promacta) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
    - Must have a platelet count <30 x 10^9/L prior to initiation

If the above criteria are met, fostamatinib disodium hexahydrate (Tavalisse) is approved for 6 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one-year intervals based upon documentation showing improvement in platelet count from baseline.

• Fostemsavir (Rukobia™) [617, 618]

Initial Authorization Criteria:
- Must be prescribed by or in consultation with an infectious disease or HIV specialist
- Must be age 18 years or older
- Must have a diagnosis of multidrug resistant HIV-1 infection with failure of current antiretroviral regimen due to resistance, intolerance, or safety considerations.
- Must be used in combination with other antiretroviral agents

If the above criteria are met, fostemsavir (Rukobia) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or stabilized while on therapy.

• Furosemide (Furoscix®) [619, 620]

Initial Authorization Criteria:
- Must be prescribed by a cardiologist or nephrologist
- Must be age 18 years or older
- Must have a diagnosis of edema with either chronic heart failure or chronic kidney disease (including nephrotic syndrome)
- Must have an adequate trial of an orally administered loop diuretic (e.g., furosemide, bumetanide, or torsemide) at maximally tolerated doses with an

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inadequate response, defined as recurring edema while adherent (PDC >80%) to therapy
- Must not have anuria
- Must have an attestation from the provider including BOTH of the following:
  - Furosemide (Furoscix) is not being used as maintenance therapy
  - Treatment with furosemide (Furoscix) is to be replaced by an oral diuretic as soon as practical for the member

If the above criteria are met, furosemide (Furoscix) is approved for 1 month for 1 course of therapy.

Reauthorization Criteria:
Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require documentation describing the previous response and clinical rationale for re-treatment.

• Furosemide (Lasix ONYU™) [621, 622]

Initial Authorization Criteria:
- Must be prescribed by or in consultation with a cardiologist
- Must be age 18 years or older
- Must have a diagnosis of edema with chronic heart failure
- Must be prescribed a dose and duration of therapy that is consistent with FDA-approved package labeling, nationally-recognized compendia, or peer-reviewed medical
- Must have an adequate trial of an orally administered loop diuretic (e.g., furosemide, bumetanide, or torsemide) at maximally tolerated doses with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
- Must not have ANY of the following:
  - Anuria
  - Hepatic coma
  - Hypersensitivity to furosemide or any of its components
- Must have an attestation from the provider including BOTH of the following:
  - Furosemide (Lasix ONYU) is not being used as maintenance therapy
  - Treatment with furosemide (Lasix ONYU) is to be replaced by an oral diuretic as soon as practical for the member

If the above criteria are met, furosemide (Lasix ONYU) is approved for 1 month for 1 course of therapy.

Reauthorization Criteria:
Additional authorizations for treatment are made on a case-by-case basis, are subject

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to the above criteria, and require documentation describing the previous response and clinical rationale for re-treatment.

- **Gabapentin Enacarbil ER (Horizant®)** [623, 624, 625, 626, 627, 628, 629, 630, 631, 632, 633, 634]

Criteria:

1. **For Restless Legs Syndrome (RLS)**
   - Must have a diagnosis of RLS
   - Must have an adequate trial of gabapentin defined as one of the following:
     - A positive response to gabapentin treatment with treatment effect not lasting throughout the day/night
     - Intolerance to gabapentin despite slow dose titration

2. **For Postherpetic Neuralgia (PHN)**
   - Must have a diagnosis of PHN
   - Must have an adequate trial and failure of generic gabapentin defined as one of the following:
     - Insufficient efficacy of gabapentin at a dose of at least 1800mg per day
     - Intolerance to gabapentin despite slow titration of dose.
   - Must have a trial and failure with inadequate response or significant side effects/toxicity or have a contraindication to ALL of the following drugs/drug classes:
     - One tricyclic antidepressant (e.g., amitriptyline)
     - Pregabalin
     - Lidocaine patch

If the above criteria are met, gabapentin enacarbil ER (Horizant) is approved.

- **Gabapentin ER (Gralise®)** [635, 636, 637]

Criteria:

- Must have a diagnosis of Postherpetic Neuralgia
- Must have an adequate trial and failure of generic gabapentin defined as one of the following:
  - Insufficient efficacy of gabapentin at a dose of at least 1800mg per day.
  - Intolerance to gabapentin despite slow titration of dose.
- Must have a trial and failure with inadequate response or significant side effects/toxicity or have a contraindication to ALL of the following drugs/drug classes:
  - One tricyclic antidepressant (e.g., amitriptyline)
  - Pregabalin
  - Lidocaine patch

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If the above criteria are met, gabapentin ER (Gralise) is approved.

- **Ganaxolone (Ztalmy®)** [638, 639, 640]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with a neurologist
- Must be age 2 years or older
- Must have a diagnosis of CDKL5 deficiency disorder confirmed by genetic testing demonstrating a mutation in the CDKL5 gene. Documentation of lab result confirming mutation is required.
- Must provide documentation of baseline seizure frequency

If the above criteria are met, ganaxolone (Ztalmy) is approved for 6 months.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's disease has improved based upon the prescriber's assessment, as evidenced by chart documentation of improvement in seizure frequency from baseline.

- **Givinostat (Duvyzat™)** [641]

Initial Authorization Criteria:

- Must be prescribed by a neurologist (specifically a neuromuscular specialist) who specializes in the treatment of muscular dystrophy
- Must be age 6 years or older
- Must have a diagnosis of Duchenne muscular dystrophy (DMD) with confirmatory genetic testing demonstrating an out-of-frame mutation [e.g. frameshift (deletion or duplication) mutation or a premature stop codon mutation] of the DMD gene (a copy of the genetic testing must be provided)
- Must provide chart documentation of baseline motor function and/or strength (e.g., Brooke upper extremity functional score, Modified Medical Research Council score for manual muscle testing, North Star Ambulatory Assessment/Timed Functional Test)
- Must provide chart documentation showing baseline forced vital capacity (FVC%)
- Must provide the following ONLY if non-ambulatory:
  - Must have a documented Brooke upper extremity score of 5 or less OR a documented FVC of at least 30%
- Must be using a dose and duration of therapy consistent with the FDA-approved package labeling

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- Must be used in combination with corticosteroid therapy (e.g., prednisone, deflazacort [Emflaza™], or vamorolone [Agamree®] unless contraindicated or intolerant)

If the above criteria are met, givinostat (Duvyzat) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon the following:

- Must have chart documentation from the provider that the member's condition has improved or stabilized, or the member continues to benefit from therapy.
- Must be prescribed by a neurologist (specifically a neuromuscular specialist) who specializes in the treatment of muscular dystrophy
- Must provide the following ONLY if non-ambulatory:
  - Must have a documented Brooke upper extremity score of 5 or less OR a documented FVC of at least 30%
- Must be using a dose and duration of therapy consistent with the FDA-approved package labeling

• Glucagon-like peptide-1 (GLP-1) receptor agonists and Glucose-dependent insulinotropic polypeptide (GIP) receptor and GLP-1 receptor agonist [642, 643, 644, 645, 646, 647, 648, 649, 650, 651, 652, 653, 654, 655, 656, 657, 658, 659, 660]

- Applies to: semaglutide (Ozempic®), semaglutide (Rybelsus®), dulaglutide (Trulicity®), liraglutide (Victoza®), tirzepatide (Mounjaro™)
- Applies to: exenatide extended-release (Bydureon®, Bydureon Bcise®), exenatide (Byetta®) (Value Choice and Open Choice formularies only)

Initial Authorization Criteria:

- The criteria for automatic coverage of formulary GLP-1 and GIP/GLP-1 combination products is as follows:
  - Must have ONE of the following:
    - A pharmacy claim history of prior therapy with an oral medication used for the treatment of diabetes, not including single-entity metformin
    - A medical claim history of a type 2 diabetes mellitus diagnosis
- For members who do not meet the automatic coverage criteria, a medical necessity review is completed, and the following criterion must be met:
  - Must have an attestation from the prescriber indicating that the requested medication is being used for a diagnosis of type 2 diabetes mellitus

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If the above criteria are met, the requested product is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals and are subject to the above criteria.

Off-label Use

1. **For noncirrhotic nonalcoholic steatohepatitis (NASH) or metabolic dysfunction-associated steatohepatitis (MASH):**
   Initial Authorization Criteria:
   - Must be prescribed by or in consultation with a hepatologist or gastroenterologist
   - Must have a diagnosis of noncirrhotic nonalcoholic steatohepatitis (NASH) or metabolic dysfunction-associated steatohepatitis (MASH) with moderate to advanced liver fibrosis (consistent with stages F2 to F3 fibrosis) confirmed by ONE of the following:
     - Liver biopsy performed within the last 3 years shows fibrosis stage F2 to F3 (chart documentation must be provided)
     - FIB-4 ≥ 1.3 AND one of the following non-invasive tests performed within the last 6 months is consistent with F2 to F3 fibrosis (a copy of the report must be provided):
       - Vibration controlled transient elastography (VCTE; e.g., Fibroscan)
       - Shear wave elastography (SWE)
       - Magnetic resonance elastography (MRE)
       - Enhanced liver fibrosis (ELF) score
   - Must not have evidence of cirrhosis (stage F4 fibrosis) by imaging or liver biopsy or one or more liver-related complications associated with cirrhosis (e.g., variceal bleeding, ascites, hepatic encephalopathy, etc.)
   - Must not have significant alcohol use (defined as alcohol consumption of more than 1 drink/day for natal females or more than 2 drinks/day for natal males).
   - Must have an attestation from the provider that the member has received counseling on dietary and lifestyle modifications
   - Must be used in conjunction with diet and exercise

   If the above criteria are met, the requested product is approved for 1 year.

   Reauthorization Criteria:

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All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation showing the following:

- The member's condition has improved or stabilized while on therapy
- Must not have evidence of cirrhosis (stage F4 fibrosis) by imaging or liver biopsy or one or more liver-related complications associated with cirrhosis (e.g., variceal bleeding, ascites, hepatic encephalopathy, etc.)
- Must not have significant alcohol use (defined as alcohol consumption of more than 1 drink/day for natal females or more than 2 drinks/day for natal males)
- Must continue to be used in conjunction with diet and exercise

- **Glucosylceramide Synthase Inhibitors (Cerdelga™, Zavesca®, Yargesa™, Opfolda™)** [661, 662, 663, 664, 665, 666, 667, 668, 669, 670, 671, 672, 673, 674, 675, 676, 677, 678, 679, 680, 681, 682, 683]

Initial Authorization Criteria:

For Cerdelga, Zavesca, Yargesa:

- Must be prescribed by or in consultation with a provider who specializes in the treatment of inherited metabolic disorders
- Must be age 18 years or older
- Must have a diagnosis of Type I Gaucher disease confirmed by chart documentation of ONE of the following:
  - Genetic test results indicating a biallelic pathogenic variant in the GBA gene.
  - Enzyme assay (e.g., beta-glucosidase leukocyte [BCL] test) showing low glucosylceramidase enzyme activity (also known as acid beta-glucosidase or glucocerebrosidase)
- Must have documentation of ONE of the following clinical features:
  - Hepatomegaly
  - Splenomegaly
  - Bone disease (e.g., pathological fracture, osteopenia, avascular necrosis, Ernlenmeyer flask deformity)
  - Anemia
  - Thrombocytopenia
- For eliglustat (Cerdelga):
  - Must have chart documentation of FDA-cleared test confirming CYP2D6 genotype as extensive metabolizer (EM), intermediate metabolizer (IM), or poor metabolizer (PM)
  - Must have an attestation from the provider that the member does not have any contraindications to this drug therapy

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        - Must have an adequate trial of miglustat (Opfolda, Zavesca, Yargesa) with an inadequate response or significant side effects/toxicity, or have a contraindication to this therapy

For Opfolda:

- Must be prescribed by or in consultation with a provider who specializes in the treatment of inherited metabolic disorders
- Must weigh ≥ 40 kg
- For late onset Pompe disease
    - Must have a diagnosis of late-onset Pompe disease confirmed by ONE of the following:
        - GAA enzyme assay from blood, skin fibroblasts, lymphocytes, or muscle (chart documentation of laboratory result including reference range is required)
        - GAA gene mutation (chart documentation of laboratory result is required)
    - Must have chart documentation of clinical signs or symptoms attributable to Pompe disease (e.g., impairment of pulmonary function or motor weakness)
    - Must have documentation from the provider that the member is not improving on their current enzyme replacement therapy (ERT)
    - Must be used in combination with cipaglucosidase alfa-atga (Pombiliti™)
- For Type I Gaucher disease
    - Must have diagnosis of Type I Gaucher disease confirmed by chart documentation of ONE of the following:
        - Genetic test results indicating a biallelic pathogenic variant in the GBA gene.
        - Enzyme assay (e.g., beta-glucosidase leukocyte [BCL] test) showing low glucosylceramidase enzyme activity (also known as acid beta-glucosidase or glucocerebrosidase)
    - Must have documentation of ONE of the following clinical features:
        - Hepatomegaly
        - Splenomegaly
        - Bone disease (e.g., pathological fracture, osteopenia, avascular necrosis, Ernlenmeyer flask deformity)
        - Anemia
        - Thrombocytopenia

Off Label Use:

1. **Niemann Pick disease type C (NPC)**
    - Must be prescribed by or in consultation with a provider who specializes in the treatment of inherited metabolic disorders or a neurologist

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- Must have a diagnosis of Niemann-Pick disease type C (NPC) confirmed by ONE of the following:
  - A mutation in both alleles of NPC1 or NPC2 gene (a copy of the genetic test result must be provided)
  - A mutation in one allele (NPC1 or NPC2) and a positive filipin-staining or elevated cholestane triol/oxysterols >2x the upper limit of normal (a copy of the genetic test result and laboratory results must be provided)
- Must be prescribed a dose and duration of therapy that is consistent with FDA-approved package labeling, nationally-recognized compendia, or peer-reviewed medical literature
- Must have neurological symptoms of NPC (e.g., cognitive decline, cerebellar ataxia, gaze palsy, dysarthria, dysphagia, dystonia, seizures, psychiatric disorders) Chart documentation of symptoms must be provided.
- Must provide clinical rationale for use of miglustat (Zavesca, Yargesa) instead of miglustat (Opfolda). Clinical rationale must include why miglustat (Opfolda) would not be expected to produce the same clinical results as miglustat (Zavesca, Yargesa) given that they are the same chemical entity.

If the above criteria are met, the requested glucosylceramide synthase inhibitor is approved for one year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one-year intervals based upon chart documentation from the prescriber that the member's condition has improved based upon the prescriber's assessment while on therapy.

• Glycopyrrolate Oral Solution (Cuvposa®) [684]

Initial Authorization Criteria:
- Must be age 3 to 16 years
- Must have a diagnosis of chronic severe drooling
- Must have a diagnosis of a neurologic condition that is associated with problem drooling (e.g. cerebral palsy)

If the above criteria are met glycopyrrolate oral solution (Cuvposa) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one-year intervals based upon documentation from the prescriber that the member's condition has improved based upon the prescriber's assessment while on therapy.

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- **Glycopyrronium (Qbrexza®)** [685]

Initial Authorization Criteria:

- Must be age 9 years or older
- Must have a diagnosis of primary axillary hyperhidrosis
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ONE of the following:
  - Aluminum Chloride 20% solution (Drysol)
  - Topical aluminum chloride

If the above criteria are met, glycopyrronium (Qbrexza) is approved for 1 year.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved while on therapy.

- **Granulocyte and Granulocyte-Macrophage Colony Stimulating Factors** [686, 687, 688, 689, 690, 691, 692, 693, 694, 695, 696, 697, 698, 699, 700, 701, 702, 703, 704, 705, 706]

  - Applies to: filgrastim (Neupogen), filgrastim-aafi (Nivestym), filgrastim-ayow (Releuko), filgrastim-txid (Nypozi), filgrastim-sndz (Zarxio), tbo-filgrastim (Granix), pegfilgrastim (Neulasta, Neulasta Onpro), pegfilgrastim-jmdb (Fulphila), pegfilgrastim-fpgk (Stimufend), pegfilgrastim-pbbk (Fylnetra), egfilgrastim-apgf (Nyvepria), pegfilgrastim-cbqv (Udenyca, Udenyca Onbody), pegfilgrastim-bmez (Ziextenzo), sargramostim (Leukine), eflapegrastim (Rolvedon) and efbemalenograstim alfa (Ryzneuta)

Initial Authorization Criteria:

- Must be used in ONE of the following clinical scenarios:
  - Diagnosis of cancer receiving antineoplastics or radiation therapy
  - Received stem cell transplantation
  - Hematopoietic cell mobilization for autologous or allogeneic donors for stem cell transplantation
  - Hematopoietic Syndrome of Acute Radiation Syndrome
  - Congenital, cyclic, or idiopathic neutropenia
  - Drug-induced agranulocytosis

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- Requests for pegfilgrastim-pbbk (Fylnetra), egfilgrastim-apgf (Nyvepria), pegfilgrastim-cbqv (Udenyca, Udenyca Onbody™), and pegfilgrastim-bmez (Ziextenzo):
  - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
    - Pegfilgrastim-jmdb (Fulphila)
    - Pegfilgrastim (Neulasta, Neulasta Onpro)
    - Pegfilgrastim-fpgk (Stimufend)
  - Must provide clinical rationale for use of the requested product instead of the prerequisite medication trials. Clinical rationale must include why the prerequisite medication trials would not be expected to produce the same clinical results as the requested product.
- Requests for Filgrastim (Neupogen), Filgrastim-aafi (Nivestym), filgrastim-ayow (Releuko), and filgrastim-txid (Nypozi)
  - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to BOTH of the following:
    - Tbo-filgrastim (Granix)
    - Filgrastim-sndz (Zarxio)
  - Must provide clinical rationale for use of the requested product instead of the prerequisite medication trials. Clinical rationale must include why the prerequisite medication trials would not be expected to produce the same clinical results as the requested product.
- Requests for eflapegrastim (Rolvedon) and efbemalenograstim alfa (Ryzneuta)
  - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
    - Pegfilgrastim-jmdb (Fulphila)
    - Pegfilgrastim (Neulasta, Neulasta Onpro)
    - Pegfilgrastim-fpgk (Stimufend)
    - Tbo-filgrastim (Granix)
    - Filgrastim-sndz (Zarxio)

If the above criteria are met, the requested medication is approved for 6 months.

Reauthorization Criteria:
Additional authorizations are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing previous response to treatment and clinical rationale for re-treatment.

- **Growth Hormone** 707, 708, 709, 710, 711, 712, 713, 714, 715, 716, 717, 718, 719, 720, 721, 722, 723, 724, 725, 726, 727, 728, 729, 730, 731, 732, 733, 734, 735, 736,

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737, 738, 739, 740, 741, 742, 743, 744, 745, 746, 747, 748, 749, 750, 751, 752, 753, 754, 755, 756, 757, 758, 759, 760, 761, 762, 763, 764, 765, 766, 767, 768, 769, 770, 771, 772, 773, 774, 775, 776, 777, 778, 779, 780, 781, 782, 783, 784, 785, 786, 787, 788, 789, 790, 791, 792, 793, 794, 795, 796, 797, 798, 799, 800, 801, 802, 803, 804, 805, 806, 807, 808, 809

Initial Authorization Criteria:

1. **For growth hormone requests other than somatropin (Norditropin®, Genotropin®) for Open Choice and Value Choice:**
   Growth hormone requests other than somatotropin (Norditropin, Genotropin) are considered medically necessary for members who meet ALL of the following criteria:
   - Must meet below criteria based on indication
   - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to BOTH of the following:
     - Somatropin (Norditropin)
     - Somatropin (Genotropin)
   - Must provide clinical rationale for use of the requested growth hormone product instead of somatropin (Norditropin, Genotropin). Clinical rationale must include why the requested growth hormone product would not be expected to produce the same clinical results as somatropin (Norditropin, Genotropin)

2. **Children and Adolescents with Classic Growth Hormone Deficiency (GHD)**
   GH replacement is considered medically necessary for members with GHD and growth failure who meet ALL of the following criteria:
   - Must be prescribed by a pediatric endocrinologist and must be used with appropriate prescriber follow-up
   - Must have documented failure to respond to 2 GH provocative tests, defined as a serum GH level (peak level) <10ng/mL, using the following stimulation tests: insulin, levodopa, arginine, clonidine, and glucagon. One abnormal GH test is sufficient in children with a history of irradiation or multiple pituitary hormone deficiency,
   - Must have at least TWO of the following:
     - Present height is <3rd percentile or >2 standard deviations (SD) below the mean for gender and age
     - Pretreatment growth velocity is <7 cm per year for children aged <3 years and <4 cm per year for children aged 3 and older OR for a child of any age the growth velocity is <10th percentile for gender and age based on at least 6 months of growth data
     - Comparison of skeletal (bone) age by x-ray of left hand and wrist is >2 standard deviations below the chronological age
   - Must have no evidence of active malignancy for the past year

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- Must not have active proliferative or severe non-proliferative diabetic retinopathy
- Must include a treatment plan outlining the dose, monitoring parameters such as when the member is seen for follow-up, methods for determining treatment response and anticipated duration of use

3. **Neonates with Classic Growth Hormone Deficiency (GHD)**
   GH replacement is considered medically necessary for neonates with GHD who meet ALL of the following criteria:
   - Must be prescribed by a pediatric endocrinologist or a pediatric nephrologist, and must be used with appropriate prescriber follow-up
   - Must be 30 days old or less at time of diagnosis
   - Must have presence of neonatal hypoglycemia in the absence of a metabolic disorder. Chart documentation indicating that other metabolic disorders have been ruled out as a cause of hypoglycemia through clinical work-up must be submitted.
   - Must have a random growth hormone level of <20 ng/mL (µg/L)
   - Must have no evidence of active malignancy for the past year
   - Must not have active proliferative or severe non-proliferative diabetic retinopathy
   - Must include a treatment plan outlining the dose, monitoring, and parameters such as when the member is seen for follow-up, methods for determining treatment response and anticipated duration of use

4. **Children with growth retardation due to chronic renal insufficiency (CRI)**
   GH replacement prior to renal transplantation is considered medically necessary for children with CRI and growth retardation who meet ALL of the following criteria:
   - Must be prescribed by a pediatric endocrinologist or a pediatric nephrologist, and must be used with appropriate prescriber follow-up
   - Must have documented diagnosis of CRI up to the time of renal transplant; growth hormone is not approved post-transplant
   - Must have at least ONE of the following:
     - Present height is <3rd or percentile or >2 standard deviations (SD) below the mean for gender and age
     - Pretreatment growth velocity is <7 cm per year for children aged <3 years and <4 cm per year for children aged 3 and older OR for a child of any age the growth velocity is <10th percentile for gender and age based on at least 6 months of growth data
   - Must have no evidence of active malignancy for the past year
   - Must not have active proliferative or severe non-proliferative diabetic retinopathy

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- Must include a treatment plan outlining the dose, monitoring, and parameters such as when the member is seen for follow-up, methods for determining treatment response and anticipated duration of use

5. **Turner Syndrome/Noonan Syndrome**
   GH replacement is considered medically necessary for natal female children with Turner's syndrome or children with Noonan Syndrome who have growth retardation and who meet ALL of the following criteria:
   - Must be prescribed by a pediatric endocrinologist and must be used with appropriate prescriber follow-up
   - Must have documented diagnosis of Turner's syndrome or Noonan Syndrome
   - Must have at least ONE of the following:
     - Present height is <5th percentile for age/sex
     - Pretreatment growth velocity is <7 cm per year for children aged <3 years and <4 cm per year for children aged 3 and older OR for a child of any age the growth velocity is <10th percentile for gender and age based on at least 6 months of growth data
   - Must have no evidence of active malignancy for the past year
   - Must not have active proliferative or severe non-proliferative diabetic retinopathy
   - Must include a treatment plan outlining the dose, monitoring parameters such as when the member is seen for follow-up, methods for determining treatment response and anticipated duration of use

6. **Children with Prader-Willi Syndrome**
   GH replacement is considered medically necessary for children with Prader-Willi syndrome who meet ALL of the following criteria:
   - Must be prescribed by a pediatric endocrinologist and must be used with appropriate prescriber follow-up
   - Must have documented diagnosis of Prader-Willi Syndrome
   - Must have at least ONE of the following:
     - Present height is <3rd or percentile or >2 standard deviations (SD) below the mean for gender and age
     - Pretreatment growth velocity is <7 cm per year for children aged <3 years and <4 cm per year for children aged 3 and older OR for a child of any age the growth velocity is <10th percentile for gender and age based on at least 6 months of growth data
   - Must not be severely obese (defined as a body mass index [BMI] ≥97th percentile for age and gender OR a BMI ≥35), have a history of upper airway obstruction, or sleep apnea, or have severe respiratory impairment
   - Must have no evidence of active malignancy for the past year
   - Must not have active proliferative or severe non-proliferative diabetic retinopathy

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- Must include a treatment plan outlining the dose, monitoring parameters such as when the member is seen for follow-up, methods for determining treatment response and anticipated duration of use

7. **Children with Short Stature Homeobox-containing Gene (SHOX) deficiency**
   GH replacement is considered medically necessary for children with short stature homeobox-containing gene deficiency and who meet ALL of the following criteria:
   - Must be prescribed by a pediatric endocrinologist and must be used with appropriate prescriber follow-up
   - Must have documented diagnosis of SHOX deficiency
     - Documentation of lab result confirming SHOX mutation is required
   - Must have at least ONE of the following:
     - Present height is <3rd or percentile or >2 standard deviations (SD) below the mean for gender and age
     - Pretreatment growth velocity is <7 cm per year for children aged <3 years, and <4 cm per year for children aged 3 and older, OR for a child of any age the growth velocity is <10th percentile for gender and age based on at least 6 months of growth data
   - Must have no evidence of active malignancy for the past year
   - Must not have active proliferative or severe non-proliferative diabetic retinopathy
   - Must include a treatment plan outlining the dose, monitoring parameters such as when the member is seen for follow-up, methods for determining treatment response and anticipated duration of use

8. **Children born Small for Gestational Age (SGA)**
   GH replacement is considered medically necessary for treatment of growth failure in children born small for gestational age (SGA) who meet ALL of the following criteria:
   - Must be prescribed by a pediatric endocrinologist and must be used with appropriate prescriber follow-up
   - Must be born SGA, as defined has ONE of the following:
     - Must have a birth weight of less than 2,500 grams at a gestational age of more than 37 weeks
     - Must have a birth weight or length below the 3rd percentile or <-2 SD for gestational age
   - Must have failed to achieve catch-up growth by ages two to four
     - Must have baseline pre-treatment height SDS <-2.5 SD for age and gender
   - Must have no evidence of active malignancy for the past year
   - Must not have active proliferative or severe non-proliferative diabetic retinopathy

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- Must include a treatment plan outlining the dose, monitoring parameters such as when the member is seen for follow-up, methods for determining treatment response and anticipated duration of use

9. Adult Growth Hormone Deficiency – Childhood Onset

GH replacement is considered medically necessary for adults with childhood-onset GHD who meet ALL of the following criteria:

- Must be prescribed by an endocrinologist
- Must have been growth hormone deficient during childhood as a result of congenital, genetic, acquired, or idiopathic causes. Chart documentation confirming member's history of growth hormone deficiency is required.
- Must have GH treatment stopped for at least one month after completion of linear growth, and then GH levels should be reassessed by stimulation test.
- GH reassessment through stimulation testing is not required for members with a high likelihood of GHD, as defined as having a serum IGF-I level <84µg/L while not receiving growth hormone therapy AND at least ONE of the following:
  - Severe GHD in childhood due to a genetic cause
  - Structural hypothalamic-pituitary disease
  - Central nervous system tumors
  - Deficiencies in at least 3 of the following pituitary hormones: adrenocorticotropin hormone (ACTH), thyroid stimulating hormone (TSH), gonadotropin deficiency (luteinizing hormone [LH] and/or follicle stimulating hormone [FSH] are counted as one deficiency), prolactin, or arginine vasopressin (VAP) deficiency
  - Severe GHD and the receipt of high-dose cranial radiation therapy
- For members with only a serum IGF-I level <84µg/L while not receiving growth hormone therapy, biochemical diagnosis of GH deficiency must be demonstrated on reassessment as determined by a negative response to ONE of the following standard GH stimulation tests (see i, ii, and iii below e)
- For members with serum IGF-levels within the given laboratory range while not on growth hormone therapy, biochemical diagnosis of GH deficiency must be demonstrated on reassessment as determined by a negative response to TWO of the following standard GH stimulation tests:
  - The insulin tolerance test (ITT) is required unless contraindicated. Negative response to the ITT is defined as a peak GH level of ≤5.0µg/L. Contraindications to the ITT include patients with known or who are at high risk for coronary artery disease or patients with a history of seizures, and severe panhypopituitarism/hypoadrenalism.
  - If ITT is contraindicated, the glucagon test is required unless contraindicated. Negative response to the glucagon test is defined as a peak GH level of ≤3µg/L. Contraindications to the glucagon test include patients who are malnourished or who have not eaten in 48

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hours, patients with pheochromocytoma or insulinoma, and severe hypocortisolemia.
    - If both ITT and glucagon tests are contraindicated, the arginine test is required. Negative response to arginine test is defined as a peak GH level of ≤0.4 µg/L
  - Must have no evidence of active malignancy for the past year
  - Must not have active proliferative or severe non-proliferative diabetic retinopathy
  - Must submit documentation that the tumor size has remained stable for a period of one year prior to initiating GH therapy if the member has a pituitary adenoma
  - Must include a treatment plan outlining the dose, monitoring parameters such as when the member is seen for follow-up, methods for determining treatment response and anticipated duration of use

10. Adult Growth Hormone Deficiency – Adult Onset

GH replacement is considered medically necessary for adults with adult-onset GHD who meet ALL of the following criteria:

  - Must be prescribed by an endocrinologist
  - Must have a diagnosis of growth hormone deficiency, either alone or associated with multiple hormone deficiencies (hypopituitarism), as a result of pituitary disease, hypothalamic disease, surgery, radiation therapy, or trauma
  - For members with panhypopituitarism, growth hormone stimulation testing is not necessary to verify the diagnosis if member has BOTH of the following:
    - Deficiencies in at least 3 of the following pituitary hormones: adrenocorticotropin hormone (ACTH), thyroid stimulating hormone (TSH), gonadotropin deficiency (luteinizing hormone [LH] and/or follicle stimulating hormone [FSH] are counted as one deficiency), prolactin, or arginine vasopressin (VAP) deficiency
    - Serum IGF-I level <84 µg/L while not receiving growth hormone therapy
  - For members without panhypopituitarism with low IGF levels, biochemical diagnosis of GH deficiency must be determined by a negative response to TWO of the following growth hormone stimulation tests:
    - The ITT is required unless contraindicated. Negative response to the ITT is defined as a peak GH level of ≤5.0µg/L.
    - If ITT is contraindicated, the glucagon test is required unless contraindicated. Negative response to the glucagon test is defined as a peak GH level of ≤3µg/L
    - If both ITT and glucagon tests are contraindicated, the arginine test is required. Negative response to arginine test is defined as a peak GH level of ≤0.4µg/L

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        - For members with a deficiency in at least one other pituitary hormone AND a serum IGF-I level <84µg/L, only one growth hormone stimulation test is required
    - Must submit documentation that the tumor size has remained stable for a period of one year prior to initiating GH therapy if the member has a pituitary adenoma
    - Must have no evidence of active malignancy for the past year
    - Must not have active proliferative or severe non-proliferative diabetic retinopathy
    - Must include a treatment plan outlining the dose, monitoring parameters such as when the member is seen for follow-up, methods for determining treatment response and anticipated duration of use

If the above criteria are met for the given indication, the requested formulary growth hormone product is approved for 1 year.

Reauthorization Criteria:

1. **Pediatric indications**
   Prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one-year intervals based upon documentation, including a recent progress report which includes growth and maturation, from the provider that the member's condition has improved based upon the prescriber's assessment while on therapy. GH replacement for pediatric indications is not considered medically necessary if any of the following discontinuation criteria are met:
   - Growth velocity while on therapy is <2.5cm/year (indicating nonresponse to therapy)
   - Expected final adult height has been reached
   - Growth plates have fused
   - Bone age in natal females reaches age 14
   - Bone age in natal males reaches age 16
   - Renal transplantation for CRI

2. **Adult indications**
   Prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon:
   - Chart documentation demonstrating that the member continues to benefit from growth hormone therapy. Specific examples of benefit must be included (e.g., normalization of IGF-1 levels, improvements in cardiovascular risk markers, bone mineral density, body composition, physical exercise tolerance, quality of life, etc.)
   - Evaluation of whether or not there have been any major changes in clinical status affecting the medical necessity of GH supplementation

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- Verification that the member continues to be compliant with GH therapy and recommended follow-up with provider

Diagnoses Not Covered
GH has not been proven to be effective for the following conditions, and thus is not covered by UPMC Insurance Services Division:

1. **Children with:**
   - Idiopathic short stature
   - Constitutionally delayed growth and development (i.e., delayed skeletal maturation with normal growth velocities and rates of bone age advancement, members who are at the lowest 5% of the growth curve at age three)
   - Steroid-induced growth failure
   - Kidney transplant recipients
   - Down syndrome
   - Fanconi's syndrome
   - Bloom syndrome

2. **Adults with:**
   - Chronic fatigue syndrome
   - Fibromyalgia
   - Obesity
   - Athletic performance enhancer
   - Anti-aging treatment
   - Sepsis
   - Burns
   - Trauma
   - Surgery
   - End stage renal disease (ESRD)
   - Wasting associated with:
     - Cancer
     - Organ failure

- **Hepatitis C Treatment Agents** [810, 811, 812, 813, 814, 815, 816, 817, 818, 819, 820, 821]

Criteria:
FDA-approved Indications:
- Must have a diagnosis of hepatitis C
- Must have a quantifiable hepatitis C virus titer (HCV RNA) at baseline. Chart documentation of baseline HCV RNA level, including date and reference range for assay, must be provided.
- Must have chart documentation of test indicating presence or absence of cirrhosis (e.g., liver biopsy, MRI, ultrasound, CT scan, FIB-4 score) and whether cirrhosis is compensated or decompensated if present

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        - If decompensated cirrhosis is present, must use requested regimen in combination with ribavirin, unless documentation of ribavirin ineligibility is provided
    - If genotype is required to confirm regimen selection and treatment duration per AASLD/IDSA hepatitis C guidelines, chart documentation of laboratory genotype result is required
    - Must have documentation of hepatitis C treatment history, including prior treatment regimen(s) and response
    - Must have chart documentation of recommended resistance-associated substitution (RAS) testing if RAS testing is recommended by AASLD/IDSA hepatitis C guidelines
    - Must be prescribed an appropriate treatment regimen based on member characteristics per the FDA-approved package labeling or AASLD/IDSA hepatitis C guidelines

Hepatitis C Treatment Regimens

- Drug selection and treatment duration follows the most recent AASLD/IDSA hepatitis C guideline recommendations.
- Mavyret and sofosbuvir-velpatasvir (Epclusa authorized generic) are the preferred hepatitis C medications for all members where recommended in the AASLD/IDSA hepatitis C guidelines. In clinical situations where the preferred medications are not indicated, an exception can be made for another guideline-supported medication.
- Alternative medications will be considered based on the level of evidence and strength of the recommendation provided in the AASLD/IDSA hepatitis C guidelines.

If the above criteria are met, the requested hepatitis C regimen is approved according to the AASLD/IDSA hepatitis C guideline duration specified for the indication.

Off-label Uses

1. Transplant with Hepatitis C infected organ:
    - Must have transplant or planned transplant with a Hepatitis C infected organ
    - Mavyret and sofosbuvir-velpatasvir (Epclusa authorized generic) are the preferred hepatitis C medications for all members where recommended in the AASLD/IDSA hepatitis C guidelines. In clinical situations where the preferred medications are not indicated, an exception can be made for another guideline-supported medication.

If the above criteria are met, the requested hepatitis C regimen is approved according to the AASLD/IDSA hepatitis C guideline duration specified for the indication.

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- **Hereditary transthyretin-mediated amyloidosis (hATTR) Products** [822, 823,824, 825, 826, 827, 828, 829, 830, 831]
  - Applies to: Wainua™
  - For Onpattro™ and Amvuttra™ requests, see RX.PA.406 Commercial, Exchange Medical Prior Authorization Policies

Initial Authorization Criteria:

- Must be prescribed by or in consultation with a neurologist who specializes in the treatment of amyloidosis
- Must be age 18 years or older
- Must have a diagnosis of polyneuropathy of hereditary transthyretin-mediated amyloidosis confirmed by BOTH of the following:
  - A pathogenic *TTR* mutation (a copy of the genetic test must be provided)
  - Chart documentation of symptomatic polyneuropathy associated with hereditary transthyretin-mediated amyloidosis including ONE of the following:
    - Peripheral sensorimotor polyneuropathy (e.g., tingling, pain, numbness, weakness in the extremities, carpal tunnel syndrome, loss of ability to sense temperature, difficulty with fine motor skills, difficulty walking)
    - Autonomic neuropathy symptoms (e.g., orthostasis, abnormal sweating, sexual dysfunction, recurrent urinary tract infection, dysautonomia [constipation and/or diarrhea, nausea, vomiting, anorexia, early satiety])
- Must have documentation of a baseline staging score that meets ONE of the following:
  - Polyneuropathy Disability (PND) score I-IIIb
  - Familial Amyloidotic Polyneuropathy (FAP) stage 1-2
- For eplontersen (Wainua):
  - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ONE of the following:
    - Vutrisiran (Amvuttra)
    - Patisiran (Onpattro)
- Must not be used in combination with another TTR-lowering agent (patisiran [Onpattro], vutrisiran [Amvuttra], eplontersen [Wainua]) or a TTR-stabilizing agent (tafamidis [Vyndaqel, Vyndamax], acoramidis [Attruby])

If the above criteria are met, the requested product is approved for 1 year.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon the following:

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- Must have chart documentation showing stabilization or improvement in clinical signs or symptoms of polyneuropathy, PND score, FAP stage, or impact on quality of life (QOL)
- Must provide current documentation of either the PND score or FAP stage
  - Must have a PND score below IV or FAP stage below 3, unless there is documentation showing improvement in QOL

• Ibrexafungerp (Brexafemme®) [832]

Initial Authorization Criteria:

- Must have ONE of the following diagnoses confirmed by lab results showing a positive culture:
  - Recurrent vulvovaginal candidiasis (RVVC) with documentation of at least 3 episodes of symptomatic vulvovaginal candidiasis within a year
  - Vulvovaginal candidiasis (VVC)
- For natal females of reproductive potential:
  - Must have documentation of a negative pregnancy test prior to starting ibrexafungerp (Brexafemme) including the date the test was taken
  - Must be using effective contraception
- Must have an adequate trial of fluconazole with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy at the indication-specific doses listed below:
  - For RVVC: must have a trial of fluconazole 150 mg weekly for at least 6 months
  - For VVC: must have a trial of 1 dose of fluconazole 150 mg

If the above criteria are met, ibrexafungerp (Brexafemme) is approved for a duration based upon diagnosis as follows:

- For RVVC: 1 treatment course of 6 months
- For VVC: 1 week

Reauthorization Criteria:

Additional authorizations for treatment are made on a case-by-case basis, require lab results confirming a recurrence, and require chart documentation describing the previous response and clinical rationale for re-treatment.

• Iloperidone (Fanapt®) [833, 834, 835, 836]

Criteria:

- Must be prescribed by or in consultation with a psychiatrist
- Must be age 18 years or older

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        - For members <12 years of age, please refer to the 'Atypical and Typical Antipsychotics – Children Under 12 years of Age' criteria above
    - Must have a diagnosis of schizophrenia or manic or mixed episodes associated with bipolar I disorder
    - Must have an adequate trial, of at least 4 weeks, with an inadequate response or significant side effect/toxicity or have a contraindication to FOUR generic atypical antipsychotics

If the above criteria are met, iloperidone (Fanapt) is approved.

- **Iloprost (Ventavis®)** [837, 838, 839, 840]

Initial Authorization Criteria:

- Must be prescribed by a cardiologist or pulmonologist who specializes in pulmonary hypertension
- Must have a diagnosis of PAH (WHO Group 1) with WHO functional class III or IV symptoms confirmed by right heart catheterization (RHC); must provide a copy of the RHC showing ALL of the following:
    - Mean pulmonary arterial pressure (mPAP) ≥25 mmHg
    - Pulmonary capillary wedge pressure (PCWP), left atrial pressure, or left ventricular end-diastolic pressure (LVEDP) ≤15 mmHg
    - Pulmonary vascular resistance >3 Wood units

If the above criteria are met, iloprost (Ventavis) is approved for 3 months.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the prescriber that the member's condition has improved or the member continues to benefit from therapy.

- **Immune Globulin – Intravenous & Subcutaneous (IVIG & SCIG)** [841, 842, 843, 844, 845, 846, 847, 848, 849, 850, 851, 852, 853, 854, 855, 856, 857, 858, 859, 860, 861, 862, 863, 864, 865, 866, 867, 868, 869, 870, 871, 872, 873, 874, 875, 876, 877, 878, 879, 880, 881, 882, 883, 884, 885, 886, 887, 888, 889, 890, 891, 892, 893, 894, 895, 896, 897, 898, 899, 900, 901, 902, 903, 904, 905, 906, 907, 908, 909, 910, 911, 912, 913, 914, 915, 916, 917, 918, 919, 920, 921, 922, 923, 924, 925, 926, 927, 928, 929, 930, 931, 932, 933, 934, 935, 936, 937, 938, 939, 940, 941, 942, 943, 944, 945, 946, 947, 948, 949, 950, 951, 952, 953, 954, 955, 956, 957, 958, 959, 960, 961, 962, 963,

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964, 965, 966, 967, 968, 969, 970, 971, 972, 973, 974, 975, 976, 977, 978, 979, 980, 981, 982, 983, 984, 985, 986, 987, 988, 989, 990, 991, 992, 993, 994, 995, 996, 997, 998, 999, 1000, 1001, 1002

- For Gamastan and Gamstan S/D requests, see RX.PA.406 Commercial, Exchange Medical Prior Authorization Policies

Criteria:

Requests for Immune Globulin Subcutaneous (Human) - hipp (Cutaquig®) and Immune Globulin Subcutaneous, Human-klhw (Xembify®):

- Must meet the indication-specific criteria listed below
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
  - Immune Globulin Subcutaneous (Human) (Cuvitru®)
  - Immune Globulin Subcutaneous (Human) (IGSC) (Hizentra®)
  - Immune Globulin Infusion (Human) with recombinant human hyaluronidase (Hyqvia®)
- Must provide clinical rationale for use of the requested product instead of the prerequisite medication trials. Clinical rationale must include why the prerequisite medication trials would not be expected to produce the same clinical results as the requested product.

Requests for Immune Globulin Intravenous, Human-stwk (Alyglo™), Immune Globulin Intravenous, Human-slra (Asceniv™), Immune Globulin Intravenous (Human) (Bivigam®), Immune Globulin (Human) Injection (Flebogamma®), Immune Globulin Intravenous [Human] (Gammaplex®), Immune Globulin Intravenous [Human] (Panzyga®), Immune Globulin Intravenous, Human-dira (Yimmugo™):

- Must meet the indication-specific criteria listed below
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
  - Immune Globulin Infusion (Human) (Gammagard®) or Immune Globulin Intravenous (Human) (Gammagard® S-D)
  - Immune Globulin Injection (Human) (Gammaked™)
  - Immune Globulin Injection (Human) (Gamunex®-C)
  - Immune Globulin Intravenous (Human) (Octagam®)
  - Immune Globulin Intravenous (Human) (Privigen®)
- Must provide clinical rationale for use of the requested product instead of the prerequisite medication trials. Clinical rationale must include why the prerequisite medication trials would not be expected to produce the same clinical results as the requested product.

FDA-approved Indications

1. For Primary Immunodeficiency
   - Syndromes may include:
     - Common Variable Immunodeficiency (Hypogammaglobulinemia)
     - Congenital Agammaglobulinemia

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        - Bruton's or X-linked Agammaglobulinemia
        - Severe Combined Immunodeficiency (SCID)
        - X-linked Hyper-IgM Syndrome
        - Wiskott-Aldrich Syndrome
        - Hypergammaglobulinemia Types
    - Must be prescribed by or in consultation with an immunologist or hematologist
    - Must have deficient antibody production, as evidenced through a documented IgG level ≤ 500mg/dL
        - Requests with IgG levels > 500mg/dL require chart documentation that provides clinical rationale for the use of IVIG or SCIG.
    - Must have history of at least 1 bacterial infection directly attributable to this deficiency
    - Approve for 1 year initially
    - Reauthorization requests are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation showing the following:
        - An updated IgG level
        - Improvement in the member's condition as a result of treatment

1. <u>For Idiopathic or Immune Thrombocytopenic Purpura (ITP)</u>
   (Platelet counts expressed per mm^3)
    - Must be prescribed by a hematologist or oncologist
    - For children with ITP:
        - Must have ONE of the following:
            - Platelet count < 20,000 and significant mucous membrane bleeding
            - Platelet count < 10,000 and minor purpura
            - Platelet count < 20,000 and inaccessibility or noncompliance is a concern
            - Any surgery, dental extractions, or other procedures likely to cause blood loss are needed
    - For adults with ITP:
        - Must have ONE of the following:
            - Platelet count < 30,000 and documented previous inadequate response or intolerance to corticosteroids
            - Surgical procedures likely to cause blood loss are needed based upon the following platelet counts: dentistry ≤ 10,000, except teeth extractions or regional dental block ≤ 30,000
            - Minor surgery ≤ 50,000
            - Major surgery ≤ 80,000
    - For pregnant natal females with ITP:

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        - Must be pregnant and have ONE of the following:
            - Platelet count is < 100,000
            - History of splenectomy
            - Previously delivered infants with autoimmune thrombocytopenia
    - Approve for 1 month
    - Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response and clinical rationale for re-treatment.

2. For Kawasaki Disease
    - Must be receiving aspirin concomitantly
    - Must be requesting treatment within the first 10 days of illness, or if > 10 days, must have persistent fever (without other explanation), aneurysms, and ongoing systemic inflammation
    - Approve for 1 dose
    - May approve second dose in patients who fail to respond to initial therapy
    - Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response and clinical rationale for re-treatment.

3. For Chronic B-cell Lymphocytic Leukemia
    - Must be prescribed by a hematologist, oncologist, or infectious disease specialist
    - Must have Hypogammaglobulinemia [Immunoglobulin G (IgG)
    - < 500 mg/dl]
    - Must have previous history of serious bacterial infection (requiring antibiotics)
    - Approve for 1 year
    - Reauthorization requests are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved while on therapy.

4. For HIV (Human Immunodeficiency Virus) in pediatric patients
    - Must be prescribed by an immunologist or infectious disease specialist
    - Must be < 13 years of age
    - Must have CD4 (cluster of differentiation 4) count ≥ 200/mm3
    - Must have ONE of the following
        - Recurrent (2 or more) serious bacterial infections such as bacteremia, meningitis, or pneumonia during a 1-year period despite administration of highly active antiretroviral therapy (HAART) and prophylactic sulfamethoxazole/trimethoprim (TMP-SMZ) or other antimicrobials

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        - Hypogammaglobulinemia with an IgG < 400 mg/dL
        - Absence of detectable antibodies to common antigens, (measles, pneumococcal, and/or haemophilus influenza Type B)
        - Bronchiectasis not optimally responsive to antibiotics and pulmonary therapy
        - A need for passive immunization for measles if Intramuscular Immune Globulin (IMIG) is contraindicated. Intramuscular (IM) injection contraindicated with severe thrombocytopenia or any coagulation disorder.
    - Approve for 1 year
    - Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response and clinical rationale for re-treatment.

5. For Chronic Inflammatory Demyelinating Polyneuropathy (CIDP)
    - Must be prescribed by a neurologist
    - Must have a diagnosis of CIDP
    - Must provide documentation of electrodiagnostic testing [an electromyography (EMG) report] which supports the diagnosis of CIDP
    - Must have significant disability in upper and lower limb function as defined as having arm disability of INCAT grade 2 or higher OR leg disability of INCAT grade 1 or higher
    - Approve for 6 months
    - Reauthorization requests are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation showing the following:
        - Improvement in the member's condition as a result of treatment as evidenced by improvement or stability in INCAT disability scores

6. For Multifocal Motor Neuropathy
    - Must be prescribed by a neurologist
    - Must provide documentation of electrodiagnostic testing (e.g., an EMG report, nerve conduction study) that demonstrates conduction block
    - Approve for 3 months
    - Reauthorization requests are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved while on therapy.

Off-label Uses

1. For Guillain-Barre Syndrome
    - Must have been diagnosed within 4 weeks of onset of neuropathic symptoms
    - Must be non-ambulating independently

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- Approve for 1 month
- May approve 1 additional month
- Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response and clinical rationale for re-treatment.

2. For Dermatomyositis
   - Must have a diagnosis of dermatomyositis confirmed by biopsy
     - Biopsy not required for pediatric members with pathogenomic skin rash (e.g., heliotrope rash, Gottron's papules, and/or Gottron's sign)
   - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to BOTH of the following:
     - Corticosteroids for 3 months
     - Concomitant adjuvant therapy (azathioprine, methotrexate, cyclosporine)
   - Approve for 2 months
   - Reauthorization requests are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved while on therapy.

3. For Systemic Lupus Erythematosus (SLE)
   - Must have severe active SLE
   - Must have tried and failed or have contraindications to ALL of the following:
     - Corticosteroids
     - Antimalarials
     - 1 additional immunosuppressant (azathioprine, cyclophosphamide, cyclosporine, methotrexate)
   - Approve for 3 months
   - Reauthorization requests are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved while on therapy.

4. For Autoimmune Mucocutaneous Blistering Disease (AMBD)
   - Must have biopsy-proven pemphigus vulgaris, pemphigus foliaceus, bullous pemphigoid, mucous membrane pemphigoid (e.g., cicatricial pemphigoid), and epidermolysis bullosa acquisita
   - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
     - Systemic corticosteroid or immunosuppressive agent (e.g., azathioprine, methotrexate, mycophenolate)
     - A rituximab product
     - For bullous pemphigoid, dupilumab (Dupixent)

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        - NOTE: an exception to the requirement for the above trials can be made in the case of rapidly progressive, extensive, or debilitating disease
    - Approve for 4 months
    - Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response and clinical rationale for re-treatment.

5. <u>For Myasthenia Gravis Syndrome</u>
   For patients age 18 years and older:
    - Must have a diagnosis of Myasthenia Gravis
    - Must be prescribed by a neurologist
    - For acute use:
        - Chart documentation of acute exacerbation and impaired function is required (e.g., respiratory insufficiency, inability to swallow)
        - Approve for 1 month
        - Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response and clinical rationale for re-treatment.
    - For temporary use as a bridge to immunotherapy:
        - Must have history of myasthenia gravis exacerbation
        - Must be recently started (within 3 months) on immunosuppressant therapy (e.g., azathioprine, mycophenolate, cyclosporine, or tacrolimus)
        - Chart documentation of use as bridge therapy is required
        - Approve for 6 months
        - Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response and clinical rationale for re-treatment.
    - For stabilization prior to surgery:
        - Must have history of myasthenia gravis with current or previous difficulty with swallowing, speech, or respiratory involvement (e.g., shortness of breath or reduced force vital capacity on pre-op pulmonary function test.) Chart documentation of symptoms is required.
        - IVIG infusion must be scheduled within 14 days of anticipated surgery date.
        - Approve for 1 month
        - Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response and clinical rationale for re-treatment.

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- For chronic use in refractory disease:
  - Must have an adequate trial with an inadequate response or significant side effects/toxicities or have contraindications to both of the following:
    - Cholinesterase inhibitors (e.g., pyridostigmine, neostigmine)
    - Corticosteroids
  - Must have an adequate trial of at least 3 months each with inadequate response significant side effects/toxicity or have a contraindication to therapy of TWO of the following:
    - Azathioprine
    - Mycophenolate mofetil
    - Cyclosporine
    - Tacrolimus
    - Methotrexate
  - Must have chart documentation of persistent symptoms that limit daily function as defined by patient and physician
  - Approve for 6 months
  - Reauthorization requests are reviewed on a case-by-case basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon chart documentation showing the following:
    - Previous response to treatment including improvement in the symptoms that limit daily function

For chronic use in refractory disease in patients age 17 years and younger:

- Must have a diagnosis of Myasthenia Gravis
- Must be prescribed by a pediatric neurologist
- Must have chart documentation of symptoms that limit daily function (e.g., muscle weakness that leads to falling, double vision, difficulty with breathing, speech, or swallowing)
- Must have an adequate trial with an inadequate response or significant side effects/toxicities or have contraindications to both of the following:
  - Cholinesterase inhibitors (e.g., pyridostigmine, neostigmine)
  - Corticosteroids
- Approve for 6 months
- Reauthorization requests are reviewed on a case-by-case basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon chart documentation showing the following:
  - Previous response to treatment including improvement in the symptoms that limit daily function

6. For Parvovirus B19 Infection

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- Must have documentation (e.g., Polymerase Chain Reaction test result) confirming presence of Human Parvovirus B-19 (HPV-B19) infection
- Must have severe anemia defined as hemoglobin level < 8 ng/dL
- Must have low reticulocyte count defined as < 8x109/L
- Must have history of immunodeficiency due to suppressive medications or HIV
- Approve for 1 month
- Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response and clinical rationale for re-treatment.

7. For Transplant Desensitization
   - Must be prescribed by a transplant specialist
   - Must be awaiting solid organ or bone marrow transplant
   - Must provide documentation of rationale for desensitization (e.g., risk of infection, panel reactive antibody (PRA) > 10%, prior transplant recipient)
   - Approve for 4 months
   - Reauthorization requests are reviewed on a case-by-case basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended for an appropriate duration based upon documentation showing the following:
     - The member has not received a transplant

8. For Renal Post- Transplant Rejection
   - Must have received a renal transplant from a living donor with post-transplant rejection
   - Approve for 1 month
   - Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response and clinical rationale for re-treatment.

9. For Allogenic or Hematopoietic Stem Cell Transplantation (HSCT or Bone Marrow Transplant)
   - Must have severe hypogammaglobulinemia (IgG < 400 mg/dL)
   - Must have history of recurrent infections
   - Approve for 6 months
   - Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response and clinical rationale for re-treatment.

10. For Autoimmune Hemolytic Anemia
    - Must have warm-type diagnosis
    - Must have a trial and failure or have contraindications to corticosteroids
    - Approve for 1 month

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- Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response and clinical rationale for re-treatment.

11. For Stiff Person Syndrome
    - Must have a diagnosis of Stiff-Person Syndrome confirmed by chart documentation of ONE of the following:
        - Electromyography (EMG) showing continuous motor unit activity
        - High titer of anti-glutamic acid decarboxylase (anti-GAD) antibodies, defined as > 10,000 IU/mL by enzyme assay
        - GAD antibodies in the cerebral spinal fluid
    - Must be prescribed by a neurologist
    - Must have an adequate trial with inadequate responses, significant side effects/toxicities, or have contraindications to THREE of the following:
        - Corticosteroids
        - Antiepileptics
        - Benzodiazepines
        - Muscle relaxants
        - Gabapentin
    - Must submit chart documentation of a clinical workup to rule out other medical and neurologic conditions
    - Approve for 4 months
    - Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response and clinical rationale for re-treatment.

12. For Myelin Oligodendrocyte Glycoprotein Antibody associated disorder (MOGAD)
    - Must be prescribed by a neurologist
    - Must have a diagnosis of MOGAD confirmed by BOTH of the following:
        - Must have myelin oligodendrocyte glycoprotein (MOG) antibodies per enzyme assay (chart documentation must be provided)
        - Must have an MRI screening and submit documentation of a clinical workup to rule out other medical and neurologic conditions (e.g., multiple sclerosis, NMOSD)
    - Must have documentation of symptomatic disease (e.g., transverse myelitis, optic neuritis, acute disseminated encephalomyelitis)
    - Must have an adequate trial of intravenous corticosteroids (e.g., methylprednisolone), including taper with an inadequate response or significant side effects/toxicities or have a contraindication to this therapy
    - Approve for 6 months
    - Reauthorization requests are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be

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extended at one-year intervals based upon documentation from the provider that the member's condition has stabilized or improved while on therapy.

13. For Hypogammaglobulinemia secondary to disease modifying therapy
    - Must have IgG < 400 mg/dL
    - Must be on medication therapy with known b-cell lowering effects (e.g., rituximab product, ocrelizumab)
    - Approve for 6 months
    - Reauthorization requests are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations are subject to the above criteria may be extended at one-year intervals based upon documentation from the provider that the member's condition has stabilized or improved while on therapy.

- **Inebilizumab-cdon (Uplizna®)** 1003, 1004, 1005, 1006

Initial Authorization Criteria:

1. **For the diagnosis of antiaquaporin-4 antibody positive (AQP4) neuromyelitis optica spectrum disorder (NMOSD)**
    - Must have a negative tuberculosis skin test [such as Tuberculin PPD (purified protein derivative) test] or Interferon-Gamma Release Assay (IGRA) whole-blood test [such as QuantiFERON®-TB Gold In-Tube test (QFT-GIT) or T-SPOT®.TB test (T-Spot)]
    - Must be prescribed by a neurologist
    - Must be age 18 years or older
    - Must have a diagnosis of antiaquaporin-4 antibody positive NMOSD. Chart documentation of a positive serological test of AQP4 antibodies is required
    - Must have an MRI screening to confirm diagnosis and rule out other neurologic disease
    - Must have symptomatic disease (e.g., myelitis, optic neuritis, postrema syndrome, diencephalic syndrome, brainstem syndromes, CNS mass lesions)
    - Must have history of a prior NMOSD exacerbation treated with corticosteroids or plasma exchange. Documentation is required.
    - Must have an adequate trial of a rituximab product with an inadequate response, significant side effect/toxicity or have a contraindication to this therapy
    - Must not use a biologic agent in combination with inebilizumab-cdon (Uplizna)
    - Must not have active Hepatitis B infection

2. **For the diagnosis of immunoglobulin G4-related disease (IgG4-RD)**
    - Must have a negative tuberculosis skin test [such as Tuberculin PPD (purified protein derivative) test] or Interferon-Gamma Release Assay (IGRA) whole-

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blood test [such as QuantiFERON®-TB Gold In-Tube test (QFT-GIT) or T-SPOT®.TB test (T-Spot)]
- Must be prescribed by a immunologist
- Must be age 18 years or older
- Must have a diagnosis of immunoglobulin G4-related disease (IgG4-RD) confirmed by biopsy. Chart documentation of biopsy is required.
- Must have a clinical workup to rule out other conditions (e.g., imaging [computed tomography or magnetic resonance imaging, laboratory testing with elevated IgG4 concentration]. Chart documentation is required.
- Must have history of IgG4-RD flare with clinical response to treatment with corticosteroids
- Must have an adequate trial of a rituximab product with an inadequate response, significant side effect/toxicity or have a contraindication to this therapy
- Must not use a biologic agent in combination with inebilizumab-cdon (Uplizna)
- Must not have active Hepatitis B infection

3. **For anti-acetylcholine receptor antibody (AchR) positive or anti-muscle specific tyrosine kinase (MuSK) antibody positive myasthenia gravis (MG)**
   - Must be prescribed by a neurologist
   - Must be age 18 years or older
   - Must have a diagnosis of anti-acetylcholine receptor (AChR) or anti-muscle specific tyrosine kinase (MuSK) antibody positive (MuSK) generalized myasthenia gravis. Chart documentation of a positive serological test with presence of autoantibodies against AChR or MuSK antibodies must be provided.
   - Must have history of myasthenia gravis with chart documentation of symptoms that impair the ability to perform daily functions and essential activities of daily living (e.g., difficulty chewing, swallowing, or breathing). Chart documentation must be provided.
   - Must have an adequate trial of corticosteroid with inadequate response, significant side effect/toxicity or have a contraindication to this therapy
   - Must have an adequate trial of at least 3 months of ONE of the following immunosuppressants with an inadequate response OR have a significant side effects/toxicity or have a contraindication to TWO of the following:
     - Azathioprine
     - Mycophenolate mofetil
     - Cyclosporine
     - Tacrolimus
   - For AchR positive generalized myasthenia gravis:
     - Must have a trial of two 4-week courses of efgartigimod alfa-fcab (Vyvgart) with an inadequate response, significant side effect/toxicity or have a contraindication to this therapy

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        - NOTE: A trial of efgartigimod alfa hyaluronidase (Vyvgart Hytrulo) is acceptable for the efgartigimod alfa-fcab (Vyvgart) trial
    - For MuSK positive generalized myasthenia gravis:
        - Must have an adequate trial of a rituximab product with inadequate response, or significant side effect/toxicity or have a contraindication to this therapy
    - Must be on concomitant therapy with conventional therapy (e.g., pyridostigmine, steroids, non-steroidal immunosuppressive therapies)
    - Must not use in combination with ANY of the following:
        - An eculizumab product
        - Efgartigimod alfa-fcab (Vyvgart)
        - Efgartigimod alfa and hyaluronidase (Vyvgart Hytrulo)
        - Rozanolixizumab-noli (Rystiggo)
        - Ravulizumab-cwvz (Ultomiris)
        - Zilucoplan (Zilbrysq)
        - Nipocalimab-aahu (Imaavy)

If the above criteria are met, inebilizumab-cdon (Uplizna) is approved for 6 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon chart documentation that the member's condition has improved while on therapy or the member continues to benefit from therapy.

• Inhaled levodopa (Inbrija™) [1007, 1008, 1009]

Initial Authorization Criteria:
- Must be prescribed by or in consultation with a neurologist
- Must have a diagnosis of Parkinson's disease with motor fluctuations and "off" episodes
- Must have continued "off" symptoms despite adherence to optimized medication therapy regimen based on the prescriber's assessment
- Must have an attestation that the member's dexterity and ability to use this device has been assessed
- Must be on concomitant therapy with carbidopa/levodopa
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to TWO of the following, taken in combination with carbidopa/levodopa for the adjunctive treatment of "off" episodes:
    - Entacapone
    - A generic MAO-B inhibitor (e.g., selegiline or rasagiline)
    - A generic dopamine agonist (e.g., ropinirole tablet, pramipexole tablet)

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        - Istradefylline (Nourianz)
    - Must not have asthma, COPD, or other chronic underlying pulmonary disease
    - Must not be on concomitant therapy with non-selective monoamine oxidase (MAO) inhibitors (e.g. phenelzine, isocarboxazid, tranylcypromine)

If the above criteria are met, inhaled levodopa (Inbrija) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for the continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved while on therapy or the member continues to benefit from therapy.

- **Injectable Calcitonin-Gene Receptor Peptide (CGRP) Antagonists** 1010, 1011, 1012, 1013, 1014, 1015, 1016, 1017, 1018, 1019, 1020, 1021
    - Applies to: erenumab-aooe (Aimovig™), fremanezumab-vfrm (Ajovy®), and galcanezumab-gnlm (Emgality™)

Initial Authorization Criteria:

1. **For migraine in adults**
    - Must be 18 years or older
    - Must have a diagnosis of migraine
        - Must have at least 4 migraine days per month
    - Must have an adequate trial of at least 2 months each within 2 prophylactic therapy classes including: beta-blockers, anticonvulsants, and antidepressants (TCAs, SSRIs, SNRIs) with an inadequate response or significant side effects/intolerance.
        - For members in whom one of these therapy classes is not clinically appropriate, additional prophylactic therapy classes to be used are calcium channel blockers or angiotensin converting enzyme inhibitors (ACEIs)
    - Must not be using concomitantly with onabotulinumtoxinA (Botox)
2. **For fremanezumab-vfrm (Ajovy®) for episodic migraine prevention in pediatrics**
    - Must be 6-17 years or older
    - Must have a diagnosis of migraine
        - Must have at least 4 migraine days per month
    - Must not be using concomitantly with onabotulinumtoxinA (Botox)

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If the above criteria are met, the requested CGRP antagonist is approved for 1 year. In addition, for galcanezumab-gnlm (Emgality), an initial loading dose of 240 mg (2 syringes) is approved for the first month.

3. **For galcanezumab-gnlm (Emgality) for Episodic Cluster Headache**
   - Must be prescribed by or in consultation with a neurologist or by a provider with certification in headache medicine from the United Council for Neurologic Subspecialties (UCNS) or the National Headache Foundation (NHF)
   - Must be age 18 years or older
   - Must have a diagnosis of episodic cluster headache with all of the following:
     - Must have severe or very severe unilateral orbital, supraorbital and/or temporal pain lasting 15-180 minutes when untreated
     - Must have at least one of the following: a sense of restlessness or agitation, conjunctival injection and/or lacrimation, nasal congestion and/or rhinorrhea, eyelid edema, forehead and facial sweating, miosis and/or ptosis
     - Must have cluster attacks that occur with a frequency between 1 every other day and 8 per day
     - Must have pain-free remission periods of ≥ 3 months between cluster attack periods
   - Must currently be in a period of cluster attacks
   - Must have a trial of 2 prophylactic therapies for episodic cluster headache (e.g., lithium, verapamil, suboccipital steroid injection, systemic steroids, melatonin, frovatriptan, valproate, topiramate)

If the above criteria are met, galcanezumab-gnlm (Emgality) is approved for a quantity of 3 fills per 6 months.

<u>Reauthorization Criteria:</u>

1. **For migraine in adults**
   - All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for the continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the prescriber that the member's condition has improved based upon the prescriber's assessment while on therapy.
2. **For fremanezumab-vfrm (Ajovy®) for episodic migraine prevention in pediatrics**
   - All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for the continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the prescriber that the member's condition has improved based upon the prescriber's assessment while on therapy.
3. **For galcanezumab-gnlm (Emgality) for Episodic Cluster Headache**

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- Authorizations may be extended for 3 fills per 6 month intervals for episodic cluster headache based upon:
  - A decrease in frequency, duration, and/or severity of cluster headache from baseline and/or a decrease in the use of medications used for the acute treatment of cluster headache (e.g. sumatriptan, zolmitriptan) as a result of therapy during a current or previous cluster attack
  - Documentation that the member is currently in a period of cluster attacks

- **Interferon Products:** [1022, 1023, 1024, 1025, 1026]
  - Applies to: interferon beta-1a (Avonex®), peginterferon beta-1a (Plegridy®), interferon beta-1a (Rebif®), interferon beta-1b (Betaseron®)
  - Applies to: interferon beta-1b (Extavia®) (Value Choice and Open Choice Formularies only)

Initial Authorization Criteria:
- Must be age 18 years or older
- Must have a diagnosis of relapsing forms of multiple sclerosis, including clinically isolated syndrome, relapsing-remitting disease, and active secondary progressive disease, in adults

If the above criteria are met, the requested product is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for the continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or stabilized while on therapy.

- **Interferon Gamma-1b (Actimmune®)** [1027, 1028, 1029, 1030, 1031]
  - Interferon gamma 1-b (Actimmune) is NOT covered for the following indications:
    - Basal cell carcinoma of the skin
    - Breast cancer
    - Burn infection
    - Chronic Myeloid Leukemia
    - Condyloma acuminatum
    - Graft vs. Host disease
    - Idiopathic Pulmonary Fibrosis
    - Kaposi's Sarcoma
    - Malignant mesothelioma
    - Mycobacteriosis
    - Ovarian cancer

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- Rheumatoid arthritis
- Scleroderma
- Chronic hepatitis B
- Whipple's disease

Initial Authorization Criteria:

1. **Chronic Granulomatous Disease**
   - Must be prescribed by an immunologist, hematologist, infectious disease physician or in consultation with one of these specialists

2. **Severe Malignant Osteopetrosis**
   - Must be prescribed by an orthopedic surgeon, hematologist, or endocrinologist or in consultation with one of these specialists
   - Must have the diagnosis confirmed by radiological evidence

If the above criteria are met, interferon gamma-1b (Actimmune) is approved for 1 year.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at 1-year intervals based upon documentation from the prescriber that the member's condition has improved as a result of treatment.

- **Iptacopan (Fabhalta®)** 1032, 1033, 1034, 1035, 1036, 1037, 1038, 1039, 1040, 1041

Initial Authorization Criteria:

1. **For paroxysmal nocturnal hemoglobinuria (PNH):**
   - Must be prescribed by or in consultation with a hematologist, oncologist, immunologist or genetic specialist
   - Must be age 18 years or older
   - Must have a laboratory confirmed diagnosis of paroxysmal nocturnal hemoglobinuria (PNH) as evidenced by having detectable Glycosyl Phosphatidyl Inositol (GPI)-deficient hematopoietic clones (Type III PNH RBC) via Flow Cytometry. Documentation of Flow Cytometry pathology report must indicate presence of PNH-type RBC (red blood cell) and must be submitted.
   - Must have a baseline LDH level of 1.5 times the upper limit of the normal range (laboratory results with reference range must be submitted)
     - NOTE: Members converting therapy must have a history of an LDH level > 1.5 times the upper limit of the normal range prior to treatment

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with current therapy (laboratory results with reference range must be submitted)
- Must have clinical symptoms of PNH (e.g., fatigue, hemoglobinuria, abdominal pain, shortness of breath, anemia, dysphagia, history of red blood cell transfusion due to PNH, history of a major vascular event [e.g., thrombosis])
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to BOTH of the following:
  - Pegcetacoplan (Empaveli)
  - Eculizumab-aagh (Epsyqli)
    - NOTE: an exception to a trial of eculizumab-aagh (Epsyqli) can be made if the member had a trial of another eculizumab product
- Must provide documentation that the member is vaccinated with immunizations for encapsulated bacteria (e.g., Streptococcus pneumoniae, Neisseria meningitidis, and Haemophilus influenzae type B [Hib]) according to current immunization guidelines at least 2 weeks prior to the administration of the first dose of iptacopan (Fabhalta) unless the risks of delaying treatment outweigh the risks of developing infection per documentation of the prescriber
- Must not use in combination with pegcetacoplan (Empaveli), an eculizumab product, ravulizumab-cwvz (Ultomiris), danicopan (Voydeya), or crovalimab-akkz (PiaSky)

2. For immunoglobulin A nephropathy (IgAN)
- Must be prescribed by a nephrologist
- Must be age 18 years or older
- Must have a diagnosis of primary immunoglobulin A nephropathy (IgAN) confirmed by a kidney biopsy. Documentation of the biopsy results is required.
- Must be at risk of rapid disease progression defined by the following (chart documentation is required):
  - Proteinuria >0.75g/day OR Urinary Protein-to-Creatinine Ratio (UPCR) ≥1.5g/g despite at least 90 days of optimized supportive care treatment with ALL the following:
    - Maximally tolerated dose of an angiotensin converting enzyme inhibitor (ACEi) or an angiotensin receptor blocker (ARB) unless there is a contraindication to these therapies
      - NOTE: for members who have experienced significant side effects/toxicities to one of these classes, an adequate trial of the other class is required (e.g., ACEi-induced cough requires a trial of an ARB)

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        - Other supportive care measures as clinically indicated (e.g., lifestyle modifications, blood pressure management, assessment of cardiovascular risk)
    - Must provide documentation that the member is vaccinated with immunizations for encapsulated bacteria (e.g., Streptococcus pneumoniae, Neisseria meningitidis, and Haemophilus influenzae type B [Hib]) according to current immunization guidelines at least 2 weeks prior to the administration of the first dose of iptacopan (Fabhalta) unless the risks of delaying treatment outweigh the risks of developing infection per documentation of the prescriber
    - Must be on a stable dose of maximally-tolerated renin-angiotensin system (RAS) inhibitor therapy (e.g., angiotensin converting enzyme inhibitor or an angiotensin receptor blocker)
    - Must have an adequate trial of at least 3 months with an inadequate response (proteinuria > 0.75 g/day OR UPCR ≥ 1.5 g/g), significant side effect/toxicity or have a contraindication to BOTH of the following:
        - A sodium-glucose co-transporter 2 (SGLT-2) inhibitor [e.g., empagliflozin (Jardiance)]
        - Sparsentan (Filspari)
    - Must have an attestation from the prescriber that the member is to remain on optimized supportive care as clinically indicated
    - Must not use in combination with the following:
        - Sparsentan (Filspari)
        - Atrasentan (Vanrafia)
        - Sibeprenlimab-szsi (Voyxact)
        - Budesonide DR (Tarpeyo)

3. **For Complement 3 Glomerulopathy (C3G)**
    - Must be prescribed by a nephrologist
    - Must be age 18 years or older
    - Must have a diagnosis of complement 3 glomerulopathy confirmed by a kidney biopsy. Chart documentation of the biopsy results is required.
    - Must have BOTH of the following despite at least 90 days of optimized supportive care (e.g., lifestyle modifications, blood pressure management) and maximally-tolerated renin-angiotensin system (RAS) inhibitor therapy (e.g., angiotensin converting enzyme inhibitor or an angiotensin receptor blocker):
        - Estimated glomerular filtration rate ≥ 30 mL/min/1.73 m^2
        - Urine protein-to-creatinine ratio ≥ 1.0 g/g
    - Must provide documentation that the member is vaccinated with immunizations for encapsulated bacteria (e.g., Streptococcus pneumoniae, Neisseria meningitidis, and Haemophilus influenzae type B [Hib]) according to current immunization guidelines at least 2 weeks prior to the administration

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of the first dose of iptacopan (Fabhalta) unless the risks of delaying treatment outweigh the risks of developing infection per documentation of the prescriber
- Must be on a stable dose of maximally-tolerated renin-angiotensin system (RAS) inhibitor therapy (e.g., angiotensin converting enzyme inhibitor or an angiotensin receptor blocker)
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
  - Glucocorticoids
  - Mycophenolate mofetil
  - Pegcetocoplan (Empaveli)
- Must have an attestation from the prescriber that the member is to remain on optimized supportive care as clinically indicated (e.g., lifestyle modifications, blood pressure management)
- Must not have received a kidney transplant
- Must not use in combination with pegcetocoplan (Empaveli)

If the above criteria are met, iptacopan (Fabhalta) is approved for 6 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon chart documentation showing the following:
For PNH:
- Must have a decrease in LDH level from baseline
- For members who required blood transfusions at baseline:
  - Must have a decreased requirement or no longer require blood transfusions
- Must not use in combination with pegcetacoplan (Empaveli), an eculizumab product, ravulizumab-cwvz (Ultomiris), danicopan (Voydeya), or crovalimab-akkz (PiaSky)

For primary IgAN:
- Must have a reduction in proteinuria OR urinary protein-to-creatinine ratio from baseline
- Must be on optimized supportive care as clinically indicated
- Must be on a stable dose of maximally-tolerated renin-angiotensin system (RAS) inhibitor therapy (e.g., angiotensin converting enzyme inhibitor or an angiotensin receptor blocker)
- Must not use in combination with the following:
  - Sparsentan (Filspari)
  - Atrasentan (Vanrafia)
  - Sibeprenlimab-szsi (Voyxact)
  - Budesonide DR (Tarpeyo)

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For Complement 3 Glomerulopathy (C3G):

- Must have a reduction in proteinuria OR urinary protein-to-creatinine ratio from baseline
- Must be on optimized supportive care as clinically indicated
- Must be on a stable dose of maximally-tolerated renin-angiotensin system (RAS) inhibitor therapy (e.g., angiotensin converting enzyme inhibitor or an angiotensin receptor blocker)
- Must not have received a kidney transplant
- Must not use in combination with pegcetocoplan (Empaveli)

- **Istradefylline (Nourianz™)** [1042, 1043]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with a neurologist
- Must have a diagnosis of Parkinson's disease (PD) and be experiencing "off" episodes
- Must have motor fluctuations with an average daily "off" time of 2 hours or longer
- Must have continued "off" symptoms despite adherence to optimized medication therapy regimen based on the prescriber's assessment
- Must be on concomitant therapy with carbidopa/levodopa
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to TWO of the following, taken in combination with carbidopa/levodopa for the adjunctive treatment of "off" episodes:
  - Entacapone
  - A generic MAO-B inhibitor (e.g., selegiline or rasagiline)
  - A generic dopamine agonist (e.g., ropinirole tablet, pramipexole tablet)

If the above criteria are met, istradfylline (Nourianz) is approved for 1 year.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for the continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved while on therapy or the member continues to benefit from therapy.

- **Itraconazole capsule (Tolsura®)** [1044, 1045, 1046, 1047, 1048, 1049, 1050, 1051]

Initial Authorization Criteria:

- Must have a diagnosis of one of the following:

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        - Aspergillosis, pulmonary and extrapulmonary, in patients who are intolerant of or who are refractory to amphotericin B therapy
        - Blastomycosis, pulmonary and extrapulmonary
        - Histoplasmosis, including chronic cavitary pulmonary disease and disseminated, non-meningeal histoplasmosis
    - Must have an adequate trial and failure of generic itraconazole capsules or a contraindication to this formulation

If the above criteria are met, itraconazole (Tolsura) capsule is approved for a timeframe dependent upon the diagnosis. (Applies to Value Choice and Open Choice formularies only)

Reauthorization Criteria:
All prior authorization renewals are reviewed on a case-by-case basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended on a case-by-case basis based on the diagnosis and documentation of the response to the previous course of treatment.

• Ivabradine (Corlanor®) [1052, 1053, 1054, 1055, 1056]

Criteria:

1. **For adult patients with symptomatic chronic heart failure**
    - Must be prescribed by a cardiologist
    - Must be age 18 years or older
    - Must have a diagnosis of stable, symptomatic chronic heart failure
    - Must have a left ventricular ejection fraction ≤ 35%
    - Must have a sinus rhythm with a resting heart rate of ≥ 70 beats per minute
    - Must currently be receiving a beta-blocker at maximally tolerated dose for heart failure treatment (e.g., metoprolol succinate sustained-release, carvedilol, bisoprolol) unless a prior trial with beta-blocker therapy resulted in significant side effect/toxicity or there is a contraindication to use of beta-blocker therapy (e.g., bronchospastic disease such as chronic obstructive pulmonary disease [COPD] and asthma, severe hypotension or bradycardia)
    - Must have had a hospitalization for heart failure within the past year
2. **For pediatric patients with symptomatic heart failure due to dilated cardiomyopathy**
    - Must be prescribed by a cardiologist
    - Must be age 6 months to 18 years
    - Must have a diagnosis of stable symptomatic heart failure due to dilated cardiomyopathy
    - Must have a left ventricular ejection fraction ≤ 45%
    - Must have a sinus rhythm with a resting heart rate of ≥ 70 beats per minute
    - Must currently be stable on therapy with a beta-blocker for heart failure unless a prior trial with beta-blocker therapy resulted in a significant side

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effect/toxicity or there is a contraindication to the use of beta-blocker therapy (e.g. bronchospastic disease such as asthma, severe hypotension, or bradycardia)
    - If unable to tolerate a beta-blocker, must currently be on stable therapy with at least one other drug used for heart failure (i.e. ACE inhibitors, angiotensin II antagonists, and/or an angiotensin II antagonist-neprilysin inhibitor)

If the above criteria are met, ivabradine (Corlanor) is approved.

- The quantity limit is 60 tablets every 30 days for maintenance dosing. A quantity of 90 tablets every 30 days for the 5mg dose can be approved for one month for initiation of therapy and dose titration.

3. For ivabradine (Corlanor) suspension

- Must meet the above criteria for ivabradine (Corlanor) for pediatric patients with symptomatic heart failure due to dilated cardiomyopathy
- Must have a weight less than 40kg or documentation of the inability to swallow an oral solid dosage form due to age (< 7 years) or medical condition (e.g. nasogastric tube).

If the above criteria are met, ivabradine (Corlanor) suspension is approved.

• Ivermectin (Stromectol®) [1057, 1058]

Initial Authorization Criteria:

- Must be used for the treatment of an infection caused by a parasite or worm (e.g., onchocerciasis, strongyloidiasis)
- Must not be used for the treatment of COVID-19 as ivermectin is not FDA-approved for this use given no evidence of ivermectin improving health outcomes for those infected with COVID-19 has been established in clinical trials

If the above criteria are met, ivermectin (Stromectol) is approved for 30 days.

Reauthorization Criteria:

Requests for additional courses of treatment are reviewed on a case-by-case basis. Requests for additional courses of treatment must:

- Meet the above criteria
- Include chart documentation describing previous response to therapy
- Include clinical rationale for re-treatment

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- **Lanreotide prefilled syringe (J1932) and lanreotide subcutaneous injection (Somatuline® Depot) (J1930)** [1059, 1060, 1061, 1062, 1063, 1064, 1065]

Initial Authorization Criteria:

1. **For acromegaly:**
   - Must be prescribed by or in consultation with an endocrinologist
   - Must be age 18 years or older
   - Must have a confirmed diagnosis of acromegaly to include one of the following baseline (pre-treatment) labs:
     - Elevated serum IGF-1 level for patient's gender and age range. Laboratory reference range must be provided.
     - Elevated growth hormone (GH) level defined as a GH level ≥1ng/mL during oral glucose tolerance test (OGTT)
   - Must have an inadequate response to surgery and/or for whom surgery is not an option
   - For lanreotide (J1932):
     - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ONE of the following:
       - Lanreotide (Somatuline Depot)
       - Octreotide acetate (Sandostatin LAR Depot)
     - Must provide clinical rationale for use of the requested product instead of the prerequisite medication trials. Clinical rationale must include why the prerequisite medication trials would not be expected to produce the same clinical results as the requested product.
2. **For GEP-NET:**
   - Must be prescribed by a hematologist, an oncologist, or a surgical oncologist within hematology/oncology
   - Must have diagnosis of unresectable, well- or moderately-differentiated, locally advanced or metastatic GEP-NET
   - For lanreotide (J1932):
     - Must have an adequate trial of lanreotide (Somatuline Depot) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
     - Must provide clinical rationale for use of the requested product instead of the prerequisite medication trials. Clinical rationale must include why the prerequisite medication trials would not be expected to produce the same clinical results as the requested product.
3. **For Carcinoid Syndrome:**
   - Must be prescribed by a hematologist, oncologist, or surgical oncologist within hematology, oncology
   - Must be age 18 years or older
   - Must have a diagnosis of carcinoid syndrome

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- Must currently not be treated with lanreotide for GEP-NET (additional doses of lanreotide should not be administered)
- For lanreotide (J1932):
  - Must have an adequate trial of lanreotide (Somatuline Depot) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
  - Must provide clinical rationale for use of the requested product instead of the prerequisite medication trials. Clinical rationale must include why the prerequisite medication trials would not be expected to produce the same clinical results as the requested product.

4. ***For off-label use* for GEP-NET when undergoing treatment with lutetium Lu 177 dotatate (Lutathera®):**
   - Must be prescribed by a hematologist, an oncologist, or a surgical oncologist within hematology/oncology
   - Must have a diagnosis of GEP-NET confirmed by chart documentation and be undergoing course of treatment with lutetium Lu 177 dotatate (Lutathera)
   - Must be using for symptom control
   - For lanreotide (J1932):
     - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ONE of the following:
       - Lanreotide (Somatuline Depot)
       - Octreotide acetate (Sandostatin LAR Depot)
     - Must provide clinical rationale for use of the requested product instead of the prerequisite medication trials. Clinical rationale must include why the prerequisite medication trials would not be expected to produce the same clinical results as the requested product.

If the above criteria are met, the requested product is approved for the following duration based upon diagnosis:

- Acromegaly, GEP-NET **without** lutetium Lu 177 dotatate (Lutathera), Carcinoid Syndrome: 6 months
- GEP-NET with lutetium Lu 177 dotatate (Lutathera): 8 months [one course of therapy with lutetium Lu 177 dotatate (Lutathera) (every 8 weeks for 4 doses)]

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved while on therapy.

- **Lecanemab (Leqembi IQLIK)** [1066, 1067, 1068, 1069, 1070, 1071, 1072]

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- For requests for lecanemab (Leqembi™) intravenous, please refer to policy RX.PA.406 Commercial, Exchange Medical Prior Authorization

Initial Authorization Criteria:

- Must be prescribed by a geriatrician, geriatric psychiatrist, neurologist, or neuropsychiatrist
- Must be 50-90 years of age
- Must have a diagnosis of mild cognitive impairment due to Alzheimer's disease or mild Alzheimer's disease dementia including chart documentation of BOTH of the following:
  - A comprehensive neuropsychological evaluation
  - A global Clinical Dementia Rating (CDR) score of 0.5 or 1, Mini Mental State Examination (MMSE) score of 20-30, or Montreal Cognitive Assessment (MoCA) score of 13-30
- Must be prescribed a dose and duration of therapy that is consistent with FDA-approved package labeling, nationally-recognized compendia, or peer-reviewed medical literature
- Must have chart documentation of progressive cognitive decline over months to years
- Must have a positive amyloid PET scan OR documentation of cerebrospinal fluid (CSF) biomarkers consistent with presence of amyloid beta plaques (e.g., elevated P-tau, low Aβ42/Aβ40, low Aβ42/T-tau). A copy of scan results or CSF laboratory results with reference range must be provided.
- Must submit chart documentation of a clinical workup to rule out any medical or neurological condition (e.g., depression, Lewy body dementia, frontotemporal dementia, vascular dementia) or medications that are the primary cause of the member's cognitive impairment
- Must not have ANY of the following:
  - Stroke or transient ischemic attack (TIA) in the past year
  - History of seizures
  - Uncontrolled bleeding disorder [e.g., platelet count less than 50 x 10^9/L, International normalized ratio (INR) greater than 1.5]
  - On anticoagulant therapy (e.g., warfarin, direct oral anticoagulants)
  - History of immunologic or inflammatory disease (e.g., rheumatoid arthritis, Crohn's disease, polymyalgia rheumatica, giant cell arteritis, psoriatic arthritis, systemic lupus erythematosus, amyloid-related angiitis, and cerebral amyloid angiopathy with inflammation)
  - Systemic treatment with immunosuppressants, immunoglobulins, monoclonal antibodies or their derivatives, or plasmapheresis
  - A contraindication to magnetic resonance imaging (MRI) (e.g., pacemaker, foreign metal)
- Must have attestation from the provider of recent (within one year) MRI without any of the following (copy of MRI report must be provided):
  - Clinically significant lesions that could indicate a dementia diagnosis other than Alzheimer's disease

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        - Macrohemorrhage
        - Greater than 4 microhemorrhages
        - Stroke involving a major vascular territory
        - >2 lacunar infarcts
        - Superficial siderosis
        - Evidence of vasogenic edema
        - Evidence of cerebral contusion, encephalomalacia, aneurysms, vascular malformations, infective lesions
        - Space occupying lesions
        - Brain tumors (except for meningiomas or arachnoid cysts that are less than 1 cm at their greatest diameter)
        - Severe white matter/small vessel disease
        - Evidence of amyloid beta-related angiitis (ABRA)
        - Evidence of cerebral amyloid angiopathy-related inflammation (CAA-ri)
    - Must not receive more than one monoclonal antibody directed against amyloid for the treatment of Alzheimer's disease concomitantly
    - Must have attestation from the provider that the member did not develop ANY of the following on brain MRI or clinical assessment with treatment with a previous anti-amyloid monoclonal antibody:
        - Any macrohemorrage
        - More than 1 area of superficial siderosis
        - More than 10 microhemorrhages
        - More than 2 episodes of amyloid-related imaging abnormalities (ARIA)
        - Severe symptoms of ARIA (e.g., seizures, encephalopathy, stupor, any symptom requiring hospitalization or high-dose glucocorticoid therapy)
    - Must be using as maintenance therapy after receiving lecanemab (Leqembi) intravenous for at least 18 months

If the above criteria are met, lecanemab (Leqembi IQLIK) is approved for 6 months.

Reauthorization Criteria:

- All prior authorization renewals are reviewed every 6 months to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at 6-month intervals based upon ALL of the following:
    - The member continues to be treated by a geriatrician, geriatric psychiatrist, neurologist, or neuropsychiatrist
    - Must be prescribed a dose and duration of therapy that is consistent with FDA-approved package labeling, nationally-recognized compendia, or peer-reviewed medical literature
    - Must not receive more than one monoclonal antibody directed against amyloid for the treatment of Alzheimer's disease concomitantly

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- Documentation that the member is being monitored for ARIA including chart documentation of clinical assessment for symptoms of ARIA (e.g., headache, confusion, delirium, altered mental status, disorientation, dizziness/vertigo, visual disturbance, nausea)
- Must show that continuation of treatment is considered medically necessary by demonstrating ALL of the following:
    - Chart documentation from the provider that the member's condition has improved or stabilized based upon the prescriber's assessment while on therapy or the member continues to benefit from therapy
    - Chart documentation of MMSE or MoCA score within the previous 12 months indicating that the member has not progressed to moderate or severe Alzheimer's disease (MMSE score less than 20 or MoCA score less than 13)
    - Attestation from the prescriber that the member does not have current symptomatic ARIA, moderate or severe radiographic ARIA-edema (ARIA-E), or unstable moderate or severe radiographic ARIA-hemorrhage (ARIA-H)
- Attestation from the provider that the member did not develop ANY of the following on brain MRI or clinical assessment (reauthorization will not be granted after the occurrence of any item below):
    - Any macrohemorrage
    - More than 1 area of superficial siderosis
    - More than 10 microhemorrhages since treatment initiation
    - More than 2 episodes of ARIA
    - Severe symptoms of ARIA (e.g. seizures, encephalopathy, stupor, any symptom requiring hospitalization or high-dose glucocorticoid therapy)
    - Patient requires treatment with an anticoagulant

- **Lenacapavir (Sunlenca®)** [1073, 1074]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with an infectious disease or HIV specialist
- Must be age 18 years or older
- Must have a diagnosis of multidrug resistant HIV-1 infection with failure of current antiretroviral regimen due to resistance, intolerance, or safety considerations. Chart documentation must be provided.
- Must be used in combination with other antiretroviral agents
- Must have a requested dose that falls within the recommended dosing guidelines from the manufacturer

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If the above criteria are met, lenacapavir (Sunlenca) is approved for a duration based upon the dosage form as follows:
- Tablets: 1 treatment course for 1 month
- Vials: 1 year

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or stabilized while on therapy.

- **Leniolisib (Joenja®)** [1075, 1076, 1077]

Initial Authorization Criteria:
- Must be prescribed by an immunologist or a geneticist
- Must be age 12 years or older
- Must weigh at least 45 kilograms. Documentation of weight must be provided.
- Must have a diagnosis of activated phosphoinositide 3-kinase delta syndrome (APDS) confirmed by chart documentation of BOTH of the following:
  - Genetic test result showing a phosphoinositide 3-kinase delta (PI3Kδ) mutation with a pathologic variant in PIK3CD or PIK3R1 genes. Copy of genetic test result must be provided.
  - Clinical symptoms consistent with APDS (e.g., history of recurrent sinopulmonary infections, recurrent herpesvirus infections, lymphadenopathy, hepatomegaly, splenomegaly, nodular lymphoid hyperplasia, autoimmunity, cytopenia, enteropathy, bronchiectasis, organ dysfunction).

If the above criteria are met, leniolisib (Joenja) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon chart documentation that the member's condition has improved while on therapy.

- **Letermovir tablet and oral pellets (Prevymis™)** [1078, 1079, 1080]
  - Requests for letermovir intravenous (Prevymis IV) please refer to policy RX.PA.406 Commercial, Exchange Medical Prior Authorization Policies

Initial Authorization Criteria:

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1. **For CMV Prophylaxis in Hematopoietic Stem Cell Transplant (HSCT) Recipients**
   - Must be prescribed by a hematologist, oncologist, infectious disease physician, or transplant physician
   - Must be age 6 months or older and weigh ≥ 6 kg
   - Must be used for prophylaxis of cytomegalovirus (CMV) infection and disease in CMV-seropositive recipients [R+] of an allogeneic hematopoietic stem cell transplant (HSCT)
   - Must not be used concomitantly with any of the following;
     - Pimozide
     - Ergot alkaloids (e.g., ergotamine, dihydroergotamine)
     - Pitavastatin and simvastatin when co-administered with cyclosporine
   - For letermovir (Prevymis) oral pellets:
     - Must have documentation of the inability to swallow an oral solid dosage form due to age (< 7 years), medical condition (e.g., nasogastric tube), or other clinical rationale for why letermovir (Prevymis) tablets cannot be used

2. **For CMV Prophylaxis in Kidney Transplant Recipients**
   - Must be prescribed by a hematologist, oncologist, infectious disease physician, or transplant physician
   - Must be age 12 years or older and weigh ≥ 40 kg
   - Must be used for prophylaxis of CMV disease in kidney transplant recipients at high risk (Donor CMV seropositive/Recipient CMV seronegative [D+/R-])
   - Must not be used concomitantly with any of the following;
     - Pimozide
     - Ergot alkaloids (e.g., ergotamine, dihydroergotamine)
     - Pitavastatin and simvastatin when co-administered with cyclosporine
   - For letermovir (Prevymis) oral pellets:
     - Must have documentation of the inability to swallow an oral solid dosage form due to age (< 7 years), medical condition (e.g., nasogastric tube), or other clinical rationale for why letermovir (Prevymis) tablets cannot be used

If the above criteria are met, the requested product is approved for 7 months for 1 course of treatment to allow for prophylaxis through Day 200 post-transplant. Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response and clinical rationale for re-treatment.

- **Levacetylleucine (Aqneursa®)** [1081, 1082, 1083, 1084, 1085]

<u>Initial Authorization Criteria:</u>

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- Must be prescribed by provider who specializes in the treatment of metabolic disorders or a neurologist
- Must have documentation of a current weight ≥ 15 kg
- Must have a diagnosis of Niemann-Pick disease type C (NPC) confirmed by ONE of the following:
  - A mutation in both alleles of NPC1 or NPC2 gene (a copy of the genetic test result must be provided)
  - A mutation in one allele (NPC1 or NPC2) and a positive filipin-staining or elevated cholestane triol/oxysterols >2x the upper limit of normal (a copy of the genetic test result and laboratory results must be provided)
- Must have neurological symptoms of NPC (e.g., cognitive decline, cerebellar ataxia, gaze palsy, dysarthria, dysphagia, dystonia, seizures, psychiatric disorders). Chart documentation of NPC symptoms must be provided.
- Must be ambulatory (e.g., walk independently or walk with assistance)
- Must not use in combination with arimoclomol (Miplyffa)
- Must be using a dose and duration of therapy consistent with the FDA-approved package labeling

If the above criteria are met, levacetylleucine (Aqneursa) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation showing the following:

- Must have improvement or stabilization in function or neurologic symptoms affected by NPC (e.g., ambulation, fine motor skills, swallowing, or speech)
- Must not use in combination with arimoclomol (Miplyffa)
- Must be using a dose and duration of therapy consistent with the FDA-approved package labeling

• Levoketoconazole (Recorlev®) [1086, 1087, 1088]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with an endocrinologist
- Must be age 18 years and older
- Must have a diagnosis of Cushing's syndrome
- Must have previously had surgical resection of the primary lesion (e.g., trans-sphenoidal resection of pituitary tumor, adrenalectomy, resection of ectopic ACTH-secreting tumor) that was not curative or not be a candidate for surgery
- Must submit a baseline 24-hour urinary free cortisol level

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- Must have chart documentation of an adequate trial and failure of ALL of the following with an inadequate response or significant side effects/toxicity or have a contraindication to these therapies:
    - Ketoconazole
    - Metyrapone (Metopirone®)
    - Cabergoline or pasireotide (Signifor®, Signifor LAR®)
        - NOTE: this trial is required for a diagnosis of Cushing's disease only
- Must provide clinical rationale to support the use of levoketoconazole (Recorlev) if ketoconazole has not been effective to treat the diagnosis
    - Rationale must include reason why ketoconazole will not produce the same clinical results as would be expected with levoketoconazole (Recorlev)
- Must provide clinical rationale to support the use of levoketoconazole (Recorlev) if the member has a contraindication to, intolerance of, or drug interaction with ketoconazole
    - Rationale must include reason why levoketoconazole (Recorlev) would not be expected to have the same contraindication, produce the same intolerance, or have the same drug interaction as ketoconazole
- Must not have any of the following contraindications:
    - Cirrhosis, acute liver disease or poorly controlled chronic liver disease, baseline AST or ALT > 3 times the ULN, recurrent symptomatic cholelithiasis, a prior history of drug induced liver injury due to ketoconazole or any azole antifungal therapy that required discontinuation of treatment, or extensive metastatic liver disease
    - Taking a mediation that causes QT prolongation
    - Prolonged QTcF interval > 470 msec at baseline, history of torsades de pointes, ventricular tachycardia, ventricular fibrillation, or prolonged QT syndrome
    - Hypersensitivity to ketoconazole
    - Drugs that are sensitive substrates of CYP3A4 and P-gp

If the above criteria are met, levoketoconazole (Recorlev) is approved for 3 months.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon chart documentation from the provider that the member's disease course has improved based on a reduction in the 24-hour urinary free cortisol level from baseline value, as well as improvements in the signs and symptoms of the disease (e.g., blood pressure, lipid levels, weight).

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- **Levomilnacipran (Fetzima®)** [1089, 1090]

Criteria:
- Must be age 18 years or older
- Must have a diagnosis of major depressive disorder (MDD)
- Must have chart documentation which shows the member has had an adequate trial and failure or an inadequate response, duration of at least 4 weeks, or an intolerance to the following:
  - Four different generic antidepressants from the SSRI or SNRI therapeutic classes
- Must currently not be taking a monoamine oxidase inhibitor (MAOI)
  - At least a 14 day washout period is required between discontinuing an MAOI and starting levomilnacipran (Fetzima)
- Must currently not be taking linezolid or intravenous methylene blue
- Must not have a diagnosis of uncontrolled narrow-angle glaucoma

If the above criteria are met, levomilnacipran (Fetzima) is approved.

- **L-glutamine oral powder (Endari™)** [1091, 1092, 1093, 1094]

Initial Authorization Criteria:
- Must be prescribed by or in consultation with a hematologist, oncologist or provider who specializes in the treatment of sickle cell disease
- Must be age 5 years or older
- Must have a diagnosis of sickle cell disease
- Must have documentation the member has experienced ≥ 1 sickle cell-related vaso-occlusive crisis within the last 12 months
- Must have an adequate trial of at least 6 months of hydroxyurea with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
- Must not be used in combination with crizanlizumab-tmca (Adakveo)

If the above criteria are met, L-glutamine (Endari) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for the continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved while on therapy.

- **Lidocaine 5% patch:** [1095, 1096, 1097, 1098, 1099, 1100, 1101, 1102, 1103, 1104, 1105]

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Initial Authorization Criteria:

- Must have ONE of the following:
  - Must have a diagnosis of post-herpetic neuralgia
  - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ONE of the following:
    - 2 prescription-strength NSAIDs
    - Generic SSRI (e.g., citalopram, escitalopram, fluoxetine, sertraline)
    - Generic SNRI (e.g., duloxetine)
    - Generic TCA (e.g., amitriptyline)
    - Generic gabapentin
    - Generic pregabalin
    - Generic muscle relaxant (e.g., cyclobenzaprine)

If the above criteria are met, the requested lidocaine patch product is approved.

• Lofexidine (Lucemyra™) [1106, 1107, 1108, 1109]

Initial Authorization Criteria:

- Must be used for the treatment of opioid withdrawal symptoms to facilitate abrupt opioid discontinuation in adults
- Must have an adequate trial of clonidine (e.g., tablets, patches) with an inadequate response or significant side effects/toxicity or have a contraindication to this therapy

If the above criteria are met, lofexidine (Lucemyra) is approved for 14 days.

• Lomitapide (Juxtapid™) [1110, 1111, 1112, 1113]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with a clinical lipidologist
- Must be age 18 years or older
- Must have a diagnosis of homozygous familial hypercholesterolemia (HoFH). Chart documentation of a clinical work-up to rule out other diagnoses and clinical rationale for the diagnosis and exclusion of other possible diagnoses must be provided. The diagnosis must be confirmed by ONE of the following:
  - Must have genetic testing that confirms functional mutation(s) in both LDL receptor alleles or alleles known to affect LDL receptor functionality
  - Must have an untreated LDL-C level >400mg/dL OR a treated LDL-C level ≥ 300mg/dL AND one of the following:
    - Presence of cutaneous or tendon xanthomas or corneal arcus in the first decade of life

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        - Evidence of heterozygous familial hypercholesterolemia (HeFH) in both parents
  - Must provide the baseline and goal LDL-C level. The date of the baseline test must be provided.
  - Must have LDL-C level above goal despite ALL of the following:
    - An adequate trial of 1 high intensity statin (such as atorvastatin 40-80mg daily and/or rosuvastatin 20-40mg daily) at maximally tolerated dose for at least 2 continuous months
      - If member cannot use statins due to intolerance, the following documentation is required:
        - Must have documented intolerable adverse reactions to statin therapy (such as myalgia or myositis)
      - If member cannot use statins due to a contraindication, the following documentation is required:
        - Must have documented active liver disease, which may include unexplained persistent elevations in hepatic transaminase levels
    - An adequate trial (of at least 4 weeks) of ezetimibe with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy. If the member can tolerate statins, this trial of ezetimibe must be in combination with a maximally tolerated statin
    - An adequate trial (of at least 3 months) of evolocumab (Repatha®) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
    - An adequate trial (of at least 6 months) of evinacumab-dgnb (Evkeeza®) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
  - Must be used in combination with other LDL-lowering therapy (e.g., statin, ezetimibe, etc.)
  - Must not have ANY of the following:
    - Must not be on concomitant treatment with moderate or strong CYP3A4 inhibitors (e.g., clarithromycin, itraconazole, ketoconazole)
    - Must not have moderate or severe hepatic impairment (Child-Pugh category B or C) or active liver disease
    - Must not be pregnant

If the above criteria are met, lomitapide (Juxtapid) is approved for 6 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one-year intervals based upon the following:

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- Must have documentation from the provider that the member's condition has improved while on therapy, evidenced by a recent LDL-C level that has decreased from baseline

Must have documentation/claims data demonstrating member is using in combination with other LDL-lowering therapy

- **Lonafarnib (Zokinvy™)** [1114, 1115, 1116]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with a physician who is knowledgeable about the treatment of Hutchinson-Gilford Progeria Syndrome and processing-deficient Progeroid Laminopathies
- Must be 12 months of age or older
- Must provide baseline body surface area (BSA) OR height and weight. Must have a BSA of 0.39-1 m^2.
- Must have a diagnosis confirmed by a geneticist including ONE of the following genetic test results (documentation of laboratory result confirming the mutation is required):
  - Hutchinson-Gilford Progeria Syndrome (HGPS) with mutation c.1824C > T (p.Gly608Gly) in exon 11 of *LMNA*
  - Processing-deficient Progeroid Laminopathy (PL) with ONE of the following
    - Heterozygous *LMNA* mutation with progerin-like protein accumulation
    - Homozygous or compound heterozygous *ZMPSTE24* mutations
- Must provide chart documentation of clinical signs and symptoms of HGPS/processing-deficient PL including BOTH of the following:
  - Documentation of growth impairment (e.g., short stature, poor weight gain, loss of subcutaneous body fat)
  - Documentation of additional characteristic manifestations from at least TWO the following categories:
    - Skin disorders including alopecia, skin atrophy, skin dryness, or scleroderma
    - Bone or joint disorders including decreased joint range of motion, joint contractures, hypodontia, osteoarthritis, shuffling gait, or decreased bone density
    - Characteristic facial features including circumoral (encircling the mouth) cyanosis, prominent eyes, thin and beaked nose, migrognathia (small lower jaw), retrognathia ("overbite"), thin lips, loss of eyebrows, prominent scalp veins
    - Cardiovascular disorders including atherosclerosis, stroke, myocardial infarction, heart failure
- Must have chart documentation or an attestation from the prescriber of provision of the appropriate supportive medical care (genetic, nutrition,

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skeletal, dental, ophthalmologic, audiology, cardiovascular, and neurologic assessments)
- Must be using a dose that is appropriate for the patient's BSA (see Table 1 and Table 2)

Zokinvy Dosing Guidelines

Table 1

Recommended dosage and administration for 115 mg/m^2 BSA-based dosing (initial 4 month starting dose, dose reduction for gastrointestinal adverse reactions or use with weak CYP3A inhibitors)

BSA (m^2)	Total Daily Dosage Rounded to Nearest 25mg	Morning Dose Number of Capsule(s)		Evening Dose Number of Capsule(s)	
		Lonafarnib 50mg	Lonafarnib 75mg	Lonafarnib 50mg	Lonafarnib 75mg
0.39-0.48	100	1		1	
0.49-0.59	125		1	1	
0.6-0.7	150		1		1
0.71-0.81	175	2			1
0.82-0.92	200	2		2	
0.93-1	225	1	1	2	

Table 2

Recommended dosage and administration for 150 mg/m^2 BSA-based dosing (recommended maintenance dose after the initial 4 months)

BSA (m^2)	Total Daily Dosage Rounded to Nearest 25mg	Morning Dose Number of Capsule(s)		Evening Dose Number of Capsule(s)	
		Lonafarnib 50mg	Lonafarnib 75mg	Lonafarnib 50mg	Lonafarnib 75mg
0.39-0.45	125		1	1	
0.46-0.54	150		1		1
0.55-0.62	175	2			1
0.63-0.7	200	2		2	
0.71-0.79	225	1	1	2	
0.8-0.87	250	1	1	1	1
0.88-0.95	275		2	1	1
0.96-1	300		2		2

If the above criteria are met, lonafarnib (Zokinvy) is approved for 1 year.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider of both of the following:
- The member is tolerating therapy

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- The member is using a dose that is appropriate for the patient's BSA (see Table 1 and Table 2)

• Lumateperone (Caplyta®) [1117, 1118, 1119, 1120, 1121]

Criteria:

- Must be prescribed by or in consultation with a psychiatrist
- Must be age 18 years or older
  - For members <12 years of age, please reference 'Atypical and Typical Antipsychotics – Children Under 12 years of Age' criteria above
- Must have one of the following diagnoses:
  - Schizophrenia and meet the following criteria:
    - Must have an adequate trial, duration of at least 4 weeks, with an inadequate response or significant side effect/toxicity or have a contraindication to FOUR generic atypical antipsychotics (e.g. olanzapine, risperidone, ziprasidone)
  - Depressive episodes associated with bipolar I or II disorder (bipolar depression) and meet the following criteria:
    - Must have an adequate trial, duration of at least 4 weeks, with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
      - Lithium or lamotrigine
      - Quetiapine or quetiapine ER
      - Lurasidone
      - Olanzapine or olanzapine/fluoxetine
  - Major Depressive Disorder and meet the following criteria:
    - Must have an adequate trial, duration of at least 4 weeks, with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
      - Monotherapy with three different antidepressant therapies from three different classes
      - Combination therapy consisting of a SSRI or SNRI antidepressant plus aripiprazole as adjunctive treatment
      - Combination therapy consisting of a SSRI or SNRI antidepressant plus quetiapine ER as adjuctive treatment
    - Must take lumateperone (Caplyta) with a SSRI or SNRI antidepressant as adjunctive treatment

If the above criteria are met, lumateperone (Caplyta) is approved.

• Lusutrombopag (Mulpleta®) [1122, 1123, 1124, 1125]

Initial Authorization Criteria:

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- Must be prescribed by a hepatologist, gastroenterologist, hematologist, oncologist, or surgeon
- Must be age 18 years or older
- Must have a diagnosis of thrombocytopenia in chronic liver disease (CLD)
- Must have documentation of a scheduled procedure
- Must have a baseline platelet count $<50x10^9/L$
- Must have an adequate trial of avatrombopag (Doptelet) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy

If the above criteria are met, lusutrombopag (Mulpleta) is approved for 1 month (one 7-day course of treatment).

Reauthorization Criteria:
Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response and clinical rationale for re-treatment.

- **Luteinizing Hormone Releasing Hormone (LHRH) Agents, Injectable** [1126, 1127, 1128, 1129, 1130, 1131, 1132, 1133, 1134, 1135, 1136, 1137, 1138, 1139, 1140, 1141, 1142, 1143, 1144, 1145, 1146, 1147, 1148, 1149, 1150, 1151, 1152, 1153]
  - For requests for leuprolide (Camcevi®), leuprolide depot (Lutrate Depot®) 22.5mg, and leuprolide acetate (Vabrinty™)please refer to policy RX.PA.406 Commercial, Exchange Medical Prior Authorization Policies

Initial Authorization Criteria:
- Must have ONE of the following diagnoses or conditions:
  - Prostate cancer
  - Endometriosis
  - Uterine leiomyomata (fibroids)
  - Central precocious puberty
    - Must have onset of secondary sexual characteristics earlier than 8 years in females and 9 years in males
    - Must be age 11 years or younger for females
    - Must be age 12 years or younger for males
  - Breast cancer
  - Endometrial-thinning agent prior to endometrial ablation for dysfunctional uterine bleeding
  - Infertility
    - Must have coverage for infertility. Members with infertility coverage can be identified through presence of infertility services included in their pharmacy rider.

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    - Must be prescribed by a reproductive or fertility specialist
    - Must have a diagnosis of infertility
- Gender affirming care services
    - For Commercial members, must have coverage for gender affirming care services. Members without coverage can be identified through presence of gender affirming care services exclusion rider on benefit.
    - Must be an adolescent or older
    - For adolescents:
        - Must be prescribed by or in consultation with a pediatric endocrinologist or a clinician trained in gender affirming health care
        - Must have a long-lasting and intense pattern of gender dysphoria/gender incongruence, which may have worsened with the onset or progression of puberty
        - Must have experienced puberty to at least Tanner Stage 2
        - Must have addressed and reasonably controlled any coexisting psychiatric and medical conditions or social problems that could interfere with treatment
        - Must have an attestation from the prescriber that the member meets ALL of the following:
            - Has been informed of the effects and side effects of treatment
            - Has the capacity to make a fully informed decision and consents to treatment
            - Parents, other caretakers or legal guardians' consent to treatment and are involved in supporting the member throughout the treatment process
            - Exceptions to this can be made on a case-by-case basis and in conjunction with the prescriber when parental consent cannot be obtained
    - For adults:
        - Must be prescribed by or in consultation with an endocrinologist or a clinician trained in gender affirming health care
        - Must have a persistent, well-documented diagnosis of gender dysphoria
        - If the member has significant medical or mental health concerns present, they must be reasonably well-controlled

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- Must have an attestation from the prescriber that the member has the capacity to make a fully informed decision and consents to treatment

For a diagnosis of prostate cancer, the requested LHRH product is approved.

For all other diagnoses, the requested LHRH product is approved based upon the diagnosis as follows:

- Endometriosis: 6 months
- Uterine leiomyomata (fibroids): 3 months
- Central precocious puberty: up to 1 year
  - Authorizations are only approved up to age 11 in females and age 12 in males
- Breast cancer: 1 year
- Dysfunctional uterine bleeding (endometrial thinning): 2 months
- Infertility: 3 months
- Gender affirming care services: 1 year

Reauthorization Criteria:

All prior authorization renewals are reviewed to determine the Medical Necessity for continuation of therapy. Authorizations may be extended for a duration depending upon the diagnosis as outlined above and based on the following:

1. **For all diagnoses not otherwise specified:**
   - Must have documentation from the provider that the member's condition has improved while on therapy
2. **For central precocious puberty:**
   - Authorizations are only approved up to age 11 in females and age 12 in males
     - Reauthorization requests for members older than the ages listed above are not granted
3. **For dysfunctional uterine bleeding (endometrial thinning):**
   - Authorizations are only approved for a one-time, two-month period
     - Reauthorization requests beyond the timeframe listed above are not granted
4. **For infertility:**
   - Must have coverage for infertility. Members with infertility coverage can be identified through presence of infertility services included in their pharmacy rider.
   - Authorizations may be extended on a case-by-case basis, are subject to the above criteria, and require documentation describing the previous response to treatment and clinical rationale for re-treatment to determine the Medical Necessity for continuation of therapy
5. **For gender affirming care services:**

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- Must have coverage for gender affirming care services. Members without coverage can be identified through presence of gender affirming care services exclusion rider on benefit.
- For FEHB and PSHB members, please refer to policy RX.011 Request for Non-Covered Medications
- Must have documentation from the provider that the member continues to benefit from therapy

• Macitentan (Opsumit®) [1154, 1155, 1156, 1157]

Initial Authorization Criteria:

- Must be prescribed by a cardiologist or pulmonologist who specializes in pulmonary hypertension
- Must have a diagnosis of PAH (WHO Group 1) confirmed by right heart catheterization (RHC); must provide a copy of the RHC showing ALL of the following:
  - Mean pulmonary arterial pressure (mPAP) ≥25 mmHg
  - Pulmonary capillary wedge pressure (PCWP), left atrial pressure, or left ventricular end-diastolic pressure (LVEDP) ≤15 mmHg
  - Pulmonary vascular resistance >3 Wood units
- Must have an adequate trial with an inadequate response, or significant side effect/toxicity, or have a contraindication to ONE of the following:
  - Ambrisentan (Letairis)
  - Bosentan (Tracleer)

If the above criteria are met, macitentan (Opsumit) is approved for 3 months.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or the member continues to benefit from therapy.

• Macitentan/tadalafil (Opsynvi®) [1158, 1159, 1160, 1161, 1162]

Initial Authorization Criteria:

- Must be prescribed by a cardiologist or pulmonologist who specializes in pulmonary hypertension
- Must be age 18 years or older
- Must have a diagnosis of pulmonary arterial hypertension (PAH) WHO Group 1 confirmed by right heart (RHC); must provide a copy of the RHC showing ALL of the following:
  - Mean pulmonary arterial pressure (mPAP) ≥25 mmHg

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        - Pulmonary capillary wedge pressure (PCWP), left atrial pressure, or left ventricular end-diastolic pressure (LVEDP) ≤15 mmHg
        - Pulmonary vascular resistance >3 Wood units
    - Must meet ONE of the following:
        - Must have an adequate trial of combination therapy with ambrisentan (Letairis) and tadalafil (Adcirca) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
        - Must be currently ongoing with at least 3 months of combination therapy with macitentan (Opsumit) and tadalafil (Adcirca) with a response per the provider
    - Must not use in combination with ANY of the following:
        - Endothelin receptor antagonists (e.g., macitentan [Opsumit], ambrisentan [Letairis], or bosantan [Tracleer])
        - Phosphodiesterase-5 inhibitors (e.g., tadalafil [Adcirca] or sildenafil [Revatio])

If the above criteria are met, macitentan/tadalafil (Opsynvi) is approved for 3 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or the member continues to benefit from therapy.

• Mannitol (Bronchitol®) [1163, 1164]

Initial Authorization Criteria:
- Must be prescribed by a pulmonologist
- Must be age 18 years or older
- Must have a diagnosis of cystic fibrosis
- Must have an attestation from the prescriber that the member has passed a Bronchitol Tolerance Test
- Must use as add-on maintenance treatment with standard therapies (e.g., bronchodilators, inhaled antibiotics) to improve pulmonary function
- Must have an adequate trial of hypertonic saline solution for inhalation with an inadequate response or significant side effects/toxicity or have a contraindication to this therapy

If the above criteria are met, mannitol (Bronchitol) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at

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one-year intervals based upon documentation from the provider that the member's condition has improved or stabilized, or the member continues to benefit from therapy.

- **Maralixibat (Livmarli™)** 1165, 1166, 1167, 1168, 1169, 1170

Initial Authorization Criteria:

1. **For progressive familial intrahepatic cholestasis (PFIC):**
   - Must be prescribed by a hepatologist or gastroenterologist
   - Must be age 12 months or older
   - Must have a diagnosis of pruritus associated with progressive familial intrahepatic cholestasis (PFIC) confirmed by ALL of the following:
     - Must provide genetic testing demonstrating a mutation associated with PFIC (a copy of the genetic test must be provided)
       - Genetic mutations associated with PFIC, include, but are not limited to: *ATP8B1*, *ABCB11*, *ABCB4*, *TJP2*, *NR1H4*, and *MYO5B*
       - Must not have genetic testing results that indicate PFIC type 2 with *ABCB11* variants resulting in nonfunctional or complete absence of bile salt export pump protein (BSEP-3)
     - Must provide chart documentation of moderate to severe pruritis that causes a negative effect on quality of life (e.g., sleep disturbances, difficulty concentrating during the day)
     - Must have a serum bile acid concentration above the upper limit of normal (chart documentation of test result including reference range must be provided)
   - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to at least TWO medications for PFIC-associated pruritis (e.g., ursodiol, rifampin, cholestyramine)
   - Must have an adequate trial of odevixibat (Bylvay) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
   - Must provide current weight and be using a dose and duration of therapy consistent with the FDA-approved package labeling

2. **For Alagille syndrome (ALGS):**
   - Must be prescribed by a hepatologist or gastroenterologist
   - Must be age 3 months or older
   - Must have a diagnosis of cholestatic pruritus associated with Alagille syndrome (ALGS) confirmed by ALL of the following:
     - Must provide genetic testing demonstrating a *JAG1* or *NOTCH2* mutation (a copy of the genetic test must be provided)

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        - Must provide chart documentation of moderate to severe pruritis that causes a negative effect on quality of life (e.g., sleep disturbances, difficulty concentrating during the day)
        - Must have a serum bile acid concentration above the upper limit of normal (chart documentation of test result including reference range must be provided)
    - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to at least TWO medications for ALGS-associated pruritis (e.g., ursodiol, rifampin, cholestyramine)
    - Must provide current weight and be using a dose and duration of therapy consistent with the FDA-approved package labeling

If the above criteria are met, maralixibat (Livmarli) is approved for 6 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon the following:
- Must have chart documentation from the provider that the member's condition has improved while on therapy
- Must provide current weight and be using a dose and duration of therapy consistent with the FDA-approved package labeling

• Maribavir (Livtencity™) [1171, 1172, 1173, 1174, 1175]

Initial Authorization Criteria:
- Must be prescribed by a hematologist, oncologist, infectious disease physician, or transplant physician
- Must be 12 years of age or older and must weigh at least 35 kg
- Must have received a solid organ transplant (SOT) or hematopoietic stem cell transplant (HSCT)
- Must be used for the treatment of post-transplant CMV infection/disease that is refractory to treatment (with or without genotypic resistance) with ganciclovir, valganciclovir, cidofovir or foscarnet. Must have chart documentation of at least ONE of the following:
    - CMV DNA level in whole blood or plasma that has not decreased by ≥1 log10 (10-fold decrease, aka reduced by 90%) after 14 days of antiviral therapy at the treatment dose
    - Dose limiting toxicity preventing the continuation of current antiviral therapy (e.g., bone marrow suppression, renal toxicity)
- Must not be using concomitantly with ganciclovir or valganciclovir
- Must not be used to treat CMV disease involving the central nervous system, including the retina

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If the above criteria are met, maribavir (Livtencity) is approved for 3 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed every 3 months to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at 3-month intervals based upon chart documentation from the provider that the member's condition has improved based upon the prescriber's assessment while on therapy or the member continues to benefit from therapy.

- **Mavacamten (Camzyos™)** [1176, 1177, 1178]

Initial Authorization Criteria:
- Must be prescribed by a cardiologist
- Must be 18 years of age or older
- Must have a diagnosis of symptomatic obstructive hypertrophic cardiomyopathy defined as ALL of the following:
  - Diagnostic imaging (e.g., echocardiography or cardiovascular magnetic resonance) that shows ONE of the following measurements for maximal left ventricular wall thickness:
    - ≥15mm
    - ≥13mm with family history of hypertrophic cardiomyopathy or in conjunction with a positive genetic test
  - Left ventricular outflow tract (LVOT) gradient ≥30mmHg (documentation must be provided)
  - NYHA Functional Class II or III symptoms with at least one clinical symptom of heart failure (e.g., chest pain, dyspnea on exertion, edema, etc.) [documentation must be provided]
  - Clinical work-up to rule out other diagnoses that cause cardiac hypertrophy and mimic symptoms of obstructive hypertrophic cardiomyopathy (e.g., amyloidosis, Fabry disease, or Noonan syndrome with left ventricular hypertrophy) [documentation must be provided]
- Must have a baseline (within the past 12 months) left ventricular ejection fraction (LVEF) ≥55%
- Must have an adequate trial with an inadequate response, or significant side effects/toxicity, or have a contraindication to ONE of the following:
  - Beta blockers (e.g., nadolol, atenolol, metoprolol)
  - Calcium channel blockers (e.g., verapamil, diltiazem)
  - NOTE: for members who have experienced significant side effects/toxicities or contraindication to one of these classes, an adequate trial of the other class is required (e.g., verapamil-induced constipation requires a trial of a beta blocker)
- Must not use in combination with ANY of the following:

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- Strong CYP2C19 inhibitors (e.g., fluconazole, fluvoxamine), moderate to strong CYP2C19 inducers (e.g., rifampin), or moderate to strong CYP3A4 inducers (e.g., phenytoin, St. John's wort)
- Disopyramide,ranolazine, or a combination of both beta blockers and calcium channel blockers
- Aficamten (Myqorzo)

If the above criteria are met, mavacamten (Camzyos) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for the continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation showing ALL of the following:

- The member's symptoms (e.g., chest pain, dyspnea on exertion, edema, etc.) have improved while on therapy or the member continues to benefit from therapy based upon the prescriber's assessment
- Must have a recent (within the past 6 months) LVEF ≥50%
- Must have a recent (within the past 6 months) LVOT gradient that has improved (decreased) from baseline

• Mavorixafor (Xolremdi®) [1179, 1180]

Initial Authorization Criteria:

- Must be prescribed by an immunologist, hematologist, dermatologist, or genetic specialist
- Must be age 12 years or older
- Must have a diagnosis of WHIM syndrome confirmed by genetic testing demonstrating a pathogenic mutation in the CXCR4 gene (a copy of the genetic test result must be provided)
- Must have an absolute neutrophil count (ANC) ≤ 400 cells/µL (chart documentation of laboratory test must be provided)
- Must not use in combination with drugs highly dependent on CYP2D6 for clearance

If the above criteria are met, mavorixafor (Xolremdi) is approved for 6 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon chart documentation showing the following:

- Must have an improvement from baseline in ANC
- Must not be using in combination with drugs highly dependent on CYP2D6 for clearance

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- **Mecasermin (Increlex®)** [1181, 1182, 1183]

Initial Authorization Criteria:

- Must be prescribed by a pediatric endocrinologist
- Must be age 2 years or older
- Must have a diagnosis of ONE of the following:
  - Severe primary IGF-1 deficiency confirmed by documentation of ALL of the following:
    - Height standard deviation score ≤ -3.0 for age and sex
    - Basal IGF-1 standard deviation ≤ -3.0 based on laboratory reference range
    - Normal or elevated growth hormone defined as stimulated serum growth hormone level (peak level) >10ng/mL
  - Growth hormone gene deletion with neutralizing antibodies to growth hormone
- Must not have secondary forms of IGF-1 deficiency, such as growth hormone deficiency, malnutrition, hypothyroidism, or chronic treatment with pharmacologic doses of anti-inflammatory corticosteroids
- Must not use concomitantly with growth hormone treatment
- Must not have ANY of the following:
  - Hypersensitivity to mecasermin (Increlex) or any component of the formulation
  - Must not be administered intravenously
  - Closed epiphyses
  - Malignant neoplasia or history of malignancy

If the above criteria are met, mecasermin (Increlex) is approved for 1 year.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon the following:

- Must have chart documentation from the provider that the member's condition has improved while on therapy (e.g., improvement in growth, height velocity)
- Must have documentation that epiphyses have not closed

- **Mepolizumab Autoinjector, Syringe (Nucala®)** [1184, 1185, 1186, 1187, 1188, 1189, 1190, 1191, 1192, 1193, 1194, 1195, 1196, 1197, 1198, 1199, 1200]
  - Requests for mepolizumab vial (Nucala) please refer to policy RX.PA.406 Commercial, Exchange Medical Prior Authorization Policies

Initial Authorization Criteria:

1. **For severe asthma**

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- Must be prescribed by or in consultation with an allergist, an immunologist, or a pulmonologist
- Must be age 6 years or older
- Must have a diagnosis of severe eosinophilic phenotype asthma with a blood eosinophil count of ≥150 cells/µL within the past year. The date the test was performed must be provided.
- Must have received at least 3 consecutive months of therapy with ONE of the following combinations for the maintenance treatment of asthma:
  - A medium-high dose inhaled corticosteroid AND one additional asthma controller medication (e.g., inhaled long-acting beta-agonist, inhaled long-acting muscarinic antagonist)
  - A maximally tolerated inhaled corticosteroid AND long-acting beta-agonist combination product
- Must have history of one asthma exacerbation in the previous year despite adherence to optimized medication therapy regimen. Asthma exacerbation is defined by ONE of the following:
  - Hospitalization for asthma
  - Emergency room visit for asthma
  - Requirement for 2 or more courses of treatment with systemic (e.g., oral, parenteral) corticosteroids
- Must be used as an add-on maintenance treatment to asthma controller therapy
- Must not be receiving combination therapy with another biologic agent (e.g., Xolair, Cinqair, Fasenra, Dupixent, Tezspire) for this diagnosis

2. For eosinophilic granulomatosis with polyangiitis (EGPA)

- Must be prescribed by or in consultation with an allergist, an immunologist, pulmonologist, or rheumatologist.
- Must be age 18 years or older
- Must have a diagnosis of relapsed, refractory eosinophilic granulomatosis with polyangiitis. Chart documentation of diagnosis must be provided.
  - Must have a history or presence of asthma
  - Must have a blood eosinophil level of 10% or an absolute eosinophil count of >1000 cells/µL
  - Must have two or more of the following criteria that are typical of eosinophilic granulomatosis with polyangiitis:
    - Histopathological evidence of eosinophilic vasculitis, perivascular eosinophilic infiltration, or eosinophil-rich granulomatous inflammation
    - Neuropathy
    - Pulmonary infiltrates
    - Sino-nasal abnormality
    - Cardiomyopathy
    - Glomerulonephritis

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        - Alveolar hemorrhage
        - Palpable purpura
        - Antineutrophil cytoplasmic antibody [ANCA] positivity
    - Must have an adequate trial ≥3 months of systemic corticosteroids (prednisone or prednisolone at a dose of ≥7.5 mg/day) with an inadequate responses or significant side effects/toxicity unless contraindicated

3. **For hypereosinophilic syndrome (HES)**
    - Must be prescribed by or in consultation with an allergist, immunologist, or hematologist
    - Must be age 12 years or older
    - Must have a diagnosis of hypereosinophilic syndrome (HES). Chart documentation confirming the diagnosis is required.
        - Must have a history of HES diagnosis for at least 6 months
        - Must have documentation that non-hematologic secondary causes have been ruled out (e.g., drug hypersensitivity, parasitic helminth infection, HIV infection, non-hematologic malignancy)
        - Must have a blood eosinophil count of ≥1,000 cells/µL
        - Must have FIP1L1-PDGFRα negative disease
    - Must have chart documentation of uncontrolled disease, defined as one of the following:
        - At least two HES flares within the past 12 months despite HES therapy (e.g., systemic corticosteroids, immunosuppressive therapy, cytotoxic therapy), flares include the following:
            - HES-related worsening of clinical symptoms
            - Blood eosinophil counts requiring an escalation in therapy
        - On daily corticosteroid with inability to taper off

4. **For Chronic Rhinosinusitis with Nasal Polyps (CRSwNP)**
    - Must be prescribed by or in consultation with an allergist, immunologist, or an otolaryngologist (ear, nose, and throat [ENT])
    - Must be age 18 years or older
    - Must have a diagnosis of chronic rhinosinusitis with nasal polyps. Chart documentation confirming the diagnosis is required as outlined below:
        - Must have documentation of one of the following:
            - A baseline endoscopic bilateral nasal polyp score (NPS) of at least 5 out of a maximum score of 8 (with a minimum score of 2 in each nasal cavity)
            - Evidence of polyposis on sinus CT
        - Must have two or more of the following symptoms present for at least 3 months:
            - Nasal obstruction (congestion)
            - Mucopurulent discharge (rhinorrhea or post-nasal drip)

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        - Facial pain or pressure
        - Reduction or loss of smell
    - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ONE of the following:
        - A generic intranasal corticosteroid
        - Fluticasone propionate (Xhance)
        - NOTE: If the member has a history of sinus surgery, medication trial must be post-surgery.
    - Must be used in combination with an intranasal corticosteroid, unless intolerant or contraindicated
    - Must not be receiving combination therapy with another biologic agent (e.g., Dupixent, Xolair, Fasenra, Cinqair, Tezspire)

5. **For Chronic Obstructive Pulmonary Disease (COPD)**
    - Must be prescribed by or in consultation with an allergist, an immunologist, or a pulmonologist
    - Must be age 18 years or older
    - Must have a diagnosis of eosinophilic phenotype COPD with ALL of the following:
        - A blood eosinophil count of $\geq$ 300 cells/µL within the past year. The date the test was performed must be provided.
        - Baseline spirometry with FEV1/FVC < 0.7
        - Predicted FEV1 > 30% and $\leq$ 80%
    - Must have received at least 3 consecutive months of therapy with ALL of the following combinations for the maintenance treatment of COPD:
        - An inhaled long-acting beta2-agonist (LABA); AND
        - An inhaled long-acting muscarinic antagonist (LAMA); AND
        - An inhaled corticosteroid (ICS) or have a significant side effect/toxicity or a contraindication to inhaled corticosteroid
    - Must have had, within the past 12 months, ONE of the following:
        - Two or more moderate exacerbations (exacerbations treated with either systemic corticosteroids and/or antibiotics)
        - One or more severe exacerbations (exacerbations requiring hospitalization or observation for over 24 hours in an emergency department or urgent care facility)
    - Must not be receiving combination therapy with another biologic agent (e.g., Dupixent, Xolair, Cinqair, Fasenra, Tezspire) for this diagnosis or for asthma

If the above criteria are met, mepolizumab autoinjector, syringe (Nucala) is approved for 1 year.

<u>Reauthorization Criteria:</u>

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All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon the following:

1. **For asthma**
   - Must have documentation from the provider that the member's condition has improved while on therapy (e.g., reduction in exacerbations, hospitalizations, emergency department visits, or requirement for oral corticosteroid therapy)

2. **For EGPA**
   - Must have documentation from the provider that the member's condition has improved while on therapy (e.g., reduction in requirement for oral corticosteroid therapy)

3. **For HES**
   - Must have documentation from the provider that the member's condition has improved while on therapy

4. **For CRSwNP**
   - Must have documentation from the provider that the member's condition has improved while on therapy (e.g., reduction in nasal polyp score (NPS), reduction in exacerbations requiring systemic corticosteroids and/or antibiotics)

5. **For COPD**
   - Must have documentation from the provider that the member's condition has improved or stabilized while on therapy (e.g., reduction in exacerbations, hospitalizations, emergency department visits, or requirement for oral corticosteroid therapy)

• Meprobamate tablet [1201, 1202, 1203]

Criteria:

- Must have diagnosis of anxiety
- Must have an adequate trial and failure or an inadequate response, or significant side effects/toxicity, or a contraindication to ALL of the following:
  - One SSRI (e.g., escitalopram, paroxetine)
  - One SNRI (e.g., duloxetine, venlafaxine)
  - One benzodiazepine (e.g., alprazolam, diazepam, lorazepam)
  - Buspirone
  - Hydroxyzine
- Must have an attestation from the prescriber that the member has been educated on potential adverse effects including the risks of abuse/overuse
- Must not have diagnosis of acute intermittent porphyria

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If the above criteria are met, meprobamate tablet is approved.

- **Methotrexate Subcutaneous Injection (Otrexup®)** [1204, 1205, 1206, 1207, 1208]

Criteria:

- Must have a diagnosis of rheumatoid arthritis, polyarticular juvenile idiopathic arthritis, or psoriasis
- Must have an adequate trial of at least 3 months to methotrexate subcutaneous auto-injector (Rasuvo®) with an inadequate response despite dose titration or significant side effects/toxicities to this therapy
- Must provide clinical rationale to support the use of methotrexate subcutaneous injection (Otrexup) when methotrexate subcutaneous auto-injector (Rasuvo) has not been effective to treat the diagnosis and/or caused significant side effects/toxicities
  - Rationale must include the reason why methotrexate subcutaneous injection (Otrexup) would not produce the same clinical results and/or result in the same side effects/toxicities as methotrexate subcutaneous auto-injector (Rasuvo) as they contain the same active ingredient

If the above criteria are met, methotrexate subcutaneous injection (Otrexup) is approved.

- **Methylnaltrexone (Relistor®)** [1209, 1210, 1211, 1212, 1213]

1. **For OIC in patients with advanced illness or pain caused by active cancer who require dosage escalation for palliative care (*applies to methylnaltrexone (Relistor) injection only)***

Initial Authorization Criteria:

- Must have a diagnosis of opioid-induced constipation (OIC)
- Must have an advanced illness or pain caused by active cancer
- Must have an adequate trial of 2 traditional laxatives for the treatment of the constipation with an inadequate response or significant side effect/toxicity or have a contraindication to these therapies
- Must not have a known or suspected mechanical gastrointestinal obstruction or be at risk for recurrent obstruction

If the above criteria are met, methylnaltrexone injection (Relistor) is approved for 1 year.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of treatment. Authorizations may be extended at

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one-year intervals upon documentation from the provider that the member's condition has improved on therapy.

2. **For OIC in patients with non-cancer pain including patients with chronic pain related to prior cancer or its treatment who do not require frequent (e.g., weekly) opioid dosage escalation *(applies to methylnaltrexone (Relistor) injection and tablet)***

Initial Authorization Criteria:

- Must be age 18 years or older
- Must have a diagnosis of opioid-induced constipation (OIC)
- Must have chronic non-cancer pain (or have pain related to previously treated cancer)
- Must not have a known or suspected mechanical gastrointestinal obstruction or be at risk for recurrent obstruction
- Must have documentation of current, ongoing opioid therapy through pharmacy claims or physician chart documentation
- Must have documentation of a consult with a pain management specialist to determine if dosage of opioid can be decreased
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to TWOof the following:
  - Stool softener (e.g., docusate)
  - Stimulant laxative (e.g., senna, bisacodyl)
  - Bulk laxative (e.g., psyllium)
  - Osmotic laxative (e.g., polyethylene glycol, lactulose)
- Must have chart documentation showing an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
  - Lubiprostone (Amitiza)
  - Naloxegol (Movantik)
  - Naldemedine (Symproic)

If the above criteria are met, the requested methylnaltrexone (Relistor) product is approved for 1 year.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of treatment. Authorizations may be extended at one-year intervals based upon documentation showing the following:

- Must have improved as a result of treatment as defined as an increase in the number of weekly stools from baseline
- Must be continuing on opioid therapy

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- **Metreleptin (Myalept™)** 1214, 1215, 1216, 1217, 1218, 1219, 1220, 1221, 1222, 1223

Initial Authorization Criteria:

- Must be prescribed by an endocrinologist
- Must have a diagnosis of congenital or acquired generalized lipodystrophy. Chart documentation of a clinical work-up to rule out other diagnoses and clinical rationale for the diagnosis and exclusion of other diagnoses must be provided.
- Must not have HIV-related lipodystrophy
- For adult members ages 18 years and older:
  - Must have a serum leptin level of < 4 ng/mL in natal females and < 3 ng/mL in natal males. Documentation of laboratory result must be provided.
- For pediatric members up to age 18 years:
  - Must have a serum leptin level < 12 ng/mL in natal females and < 8 ng/mL in natal males. Documentation of laboratory result must be provided.
- Must have ONE of the following:
  - Severe insulin resistance resulting in diabetes mellitus not adequately controlled with other therapies
    - Must have a hemoglobin A1c ≥ 7 % or fasting plasma glucose ≥ 126 mg/dL
    - Must have an adequate trial and failure of lifestyle modification with diet and exercise
    - Must have an adequate trial of pharmacotherapy for diabetes (e.g., metformin, insulin). Documentation that member was not able to achieve adequate blood glucose control with optimized medication regimen is required.
  - Severe hypertriglyceridemia not adequately controlled with other therapies
    - Must have a triglyceride level ≥ 500 mg/dL
    - Must have an adequate trial and failure of lifestyle modification with diet and exercise
    - Must have an adequate trial of lipid-lowering pharmacotherapy (e.g., fibrates, fish oil, statins). Documentation that member was not able to achieve adequate triglyceride control with optimized medication regimen is required.

If the above criteria are met, metreleptin (Myalept) is approved for 1 year.

Reauthorization Criteria:

All prior authorizations renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at

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one-year intervals based upon documentation from the provider showing that the member has benefited from metreleptin therapy, as evidenced by decreased hemoglobin A1c, fasting plasma glucose, and/or triglyceride levels from baseline.

- **Metyrapone (Metopirone®):** 1224, 1225, 1226, 1227
  - Metyrapone (Metopirone) is considered for coverage under the pharmacy benefit only for the treatment of Cushing's syndrome.
  - Metyrapone (Metopirone) is not considered for coverage under the pharmacy benefit when used for diagnostic testing.

Initial Authorization Criteria:
Treatment of Cushing's Syndrome:
- Must be prescribed by or in consultation with an endocrinologist
- Must have a diagnosis of Cushing's syndrome
- Must submit a baseline 24-hour urinary free cortisol level
- Must have had an adequate trial and failure of ketoconazole with an inadequate response or significant side effects/toxicity or have a contraindication to this therapy

If the above criteria are met, metyrapone (Metopirone) is approved for 3 months.

Reauthorization Criteria:
Prior authorization renewals for Cushing's syndrome are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one year intervals based upon chart documentation from the provider that the member's disease course has improved based on a reduction in the 24-hour urinary free cortisol level from baseline value, as well as improvements in the signs and symptoms of the disease (e.g., blood pressure, lipid levels, weight).

- **Mifepristone (Korlym™)** 1228, 1229, 1230, 1231, 1232, 1233, 1234

Initial Authorization Criteria:
- Must be prescribed by or in consultation with an endocrinologist
- Must be age 18 years or older
- Must be used to control hyperglycemia secondary to hypercortisolism in patients with endogenous Cushing's syndrome who have type 2 diabetes mellitus or glucose intolerance
- Must have failed surgery or not be a candidate for surgery (e.g., trans-sphenoidal resection of pituitary tumor, adrenalectomy, resection of ectopic ACTH-secreting tumor))
- Must meet ALL of the following:
  - Natal female members:

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        - Must have a baseline (within previous month) negative pregnancy test prior to initiation of therapy if of reproductive potential. The date of the test must be provided.
        - Must use a non-hormonal medically acceptable method of contraception (unless surgically sterilized) during and for one month after mifepristone therapy if of reproductive potential
        - Must not have a history of unexplained vaginal bleeding
        - Must not have a history of endometrial hyperplasia with atypia or endometrial carcinoma
    - Must not be on concomitant therapy with simvastatin, lovastatin, or CYP 3A substrates with a narrow therapeutic range (e.g., cyclosporine, dihydroergotamine, ergotamine, fentanyl, pimozide, quinidine, sirolimus, tacrolimus)
    - Must not be on concurrent long-term corticosteroid therapy
  - Must have attestation from the prescriber that a recent (within the previous month) medication reconciliation has occurred and clinically significant drug interactions with mifepristone (Korlym) have been addressed
  - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to BOTH of the following:
    - Ketoconazole
    - Metyrapone (Metopirone)
  - Must have an adequate trial and failure of conventional anti-hyperglycemic medication. Chart documentation is required.
  - Must have a baseline hemoglobin A1c (HbA1c) level

If the above criteria are met, mifepristone (Korlym) is approved for 3 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the medical necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the prescriber that the member's condition has improved while on therapy. In addition, the following criteria must be met:

  - Must have documentation of improvement in hyperglycemia control as evidenced by a reduction in blood glucose levels, HbA1c, or anti-hyperglycemic medication doses or number of medications. Chart documentation is required.
  - Must have attestation from the prescriber that a recent (within the previous year) medication reconciliation has occurred and clinically significant drug interactions with mifepristone (Korlym) have been addressed.

- **Migalastat (Galafold™)** [1235, 1236, 1237, 1238, 1239, 1240]

Initial Authorization Criteria:

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- Must be prescribed by or in consultation with a nephrologist or provider who specializes in the treatment of inherited metabolic disorders
- Must be age 18 years or older
- Must have a diagnosis of Fabry disease confirmed by ONE of the following:
  - For natal male members:
    - Must have chart documentation of ONE of the following:
      - Enzyme assay demonstrating reduced or deficient alpha-galactosidase A activity. Chart documentation of laboratory result including reference range is required.
      - Genetic testing demonstrating a mutation in the galactosidase alpha (*GLA*) gene. Chart documentation of test result is required.
  - For natal female members:
    - Must have chart documentation of ONE of the following:
      - Genetic testing demonstrating a mutation in the galactosidase alpha (*GLA*) gene. Chart documentation of test result is required.
      - Family history of Fabry disease
      - Enzyme assay demonstrating reduced or deficient alpha-galactosidase A activity. Chart documentation of laboratory result including reference range is required.
    - Must have documentation of clinical signs or symptoms attributable to Fabry disease (e.g., angiokeratomas, acroparesthesias, corneal verticillata, etc.)
- Must have an amenable galactosidase alpha gene (*GLA*) variant based on in vitro assay data (see prescribing information for a list of amenable *GLA* variants). Chart documentation of test result is required.
- Must not have severe renal impairment (eGFR < 30 mL/min/1.73m^2) or end-stage renal disease requiring dialysis
- Must not be using in combination with enzyme replacement therapy (e.g., agalsidase beta [Fabrazyme], pegunigalsidase alfa-iwxj [Elfabrio])

If the above criteria are met, migalastat (Galafold) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or stabilized while on therapy.

• Milnacipran (Savella®) [1241, 1242, 1243]

Criteria:

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- Must have a definitive diagnosis of fibromyalgia as evidenced by chart documentation showing a history of widespread pain involving the extremities for three months and localized areas of tenderness
- Must have a trial and failure with an inadequate response or significant side effects unless there is a documented contraindication to the following:
  - Gabapentin at a dose of at least 1200mg daily, which is documented in pharmacy claims or through physician chart documentation
  - Pregabalin (Lyrica)
  - Tricyclic antidepressant (e.g., amitriptyline) or muscle relaxant (e.g., cyclobenzaprine) or duloxetine (Cymbalta®)
- Must have a trial and failure of exercise or physical therapy for the treatment of fibromyalgia as evidenced by physician chart documentation

If the above criteria are met, milnacipran (Savella) is approved.

• Mitapivat (Pyrukynd®) [1244, 1245]

Initial Authorization Criteria:

- Must be prescribed by a hematologist or geneticist
- Must be age 18 years or older
- Must have a confirmed diagnosis of hemolytic anemia due to pyruvate kinase deficiency with documentation of ALL of the following:
  - Genetic testing demonstrating presence of at least two mutant alleles in the PLKR gene, of which at least 1 is a missense mutation (documentation of laboratory result is required)
    - Must not be homozygous for the R479H mutation or have two non-missense mutations, without the presence of another missense mutation, in the *PLKR* gene
  - Decreased pyruvate kinase enzyme activity on enzyme activity assay (documentation of laboratory result with reference range is required)
  - Presence of hemolytic anemia as evidenced by laboratory results (must provide baseline hemoglobin level and markers of hemolysis including bilirubin, lactate dehydrogenase [LDH], haptoglobin, and/or reticulocyte count)
- Must have a recent hemoglobin level ≤ 10 g/dL OR has required red blood cell (RBC) transfusions within the last year (documentation of baseline hemoglobin level and/or RBC transfusion history is required)

If the above criteria are met, mitapivat (Pyrukynd) is approved for 6 months.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at

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one-year intervals based upon chart documentation from the provider that the member's condition has improved as evidenced by an improvement from baseline in hemoglobin level, hemolysis markers, and/or decrease from baseline in RBC transfusion requirements.

- If improvement has not been observed, mitapivat (Pyrukynd) should be discontinued. Authorizations for these members may be extended for one additional month to allow for dose taper based on the prescribing information.

• Monomethyl fumarate (Bafiertam) [1246]

Initial Authorization Criteria:

- Must be prescribed by a neurologist
- Must be age 18 years or older
- Must have a diagnosis of relapsing form of multiple sclerosis, to include clinically isolated syndrome, relapsing-remitting disease, and active secondary progressive disease
- Must have an adequate trial of dimethyl fumarate (Tecfidera) with an inadequate response or significant side effect/toxicity, or have a contraindication to this this therapy
- Must have an adequate trial with an inadequate response, significant side effect/toxicity or contraindication to ONE of the following:
    - Interferon beta-1a (Avonex®, Rebif®)
    - Peginterferon beta-1a (Plegridy®)
    - Interferon beta-1b (Betaseron®)
    - Siponimod (Mayzent®)
    - Ozanimod (Zeposia®)
    - Ofatumumab (Kesimpta®)
    - Diroximel fumarate (Vumerity)
- Must provide clinical rationale for use of monomethyl fumarate (Bafiertam) instead of dimethyl fumarate (Tecfidera) or diroximel fumarate (Vumerity). Clinical rationale must include why dimethyl fumarate (Tecfidera) or diroximel fumarate (Vumerity) would not be expected to produce the same clinical results as monomethyl fumerate (Bafiertam)
- Must not be on concomitant therapy with a disease modifying drug for multiple sclerosis

If the above criteria are met, monomethyl fumarate (Bafiertam) is approved for 1 year.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or stabilized while on therapy.

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- **Naldemidine (Symproic®)** [1247, 1248]

Initial Authorization Criteria:

- Must be age 18 years or older
- Must have a diagnosis of opioid-induced constipation (OIC)
- Must have chronic non-cancer pain (or have pain related to previously treated cancer)
- Must not have a known or suspected mechanical gastrointestinal obstruction
- Must have documentation of current, ongoing opioid therapy through pharmacy claims or physician chart documentation
- Must have documentation of a consult with a pain management specialist to determine if dosage of opioid can be decreased
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to TWO of the following:
  - Stool softener (e.g., docusate)
  - Stimulant laxative (e.g., senna, bisacodyl.)
  - Bulk laxative (e.g., psyllium
  - Osmotic laxative (e.g., polyethylene glycol, lactulose)
- Must have chart documentation showing an adequate trial of naloxegol (Movantik) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy.

If the above criteria are met, naldemedine (Symproic) is approved for 1 year.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation showing the following:

- Must have improved as a result of treatment as defined as an increase in the number of weekly stools from baseline
- Must be continuing on opioid therapy

- **Narcotic Cough and Cold Products in Children and Adolescents** [1249, 1250, 1251, 1252, 1253, 1254,1255, 1256, 1257]

Criteria:

- Prior authorization will apply to cough and cold medications containing opioids prescribed to members under the age of 18 for Commercial and Exchange:
  - Must provide documentation of an evaluation to rule out other diagnoses, (e.g., allergies, bronchitis, pneumonia), for symptoms lasting more than 1 week

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        - Must provide documentation showing specific examples of how quality of life is impacted by symptoms (e.g., sleep disturbances, school/work disruption, etc.)
        - Must provide documentation indicating the member has tried and failed non-pharmacologic therapies (e.g., saline nasal spray, vaporizer, fluids, rest)
        - Must provide documentation showing the Prescription Drug Monitoring Program has been reviewed

If the above criteria are met, the requested cough and cold medication is approved for 1 month.

Reauthorization Criteria:
All prior authorization renewals are reviewed to determine the Medical Necessity for continuation of therapy. Authorization may be extended at 1 month intervals on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response to treatment and clinical rationale for re-treatment.

- **Natalizumab (Tysabri®)** 1258, 1259, 1260, 1261, 1262, 1263, 1264, 1265, 1266, 1267, 1268

Initial Authorization Criteria:
1. **Multiple Sclerosis (MS)**
    - Must be prescribed by a neurologist
    - Must have a diagnosis of a relapsing form of MS, to include clinically isolated syndrome, relapsing-remitting disease, and active secondary progressive disease
    - Must be age 18 years or older
    - Must have an adequate trial with an inadequate response, significant side effect/toxicity, or have a contraindication to ONE of the following:
        - Glatiramer acetate/Glatopa® (Copaxone®)
        - Dimethyl fumarate (Tecfidera®)
        - Fingolimod (Gilenya®)
        - Teriflunomide (Aubagio®)
    - Must have an adequate trial with an inadequate response, significant side effect/toxicity, or have a contraindication to ONE of the following:
        - Interferon beta-1a (Avonex®, Rebif®)
        - Peginterferon beta-1a (Plegridy®)
        - Interferon beta-1b (Betaseron®)
        - Siponimod (Mayzent®)
        - Ozanimod (Zeposia®)
        - Ofatumumab (Kesimpta®)
    - An exception to ALL drug trials can be made in one of the following circumstances:

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        - The member has rapidly evolving severe relapsing remitting MS as assessed by the neurologist. Chart documentation current symptoms, exacerbation history, and rationale for initial use is required.
        - The member has 3 or more predictive factors of poor prognosis:
            - Age of onset 40 years or older
            - Motor system involvement at onset including weakness of the extremities or ataxia
            - 4 or more T2-weighted lesions suggestive of MS seen on MRI
            - 2.5 years or less between the first 2 relapses
            - 2 or more relapses in the first year of disease
            - Poor recovery from the initial 2 relapses defined as an EDSS of 1.5 or higher sustained for at least 1 year
    - Must not be on concomitant therapy with an oral targeted anti-inflammatory agent, biologic agent, or a disease modifying drug for multiple sclerosis

If the above criteria are met, natalizumab (Tysabri) is approved for 1 year.

2. Crohn's Disease
- Must be prescribed by a gastroenterologist
- Must have a diagnosis of moderately to severely active CD with inflammation
- Must be age-appropriate according to FDA-approved package labeling, nationally recognized compendia, or peer-reviewed medical literature
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to of ONE of these conventional therapies:
    - Corticosteroids
    - An immunosuppressant (e.g., azathioprine, 6-mercaptopurine, methotrexate – an adequate trial is defined as 3 months)
- Must have a 3 month trial of a TNF blocking agent (e.g., infliximab product, adalimumab product [Humira, Adalimumab-adbm, Cyltezo, Adalimumab-ryvk, Simlandi], certolizumab pegol [Cimzia]) with an inadequate response orsignificant side effect/toxicity, or contraindication to this therapy
- Must not be on concomitant therapy with an oral targeted anti-inflammatory agent, biologic agent, or a disease modifying drug for multiple sclerosis

If the above criteria are met, natalizumab (Tysabri) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of treatment. Authorization may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or stabilized while on therapy.

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- **Nedosiran (Rivfloza®)** [1269, 1270, 1271, 1272, 1273]

Initial Authorization Criteria:

- Must be prescribed by a nephrologist, urologist, or medical geneticist specializing in the treatment of primary hyperoxalurias
- Must be age 2 years or older
- Must have preserved kidney function (e.g. estimated glomerular filtration rate [eGFR] ≥30 mL/min/1.73 m2). Chart documentation of eGFR is required.
- Must have a diagnosis of primary hyperoxaluria type 1 (PH1) confirmed by BOTH of the following:
  - A pathogenic AGXT gene mutation (copy of genetic test result must be provided)
  - A baseline oxalate measurement consisting of ONE of the following (copy of laboratory test result must be provided):
    - A 24-hour urine collection demonstrating an elevated urine oxalate excretion of ≥ 0.70 mmol/24-hr/1.73m^2
    - A spot urine collection demonstrating an oxalate:creatinine ratio above the age-specific upper limit of normal
    - A plasma oxalate level ≥ 20 µmol/L
- Must have an adequate trial of lumasiran (Oxlumo) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
- Must have an attestation from the prescriber that the medical standards of care are being followed (e.g., fluid intake, vitamin B6/pyridoxine, calcium-oxalate crystallization inhibitor such as potassium citrate)
- Must not have a history of a liver transplant
- Must not use in combination with lumasiran (Oxlumo)

If the above criteria are met, nedosiran (Rivfloza) is approved for 6 months

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation showing the following:

- The member's condition has improved based upon on a reduction in urine oxalate excretion or plasma oxalate as compared with baseline levels
- The member does not have a history of a liver transplant
- Must not use in combination with lumasiran (Oxlumo)

- **Nintedanib (Ofev®), Pirfenidone (Esbriet®), and Nerandomilast (Jascayd®)** [1274, 1275, 1276, 1277, 1278, 1279, 1280, 1281, 1282, 1283, 1284, 1285]

Initial Authorization Criteria:

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1. **For idiopathic pulmonary fibrosis (IPF):**
   - Must be prescribed by a pulmonologist
   - Must be age 18 years or older
   - Must have a diagnosis of idiopathic pulmonary fibrosis (IPF) confirmed by BOTH of the following:
     - Must have a radiographic or pathologic diagnosis of usual interstitial pneumonia (UIP) or probable UIP confirmed by ONE of the following:
       - High-resolution computed tomography (HRCT)
       - Surgical lung biopsy
     - Must have all other diagnoses ruled out (e.g., domestic and occupational environmental exposures, connective tissue disease, and drug toxicity)
   - Must have BOTH of the following lung values:
     - Forced vital capacity (FVC) ≥50% of predicted
     - Diffusing capacity for carbon monoxide (DLCO) 30% to 90% of predicted
   - Must submit documentation of a recent urine or serum nicotine screening confirming that member is NOT currently smoking or recently stopped smoking
   - Must use a dose and duration of therapy consistent with the FDA-approved package labeling
   - Must not use nintedanib (Ofev) and pirfenidone (Esbriet) in combination
   - For nintedanib (Ofev):
     - Must have an adequate trial with generic pirfenidone (Esbriet) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
   - For pirfenidone (Esbriet) 534 mg tablets:
     - Must provide clinical rationale for use of the 534 mg tablets instead of the 267 mg tablets or capsules. Clinical rationale must include why the 534 mg tablets would not be expected to produce the same clinical results as the 267 mg tablets or capsules given that they are the same chemical entity.
   - For nerandomilast (Jascayd):
     - Must have an adequate trial with generic pirfenidone (Esbriet) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
     - Must use 18 mg dose twice a day when using in combination with pirfenidone; must not reduce dose in patients taking concomitant pirfenidone as efficacy was not observed with 9 mg twice a day
     - Must not use in combination with moderate or strong CYP3A inducers

If the above criteria are met, the requested medication is approved for 6 months.

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2. **For systemic sclerosis-associated interstitial lung disease (SSc-ILD) – Applies only to nintedanib (Ofev):**
   - Must be prescribed by a pulmonologist or a rheumatologist
   - Must be age 18 years or older
   - Must have a diagnosis of systemic sclerosis-associated interstitial lung disease (SSc-ILD) confirmed by both of the following:
     - Must meet the 2013 ACR/EULAR criteria for definite systemic sclerosis (a total score of 9 or greater) [see definition section for scoring]
     - Must have interstitial lung disease confirmed by computed tomography (CT) with evidence of fibrosis
   - Must have ALL of the following lung values:
     - Forced vital capacity (FVC) ≥40% of predicted
     - Diffusing capacity for carbon monoxide (DLCO) 30% to 89% of predicted
   - Must submit documentation of a recent urine or serum nicotine screening confirming that member is NOT currently smoking or recently stopped smoking
   - Must be on concurrent immunosuppressive therapy (e.g., mycophenolate, cyclophosphamide, etc.) unless there is evidence of significant side effects/toxicity or a contraindication to this therapy
     - NOTE: An exception to the requirement of concurrent immunosuppressive therapy can be made if prescriber provides clinical rationale for Ofev monotherapy (e.g., significant side effects/toxicity or contraindication to immunosuppressive therapy, member with limited cutaneous scleroderma, no other internal organ involvement, or not rapidly progressive ILD)
   - Must not be using nintedanib (Ofev) and pirfenidone (Esbriet) in combination

If the above criteria are met, the requested medication is approved for 6 months.

3. **For chronic fibrosing interstitial lung diseases (ILDs) with a progressive phenotype (e.g., progressive pulmonary fibrosis [PPF]) – Applies only to nintedanib (Ofev) and nerandomilast (Jascayd):**
   - Must be prescribed by a pulmonologist
   - Must be age 18 years or older
   - Must have a diagnosis of chronic, progressive, fibrosing interstitial lung disease (ILD) (e.g., progressive pulmonary fibrosis [PPF]) confirmed by BOTH of the following:
     - Must have interstitial lung disease confirmed by computed tomography (CT) with evidence of fibrosis

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        - Must have ONE of the following in the previous 24 months despite treatment for ILD (e.g., azathioprine, mycophenolate mofetil, cyclosporine, rituximab) [documentation must be provided]:
            - FVC decline ≥10%
            - FVC decline ≥5% to <10% and worsening of respiratory symptoms
            - FVC decline ≥5% to <10% and increased extent of fibrosis on imaging
            - Worsening of respiratory symptoms and an increased extent of fibrosis on imaging
    - Must have ALL of the following lung values:
        - Forced vital capacity (FVC) ≥45% of predicted
        - Diffusing capacity for carbon monoxide (DLCO) ≥25% of predicted
    - Must submit documentation of a recent urine or serum nicotine screening confirming that member is NOT currently smoking or recently stopped smoking
    - For Ofev:
        - Must not use nintedanib (Ofev) and pirfenidone (Esbriet) in combination
    - For Jascayd:
        - Must have an adequate trial of nintedanib (Ofev) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy

If the above criteria are met, the requested medication is approved for 6 months.

Reauthorization Criteria (all diagnoses):
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one-year intervals based upon the following:

- Must provide chart documentation from the prescriber showing that the member's condition has stabilized based upon the prescriber's assessment while on therapy (e.g., has anticipated decline in forced vital capacity or six-minute walk distance time, has expected severity and number of idiopathic pulmonary fibrosis exacerbations compared to baseline)
- Must provide current documentation of each of the following lung values: FVC, DLCO
- Must show that continuation of treatment is considered medically necessary by demonstrating at least one of the following:
    - <10% decline in FVC since the previous authorization
    - <15% decline in DLCO since the previous authorization
    - If member has ≥10% decline in FVC OR ≥15% decline in DLCO since the previous authorization, must provide clinical rationale to support continuation of therapy.

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- **Nitisinone (Orfadin®, Nityr®)** [1286, 1287, 1288, 1289, 1290, 1291]

Initial Authorization Criteria:

Nitisinone (Orfadin) capsule

1. **For hereditary tyrosinemia type 1 (HT-1)**
   - Must be prescribed by or in consultation with a physician who specializes in the treatment of inherited metabolic disorders
   - Must have diagnosis of hereditary tyrosinemia type 1 (HT-1) confirmed by chart documentation of ONE of the following:
     - Newborn screening for HT-1 with a positive succinylacetone test (test result with reference range is required)
     - Elevated blood or urine succinylacetone or succinylacetoacetate (SA) level (test result with reference range is required)
     - Genetic test result showing biallelic pathogenic variants in the fumarylacetoacetate hydrolase (FAH) gene (test result is required)
   - Must be used in combination with dietary restriction of tyrosine and phenylalanine
   - Must be prescribed a dose and duration of therapy that is consistent with FDA-approved package labeling, nationally recognized compendia, or peer-reviewed medical literature

2. **Off-label use for alkaptonuria (AKU)**
   - Must be prescribed by or in consultation with a physician who specializes in the treatment of inherited metabolic disorders
   - Must have diagnosis of alkaptonuria (AKU) confirmed by chart documentation of ONE of the following:
     - Elevated blood or urine homogentisic acid (HGA) level (test result with reference range is required)
     - Genetic test result showing biallelic pathogenic variants in the homogentisate 1,2 dioxygenase (HGD) gene (test result is required)
   - Must be prescribed a dose and duration of therapy that is consistent with FDA-approved package labeling, nationally recognized compendia, or peer-reviewed medical literature

Nitisinone (Orfadin) oral suspension

1. **For hereditary tyrosinemia type 1 (HT-1)**
   - Must be prescribed by or in consultation with a physician who specializes in the treatment of inherited metabolic disorders
   - Must have diagnosis of hereditary tyrosinemia type 1 (HT-1) confirmed by chart documentation of ONE of the following:
     - Newborn screening for HT-1 with a positive succinylacetone test (test result with reference range is required)

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        - Elevated blood or urine succinylacetone or succinylacetoacetate (SA) level (test result with reference range is required)
        - Genetic test result showing biallelic pathogenic variants in the fumarylacetoacetate hydrolase (FAH) gene (test result is required)
    - Must be used in combination with dietary restriction of tyrosine and phenylalanine
    - Must be prescribed a dose and duration of therapy that is consistent with FDA-approved package labeling, nationally recognized compendia, or peer-reviewed medical literature
    - Must have documentation of the inability to swallow an oral solid dosage form due to age (< 7 years), medical condition (e.g. nasogastric tube), or other clinical rationale for why nitisinone (Orfadin) capsule cannot be used

2. **Off-label use for alkaptonuria (AKU)**
    - Must be prescribed by or in consultation with a physician who specializes in the treatment of inherited metabolic disorders
    - Must have diagnosis of alkaptonuria (AKU) confirmed by chart documentation of ONE of the following:
        - Elevated blood or urine homogentisic acid (HGA) level (test result with reference range is required)
        - Genetic test result showing biallelic pathogenic variants in the homogentisate 1,2 dioxygenase (*HGD*) gene (test result is required)
    - Must be prescribed a dose and duration of therapy that is consistent with FDA-approved package labeling, nationally recognized compendia, or peer-reviewed medical literature
    - Must have documentation of the inability to swallow an oral solid dosage form due to age (< 7 years), medical condition (e.g. nasogastric tube), or other clinical rationale for why nitisinone (Orfadin) capsule cannot be used

Nitisinone (Nityr) tablet

1. **For hereditary tyrosinemia type 1 (HT-1)**
    - Must be prescribed by or in consultation with a physician who specializes in the treatment of inherited metabolic disorders
    - Must have diagnosis of hereditary tyrosinemia type 1 (HT-1) confirmed by chart documentation of ONE of the following:
        - Newborn screening for HT-1 with a positive succinylacetone test (test result with reference range is required)
        - Elevated blood or urine succinylacetone or succinylacetoacetate (SA) level (test result with reference range is required)
        - Genetic test result showing biallelic pathogenic variants in the fumarylacetoacetate hydrolase (FAH) gene (test result is required)
    - Must be used in combination with dietary restriction of tyrosine and phenylalanine

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- Must be prescribed a dose and duration of therapy that is consistent with FDA-approved package labeling, nationally recognized compendia, or peer-reviewed medical literature
- Must have an adequate trial of generic nitisinone capsule with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
- Must provide clinical rationale for use of nitisinone (Nityr) tablets instead of generic nitisinone capsule. Clinical rationale must include why generic nitisinone capsule would not be expected to produce the same clinical results as nitisinone (Nityr) tablet given that they are the same chemical entity

2. **Off-label use for alkaptonuria (AKU)**
   - Must be prescribed by or in consultation with a physician who specializes in the treatment of inherited metabolic disorders
   - Must have diagnosis of alkaptonuria (AKU) confirmed by chart documentation of ONE of the following:
     - Elevated blood or urine homogentisic acid (HGA) level (test result with reference range is required)
     - Genetic test result showing biallelic pathogenic variants in the homogentisate 1,2 dioxygenase (*HGD*) gene (test result is required)
   - Must be prescribed a dose and duration of therapy that is consistent with FDA-approved package labeling, nationally recognized compendia, or peer-reviewed medical literature
   - Must have an adequate trial of generic nitisinone capsule with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
   - Must provide clinical rationale for use of nitisinone (Nityr) tablets instead of generic nitisinone capsule. Clinical rationale must include why generic nitisinone capsule would not be expected to produce the same clinical results as nitisinone (Nityr) tablet given that they are the same chemical entity

If the above criteria are met, nitisinone (Orfadin, Nityr) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one-year intervals based upon documentation from the prescriber that the member's disease has stabilized or improved based upon the prescriber's assessment while on therapy.

- **Nitroglycerin Ointment 0.4% (Rectiv®)** [1292, 1293, 1294, 1295, 1296, 1297]

Initial Authorization Criteria:

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- Must have a diagnosis of chronic anal fissure
- Must have an adequate trial and failure of conservative treatment through local wound care and relief of constipation
- Must have an adequate trial and failure (duration of at least 3 months) of compounded topical diltiazem or compounded nifedipine with an inadequate response or significant side effect/toxicity or have a contraindication to topical calcium channel blocker therapy
- Must not have any of the following contraindications:
  - Concomitant use of a phosphodiesterase type 5 (PDE5) inhibitor such as sildenafil (Revatio®, Viagra®), tadalafil (Adcirca®, Cialis®), vardenafil (Levitra®, Staxyn®)
  - Severe anemia (defined as hemoglobin < 8g/dL)
  - Increased intracranial pressure

If the above criteria are met, nitroglycerin ointment 0.4% (Rectiv) is approved for 1 month.

Reauthorization Criteria:
All prior authorization renewals are reviewed on a case-by-case basis to determine the Medical Necessity for continuation of treatment. Authorization may be extended based upon the following:

- Documentation of improvement in the condition based upon the prescriber's assessment while on treatment
- Clinical rationale for continuation of topical therapy versus surgery

• Obeticholic acid (Ocaliva™) [1298, 1299]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with a gastroenterologist, hepatologist, or liver transplant specialist
- Must be age 18 years or older
- Must have a diagnosis of primary biliary cholangitis (PBC) confirmed by chart documentation of TWO of the following:
  - Elevated alkaline phosphatase (ALP) above the upper limit of normal (ULN) for at least 6 months based on the reference range provided by the laboratory
  - Positive anti-mitochondrial antibodies (AMA) or other PBC-specific auto-antibodies, if AMA is negative
  - Liver biopsy consistent with PBC
- Must have an adequate trial of at least 12 months with ursodiol at a dose of 13-15 mg/kg/day with an inadequate response defined as ALP 1.5-times the ULN or significant side effect/toxicity or have a contraindication to this therapy

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- Must be used in combination with ursodiol unless there is a clinical contraindication or intolerance to ursodiol
- Must not be used in combination with elafibranor (Iqirvo) or seladelpar (Livdelzi)
- Must not have ANY of the following:
  - Decompensated cirrhosis (e.g., Child-Pugh Class B or C) or a prior decompensation event
  - Compensated cirrhosis with evidence of portal hypertension (e.g., ascites, gastroesophageal varices, persistent thrombocytopenia)
  - Complete biliary obstruction

If the above criteria are met, obeticholic acid (Ocaliva) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one-year intervals based upon chart documentation showing the following:

- The member's condition has improved while on therapy
- Must have an assessment for disease progression including laboratory or clinical evidence of decompensated cirrhosis (e.g., Child-Pugh Class B or C), compensated cirrhosis with evidence of portal hypertension (e.g., ascites, gastroesophageal varices, persistent thrombocytopenia), or complete biliary obstruction
  - NOTE: authorizations may not be extended for members who develop one of these conditions

• Ocrelizumab (Ocrevus™), Ocrelizumab and hyaluronidase-ocsq (Ocrevus Zunovo™) [1300, 1301, 1302, 1303]

Initial Authorization Criteria:

- Must be prescribed by a neurologist
- Must be age 18 years or older
- Must have a diagnosis of ONE of the following:
  - Relapsing forms of multiple sclerosis to include clinically isolated syndrome, relapsing-remitting disease, and active secondary progressive disease
  - Primary progressive multiple sclerosis (PPMS)
- Must not be on concomitant therapy with a disease modifying drug for multiple sclerosis

If the above criteria are met, the requested medication is approved for 6 months.

Reauthorization Criteria:

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All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or stabilized while on therapy.

- **Ocriplasmin (Jetrea®)** [1304, 1305, 1306]

Criteria:
- Must have a diagnosis of vitreomacular adhesion
- Must be prescribed by a retinal specialist
- Must not have had a prior vitrectomy
- Must not be receiving treatment in both eyes within 7 days of each other

If the above criteria are met, ocriplasmin (Jetrea) is approved for one injection per eye per lifetime.

- **Octreotide acetate intramuscular suspension (Sandostatin® LAR Depot)** [1307, 1308, 1309, 1310, 1311, 1312, 1313]

Initial Authorization Criteria:

1. **For acromegaly:**
   - Must be prescribed by or in consultation with an endocrinologist
   - Must be age 18 years or older
   - Must have a confirmed diagnosis of acromegaly to include one of the following baseline (pre-treatment) labs:
     - Elevated serum IGF-1 level for patient's gender and age range. Laboratory reference range must be provided.
     - Elevated growth hormone (GH) level defined as a GH level ≥1ng/mL during oral glucose tolerance test (OGTT)
   - Must have an inadequate response to surgery and/or not have surgery as an option
2. **For severe diarrhea and flushing episodes associated with metastatic carcinoid tumors:**
   - Must be prescribed by a hematologist, oncologist, or surgical oncologist within hematology, oncology
   - Must have a diagnosis of metastatic carcinoid tumor with associated severe diarrhea and flushing episodes
3. **For profuse watery diarrhea associated with vasoactive intestinal peptide (VIP) secreting tumors:**
   - Must be prescribed by a hematologist, an oncologist, or a surgical oncologist within hematology/oncology
   - Must have a diagnosis of vasoactive intestinal peptide secreting tumors with associated profuse watery diarrhea

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4. ***For off-label use* for GEP-NET when undergoing treatment with lutetium Lu 177 dotatate (Lutathera®):**
   - Must be prescribed by a hematologist, an oncologist, or a surgical oncologist within hematology/oncology
   - Must have a diagnosis of GEP-NET confirmed by chart documentation and be undergoing course of treatment with lutetium Lu 177 dotatate (Lutathera)
   - Must be using for symptom control

If the above criteria are met, octreotide acetate (Sandostatin LAR) is approved for the following duration based upon diagnosis:
- Acromegaly, severe diarrhea/flushing associated with metastatic carcinoid tumor, profuse diarrhea associated with VIP-secreting tumor: 6 months
- GEP-NET with lutetium Lu 177 dotatate (Lutathera): 8 months [one course of therapy with lutetium Lu 177 dotatate (Lutathera) (every 8 weeks for 4 doses)]

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved while on therapy.

- **Octreotide delayed release capsules (Mycapssa®)** [1314, 1315, 1316, 1317, 1318]

Initial Authorization Criteria:
- Must be prescribed by or in consultation with an endocrinologist
- Must be age 18 years or older
- Must have a confirmed diagnosis of acromegaly to include one of the following baseline (pre-treatment) labs:
  - Elevated serum IGF-1 level for patient's natal gender and age range. Laboratory reference range must be provided.
  - Elevated growth hormone (GH) level defined as a GH level ≥1ng/mL during oral glucose tolerance test (OGTT)
- Must have an inadequate response to surgery and/or not have surgery as an option
- Must have responded to and tolerated treatment with injectable octreotide or lanreotide
- Must provide clinical rationale for use of octreotide delayed release capsules (Mycapssa) instead of lanreotide acetate (Somatuline Depot) or octreotide acetate long-acting release (Sandostatin LAR). Clinical rationale must include why lanreotide acetate (Somatuline Depot) or octreotide acetate long-acting release (Sandostatin LAR) would not be expected to produce the same clinical results as octreotide delayed release capsules (Mycapssa).

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- Must not be used concurrently with an injectable somatostatin analog [e.g.- octreotide, octreotide LAR (Sandostatin® LAR Depot), lanreotide (Somatuline® Depot), pasireotide (Signifor®) or pasireotide (Signifor® LAR)]

If the above criteria are met, octreotide delayed release capsule (Mycapssa) is approved for 6 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon:

- Documentation from the provider that the member's condition has improved based upon the prescriber's assessment while on therapy or the member continues to benefit from therapy
- Must not be used concurrently with an injectable somatostatin analog [e.g.- octreotide, octreotide LAR (Sandostatin LAR Depot), lanreotide (Somatuline Depot), pasireotide (Signifor) or pasireotide (Signifor LAR)]

- **Odevixibat (Bylvay®)** 1319, 1320, 1321, 1322, 1323, 1324, 1325, 1326, 1327, 1328

Initial Authorization Criteria:

1. **For progressive familial intrahepatic cholestasis (PFIC):**
   - Must be prescribed by a hepatologist or gastroenterologist
   - Must be age 3 months or older
   - Must have a diagnosis of pruritus associated with progressive familial intrahepatic cholestasis (PFIC) confirmed by ALL of the following:
     - Must provide genetic testing demonstrating a mutation associated with PFIC (a copy of the genetic test must be provided)
       - Genetic mutations associated with PFIC, include, but are not limited to: *ATP8B1*, *ABCB11*, *ABCB4*, *TJP2*, *NR1H4*, and *MYO5B*
       - Must not have genetic testing results that indicate PFIC type 2 with *ABCB11* variants resulting in nonfunctional or complete absence of bile salt export pump protein (BSEP-3)
     - Must provide chart documentation of moderate to severe pruritis that causes a negative effect on quality of life (e.g., sleep disturbances, difficulty concentrating during the day)
     - Must have a serum bile acid concentration above the upper limit of normal (chart documentation of test result including reference range must be provided)

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- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to at least TWO medications for PFIC-associated pruritis (e.g., ursodiol, rifampin, cholestyramine)
- Must provide current weight and be using a dose and duration of therapy consistent with the FDA-approved package labeling
- Must not exceed a total daily dose of 6 mg
- Requests for doses above 40 mcg/kg/day:
  - Must have an adequate trial of at least three months with inadequate response at the 40 mcg/kg/day dose
  - Must provide chart documentation that pruritis continues to affect quality of life despite treatment at the 40 mcg/kg/day dose

2. For Alagille syndrome (ALGS):

- Must be prescribed by a hepatologist or gastroenterologist
- Must be age 12 months or older
- Must have a diagnosis of cholestatic pruritus associated with Alagille syndrome (ALGS) confirmed by ALL of the following:
  - Must provide genetic testing demonstrating a *JAG1* or *NOTCH2* mutation (a copy of the genetic test must be provided)
  - Must provide chart documentation of moderate to severe pruritis that causes a negative effect on quality of life (e.g., sleep disturbances, difficulty concentrating during the day)
  - Must have a serum bile acid concentration above the upper limit of normal (chart documentation of test result including reference range must be provided)
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to at least TWO medications for ALGS-associated pruritis (e.g., ursodiol, rifampin, cholestyramine)
- Must have an adequate trial of maralixibat (Livmarli) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
- Must provide current weight and be using a dose and duration of therapy consistent with the FDA-approved package labeling

If the above criteria are met, odevixibat (Bylvay) is approved for 6 months.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon the following:

- Must have chart documentation from the provider that the member's condition has improved while on therapy
- Must provide current weight and be using a dose and duration of therapy consistent with the FDA-approved package labeling

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- For a diagnosis of PFIC:
  - Must not exceed a total daily dose of 6 mg
  - Requests for doses above 40 mcg/kg/day:
    - Must have an adequate trial of at least three months with inadequate response at the 40 mcg/kg/day dose
    - Must provide chart documentation that pruritis continues to affect quality of life despite treatment at the 40 mcg/kg/day dose

• Ofatumumab (Kesimpta®) [1329]

Initial Authorization Criteria

- Must be prescribed by a neurologist
- Must be age 18 years or older
- Must have a diagnosis of relapsing form of multiple sclerosis, to include clinically isolated syndrome, relapsing-remitting disease, and active secondary progressive disease
- Must not be on concomitant therapy with a disease modifying drug for multiple sclerosis

If the above criteria are met, ofatumumab (Kesimpta) is approved for 1 year.

Reauthorization Criteria

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or stabilized while on therapy.

• Olanzapine and Samidorphan (Lybalvi®) [1330, 1331, 1332, 1333, 1334, 1335]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with a psychiatrist
- Must be age 18 years or older
  - For members < 12 years of age, please reference 'Atypical and Typical Antipsychotics – Children Under 12 years of Age' criteria
- Must have a diagnosis of schizophrenia or bipolar I disorder
- Must have an adequate trial, duration of at least 4 weeks, with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
  - Olanzapine
    - Must not have failed olanzapine due to lack of efficacy or adverse effects other than weight gain AND must provide

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documentation demonstrating unacceptable weight gain while on olanzapine
    - THREE generic atypical antipsychotics (e.g., aripiprazole, quetiapine or quetiapine ER, risperidone)
  - Must not currently be using opioids or undergoing acute opioid withdrawal
  - Must delay initiation of olanzapine and samidorphan for at least 7 days after last use of short-acting opioids and at least 14 days after last use of long-acting opioids

If the above criteria are met, olanzapine and samidorphan (Lybalvi) is approved for 1 year.

Reauthorization criteria:
All prior authorization requests are reviewed on an annual basis to determine the Medical Necessity for continuation of treatment. Authorizations may be extended at one-year intervals based upon the following:
  - Documentation from the prescriber that the member's condition has improved based upon the prescriber's assessment while on therapy.
  - Annual monitoring of weight and BMI
  - Must not currently be using opioids or undergoing acute opioid withdrawal

• Olezarsen (Tryngolza™) [1336, 1337]

Initial Authorization Criteria:
  - Must be prescribed by or in consultation with a cardiologist, endocrinologist, or lipidologist
  - Must be age 18 years or older
  - Must have a diagnosis of familial chylomicronemia syndrome (FCS) confirmed by BOTH of the following:
    - Genetic testing demonstrating biallelic pathogenic variants in the *LPL*, *GPIHBP1*, *APOA5*, *APOC2*, or *LMF1* genes (a copy of the genetic test must be provided)
    - Fasting triglyceride level ≥ 880 mg/dL despite dietary modifications and triglyceride-lowering therapy (e.g., omega-3 fatty acids, fibrates or other lipid-lowering medications). Chart documentation must be provided.
  - Must be used in conjunction with a low-fat diet
  - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to plozasiran (Redemplo).

If the above criteria are met, olezarsen (Tryngolza) is approved for 1 year.

Reauthorization Criteria:

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All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon chart documentation showing the following:

- The member's condition has improved while on therapy
- Must have a reduction in fasting triglyceride levels from baseline
- Must continue to be used in conjunction with a low-fat diet

• Olsalazine (Dipentum®) and Mesalamine CR (Pentasa®)

1338, 1339, 1340, 1341, 1342, 1343, 1344, 1345, 1346, 1347, 1348, 1349

Initial Authorization Criteria:

- Must have a diagnosis of ulcerative colitis
- Must have had an adequate trial to TWO of the following with an inadequate response or significant side effects/toxicity or have a contraindication to these therapies:
  - Balsalazide (Colazal®)
  - Mesalamine DR (Lialda®, Asacol® HD, Delzicol)
  - Mesalamine ER (Apriso®)
  - Sulfasalazine
  - Sulfasalazine DR

If the above criteria are met, the requested product is approved.

Off-label use for mesalamine CR (Pentasa):

- Must have a diagnosis of Crohn's disease
- Must have an adequate trial to ONE of the following with an inadequate response or significant side effects/toxicity or have a contraindication to these therapies:
  - Corticosteroids
  - Azathioprine
  - 6-mercaptopurine
  - Methotrexate
  - Sulfasalazine
  - Sulfasalazine DR

If the above criteria are met, mesalamine CR (Pentasa) is approved.

• Omadacycline (Nuzyra®) 1350, 1351, 1352, 1353

Criteria:

1. Community-acquired pneumonia (CAP)

- Must be prescribed by or in consultation with an infectious disease specialist.

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- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to all formulary drugs used to treat this indication.
  - NOTE: an exception to the requirement for a trial of all formulary drugs used to treat CAP infections can be made if the member has a history of *Clostridioides difficile* infection (CDI)

If the above criteria are met, omadacycline (Nuzyra) is approved for 1 month.

2. Skin and Soft Tissue Infections (SSTIs)
- Must be prescribed by or in consultation with an infectious disease specialist.
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to all formulary drugs used to treat this indication.

If the above criteria are met, omadacycline (Nuzyra) is approved for 1 month.

Off-label Uses
1. Nontuberculous Mycobacteria (NTM) Infections
- Must be prescribed by or in consultation with an infectious disease specialist.

If the above criteria are met, omadacycline (Nuzyra) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on a case by case basis for medical necessity and are subject to the above initial authorization criteria.

- **Omalizumab Autoinjector, Syringe (Xolair®)** [1354, 1355, 1356, 1357, 1358, 1359, 1360, 1361, 1362, 1363, 1364]
  - Requests for omalizumab vial (Xolair) please refer to policy RX.PA.406 Commercial, Exchange Medical Prior Authorization Policies

1. For Asthma:
Initial Authorization Criteria:
- Must be prescribed by or in consultation with an allergist, an immunologist, or a pulmonologist
- Must be age 6 years or older
- Must have a diagnosis of moderate to severe persistent asthma
- Must have a baseline IgE level ≥30IU/mL
- Must have a positive skin test or *in vitro* testing [i.e., a blood test for allergen-specific IgE antibodies such as the radioallergosorbent test (RAST)] for one or more perennial aeroallergens (e.g., house dust mite, animal dander, cockroach, feathers, mold spores) AND/OR for one or more seasonal aeroallergens (grass, pollen, weeds)

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- Must have received at least 3 consecutive months of therapy with ONE of the following combinations for the maintenance treatment of asthma:
  - A medium-high dose inhaled corticosteroid AND one additional asthma controller medication (e.g., inhaled long-acting beta-agonist, inhaled long-acting muscarinic antagonist)
  - A maximally tolerated inhaled corticosteroid AND long-acting beta-agonist combination product
- Must have history of one asthma exacerbation in the previous year despite adherence to optimized medication therapy regimen. Asthma exacerbation is defined by ONE of the following:
  - Hospitalization for asthma
  - Emergency room visit for asthma
  - Requirement for 2 or more courses of treatment with systemic (e.g., oral, parenteral) corticosteroids
- Must have a requested dose, based on IgE level and weight, that falls within the recommended dosing guidelines from the manufacturer (See Table 1, Table 2, and Table 3 below)
- Must be used as an add-on maintenance treatment to asthma controller therapy
- Must not be receiving combination therapy with another biologic agent (e.g., Dupixent, Cinqair, Fasenra, Nucala, Tezspire) for this diagnosis

Xolair Dosing Guidelines
Table 1
ADMINISTRATION EVERY 4 WEEKS
Xolair® Doses (milligrams) Administered by Subcutaneous Injection Every 4 Weeks for Adults and Adolescents (12 Years of Age and Older) with Asthma

Pre-treatment Serum IgE (IU/mL)	Body Weight (kg)			
	30-60	> 60-70	> 70-90	> 90-150
≥30-100	150	150	150	300
> 100-200	300	300	300	**SEE TABLE 2**
> 200-300	300	**SEE TABLE 2**		
> 300-400	**SEE TABLE 2**			
> 400-500				
> 500-600				

Table 2
ADMINISTRATION EVERY 2 WEEKS
Xolair® Doses (milligrams) Administered by Subcutaneous Injection Every 2 Weeks for Adults and Adolescents (12 Years of Age and Older) with Asthma

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Pre-treatment Serum IgE (IU/mL)	Body Weight (kg)			
	30-60	> 60-70	> 70-90	> 90-150
≥30-100	**SEE TABLE 1**			**SEE TABLE 1**
> 100-200				225
> 200-300	**SEE TABLE 1**	225	225	300
> 300-400	225	225	300	**DO NOT DOSE**
> 400-500	300	300	375	
> 500-600	300	375	**DO NOT DOSE**	
> 600-700	375	**DO NOT DOSE**		

Table 3

Xolair® Doses (milligrams) Administered by Subcutaneous Injection Every 2 or 4 Weeks for Pediatric Patients with Asthma Between the Ages of 6 to <12 Years*

Pre-treatment Serum IgE (IU/mL)	Dosing Freq.	Body Weight (kg)									
		20-25	>25-30	>30-40	>40-50	>50-60	>60-70	>70-80	>80-90	>90-125	>125-150
		Dose (mg)									
30-100	Every 4 weeks	75	75	75	150	150	150	150	150	300	300
>100-200		150	150	150	300	300	300	300	300	225	300
>200-300		150	150	225	300	300	225	225	225	300	375
>300-400		225	225	300	225	225	225	300	300		
>400-500		225	300	225	225	300	300	375	375		
>500-600		300	300	225	300	300	375				
>600-700		300	225	225	300	375					
>700-800	Every 2 weeks	225	225	300	375						
>800-900		225	225	300	375						
>900-1000		225	300	375							
>1000-1100		225	300	375							
>1100-1200		300	300								
>1200-1300		300	375								

DO NOT DOSE

***Dosing frequency:**

- ■ Subcutaneous doses to be administered every 4 weeks
- □ Subcutaneous doses to be administered every 2 weeks

If the above criteria are met, omalizumab autoinjector, syringe (Xolair) is approved for 1 year.

<u>Reauthorization Criteria:</u>

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All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved while on therapy (e.g., reduction in exacerbations, hospitalizations, emergency department visits, or requirement for oral corticosteroid therapy).

2. For Chronic Spontaneous Urticaria (CSU):
Initial Authorization Criteria:

- Must be prescribed by an allergist, immunologist, or dermatologist
- Must be age 12 years or older
- Must have a diagnosis of chronic spontaneous urticaria confirmed by BOTH of the following. Chart documentation must be provided:
  - A 6-week history of urticaria with the presence of hives
  - A clinical work-up to rule out other diagnoses (e.g., cold or heat urticaria, cholinergic urticaria, contact urticaria)
- Must have Urticaria Activity Score over 7 days (UAS7) ≥ 16
- Must have symptoms that continue to be uncontrolled despite adherence to one of the following at four-fold the starting dose: cetirizine, fexofenadine, or levocetirizine
- Must continue treatment with cetirizine, fexofenadine, or levocetirizine at maximally tolerated dose
- Must not be receiving combination therapy with another biologic (e.g., Dupixent)

If the above criteria are met, omalizumab autoinjector, syringe (Xolair) is approved for 6 months. Dosing above 300mg every 4 weeks is not covered for a diagnosis of urticaria.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon the following:

- Documentation from the provider that the member's condition has improved while on therapy
- An evaluation of the continued need for omalizumab (Xolair). Must provide clinical rationale for why unable to taper or discontinue therapy

3. For Chronic Rhinosinusitis with Nasal Polyps:
Initial Authorization Criteria:

- Must be prescribed by or in consultation with an allergist, immunologist, or an otolaryngologist (ear, nose, and throat [ENT])
- Must be age 18 years or older

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- Must have a diagnosis of chronic rhinosinusitis with nasal polyps. Chart documentation confirming the diagnosis is required as outlined below:
  - Must have documentation of one of the following:
    - A baseline endoscopic bilateral nasal polyp score (NPS) of at least 5 out of a maximum score of 8 (with a minimum score of 2 in each nasal cavity)
    - Evidence of polyposis on sinus CT
  - Must have two or more of the following symptoms present for at least 3 months:
    - Nasal obstruction (congestion)
    - Mucopurulent discharge (rhinorrhea or post-nasal drip)
    - Facial pain or pressure
    - Reduction or loss of smell
- Must have a baseline IgE level ≥30 IU/mL
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ONE of the following:
  - A generic intranasal corticosteroid
  - Fluticasone propionate (Xhance)
  - NOTE: If the member has a history of sinus surgery, medication trial must be post-surgery.
- Must be used in combination with an intranasal corticosteroid, unless intolerant or contraindicated
- Must not be receiving combination therapy with another biologic agent (e.g., Dupixent, Cinqair, Fasenra, Nucala, Tezspire)
- Must have a requested dose, based on IgE level and weight, that falls within the recommended dosing guidelines from the manufacturer (See Table 4)

Table 4
Xolair Dosing Guidelines
Subcutaneous Xolair Doses Every 2 or 4 Weeks for Adult Patients with Nasal Polyps*

Pretreatment Serum IgE (IU/mL)	Dosing Freq.	Bodyweight							
		>30-40 kg	>40-50 kg	>50-60 kg	>60-70 kg	>70-80 kg	>80-90 kg	>90-125 kg	> 125-150 kg
		Dose (mg)							
30-100	Every 4 weeks	75	150	150	150	150	150	300	300
>100-200		150	300	300	300	300	300	450	600
>200-300		225	300	300	450	450	450	600	375
>300-400		300	450	450	450	600	600	450	525
>400-500		450	450	600	600	375	375	525	600
>500-600		450	600	600	375	450	450	600	
>600-700		450	600	375	450	450	525		
>700-800	Every 2 weeks	300	375	450	450	525	600		
>800-900		300	375	450	525	600			
>900-1000		375	450	525	600				

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>1000-1100		375	450	600	**In Insufficient Data to Recommend a Dose**
>1100-1200		450	525	600	
>1200-1300		450	525		
>1300-1500		525	600		

***Dosing frequency:**
- ■ Subcutaneous doses to be administered every 4 weeks
- □ Subcutaneous doses to be administered every 2 weeks

If the above criteria are met, omalizumab autoinjector, syringe (Xolair) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved while on therapy (e.g., reduction in nasal polyp score (NPS), reduction in exacerbations requiring systemic corticosteroids and/or antibiotics).

4. For Ig-E Mediated Food Allergy
Initial Authorization Criteria:
- Must be prescribed by an allergist or immunologist
- Must be age 1 year or older
- Must have a diagnosis of Ig-E mediated food allergy confirmed by the following. Chart documentation must be provided:
  - Must have a history of allergic reaction (e.g., hives, lip swelling, itching, anaphylaxis) attributed to the allergen food
  - Must have ONE of the following:
    - Positive skin prick test (SPT) response to the allergen food
    - Positive serum allergen specific IgE (psIgE) test
    - Positive medically supervised oral food challenge response
- Must be used in conjunction with food allergen avoidance
- Must not be receiving combination therapy with another biologic agent (e.g., Dupixent, Cinqair, Fasenra, Nucala, Tezspire) or with Peanut Allergen-dnfp (Palforzia)
- Must have a requested dose, based on IgE level and weight, that falls within the recommended dosing guidelines from the manufacturer (See Table 5 below)

Table 5
Xolair Dosing Guidelines
Subcutaneous Xolair Doses Every 2 or 4 Weeks for Adult and Pediatric Patients 1 Year of Age and Older with IgE-Mediated Food Allergy*

		Bodyweight (kg)

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Pretreatment Serum IgE (IU/mL)	Dosing Freq.	≥ 10-12	>12-15	>15- 20	>20–25	>25-30	>30-40	>40-50	>50-60	>60-70	>70-80	>80-90	>90-125	>125-150
		Dose (mg)												
≥30-100	Every 4 weeks	75	75	75	75	75	75	150	150	150	150	150	300	300
>100-200		75	75	75	150	150	150	300	300	300	300	300	450	600
>200-300		75	75	150	150	150	225	300	300	450	450	450	600	375
>300-400		150	150	150	225	225	300	450	450	450	600	600	450	525
>400-500		150	150	225	225	300	450	450	600	600	375	375	525	600
>500-600		150	150	225	300	300	450	600	600	375	450	450	600	
>600-700		150	150	225	300	225	450	600	375	450	450	525		
>700-800	Every 2 weeks	150	150	150	225	225	300	375	450	450	525	600		
>800-900		150	150	150	225	225	300	375	450	525	600			
>900-1000		150	150	225	225	300	375	450	525	600				
>1000-1100		150	150	225	225	300	375	450	600					
>1100-1200		150	150	225	300	300	450	525	600					
>1200-1300		150	225	225	300	375	450	525						
>1300-1500		150	225	300	300	375	525	600						
>1500-1850			225	300	375	450	600							

Insufficient Data to Recommend a Dose

***Dosing frequency:**
- Subcutaneous doses to be administered every 4 weeks
- Subcutaneous doses to be administered every 2 weeks

If the above criteria are met, omalizumab autoinjector, syringe (Xolair) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one-year intervals based upon chart documentation showing the following:
- Must have documentation from the provider that the member's condition has improved or the member continues to benefit from therapy (e.g., fewer episodes of allergic reaction/anaphylaxis, improved quality of life)
- Must be used in conjunction with food allergen avoidance
- Must have a requested dose, based on IgE level and weight, that falls within the recommended dosing guidelines from the manufacturer
- Must have an attestation from the provider that the member has been evaluated for the need for continued therapy

• Omaveloxolone (Skyclarys™) [1365, 1366, 1367]

Initial Authorization Criteria:
- Must be prescribed by a neurologist
- Must be 16 to 40 years of age

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- Must have a diagnosis of Friedreich's ataxia confirmed by ALL of the following:
  - Genetic testing showing ONE of the following abnormalities of the frataxin (FXN) gene (documentation of the genetic testing must be provided):
    - Biallelic guanine-adenine-adenine (GAA) repeat expansion
    - One allele with GAA repeat expansion and one allele with a point mutation or deletion
  - Chart documentation of a history of cerebellar ataxia (e.g., imbalance, incoordination, vertigo)
- Must not be taking concomitantly with a strong or moderate CYP 3A4 inducer (e.g., rifampin, carbamazepine, phenytoin, cenobamate, phenobarbital, primidone)

If the above criteria are met, omaveloxolone (Skyclarys) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon ALL of the following:

- Chart documentation from the provider that the member's condition has improved or stabilized, or the member continues to benefit from therapy
- Must not be taking concomitantly with a strong or moderate CYP 3A4 inducer (e.g., rifampin, carbamazepine, phenytoin, cenobamate, phenobarbital, primidone)

• Opicapone (Ongentys®) [1368, 1369, 1370, 1371, 1372]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with a neurologist
- Musts have a diagnosis of Parkinson's disease (PD) and be experiencing "off" episodes
- Must have motor fluctuations with an average daily "off" time of 1.5 hours or longer
- Must have continued "off" symptoms despite adherence to optimized medication therapy regimen based on the prescriber's assessment
- Must be on concomitant therapy with carbidopa/levodopa
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to TWO of the following, taken in combination with carbidopa/levodopa for the adjunctive treatment of "off" episodes:
  - Entacapone

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        - Either one generic MAO-B inhibitor (e.g., selegiline or rasagiline) OR one generic dopamine agonist (e.g., ropinirole tablet, pramipexole tablet)
    - Must not be on concomitant therapy with non-selective monoamine oxidase (MAO) inhibitors (e.g. phenelzine, isocarboxazid, tranylcypromine)
    - Must not have a history of pheochromocytoma, paraganglioma, or other catecholamine secreting neoplasms

If the above criteria are met, opicapone (Ongentys) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one-year intervals based upon documentation from the prescriber that the member's condition has improved while on therapy or the member continues to benefit from therapy.

- **Opioid Analgesics, Long-acting** [1373, 1374, 1375, 1376, 1377]
    - For oxycodone extended-release tablet for Open Choice and Value Choice members, in addition to below criteria for Opioid Analgesics, Long-acting, see Oxycodone Extended-Release Tablet
    - For methadone products, the below criteria only apply to the dosage forms and strengths on the formulary when being used to treat pain. Methadone products when used for the treatment of opioid addiction in detoxification or maintenance programs are only to be dispensed by certified opioid treatment programs as stipulated in the Federal Opioid Treatment Standards (42 CFR 8.12) and are covered on the medical benefit only.

Initial Authorization Criteria:

1. **For adults 18 years of age and older**
    - Must be used for active cancer, sickle cell, as part of end-of-life care, or must have all the following:
        - Must have a diagnosis of severe pain due to a medical condition
        - Must have a history of a therapeutic failure or an intolerance or contraindication to the following:
            - 3 non-opioid analgesics (e.g., acetaminophen, NSAID, antidepressant, anticonvulsant) appropriate for the member's condition
            - A short-acting opioid analgesic
            - NOTE: the above medication trials do not apply to requests for buprenorphine products
        - Must be opioid-tolerant

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- Must have a prescribed dose that is appropriate based on the FDA-approved package labeling, nationally recognized compendia, or peer-reviewed medical literature
- Must have been assessed for potential risk of opioid misuse or use disorder by the provider
- For concurrent use with benzodiazepines, must provide ONE of the following:
  - Documentation of discontinuation/taper plan
  - Clinical rationale for concurrent use
- Must not be used in combination with other long-acting opioids
- Based on the clinical assessment of the reviewer, the following may also be considered:
  - Education to the provider on appropriate prescribing of opioids, clinical guidelines, and/or care management programs
  - Assessment of the need for a specialist referral
  - Referral to care management if member is identified as high risk (e.g., concurrent benzodiazepine use, overdose history, other behavioral health conditions, illegal drug use, history of substance abuse disorder)

2. For children and adolescents age 17 years and younger

- Must be used for active cancer, sickle cell, as part of end-of-life care, neonatal abstinence syndrome, or must have all of the following:
  - Must have a diagnosis of severe pain due to a medical condition
  - Must have a history of a therapeutic failure or an intolerance or contraindication to the following:
    - 2 non-opioid analgesics (e.g., acetaminophen, NSAID, antidepressant, anticonvulsant) appropriate for the member's condition
    - A short-acting opioid analgesic
    - NOTE: the above medication trials do not apply to requests for buprenorphine products
  - Must be opioid-tolerant
  - Must have a prescribed a drug and dose that is appropriate based on the member's age, weight, and concurrent medical conditions and is consistent with the FDA-approved package labeling, nationally recognized compendia, or peer-reviewed medical literature
  - The member, parent and/or guardian must have been assessed for potential risk of opioid misuse or use disorder by the provider
  - For concurrent use with benzodiazepines, must provide ONE of the following:
    - Documentation of discontinuation/taper plan
    - Clinical rationale for concurrent use

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        - Must not be used in combination with other long-acting opioids
        - Based on the clinical assessment of the reviewer, the following may also be considered:
            - Education to the provider on appropriate prescribing of opioids, clinical guidelines, and/or care management programs
            - Assessment of the need for a specialist referral
            - Referral to care management if member is identified as high risk (e.g., concurrent benzodiazepine use, overdose history, other behavioral health conditions, illegal drug use, history of substance abuse disorder)

If the above criteria are NOT met, exceptions may be allowed for up to 3 months to avoid abrupt discontinuation of current therapy based on the clinical assessment of the reviewer.

If the above criteria are met, the requested opioid analgesic is approved for a duration based upon diagnosis as follows:

- For cancer, sickle cell, end of life care, or neonatal abstinence syndrome: 1 year
- For all other pain related diagnoses: up to 1 year

Reauthorization Criteria:

All prior authorization renewals are reviewed to determine the Medical Necessity for continuation of therapy. Authorization may be extended on a case-by-case basis for up to 1 year based upon documentation from the provider showing the following:

- The member's condition has improved or stabilized while on therapy
- Must provide an attestation showing improved pain control and improved level of functioning
- The member, parent (if for pediatric members), and/or guardian (if for pediatric members) must have been reassessed for potential risk of opioid misuse or use disorder by the provider
- For concurrent use with benzodiazepines, must provide ONE of the following:
    - Documentation of discontinuation/taper plan
    - Clinical rationale for concurrent use
- Based on the clinical assessment of the reviewer, the following may also be considered:
    - Education to the provider on appropriate prescribing of opioids, clinical guidelines, and/or care management programs
    - Assessment of the need for a specialist referral
    - Referral to care management if member is identified as high risk (e.g., concurrent benzodiazepine use, overdose history, other behavioral health conditions, illegal drug use, history of substance abuse disorder)

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If the above criteria are NOT met, exceptions may be allowed for up to 3 months to avoid abrupt discontinuation of current therapy based on the clinical assessment of the reviewer.

- **Opioid Analgesics, Short-Acting** [1378, 1379, 1380, 1381, 1382, 1383]
  - For transmucosal fentanyl citrate products, in addition to below criteria for Opioid Analgesics, Short-acting, see Transmucosal Fentanyl Citrate

Initial Authorization Criteria:

1. **For adults 18 years of age and older**
   - Prior authorization applies to short-acting opioid analgesics prescribed for greater than a 10-day supply or if the member has a claim history for a short-acting opioid in the past 180 days
   - Must be used for active cancer, sickle cell, as part of end-of-life care, post-operative treatment, following a traumatic injury, or must have all the following:
     - Must have a diagnosis of moderate to severe pain due to a medical condition
     - Must have a history of a therapeutic failure or an intolerance or contraindication to the following:
       - 3 non-opioid analgesics (e.g., acetaminophen, NSAID, antidepressant, anticonvulsant) appropriate for the member's condition
     - Must have a prescribed dose that is appropriate based on the FDA-approved package labeling, nationally recognized compendia, or peer-reviewed medical literature
     - Must have been assessed for potential risk of opioid misuse or use disorder by the provider
     - For concurrent use with benzodiazepines, must provide ONE of the following:
       - Documentation of discontinuation/taper plan
       - Clinical rationale for concurrent use
     - Based on the clinical assessment of the reviewer, the following may also be considered:
       - Education to the provider on appropriate prescribing of opioids, clinical guidelines, and/or care management programs
       - Assessment of the need for a specialist referral
       - Referral to care management if member is identified as high risk (e.g., concurrent benzodiazepine use, overdose history, other behavioral health conditions, illegal drug use, history of substance abuse disorder)

2. **For children and adolescents 17 years of age and younger**

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- Prior authorization applies to short-acting opioid analgesics prescribed for greater than a 5-day supply or if the member has a claim history for a short-acting opioid in the 180 days
- Must be used for active cancer, sickle cell, as part of end-of-life care, post-operative treatment, following a traumatic injury, neonatal abstinence syndrome, or must have all of the following:
    - Must have a diagnosis of moderate to severe pain due to a medical condition
    - Must have a history of a therapeutic failure or an intolerance or contraindication to the following:
        - 2 non-opioid analgesics (e.g., acetaminophen, NSAID, antidepressant, anticonvulsant) appropriate for the member's condition
    - Must have a prescribed a drug and dose that is appropriate based on the member's age, weight, and concurrent medical conditions and is consistent with FDA-approved package labeling, nationally recognized compendia, or peer-reviewed medical literature
    - The member, parent, and/or guardian must have been assessed for potential risk of opioid misuse or use disorder by the provider
    - For concurrent use with benzodiazepines, must provide ONE of the following:
        - Documentation of discontinuation/taper plan
        - Clinical rationale for concurrent use
    - Based on the clinical assessment of the reviewer, the following may also be considered:
        - Education to the provider on appropriate prescribing of opioids, clinical guidelines, and/or care management programs
        - Assessment of the need for a specialist referral
        - Referral to care management if member is identified as high risk (e.g., concurrent benzodiazepine use, overdose history, other behavioral health conditions, illegal drug use, history of substance abuse disorder)
- Additionally, for codeine and tramadol containing products:
    - Must provide clinical rationale for prescribing these medications since they carry safety risks and prescribing restrictions in children and adolescents

If the above criteria are NOT met, exceptions may be allowed for up to 3 months to avoid abrupt discontinuation of current therapy based on the clinical assessment of the reviewer.

If the above criteria are met, the requested opioid analgesic is approved for a duration based upon diagnosis as follows:

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- For cancer, sickle cell, end of life care, and neonatal abstinence syndrome: 1 year
- For all other pain related diagnoses: up to 1 year

Reauthorization Criteria:
All prior authorization renewals are reviewed to determine the Medical Necessity for continuation of therapy. Authorization may be extended on a case-by-case basis for up to 1 year based upon documentation from the provider showing the following:

- The member's condition has improved or stabilized while on therapy
- Must provide an attestation showing improved pain control and improved level of functioning
- The member, parent (if for pediatric members), and/or guardian (if for pediatric members) must have been reassessed for potential risk of opioid misuse or use disorder by the provider
- For concurrent use with benzodiazepines, must provide ONE of the following:
  - Documentation of discontinuation/taper plan
  - Clinical rationale for concurrent use
- Based on the clinical assessment of the reviewer, the following may also be considered:
  - Education to the provider on appropriate prescribing of opioids, clinical guidelines, and/or care management programs
  - Assessment of the need for a specialist referral
  - Referral to care management if member is identified as high risk (e.g., concurrent benzodiazepine use, overdose history, other behavioral health conditions, illegal drug use, history of substance abuse disorder)

If the above criteria are NOT met, exceptions may be allowed for up to 3 months to avoid abrupt discontinuation of current therapy based on the clinical assessment of the reviewer.

• Oral and Topical Oncology Agents

- Requests for off-label use are reviewed in accordance with RX.013.1 Off-Label Use of Drugs and Biologicals in an Anti-Cancer Chemotherapeutic Regimen – Commercial, Exchange
- Members on ongoing therapy are not required to complete prerequisite medication trials where applicable as required by the policy and requested use is determined to be for an FDA-approved indication or a medically-accepted indication based on appropriate literature/compendia support.
- See Appendix A. for affected products and additional criteria

Initial Authorization Criteria:

- Must be prescribed by an oncologist, a hematologist, or a provider who specializes in the condition being treated
- Must have a diagnosis consistent with the FDA-approved package labeling

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        - If indication for use includes a specific genetic mutation, must have chart documentation of lab result confirming mutation
    - Requested dose and treatment duration must fall within the recommended dosing guidelines in FDA-approved labeling, clinical literature, and/or applicable compendia

If the above criteria are met, the requested medication is approved for up to 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on a case-by-case basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at up to one-year intervals based upon documentation from the provider that the member's condition has stabilized or improved while on therapy.

• Oral Delayed-Release Cysteamine Bitartrate (Procysbi®)

1384, 1385, 1386, 1387, 1388, 1389, 1390, 1391, 1392

Initial Authorization Criteria:
- Must be prescribed by or in consultation with a provider who specializes in the treatment of inherited metabolic disorders
- Must have a diagnosis of nephropathic cystinosis confirmed by ONE of the following (chart documentation is required):
    - Elevated baseline white blood cell (WBC) cystine levels >1 nmol/half-cystine/mg protein
    - Elevated baseline granulocyte cystine level >2 nmol/half-cystine/mg protein
    - *CTNS* gene mutation
- Must have an adequate trial of immediate-release cysteamine (Cystagon) with an inadequate response defined as inadequately controlled WBC cystine levels despite dose titration and compliance with therapy (documentation of laboratory results are required)
- Must provide clinical rationale for use of delayed-release cysteamine (Procysbi) instead of immediate-release cysteamine (Cystagon). Clinical rationale must include why immediate-release cysteamine (Cystagon) would not be expected to produce the same clinical results as delayed-release cysteamine (Procysbi) given that they are the same chemical entity

If the above criteria are met, delayed-release cysteamine (Procysbi) is approved for 3 months.

Reauthorization Criteria:
All authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of treatment. Authorization may be extended at one-year intervals based upon chart documentation showing the following:

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- Must be adherent (PDC >80%) to delayed-release cysteamine (Procysbi)
  - If the member is unable to adhere to the requested regimen, must submit documentation of BOTH of the following:
    - Methods used to increase adherence
    - Adherence counseling has been performed by the provider
- Laboratory results showing reduction in WBC cystine levels since starting treatment with delayed-release cysteamine (Procysbi)

• Osilodrostat (Isturisa®) [1393, 1394, 1395, 1396]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with an endocrinologist
- Must be age 18 years and older
- Must have a diagnosis of Cushing's syndrome
- Must have previously had surgical resection of the primary lesion (e.g., trans-sphenoidal resection of pituitary tumor, adrenalectomy, resection of ectopic ACTH-secreting tumor) that was not curative or not be a candidate for surgery
- Must submit a baseline 24-hour urinary free cortisol level
- Must have had an adequate trial and failure of ALL of the following with an inadequate response or significant side effects/toxicity or have a contraindication to these therapies:
  - Ketoconazole
  - Metyrapone (Metopirone®)
  - Cabergoline or pasireotide (Signifor®, Signifor LAR®)
    - NOTE: this trial is required for a diagnosis of Cushing's disease only

If the above criteria are met, osilodrostat (Isturisa) is approved for 3 months.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one year intervals based upon chart documentation from the provider that the member's disease course has improved based on a reduction in the 24-hour urinary free cortisol level from baseline value, as well as improvements in the signs and symptoms of the disease (e.g., blood pressure, lipid levels, weight).

• Oteseconazole (Vivjoa™) [1397]

Initial Authorization Criteria:

- Must have a diagnosis of acute vulvovaginal candidiasis confirmed by lab results of the culture
- Must have recurrent vulvovaginal candidiasis (RVVC) with documentation of at least 3 episodes of symptomatic vulvovaginal candidiasis within a year

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- Must be a natal female who is not of reproductive potential (e.g., postmenopausal, permanent infertility from tubal ligation, hysterectomy, salpingo-oophorectomy)
- Must have an adequate trial of maintenance fluconazole (150 mg weekly for at least 6 months) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
- Must not be pregnant or lactating

If the above criteria are met, oteseconazole (Vivjoa) is approved for 1 treatment course of 14 weeks.

Reauthorization Criteria:
Additional authorizations for treatment are made on a case-by-case basis, require lab results confirming a recurrence, and require chart documentation describing the previous response and clinical rationale for re-treatment.

• Oxymetazoline Hydrochloride (Upneeq™) [1398, 1399, 1400]

Initial Authorization Criteria:

- Must be prescribed by an ophthalmologist
- Must be age 18 years or older
- Must have a diagnosis of acquired blepharoptosis
- Must have chart documentation that a diagnostic test (such as Marginal reflex distance of the upper lid [MRD_1] or levator function) was used to confirm the diagnosis
- Must have documentation that the diagnosis causes vision impairment
- Must have an assessment of the underlying cause of blepharoptosis and be receiving appropriate treatment

If the above criteria are met, oxymetazoline (Upneeq) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed to determine the Medical Necessity for continuation of therapy. Authorization may be extended based upon chart documentation from the prescriber that the member's condition has improved based upon the prescriber's assessment while on therapy.

If the above criteria are met, oxymetazoline (Upneeq) is approved.

• Paliperidone (Invega®) [1401]

Criteria:

- For members < 12 years of age, please reference 'Atypical and Typical Antipsychotics – Children Under 12 years of Age' criteria

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- Must have a diagnosis of schizophrenia or schizoaffective disorder
- Must have an adequate trial, of at least 4 weeks, with an inadequate response or significant side effect/toxicity or have a contraindication to TWO generic atypical antipsychotics.

If the above criteria are met, paliperidone (Invega) is approved.

• Palivizumab (Synagis®) [1402, 1403, 1404]

Initial Authorization Criteria:

- Must meet ONE of the following age groups with specified conditions during the designated yearly RSV season:
  - Must be under the age of 12 months at the beginning of the RSV season and meet ONE of the following conditions:
    - Must have gestational age less than 29 weeks
    - Must have neuromuscular disease or congenital anomaly that impairs the ability to clear secretions from the upper airway because of ineffective cough
    - Must have chronic lung disease (CLD) of prematurity defined as gestational age < 32 weeks, 0 days and a requirement for > 21% oxygen for at least the first 28 days of life
    - Must have congenital heart disease (CHD) defined as ONE of the following:
      - Hemodynamically significant (cyanotic) heart disease
        - Must receive medication to control heart failure
      - Hemodynamically nonsignificant (acyanotic) heart disease with BOTH of the following:
        - Must receive medication to control heart failure
        - Must require a corrective cardiac surgical procedure
      - Heart failure that requires ongoing medication after cardiac lesions adequately corrected by surgery
      - Moderate to severe pulmonary hypertension
    - Must have cystic fibrosis with nutritional compromise (Routine use of palivizumab prophylaxis in patients with cystic fibrosis, including neonates diagnosed with cystic fibrosis by newborn screening, is not recommended unless nutritional compromise is present)
    - Must be profoundly immunocompromised (e.g., diagnosis of severe combined immunodeficiency syndrome [SCID], severe T-cell deficiency, severe acquired immunodeficiency syndrome, acute myeloid leukemia, acute lymphoblastic leukemia, receiving chemotherapy, or have had hematopoietic stem cell transplant)

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        - Must be 12 months to 24 months of age and meet ONE of the following conditions:
            - Must have CLD of prematurity (defined as gestational age < 32 weeks, 0 days and a requirement for > 21% oxygen for at least the first 28 days of life) AND must require ONE of the following within 6 months of the start of the RSV season:
                - Supplemental oxygen
                - Chronic systemic corticosteroid treatment
                - Diuretic medication
            - Must be profoundly immunocompromised during the RSV season, (e.g., diagnosis of severe combined immunodeficiency syndrome [SCID], severe T-cell deficiency, severe acquired immunodeficiency syndrome, acute myeloid leukemia, acute lymphoblastic leukemia, receiving chemotherapy, or have had hematopoietic stem cell transplant)
            - Must have cystic fibrosis with chart documentation of ONE of the following manifestations of severe lung disease:
                - Previous hospitalization for pulmonary exacerbation in the first year of life
                - Abnormalities on chest radiography or chest computed tomography that persist when stable
                - Weight for length < 10th percentile
    - Must provide weight and have a requested dose that falls within the recommended dosing guidelines from the manufacturer
    - Must not have RSV hospitalization while receiving palivizumab (Synagis) in the current season
    - Must not have received nirsevimab (Beyfortus) for the current RSV season
    - Must provide clinical rationale for use of palivizumab (Synagis) instead of nirsevimab (Beyfortus). Clinical rationale must include why nirsevimab (Beyfortus) would not be expected to produce the same clinical results as palivizumab (Synagis)

If the above criteria are met, palivizumab (Synagis) is approved for a maximum of 5 doses per RSV season.

Reauthorization Criteria:
All prior authorization renewals are reviewed on a case by case basis for medical necessity and are subject to the above initial authorization criteria.

• Palopegteriparatide (Yorvipath®) [1405, 1406, 1407, 1408]

Initial Authorization Criteria:
- Must be prescribed by or in consultation with an endocrinologist
- Must be age 18 years or older

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- Must have a diagnosis of hypoparathyroidism confirmed by a history of BOTH of the following levels below the laboratory reference range on two occasions at least two weeks apart.
  - Albumin-corrected serum calcium or ionized serum calcium
  - Parathyroid hormone (PTH)
- Must have an adequate trial with an inadequate response (defined as uncontrolled hypocalcemia or inability to maintain stable serum or urinary calcium levels despite at least 12 weeks of therapy) or significant side effect/toxicity or have a contraindication to concomitant therapy with BOTH of the following:
  - Oral calcium at maximally tolerated dose
  - Oral calcitriol at a dose of ≥0.5 mcg daily
- Must have the following baseline laboratory levels before initiating therapy with palopegteriparatide (Yorvipath):
  - Serum 25(OH)-vitamin D: 20-80 ng/mL
  - Albumin-corrected serum calcium: ≥7.8 mg/dL or ionized serum calcium: ≥4.4 mg/dL
- Must not be used for acute post-surgical hypoparathyroidism

If the above criteria are met, palopegteriparatide (Yorvipath) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon the following:

- Must have reduction in oral calcium dose, reduction in oral active vitamin D dose, and/or improvement in serum calcium level while on therapy

• Palovarotene (Sohonos™): [1409, 1410]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with a rheumatologist, orthopedist, or provider who specializes in the treatment of rare connective tissue disorders or bone disease
- Must meet ONE of the following:
  - Must be age 8 years or older if natal female
  - Must be age 10 years or older if natal male
- Must have a diagnosis of fibrodysplasia ossificans progressiva (FOP) confirmed by BOTH of the following:
  - Genetic test with confirmed *ACVR1* R206H mutation (a copy of the genetic test must be provided)
  - Must have chart documentation of clinical signs or symptoms attributable to FOP (e.g., malformed great toes, soft tissue swelling, heterotopic ossification)

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- Must be prescribed a dose and duration of therapy that is consistent with FDA-approved package labeling
- For flare-up treatment requests:
  - Must have ONE of the following:
    - Must have documentation the member is currently experiencing a symptom of a FOP flare-up (e.g., localized pain, soft tissue swelling/inflammation, redness, warmth, decreased joint range of motion, stiffness)
    - Must have documentation the member is undergoing a substantial high-risk traumatic event likely to lead to a flare-up (e.g., surgery, intramuscular immunization, mandibular block for dental procedure, muscle fatigue, blunt muscle trauma from bumps, bruises, falls, or an influenza-like viral illness)
- Must not have ANY of the following:
  - Pregnancy
  - History of allergy or hypersensitivity to retinoids or any component of palovarotene

If the above criteria are met, palovarotene (Sohonos) is approved for a duration as follows:

- Maintenance treatment: 1 year
- Flare-up treatment: 12 weeks

Reauthorization Criteria:

All prior authorization renewals are reviewed to determine the Medical Necessity for continuation of therapy. Authorizations may be extended based upon documentation showing the following criteria have been met:

- For maintenance treatment:
  - Must have chart documentation from the provider that the member's condition has improved or stabilized, or the member continues to benefit from therapy
  - Authorizations may be extended at one-year intervals
- For flare-up treatment:
  - For members who experience marked worsening of the original flare-up site or another flare-up at a new location during a course of flare-up treatment:
    - Must be prescribed a dose and duration of therapy that is consistent with FDA-approved package labeling
    - Authorization may be extended for 12 weeks
  - For members who completed a 12-week course of flare-up treatment but symptoms have not resolved:
    - Must be prescribed a dose and duration of therapy that is consistent with FDA-approved package labeling

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        - Authorization may be extended in 4-week intervals until flare-up symptoms resolve.
    - For members who experience new flare-up symptoms:
        - Must have ONE of the following:
            - Must have documentation the member is currently experiencing a symptom of a FOP flare-up (e.g., localized pain, soft tissue swelling/inflammation, redness, warmth, decreased joint range of motion, stiffness)
            - Must have documentation the member is undergoing a substantial high-risk traumatic event likely to lead to a flare-up (e.g., surgery, intramuscular immunization, mandibular block for dental procedure, muscle fatigue, blunt muscle trauma from bumps, bruises, falls, or an influenza-like viral illness)
        - Must be prescribed a dose and duration of therapy that is consistent with FDA-approved package labeling
        - Authorization is approved for 12 weeks.

- **Paltusotine (Palsonify™)** 1411, 1412, 1413, 1414, 1415

Initial Authorization Criteria:

- Must be prescribed by or in consultation with an endocrinologist
- Must be age 18 years or older
- Must have a confirmed diagnosis of acromegaly to include one of the following baseline (pre-treatment) labs:
    - Elevated serum IGF-1 level for patient's gender and age range. Laboratory reference range must be provided.
    - Elevated growth hormone (GH) level defined as a GH level ≥1ng/mL during oral glucose tolerance test (OGTT)
- Must have an inadequate response to surgery and/or not have surgery as an option
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to BOTH of the following:
    - Lanreotide acetate (Somatuline Depot)
    - Octreotide acetate long-acting release (Sandostatin LAR)
- Must provide clinical rationale for use of paltusotine (Palsonify) instead of lanreotide acetate (Somatuline Depot) or octreotide acetate long-acting release (Sandostatin LAR). Clinical rationale must include why lanreotide acetate (Somatuline Depot) or octreotide acetate long-acting release (Sandostatin LAR) would not be expected to produce the same clinical results as paltusotine (Palsonify).

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- Must not use in combination with an injectable somatostatin analog [e.g.- octreotide, octreotide LAR (Sandostatin® LAR Depot), lanreotide (Somatuline® Depot), pasireotide (Signifor®) or pasireotide (Signifor® LAR)]

If the above criteria are met, paltusotine (Palsonify) is approved for 3 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon the following:
- Documentation from the provider that the member's condition has improved while on therapy
- Must not be used in combination with an injectable somatostatin analog [e.g.- octreotide, octreotide LAR (Sandostatin LAR Depot), lanreotide (Somatuline Depot), pasireotide (Signifor) or pasireotide (Signifor LAR)]

- **Pasireotide (Signifor®)** [1416, 1417, 1418, 1419, 1420, 1421, 1422, 1423, 1424, 1425]
  - Requests for pasireotide (Signifor LAR®) intramuscular injection: please refer to Pasireotide intramuscular suspension (Signifor LAR) criteria

Initial Authorization Criteria:
- Must be prescribed by or in consultation with an endocrinologist
- Must be age 18 years and older
- Must have a diagnosis of Cushing's disease
- Must have a confirmed pituitary source of Cushing's syndrome (chart documentation required)
- Must submit a baseline 24-hour urinary free cortisol level
- Must have previously had pituitary surgery (e.g., transsphenoidal surgery) that was not curative or not be a candidate for surgery
- Must provide recent (within 6 months) hemoglobin A1c
  - For members with a hemoglobin A1c value greater than 8%, documentation that anti-diabetic therapy has been optimized must be provided

If the above criteria are met, pasireotide (Signifor) is approved for 3 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one-year intervals based upon chart documentation from the provider that the member's

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disease course has improved based on a reduction in the 24-hour urinary free cortisol level from baseline value, as well as improvements in the signs and symptoms of the disease (e.g., blood pressure, lipid levels, weight).

- **Pasireotide intramuscular suspension (Signifor® LAR)**
  1426, 1427, 1428, 1429, 1430, 1431

Initial Authorizatoin Criteria:

1. **For acromegaly:**
   - Must be prescribed by or in consultation with an endocrinologist
   - Must be age 18 years or older
   - Must have a confirmed diagnosis of acromegaly to include one of the following baseline (pre-treatment) labs:
     - Elevated serum IGF-1 level for patient's gender and age range. Laboratory reference range must be provided.
     - Elevated growth hormone (GH) level defined as a GH level ≥1ng/mL during oral glucose tolerance test (OGTT)
   - Must have an inadequate response to surgery and/or not have surgery as an option
   - Must provide recent (within 6 months) hemoglobin A1c
     - For members with a hemoglobin A1c value greater than 8%, documentation that anti-diabetic therapy has been optimized must be provided
   - Must have previously had an inadequate response or intolerance to Somatuline Depot (lanreotide acetate) or Sandostatin LAR Depot (octreotide acetate long-acting release)
2. **For Cushing's disease:**
   - Must be prescribed by or in consultation with an endocrinologist
   - Must be age 18 years or older
   - Must have a diagnosis of Cushing's disease
   - Must have a confirmed pituitary source of Cushing's syndrome (chart documentation required)
   - Must submit a baseline 24-hour urinary free cortisol level
   - Must have previously had pituitary surgery (e.g., transsphenoidal surgery) that was not curative or not be a candidate for surgery
   - Must provide recent (within 6 months) hemoglobin A1c
     - For members with a hemoglobin A1c value greater than 8%, documentation that anti-diabetic therapy has been optimized must be provided

If the above criteria are met, pasireotide intramuscular suspension (Signifor LAR) is approved for the following duration based upon diagnosis:

- Acromegaly: 6 months
- Cushing's: 4 months

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Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's disease has improved based upon the prescriber's assessment while on therapy. Specifically for Cushing's disease, must have chart documentation from the provider that the member's disease has improved based on a reduction in the 24-hour urinary free cortisol level from baseline value, as well as improvements in the signs and symptoms of the disease (e.g., blood pressure, lipid levels, weight).

- **Peanut (*Arachis hypogaea*) Allergen Powder-dnfp (Palforzia®)** [1432, 1433, 1434]

Initial Authorization Criteria:
- Must be prescribed by an allergist/immunologist
- For initial dose escalation:
  - Must be 1 to 17 years of age
- For up-dosing and maintenance dosing:
  - Must be ≥ 1 year of age
- Must have a diagnosis of peanut allergy confirmed by a history of an allergic reaction attributed to peanut (e.g. hives, lip swelling, itching, anaphylaxis) and one of the following:
  - Positive skin prick test (SPT) response to peanut
  - Positive serum peanut-specific IgE (psIgE) test
  - Medically supervised food challenge
- Must not have had severe or life-threatening anaphylaxis within the 60 days prior to initiating therapy with Palforzia
- Must have concomitant injectable epinephrine and documentation that the member was instructed and trained on appropriate use
- Must not have the following contraindications:
  - Uncontrolled asthma
  - A history of eosinophilic esophagitis or other eosinophilic gastrointestinal disease
- Must have the initial dose escalation and the first dose of each up-dosing level administered in a health care setting
- Must be monitored for at least 60 minutes after administration of initial dose escalation and the first dose of each up-dosing level
- Must be using Palforzia in combination with a peanut avoidant diet per the prescriber
- Must have an assessment of both the member's ability to comply with prescribed regimen, as well as potential risk of anaphylaxis

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If the above criteria are met, peanut (*Arachis hypogaea*) allergen powder-dnfp (Palforzia) is approved for 6 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on a 6-month or annual basis depending on current dosing phase to determine the Medical Necessity for continuation of therapy. Authorizations may be extended based upon documentation from the prescriber of the following:

- For all reauthorizations the member must continue to:
  - Follow a peanut avoidant diet
  - Have access to injectable epinephrine
  - Remain compliant with therapy
- Members who have not yet reached the maintenance dose phase receive additional authorizations at 6-month intervals provided the prescriber attests that the member continues up-dosing as tolerated to reach the maintenance dose.
- Members who have stabilized on the maintenance dose phase receive additional authorizations at 1-year intervals.

• Pegcetacoplan (Empaveli®) [1435, 1436, 1437, 1438, 1439]

Initial Authorization Criteria:

1. For paroxysmal nocturnal hemoglobinuria (PNH):

- Must be prescribed by or in consultation with a hematologist, oncologist, immunologist or genetic specialist
- Must be age 18 years or older
- Must have a laboratory confirmed diagnosis of paroxysmal nocturnal hemoglobinuria (PNH) as evidenced by having detectable Glycosyl Phosphatidyl Inositol (GPI)-deficient hematopoietic clones (Type III PNH RBC) via Flow Cytometry. Documentation of Flow Cytometry pathology report must indicate presence of PNH-type RBC (red blood cell) and must be submitted.
- Must have a baseline LDH level of 1.5 times the upper limit of the normal range (laboratory results with reference range must be submitted)
  - NOTE: Members converting therapy must have a history of an LDH level > 1.5 times the upper limit of the normal range prior to treatment with current therapy (laboratory results with reference range must be submitted)
- Must have clinical symptoms of PNH (e.g., fatigue, hemoglobinuria, abdominal pain, shortness of breath, anemia, dysphagia, history of red blood cell transfusion due to PNH, history of a major vascular event [e.g., thrombosis])
- Must provide documentation that the member is vaccinated with immunizations for encapsulated bacteria (e.g., Streptococcus pneumoniae,

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Neisseria meningitidis, and Haemophilus influenzae type B [Hib]) according to current immunization guidelines at least 2 weeks prior to the administration of the first dose of pegcetacoplan (Empaveli) unless the risks of delaying treatment outweigh the risks of developing infection per documentation of the prescriber
- Must have an adequate trial of eculizumab-aagh (Epysqli) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
    - NOTE: an exception to a trial of eculizumab-aagh (Epysqli) can be made if the member had a trial of another eculizumab product
- Must not use in combination with an eculizumab product, ravulizumab-cwvz (Ultomiris), iptacopan (Fabhalta), danicopan (Voydeya), or crovalimab-akkz (PiaSky)
    - Members transitioning to pegcetacoplan (Empaveli) from an eculizumab product may use concurrent therapy with an eculizumab product for 4 weeks according to the dosing instruction per pegcetacoplan (Empaveli) prescribing information

2. **For Complement 3 Glomerulopathy (C3G):**
    - Must be prescribed by a nephrologist
    - Must be age 12 years or older
    - Must have a diagnosis of complement 3 glomerulopathy confirmed by a kidney biopsy. Chart documentation of the biopsy results is required.
    - Must have BOTH of the following despite at least 90 days of optimized supportive care (e.g., lifestyle modifications, blood pressure management) and maximally-tolerated renin-angiotensin system (RAS) inhibitor therapy (e.g., angiotensin converting enzyme inhibitor or an angiotensin receptor blocker):
        - Estimated glomerular filtration rate ≥ 30 mL/min/1.73 m^2
        - Urine protein-to-creatinine ratio ≥ 1.0 g/g
    - Must provide documentation that the member is vaccinated with immunizations for encapsulated bacteria (e.g., Streptococcus pneumoniae, Neisseria meningitidis, and Haemophilus influenzae type B [Hib]) according to current immunization guidelines at least 2 weeks prior to the administration of the first dose of pegcetacoplan (Empaveli) unless the risks of delaying treatment outweigh the risks of developing infection per documentation of the prescriber
    - Must be on a stable dose of maximally-tolerated renin-angiotensin system (RAS) inhibitor therapy (e.g., angiotensin converting enzyme inhibitor or an angiotensin receptor blocker)
    - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to BOTH of the following:
        - Glucocorticoids
        - Mycophenolate mofetil

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- Must have an attestation from the prescriber that the member is to remain on optimized supportive care as clinically indicated (e.g., lifestyle modifications, blood pressure management)
- Must not use in combination with iptacopan (Fabhalta)

3. **For Primary Immune-Complex Membranoproliferative Glomerulonephritis (IC-MPGN):**
   - Must be prescribed by a nephrologist
   - Must be age 12 years or older
   - Must have a diagnosis of primary IC-MPGN confirmed by a kidney biopsy. Chart documentation of the biopsy results is required.
   - Must have BOTH of the following despite at least 90 days of optimized supportive care (e.g., lifestyle modifications, blood pressure management) and maximally-tolerated renin-angiotensin system (RAS) inhibitor therapy (e.g., angiotensin converting enzyme inhibitor or an angiotensin receptor blocker):
     - Estimated glomerular filtration rate ≥ 30 mL/min/1.73 m^2
     - Urine protein-to-creatinine ratio ≥ 1.0 g/g
   - Must provide documentation that the member is vaccinated with immunizations for encapsulated bacteria (e.g., Streptococcus pneumoniae, Neisseria meningitidis, and Haemophilus influenzae type B [Hib]) according to current immunization guidelines at least 2 weeks prior to the administration of the first dose of pegcetacoplan (Empaveli) unless the risks of delaying treatment outweigh the risks of developing infection per documentation of the prescriber
   - Must be on a stable dose of maximally-tolerated renin-angiotensin system (RAS) inhibitor therapy (e.g., angiotensin converting enzyme inhibitor or an angiotensin receptor blocker)
   - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to BOTH of the following:
     - Corticosteroids (e.g., prednisone)
     - Immunosuppressants (e.g., calcineurin inhibitors, mycophenolate, cyclophosphamide)
   - Must have an attestation from the prescriber that the member is to remain on optimized supportive care as clinically indicated (e.g., lifestyle modifications, blood pressure management)

If the above criteria are met, the requested product is approved for 6 months.

<u>Reauthorization Criteria:</u>
Prior authorization renewals are reviewed to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one-year intervals based upon chart documentation showing the following:

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For PNH

- Must have a decrease in LDH level from baseline
- For members who required blood transfusions at baseline:
  - Must have a decreased requirement or no longer require blood transfusions
- Must not use in combination with an eculizumab product, ravulizumab-cwvz (Ultomiris), iptacopan (Fabhalta), danicopan (Voydeya), or crovalimab-akkz (PiaSky)

For C3 G and IC-MPGN:

- Must have a reduction in proteinuria OR urinary protein-to-creatinine ratio from baseline
- Must be on optimized supportive care as clinically indicated
- Must be on a stable dose of maximally-tolerated renin-angiotensin system (RAS) inhibitor therapy (e.g., angiotensin converting enzyme inhibitor or an angiotensin receptor blocker)
- Must not use in combination with iptacopan (Fabhalta)

• Pegvaliase (Palynziq™) [1440, 1441, 1442, 1443, 1444]

Initial Authorization Criteria:

- Must be prescribed by a provider who specializes in the treatment of inherited metabolic disorders
- Must be age 16 years or older
- Must have a diagnosis of hyperphenylalaninemia due to phenylalanine hydroxylase (PAH) deficiency (including classic phenylketonuria) confirmed by chart documentation of genetic test result showing biallelic pathogenic variants in the phenylalanine hydroxylase (PAH) gene (test result is required)
- Must have chart documentation of uncontrolled blood phenylalanine level > 600 micromol/L on existing management (e.g., dietary therapy with restriction of dietary phenylalanine, sapropterin [Kuvan®, Javygtor™, Zelvysia™], sepiapterin [Sephience™])Must have an adequate trial of sapropterin (Kuvan, Javygtor, Zelvysia) at a dose of 20 mg/kg per day with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
  - Note: an exception to the requirement for a trial of sapropterin (Kuvan, Javygtor, Zelvysia) may be made if documentation is provided that the member has biallelic null mutations in the PAH gene OR if the member has previously had an adequate trial of sepiapterin (Sephience) with an inadequate response or significant side effect/toxicity
- Must use a dose and duration of therapy consistent with the FDA-approved package labeling
- Must not use in combination with sapropterin (Kuvan, Javygtor, Zelvysia) or sepiapterin (Sephience)

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- Note: an exception may be made during the initial authorization period of pegvaliase (Palynziq) to allow for dose titration.

If the above criteria are met, pegvaliase (Palynziq) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon the following:

- Must have chart documentation of blood phenylalanine control evidenced by blood phenylalanine level ≤ 600 micromol/L
  - If member has not achieved blood phenylalanine control (blood phenylalanine level ≤ 600 micromol/L), must meet ONE of the following:
    - Must have documentation member has not been titrated to the maximum dose of 60 mg/day **OR**
    - Must have received less than 16 continuous weeks of therapy at the maximum dose of 60 mg/day
  - Note: Authorizations for these members may be extended for an additional 6 months to allow for dose titration. If members have not achieved blood phenylalanine control after 16 continuous weeks of therapy at the 60 mg/day dose, authorizations may not be extended.
- Must use a dose and duration of therapy consistent with the FDA-approved package labeling
- Must not use in combination with sapropterin (Kuvan, Javygtor, Zelvysia) or sepiapterin (Sephience)

• Pegvisomant (Somavert®) [1445, 1446, 1447]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with an endocrinologist
- Must be age 18 years or older
- Must have a confirmed diagnosis of acromegaly to include one of the following baseline (pre-treatment) labs:
  - Elevated serum IGF-1 level for patient's gender and age range. Laboratory reference range must be provided
  - Elevated growth hormone (GH) level defined by a GH level ≥1 ng/mL during oral glucose tolerance test (OGTT)
- Must have an inadequate response to surgery and/or not have surgery as an option
- Must have an inadequate response to medical therapy with a somatostatin analogue (octreotide, Sandostatin LAR, Somatuline Depot, Mycapssa) or documentation that these therapies are not appropriate

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If the above criteria are met, pegvisomant (Somavert) is approved for three months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's disease has improved based upon the prescriber's assessment while on therapy.

- **Penicillamine (Cuprimine®, Depen®, D-Penamine®)** [1448, 1449, 1450, 1451, 1452, 1453, 1454, 1455, 1456, 1457, 1458, 1459, 1460, 1461, 1462, 1463, 1464]

Initial Authorization Criteria:

1. **For Wilson's Disease**
   - Must be prescribed by or in consultation with a provider who specializes in the treatment of inherited metabolic disorders or by a hepatologist
   - Must have a diagnosis of Wilson's Disease confirmed by chart documentation of ONE of the following:
     - Genetic test results indicating biallelic pathogenic mutation in ATP7B gene
     - Hepatic parenchymal copper content of ≥ 250 µg/g dry weight (equivalent to ≥ 4 µmol/g)
     - Presence of Kayser-Fleischer Ring(s) in cornea(s) with ONE of the following:
       - 24-hour urinary copper excretion > 40 mcg/24 hours (0.64 micromol /24 hours) in addition to serum ceruloplasmin level < 20 mg/dL (200 mg/L)
       - 24-hour urinary excretion of copper > 100 µg (1.6 µmol/24 hours)
   - For penicillamine tablet (Depen, D-Penamine)
     - Must have an adequate trial of at least 3 months of penicillamine capsule (Cuprimine) with an inadequate response or significant side effects/toxicity. Chart documentation of response to penicillamine capsule (Cuprimine) trial is required
     - Must provide clinical rationale for use of pencillamine tablet (Depen, D-Penamine) instead of penicillamine capsules (Cuprimine). Clinical rationale must include why penicillamine capsule (Cuprimine) would not be expected to produce the same clinical results as pencillamine tablet (Depen, D-Penamine)
   - Must not have a history of penicillamine-related aplastic anemia or agranulocytosis

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If the above criteria are met, penicillamine (Depen, D-Penamine, Cuprimine) is approved for 6 months.

2. For Cystinuria

- Must be prescribed by or in consultation with a provider who specializes in the treatment of inherited metabolic disorders or a nephrologist
- Must have a diagnosis of cystinuria confirmed by lab values indicating urinary cystine concentration greater than 250 mg/L or urinary cystine excretion greater than 500 mg per day despite conservative cystinuria management (e.g., increased fluid intake, restricted protein and sodium diet, potassium citrate to alkanize urine) Chart documentation is required.
- For penicillamine tablet (Depen, D-Penamine)
  - Must have an adequate trial of at least 3 months of penicillamine capsule (Cuprimine) with an inadequate response or significant side effects/toxicity. Chart documentation of response to penicillamine capsule (Cuprimine) trial is required
  - Must provide clinical rationale for use of penicillamine tablet (Depen, D-Penamine) instead of penicillamine capsules (Cuprimine). Clinical rationale must include why pencillamine capsule (Cuprimine) would not be expected to produce the same clinical results as pencillamine tablet (Depen, D-Penamine)
- Must not have a history of penicillamine-related aplastic anemia or agranulocytosis

If the above criteria are met, penicillamine (Depen, D-Penamine, Cuprimine) is approved for 6 months.

3. For Rheumatoid Arthritis

- Must be prescribed by a rheumatologist
- Must have a diagnosis of severely active rheumatoid arthritis
- Must have an adequate trial of at least 3 months with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
  - Methotrexate (members with significant side effects/toxicity or who have a contraindication to methotrexate must have an adequate trial of at least 3 months of a conventional DMARD [e.g., leflunomide, hydroxychloroquine, or sulfasalazine])
  - Oral targeted anti-inflammatory agent (e.g., Janus kinase (JAK) inhibitor)
  - Biologic therapy (e.g., tumor necrosis factor inhibitor, interleukin 6 inhibitor, T-cell inhibitor, B-cell inhibitor)
- For penicillamine tablet (Depen, D-Penamine):
  - Must have an adequate trial of at least 3 months of penicillamine capsule (Cuprimine) with an inadequate response or significant side

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effects/toxicity. Chart documentation of response to penicillamine capsule (Cuprimine) trial is required
    - Must provide clinical rationale for use of penicillamine tablet (Depen, D- Penamine) instead of penicillamine capsules (Cuprimine). Clinical rationale must include why pencillamine capsule (Cuprimine) would not produce the same clinical results as pencillamine tablet (Depen, D-Penamine)
  - Must not have ANY of the following:
    - History of penicillamine-related aplastic anemia or agranulocytosis
    - History or other evidence of renal insufficiency

If the above criteria are met, penicillamine (Depen, D-Penamine, Cuprimine) is approved for 6 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of treatment. Authorizations may be extended at one-year intervals based upon chart documentation showing:
  - **For Wilson's Disease and rheumatoid arthritis**
    - Must have improvement in the member's condition while on therapy
  - **For cystinuria**
    - Must have improvement in the member's condition demonstrated by ONE of the following:
      - Reduction in urine cystine levels
      - Positive cystine capacity
      - No cystine stone formation
    - Must be using penicillamine (Depen, D-Penamine, Cuprimine) in combination with conservative cystinuria management (e.g., increased fluid intake, restricted protein and sodium diet, potassium citrate to alkalize urine)

- **Perampanel (Fycompa™)** 1465, 1466, 1467, 1468, 1469, 1470, 1471, 1472, 1473, 1474, 1475, 1476, 1477

Initial Authorization Criteria:
1. **For perampanel (Fycompa) tablet**
  - Must be prescribed by or in consulation with a neurologist
  - Must be age 4 years or older
  - Must have a diagnosis of partial-onset seizures or primary generalized tonic-clonic seizures
  - For primary generalized tonic-clonic seizure:
    - Must be on concomitant therapy with at least one other anticonvulsant medication

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- Must have an adequate trial of at least 2 generic antiepileptic medications with an inadequate response or significant side effect/toxicity or have a contraindication to these therapies

2. **For perampanel (Fycompa) oral suspension**
   - Must meet above criteria for perampanel (Fycompa) tablet
   - Must have documentation of the inability to swallow an oral solid dosage form due to age (< 7 years), medical condition (e.g., nasogastric tube), or other clinical rationale for why perampanel (Fycompa) tablet cannot be used

If the above criteria are met, perampanel (Fycompa) is approved.

• Pheochromocytoma Products [1478, 1479, 1480, 1481, 1482, 1483, 1484]

- Applies to: phenoxybenzamine (Dibenzyline®) and metyrosine (Demser®)

Initial Authorization Criteria:

- Must be prescribed by or in consultation with an endocrinologist or an endocrine surgeon
- Must have a diagnosis of pheochromocytoma
- Must have surgical resection planned, have a contraindication to surgery, or have malignant pheochromocytoma
- For metyrosine (Demser) only:
  - Must have an adequate trial of phenoxybenzamine (Dibenzyline) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
  - Must be prescribed concomitantly with an alpha-adrenergic antagonist or a calcium channel blocker

If the above criteria are met, phenoxybenzamine (Dibenzyline) or Metyrosine (Demser) is approved for 3 months.

Reauthorization Criteria:

All prior authorizations renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or stabilized while on therapy.

• Pilocarpine (Qlosi™) [1485, 1486, 1487]

Initial Authorization Criteria:

- Must be prescribed by an optometrist or ophthalmologist
- Must be 45 to 64 years of age at the initiation of treatment
- Must have a diagnosis of presbyopia confirmed by a refraction assessment. Chart documentation must be provided.

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- Must have a documented history of a failure of corrective eyeglasses and contact lenses to solve symptoms of presbyopia
  - Must provide clinical rationale to support why corrective eyeglasses and contact lenses are not able to treat symptoms of presbyopia despite having an up-to-date prescription and an assessment completed to determine whether a different type of contact lenses or eyeglasses would be an appropriate alternative
- Must have an attestation from the provider that the member has been counseled on the risk of using pilocarpine (Qlosi) in conditions with poor illumination (e.g., night driving)
- Must be using a dose and duration of therapy consistent with the FDA-approved package labeling

If the above criteria are met, pilocarpine (Qlosi) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved, or the member continues to benefit from therapy.

• Pilocarpine (Vuity™) [1488, 1489, 1490]

Initial Authorization Criteria:
- Must be prescribed by an optometrist or ophthalmologist
- Must have chart documentation of a diagnosis of presbyopia confirmed by a refraction assessment
- Must be between the ages of 40 to 55 at the initiation of treatment
- Must have a documented history of a failure of corrective eyeglasses and contact lenses to solve symptoms of presbyopia
  - Must provide clinical rationale to support why corrective eyeglasses and contact lenses are not able to treat symptoms of presbyopia despite having an up-to-date prescription and an assessment completed to determine whether a different type of contact lenses or eyeglasses would be an appropriate alternative
- Must provide chart documentation that the member has been counseled on the risk of using pilocarpine (Vuity) in conditions with poor illumination (e.g., night driving)
- Must be prescribed a dose that is consistent with FDA-approved package labeling, nationally-recognized compendia, or peer-reviewed medical literature

If the above criteria are met, pilocarpine (Vuity) is approved for 1 year.

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Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved based upon the prescriber's assessment while on therapy or the member continues to benefit from therapy.

- **Pimavanserin (Nuplazid®)** [1491, 1492, 1493, 1494, 1495, 1496, 1497, 1498]

Initial Authorization Criteria:
- Must be prescribed by or in consultation with a neurologist who specializes in the treatment of movement disorders
- Must have a diagnosis of Parkinson's disease psychosis
- Must have the presence of hallucinations or delusions (which may include illusions or a false sense of presence) on a recurrent or continuous basis for at least 1 month
  - Hallucinations or delusions must have developed after the onset of Parkinson's disease
- Must provide chart documentation of clinical work-up to rule out other diagnoses (e.g., dementia with Lewy bodies, psychiatric disorders such as schizophrenia, schizoaffective disorder, delusional disorder, or mood disorder with psychotic features, visual processing deficits/loss of visual acuity sufficient to cause hallucinations, infectious causes, or a general medical condition including delirium). Clinical rationale for diagnosis and exclusion of other diagnoses must be provided.
- Must have tried to discontinue or reduce the dose of any medication(s) that may cause or contribute to hallucinations and delusions (e.g., dopamine agonist, amantadine, monoamine oxidase B inhibitors, anticholinergics), or provide clinical rationale indicating why dose reduction or discontinuation of applicable medications would not be appropriate

If the above criteria are met, pimavanserin (Nuplazid) is approved for 1 year.
Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for the continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved based upon the prescriber's assessment while on therapy or the member continues to benefit from therapy.

- **Pimecrolimus (Elidel®)** [1499,1500, 1501, 1502,1503,1504,1505]

Criteria:
- Must have a diagnosis of atopic dermatitis (eczema), plaque psoriasis, prurigo nodularis, or vitiligo

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- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to BOTH of the following:
  - Tacrolimus ointment
  - A topical corticosteroid
    - NOTE: for members with atopic dermatitis (eczema) on the face or in intertriginous areas this trial is not required

If the above criteria are met, pimecrolimus (Elidel) is approved.

• Pitolisant (Wakix®) [1506, 1507, 1508, 1509, 1510, 1511]

Initial Authorization Criteria:

1. For Narcolepsy

- Must be prescribed by a board-certified sleep specialist, a pulmonologist, or a neurologist
- Must have polysomnographic evaluation (i.e., sleep study) with subsequent multiple sleep latency test (MSLT) that shows a mean sleep latency of less than 8 minutes and two or more sleep onset rapid eye movement periods (SOREMPs) including any SOREMP on the polysomnographic evaluation from the preceding night (copy of sleep study must be provided)
- For cataplexy with narcolepsy in adult member must have ALL of the following:
  - Must have a diagnosis of cataplexy with narcolepsy
  - Must have an adequate trial with inadequate response or significant side effect/toxicity or have a contraindication to ONE of the following:
    - A tricyclic antidepressant
    - Venlafaxine
    - Atomoxetine
    - Fluoxetine
- For excessive daytime sleepiness (EDS) associated with narcolepsy in adult members must have ALL of the following:
  - Must be age 18 years or older
  - Must have chart documentation of a history of excessive daytime sleepiness
  - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to BOTH of the following:
    - Modafinil OR armodafinil
    - Solriamfetol (Sunosi™)
- For excessive daytime sleepiness (EDS) associated with narcolepsy in pediatric members must have ALL of the following:
  - Must be 6 to 17 years of age
  - Must have chart documentation of a history of excessive daytime sleepiness

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        - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ONE of the following:
            - A central nervous system (CNS) stimulant (e.g., methylphenidate, dextroamphetamine, amphetamine/dextroamphetamine)
            - Modafinil
    - Must not have a severe hepatic impairment (Child Pugh C)

If the above criteria are met, pitolisant (Wakix) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one year intervals based upon documentation from the provider that the member's condition has improved.

• Plozasiran (Redemplo®) [1512, 1513]

Initial Authorization Criteria:
- Must be prescribed by or in consultation with a cardiologist, endocrinologist, or lipidologist
- Must be age 18 years or older
- Must have a diagnosis of familial chylomicronemia syndrome (FCS) confirmed by BOTH of the following:
    - Genetic testing demonstrating biallelic pathogenic variants in the *LPL*, *GPIHBP1*, *APOA5*, *APOC2*, or *LMF1* genes (a copy of the genetic test must be provided)
    - Fasting triglyceride level ≥ 880 mg/dL despite dietary modifications and triglyceride-lowering therapy (e.g., omega-3 fatty acids, fibrates or other lipid-lowering medications). Chart documentation must be provided.
- Must be used in conjunction with a low-fat diet

If the above criteria are met, plozasiran (Redemplo) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon chart documentation showing the following:
- The member's condition has improved while on therapy
- Must have a reduction in fasting triglyceride levels from baseline
- Must continue to be used in conjunction with a low-fat diet

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- **Ponesimod (Ponvory™)** [1514, 1515]

Initial Authorization Criteria:

- Must be prescribed by a neurologist
- Must be age 18 years or older
- Must have a diagnosis of relapsing form of multiple sclerosis, to include clinically isolated syndrome, relapsing-remitting disease, and active secondary progressive disease
- Must have an adequate trial with an inadequate response or significant side effect/toxicity, or have a contraindication to ONE of the following:
  - Glatiramer acetate/Glatopa® (Copaxone®)
  - Dimethyl fumarate (Tecfidera®)
  - Fingolimod (Gilenya®)
  - Teriflunomide (Aubagio®)
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ONE of the following:
  - Interferon beta-1a (Avonex®, Rebif®)
  - Peginterferon beta-1a (Plegridy®)
  - Interferon beta 1-b (Betaseron®)
  - Siponimod (Mayzent®)
  - Ozanimod (Zeposia®)
  - Ofatumumab (Kesimpta®)
- Must not be on concomitant therapy with a disease modifying drug for multiple sclerosis

If the above criteria are met, ponesimod (Ponvory) is approved for 1 year.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for the continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or stabilized while on therapy.

- **Posaconazole delayed-release tablet, oral suspension, and PowderMix for delayed-release oral suspension (Noxafil®)** [1516]

Initial Authorization Criteria:

- For prophylaxis of *Aspergillus* and *Candida* infections:
  - Must be severely immunocompromised (e.g., hematologic malignancy with prolonged neutropenia from chemotherapy, graft versus host disease)
  - For posaconazole delayed-release tablet (Noxafil):

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        - Must be 2 years or older and weigh greater than 40 kg
    - For posaconazole oral suspension (Noxafil):
        - Must be 13 years or older
    - For Posaconazole PowderMix for delayed-release oral suspension (Noxafil):
        - Must be 2 years or older and weigh 40 kg or less
        - Must not have known or suspected hereditary fructose intolerance (HFI)
  - For treatment of oropharyngeal candidiasis:
    - Must be age 13 years or older
    - Must have tried fluconazole and/or itraconazole for at least 2 weeks
    - Must use posaconazole oral suspension (Noxafil)
  - For the treatment of invasive aspergillosis:
    - Must be age 13 years or older
    - Must use posaconazole delayed-release tablet (Noxafil)
    - Must provide clinical rationale for use of posaconazole delayed-release tablet (Noxafil) instead of voriconazole tablet (Vfend®). Clinical rationale must include why voriconazole tablet (Vfend) would not be expected to produce the same clinical results as posaconazole delayed-release tablet (Noxafil).
  - Must not use with ANY of the following:
    - CYP3A4 substrates that prolong the QT interval (i.e., pimozide, quinidine)
    - Sirolimus (Rapamune)
    - HMG-CoA reductase inhibitors that are primarily metabolized through CYP3A4 (e.g., atorvastatin, lovastatin, simvastatin)
    - Ergot alkaloids
    - Venetoclax at initiation and during the ramp-up phase in patients with chronic lymphocytic leukemia (CLL) or small lymphocytic lymphoma (SLL)

If the above criteria are met, the following is approved:
  - Prophylaxis of *Aspergillus* and *Candida* infections: 6 months
  - Oropharyngeal candidiasis: 1 month
  - Treatment of invasive aspergillosis: 12 weeks

Reauthorization Criteria:
All prior authorization renewals are reviewed on a case-by-case basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended on a case-by-case basis based upon the above criteria and the prescriber's clinical rationale for continuation or re-treatment.

- **Pretomanid** [1517, 1518, 1519, 1520]

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Initial Authorization Criteria:

- Must be prescribed by or in consultation with an infectious disease specialist or pulmonologist
- Must be age 18 years or older
- Must have a diagnosis of pulmonary tuberculosis with documentation of susceptibility testing of Mycobacterium tuberculosis isolates demonstrating ONE of the following:
  - Resistance to isoniazid, rifamycins, a fluoroquinolone and a second line injectable antibacterial drug (i.e., amikacin, kanamycin, or capreomycin)
  - Resistance to isoniazid and rifampin in members who are treatment-intolerant or nonresponsive to standard therapy
  - Resistance to isoniazid, rifampin, and at least one fluoroquinolone or injectable medication (i.e., amikacin, kanamycin, capreomycin)
- Must be used in combination with bedaquiline (Sirturo) and linezolid
- Must not have known resistance to any component of the combination
- Must indicate that the member understands the importance of compliance to the full medication regimen

If the above criteria are met, pretomanid is approved for 26 weeks.

Reauthorization Criteria:

All prior authorizations renewals are reviewed on a case-by-case basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at six-month intervals based upon the following:

- Must have documentation of the member's initial response to therapy and clinical rationale for continuation of treatment or re-treatment
- Must have documentation that the member was compliant with previous ave course of therapy
- Must have documentation of susceptibility testing of Mycobacterium tuberculosis isolates demonstrating continued susceptibility to pretomanid
- Must be used in combination with bedaquiline (Sirturo) and linezolid
- Must not have known resistance to any component of the combination

- **Prophylactic Hereditary Angioedema Products** [1521, 1522, 1523, 1524, 1525, 1526, 1527, 1528, 1529, 1530, 1531, 1532, 1533, 1534, 1535, 1536, 1537, 1538,1539]
  - Applies to: C1 inhibitor [human] intravenous (Cinryze), C1 inhibitor [human] subcutaneous (Haegarda), lanadelumab-flyo (Takhzyro), berotralstat (Orladeyo), garadacimab-gxii (Andembry), donidalorsen (Dawnzera)

Initial Authorization Criteria:

- Must be prescribed by or in consultation with a HAE specialist
- Must meet the following age requirements:

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        - Cinryze: age 6 years or older
        - Haegarda: age 6 years or older
        - Takhzyro: age 2 years or older
        - Orladeyo: age 12 years or older
        - Andembry: age 12 years or older
        - Dawnzera: age 12 years or older
    - Must be used as prophylactic therapy for the prevention of HAE attacks
    - Must have a diagnosis of HAE confirmed by ALL of the following laboratory values (copy of laboratory reports required, must include reference ranges):
        - Low C4 complement level (mg/dL)
        - Normal C1q complement component level (mg/dL)
            - C1q level is not required for members with a documented family history of HAE OR symptom onset before age 30
        - Either of the following:
            - Low C1 esterase inhibitor antigenic level (mg/dL)
            - Low C1 esterase inhibitor functional level (percent)
    - Must be a candidate for HAE prophylaxis therapy, demonstrating at least ONE of the following (chart documentation must be provided):
        - History of frequent HAE attacks defined as two or more HAE attacks per month
        - History of severe HAE attacks defined as one or more abdominal attacks in the past 12 months
        - History of any attack of the respiratory tract which compromised the airway
    - Must NOT be using the requested product in combination with another HAE gent used for long-term prophylaxis (e.g., Cinryze, Haegarda, Takhzyro, Orladeyo, Andembry,Dawnzera)
    - Must be using a dose and duration of therapy consistent with the FDA-approved package labeling

If the above criteria are met, the requested product is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one-year intervals based upon the following:

- Must have chart documentation from the provider that the member's condition has improved while on therapy
- Must NOT be using the requested product in combination with another HAE agent used for long-term prophylaxis (e.g., Cinryze, Haegarda, Takhzyro, Orladeyo, Andembry, Dawnzera)
- For lanadelumab-flyo (Takhzyro) ONLY:

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- If the member is well-controlled (e.g., attack free) on every 2-week dosing for more than 6 months, confirm if the dosing can be extended to every 4 weeks

• Pyridostigmine bromide oral syrup (Mestinon®) [1540]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with a neurologist
- Must have a diagnosis of myasthenia gravis
- Must have documentation of the inability to swallow an oral solid dosage form due to age (< 7 years), medical condition (e.g., nasogastric tube), or other clinical rationale for why pyridostigmine (Mestinon) tablet cannot be used

If the above criteria are met, pyridostigmine (Mestinon) solution is approved.

• Pyrimethamine (Daraprim™) [1541, 1542]

Initial Authorization Criteria:

1. **For acute treatment or secondary prophylaxis (chronic maintenance therapy) of toxoplasmosis**
   - Must be prescribed by or in consultation with an infectious disease specialist
   - Must not have megaloblastic anemia due to folate deficiency
   - Must use as part of a treatment regimen that is appropriate for the member's condition according to FDA-approved package labeling or treatment guidelines

2. **For primary prophylaxis of toxoplasmosis**
   - Must be prescribed by or in consultation with an infectious disease specialist
   - Must not have megaloblastic anemia due to folate deficiency
   - Must use as part of a treatment regimen that is appropriate for the member's condition according to FDA-approved package labeling or treatment guidelines
   - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to trimethoprim/sulfamethoxazole

3. **For acute treatment or secondary prophylaxis (chronic maintenance therapy) of Cystoisosporiasis**
   - Must be prescribed by or in consultation with an infectious disease specialist
   - Must not have megaloblastic anemia due to folate deficiency
   - Must use as part of a treatment regimen that is appropriate for the member's condition according to FDA-approved package labeling or treatment guidelines

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- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to trimethoprim/sulfamethoxazole

4. **For primary prophylaxis or secondary prophylaxis (chronic maintenance therapy) of Pneumocystis pneumonia**
   - Must be prescribed by or in consultation with an infectious disease specialist
   - Must not have megaloblastic anemia due to folate deficiency
   - Must use as part of a treatment regimen that is appropriate for the member's condition according to FDA-approved package labeling or treatment guidelines
   - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to trimethoprim/sulfamethoxazole

If the above criteria are met, pyrimethamine (Daraprim) is approved for 3 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or stabilized based upon the prescriber's assessment while on therapy.

• Relugolix (Orgovyx™) 1543, 1544, 1545, 1546

Initial Authorization Criteria:
- Must be prescribed by or in consultation with an oncologist or hematologist
- Must have a diagnosis of prostate cancer
- Must have ONE of the following:
  - An adequate trial and failure of an injectable gonadotropin releasing hormone (GnRh) agonist (e.g., Lupron Depot, Eligard, Trelstar, Zoladex) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
  - A history of myocardial infarction, ischemic stroke, hemorrhagic stroke, or coronary artery disease

If the above criteria are met, relugolix (Orgovyx) is approved.

• Relugolix/estradiol/norethindrone acetate (Myfembree®) 1547, 1548, 1549

Initial Authorization Criteria:
1. **For management of heavy menstrual bleeding associated with uterine leiomyomas**

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- Must be prescribed by or in consultation with a gynecologist
- Must have a diagnosis of heavy menstrual bleeding associated with uterine leiomyomas (fibroids)
    - Documentation of imaging confirming the diagnosis must be provided (e.g., ultrasound or MRI)
- Must have tried and failed or have a documented contraindication to all of the following:
    - Non-steroidal anti-inflammatory drugs (NSAIDs)
    - Hormonal contraceptives or progestins
    - Tranexamic acid (Lysteda)
- Must have a behavioral health assessment prior to the use of relugolix/estradiol/norethindrone acetate (Myfembree) if the member has a history of depression and/or suicidal thoughts or behaviors, or if the member is currently undergoing treatment for depression and/or suicidal thoughts or behaviors. These members must also be followed while on relugolix/estradiol/norethindrone acetate (Myfembree).

2. **For management of moderate to severe pain associated with endometriosis**
    - Must be prescribed by or in consulation with a gynecologist
    - Must have a diagnosis of endometriosis
        - Must be confirmed by laparoscopy
        - If the diagnosis is not confirmed by surgery, then documentation of an adequate work-up and the clinical rationale for the diagnosis must be provided
    - Must have tried and failed or have a documented contraindication to all of the following:
        - Non-steroidal anti-inflammatory drugs (NSAIDs)
        - Hormonal contraceptives or progestins
        -
    - Must have a behavioral health assessment prior to the use of relugolix/estradiol/norethindrone acetate (Myfembree) if the member has a history of depression and/or suicidal thoughts or behaviors, or if the member is currently undergoing treatment for depression and/or suicidal thoughts or behaviors. These members must also be followed while on relugolix/estradiol/norethindrone acetate (Myfembree).

If the above criteria are met, relugolix/estradiol/norethindrone acetate (Myfembree) is approved for 2 years per lifetime. No reauthorization is granted after this period of coverage. Additionally, cumulative use of any of the following for more than 2 years will not be approved:

- Relugolix/estradiol/norethindrone acetate (Myfembree)
- Elagolix/estradiol/norethindrone acetate and elagolix (Oriahnn)
- Elagolix (Orilissa) 150mg

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        - NOTE: Elagolix (Orilissa) 200mg is only approved for 6 months per lifetime. Treatment with another GnRH antagonist (e.g., Myfembree, Oriahnn, Orilissa) will not be approved.

- **Repository Corticotropin Injection (H.P. Acthar Gel®, Purified Cortrophin Gel®)** 1550, 1551, 1552, 1553, 1554, 1555, 1556, 1557, 1558, 1559, 1560, 1561, 1562, 1563, 1564, 1565, 1566, 1567, 1568, 1569
    - Repository corticotropin injection (H.P. Acthar Gel) and repository corticotrophin injection, USP (Purified Cortrophin Gel) are not considered medically necessary for other indications.

Initial Authorization Criteria:
- Must be prescribed by a pediatric neurologist
- Must have a diagnosis of infantile spasms confirmed by EEG
- Must be under the age of 2 years
- Must have no evidence of infection

If the above criteria are met, the requested product is approved for 1 month.

Reauthorization Criteria:
All prior authorizations renewals are reviewed on a case-by-case basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-month intervals based upon the member's initial response to therapy.

- **Resmetirom (Rezdiffra™)** 1570, 1571, 1572, 1573, 1574, 1575, 1576, 1577, 1578

Initial Authorization Criteria:
- Must be prescribed by or in consultation with a hepatologist or gastroenterologist
- Must be age 18 years or older
- Must have a diagnosis of noncirrhotic nonalcoholic steatohepatitis (NASH) or metabolic dysfunction-associated steatohepatitis (MASH) with moderate to advanced liver fibrosis (consistent with stages F2 to F3 fibrosis) confirmed by ALL of the following:
    - Must have at least ONE of the following cardiometabolic risk factors:
        - Overweight or obesity
        - Type 2 diabetes or prediabetes
        - Hypertension
        - Hypertriglyceridemia
        - Reduced high-density lipoprotein cholesterol (HDL-C)

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        - Must have moderate to advanced liver fibrosis (consistent with stages F2 to F3 fibrosis) confirmed by ONE of the following:
            - Liver biopsy performed within the last 3 years shows fibrosis stage F2 to F3 (chart documentation must be provided)
            - FIB-4 ≥ 1.3 AND one of the following non-invasive tests performed within the last 6 months is consistent with F2 to F3 fibrosis (a copy of the report must be provided):
                - Vibration controlled transient elastography (VCTE; e.g., Fibroscan)
                - Shear wave elastography (SWE)
                - Magnetic resonance elastography (MRE)
                - Enhanced liver fibrosis (ELF) score
    - Must not have evidence of cirrhosis (stage F4 fibrosis) by imaging or liver biopsy or one or more liver-related complications associated with cirrhosis (e.g., variceal bleeding, ascites, hepatic encephalopathy, etc.)
    - Must not have significant alcohol use (defined as alcohol consumption of more than 1 drink/day for natal females or more than 2 drinks/day for natal males). Chart documentation must be provided.
    - Must have an attestation from the provider that the member has received counseling on dietary and lifestyle modifications
    - Must be used in conjunction with diet and exercise

If the above criteria are met, resmetirom (Rezdiffra) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon chart documentation showing the following:
    - The member's condition has improved or stabilized while on therapy
    - Must not have evidence of cirrhosis (stage F4 fibrosis) by imaging or liver biopsy or one or more liver-related complications associated with cirrhosis (e.g., variceal bleeding, ascites, hepatic encephalopathy, etc.)
    - Must not have significant alcohol use (defined as alcohol consumption of more than 1 drink/day for natal females or more than 2 drinks/day for natal males)
    - Must continue to be used in conjunction with diet and exercise

- **Retapamulin 1% ointment (Altabax®):** [1579, 1580, 1581]

Initial Authorization Criteria:
    - Must be age 9 months or older
    - Must have a diagnosis of impetigo due to *Staphylococcus aureus* (methicillin-susceptible isolates only) or *Streptococcus pyogenes*

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- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to BOTH of the following:
  - Mupirocin 2% ointment
  - Ozenoxacin 1% (Xepi) cream
- Must have an attestation from the prescriber that total affected area does not exceed 100 cm^2 for adults or 2% total body surface area for pediatric members

If the above criteria are met, retapamulin 1% ointment (Altabax) is approved for 1 month to allow for 1 treatment course.

Reauthorization Criteria:
Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response and clinical rationale for re-treatment.

- **Rifaximin (Xifaxan®) 550mg Tablet** [1582, 1583, 1584, 1585, 1586, 1587]

Initial Authorization Criteria:

1. **Reduction of risk hepatic encephalopathy (HE) recurrence in adults**
   - Must be age 18 years or older
   - Must have a diagnosis of hepatic encephalopathy
   - Must have had a previous trial and failure of lactulose

If the above criteria are met, rifaximin (Xifaxan) 550mg tablet is approved.

2. **Treatment of irritable bowel syndrome with diarrhea (IBS-D) in adults**
   - Must be prescribed by a gastroenterologist
   - Must be age 18 years or older
   - Must have a diagnosis of diarrhea-predominant IBS with chronic IBS symptoms
   - Must provide documentation showing that nutritional interventions (such as low FODMAP diet, dietary counseling, nutritional assessment, etc.) did not result in a decreased occurrence of symptoms
   - Must provide an assessment from the physician for the need of behavioral modifications (such as cognitive behavioral therapy or psychotherapy). If behavioral modifications recommended, must have documentation showing that the modifications did not result in a decreased occurrence of symptoms.
   - Must have an adequate trial and failure of loperamide or antispasmodics (such as dicyclomine or hyoscyamine) with inadequate response or significant side effect/toxicity or have a contraindication to these therapies

If the above criteria are met, rifaximin (Xifaxan) 550mg tablet is approved for 2 weeks.

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Reauthorization Criteria:
Authorizations may be extended at two week intervals if the member has had a recurrence of IBS-D symptoms after a successful treatment course based upon documentation from the provider. A maximum of two additional retreatment courses may be granted per lifetime.

3. **Treatment of small intestinal bacterial overgrowth (SIBO) syndrome**
   - Must be prescribed by a gastroenterologist
   - Must be 18 years of age or older
   - Must have a diagnosis of small intestinal bacterial overgrowth (SIBO) syndrome confirmed by EITHER of the following:
     - A carbohydrate substrate-based breath test (i.e., glucose or lactulose) demonstrating an increase in hydrogen concentrations of ≥20 ppm from baseline
     - A small bowel aspiration and culture demonstrating a colony count of $\geq 10^3$ CFU/mL
   - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to one generic antibiotic used for the treatment of SIBO (i.e., ciprofloxacin, metronidazole, or trimethoprim-sulfamethoxazole)

If the above criteria are met, rifaximin (Xifaxan) 550mg tablet is approved for 2 weeks.

Reauthorization Criteria:
Authorizations may be extended at two week intervals if the member has had a recurrence of SIBO symptoms after a successful treatment course based upon documentation from the provider.

- **Rilonacept (Arcalyst®)** 1588, 1589, 1590, 1591, 1592, 1593, 1594, 1595, 1596, 1597, 1598, 1599, 1600, 1601, 1602, 1603, 1604

Initial Authorization Criteria:
1. **For Muckle-Wells Syndrome (MWS)**
   - Must have a negative tuberculosis skin test [such as Tuberculin PPD (purified protein derivative) test] or Interferon-Gamma Release Assay (IGRA) whole-blood test [such as QuantiFERON®-TB Gold In-Tube test (QFT-GIT) or T-SPOT®.TB test (T-Spot)]
   - Must be prescribed by a rheumatologist, dermatologist, immunologist, or genetic specialist
   - Must be age 12 years or older

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- Must have a diagnosis of Muckle-Wells syndrome (MWS). Chart documentation of a clinical work-up to rule out other diagnoses and clinical rationale for the diagnosis and exclusion of other diagnoses must be provided. The diagnosis must either be confirmed by genetic testing or a clinical diagnosis defined as one of the following scenarios:
  - Must have a mutation in the NLRP3 (formerly CIAS1) gene. Documentation of lab result confirming mutation is required.
  - Must meet 3 of the following diagnostic criteria (chart documentation required):
    - History of intermittent episodes of fever for at least 24 hours duration
    - Autosomal dominant pattern of disease inheritance
    - Presence of severe fatigue
    - Presence of musculoskeletal symptoms (e.g. arthralgia, arthritis, myalgia)
    - Presence of ocular symptoms (e.g. conjunctivitis, anterior uveitis, papilledema)
    - Presence of erythematous rash
    - Presence of amyloidosis
    - Presence of hearing loss
- Must have baseline lipid panel assessment
  - Must have an adequate trial of anakinra (Kineret) with an inadequate response or significant side effects/toxicity or have a contraindication to this therapy
- Must currently not be using in combination with a Tumor Necrosis Factor (TNF) blocking agent or other biologic agent
- Must have no evidence of infection

2. For Familial Cold Autoinflammatory Syndrome (FCAS)

- Must have a negative tuberculosis skin test or IGRA whole-blood test
- Must be prescribed by a rheumatologist, dermatologist, immunologist, or genetic specialist
- Must be age 12 years or older
- Must have a diagnosis of Familial Cold Autoinflammatory Syndrome (FCAS). Chart documentation of a clinical work-up to rule out other diagnoses and clinical rationale for the diagnosis and exclusion of other diagnoses must be provided. The diagnosis must either be confirmed by genetic testing or a clinical diagnosis defined as one of the following scenarios:
  - Must have a mutation in the NLRP3 (formerly CIAS1) gene. Documentation of lab result confirming mutation is required.
  - Must meet 3 of the following diagnostic criteria (chart documentation required):

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        - History of intermittent episodes of fever and rash that primarily follow natural, experimental, or both types of generalized cold exposures for less than 24 hours duration
        - Autosomal dominant pattern of disease inheritance
        - Age of onset less than 6 months of age
        - Presence of conjunctivitis associated with attacks
        - Absence of deafness, periorbital edema, lymphadenopathy, and serositis
  - Must have baseline lipid panel assessment
    - Must have an adequate trial of anakinra (Kineret) with an inadequate response or significant side effects/toxicity or have a contraindication to this therapy
  - Must currently not be using in combination with a Tumor Necrosis Factor (TNF) blocking agent or other biologic agent
  - Must have no evidence of infection

3. **For Deficiency of Inerleukin-1 Receptor Antagonist (DIRA)**
  - Must have a negative tuberculosis skin test or IGRA whole-blood test
  - Must be prescribed by a rheumatologist, dermatologist, immunologist, or genetic specialist
  - Must have a diagnosis of DIRA. The diagnosis must be confirmed by one of the following:
    - Must have *IL1RN* gene mutation. Documentation of lab result confirming mutation is required.
    - Must have at least 2 of the following clinical symptoms (documentation is required):
        - Skin inflammation with redness
        - Skin pustules of varying size
        - Scaling skin
        - Bone pain
        - Fever
  - Must provide chart documentation of clinical work-up to rule-out other diagnoses. Clinical rationale for the diagnosis and exclusion of other diagnoses must be provided
  - Must be ongoing with treatment with anakinra (Kineret) with a positive response (e.g., decreased inflammation, skin healing, pain relief) resulting in disease remission. Documentation of response and confirmation of disease remission while on therapy with anakinra (Kineret) is required.
  - Must provide documentation of clinical rationale for changing treatment with anakinra (Kineret) to treatment with rilonacept (Arcalyst) given remission of disease occurred with anakinra (Kineret). Documentation of one of the following is required:
    - Significant side effect/toxicity to anakinra (Kineret)

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        - Clinical rationale for discontinuing anakinra (Kineret) including rationale why anakinra (Kineret) will not produce the same clinical results as would be expected with rilonacept (Arcalyst)
    - Must have baseline lipid panel assessment
    - Must currently not be using in combination with a Tumor Necrosis Factor (TNF) blocking agent or other biologic agent
    - Must have no evidence of infection

4. For Recurrent Pericarditis

- Must be prescribed by a cardiologist or rheumatologist
- Must be age 12 years or older
- Must have a diagnosis of recurrent pericarditis. Chart documentation of a clinical work-up to rule out other diagnoses and clinical rationale for the diagnosis and exclusion of other diagnoses must be provided. The diagnosis must be confirmed by clinical diagnosis and by imaging or laboratory testing as outlined below
    - Must have documentation of at least one prior episode of acute pericarditis
    - Must have documentation of one or more of the following clinical symptoms of pericarditis
        - Pericarditic chest pain
        - Pericardial rubs
        - Pericardial effusion
        - ST segment elevation or PR depression on ECG
    - Must have documentation of one of the following
        - Evidence of pericardial inflammation by an imaging technique (e.g. CT or cardiac magnetic resonance [CMR])
        - Elevated markers of inflammation (i.e. C-reactive protein, erythrocyte sedimentation rate, or white blood cell count) based on reference range provided by lab
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to the following:
    - Two standard of therapy drugs used for the treatment of recurrent pericarditis (i.e., colchicine, NSAIDs or aspirin, corticosteroids, or immunosuppressants)
- Must not be using in combination with another biologic agent (such as another anti-IL-1 blocking drug)

If the above criteria are met, rilonacept (Arcalyst) is approved for 3 months.

<u>Reauthorization Criteria:</u>

All prior authorization renewals are reviewed on an annual basis to determine the medical necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon the following:

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- Chart documentation from the prescriber that the member's disease has improved based upon the prescriber's assessment while on therapy
- Documentation that there is no evidence of infection
- Documentation that a lipid panel has been assessed within 3 months of initiation of therapy (for initial re-authorization) and at regular intervals thereafter (for annual reauthorizations)

- **Riluzole (Tiglutik™, Exservan™)** 1605, 1606, 1607, 1608, 1609, 1610, 1611, 1612, 1613, 1614

Initial Authorization Criteria:

- Must be prescribed by a neurologist
- Must have a diagnosis of definite or probable amyotrophic lateral sclerosis (ALS)
- Must have documentation of the inability to swallow an oral solid dosage form due to age (< 7 years), medical condition (e.g. nasogastric tube), or other clinical rationale for why riluzole (Rilutek) tablet cannot be used

If the above criteria are met, the requested drug is approved for 1 year.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member still is a candidate for treatment with riluzole (Tiglutik, Exservan) based upon the prescriber's assessment while on therapy.

- **Rimegepant (Nurtec™ ODT)** 1615, 1616, 1617, 1618, 1619, 1620, 1621, 1622, 1623, 1624, 1625, 1626

Initial Authorization Criteria:

1. **For acute treatment of migraine:**
   - Must be 18 years or older
   - Must be used for the acute treatment of migraine headaches
     - Must have at least 4 migraines per month
   - Must not be taking other acute, migraine-specific therapy (e.g., triptans, ergots, lasmiditan [Reyvow])
   - Must have an adequate trial of a generic triptan (e.g., naratriptan, rizatriptan, sumatriptan) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy

If the above criteria are met, rimegepant (Nurtec ODT) is approved.

2. **For preventive treatment of episodic migraine**

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- Must be 18 years or older
- Must be used for the preventive treatment of migraine with ONE of the following:
  - A diagnosis of episodic migraine OR
  - 4 to 14 migraines per month and less than 15 headache days per month
- Must have an adequate trial of at least 2 months each within 2 prophylactic therapy classes including: beta-blockers, anticonvulsants, and antidepressants (TCAs, SSRIs, SNRIs) with an inadequate response or significant side effects/intolerance
  - For members in whom one of these therapy classes is not clinically appropriate, additional prophylactic therapy classes to be used are calcium channel blockers or angiotensin converting enzyme inhibitors (ACEIs)
- Must not be using concomitantly with atogepant (Qulipta), onabotulinumtoxinA (Botox) or an injectable CGRP antagonist [e.g., erenumab (Aimovig), galcanezumab (Emgality), fremanezumab (Ajovy), eptinezumab (Vyepti)] for prevention of migraine

If the above criteria are met, rimegepant (Nurtec ODT) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for the continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the prescriber that the member's condition has improved based upon the prescriber's assessment while on therapy.

• Riociguat (Adempas®) [1627, 1628, 1629, 1630, 1631, 1632]

Initial Authorization Criteria:

- Must be prescribed by a cardiologist or pulmonologist who specializes in pulmonary hypertension
- Must have a confirmed diagnosis of ONE of the following:
  - PAH (WHO Group 1)
  - Chronic Thromboembolic Pulmonary Hypertension (CTEPH) (WHO Group 4)
- For PAH (WHO Group 1):
  - Must have diagnosis of PAH (WHO Group 1) confirmed by right heart catheterization (RHC); must provide a copy of the RHC showing ALL of the following
    - Mean pulmonary arterial pressure (mPAP) ≥25 mmHg
    - Pulmonary capillary wedge pressure (PCWP), left atrial pressure, or left ventricular end-diastolic pressure (LVEDP) ≤15 mmHg
    - Pulmonary vascular resistance >3 Wood units

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        - Must have an adequate trial of sildenafil (Revatio) tablet with an inadequate response, significant side effect/toxicity or have a contraindication to this therapy
    - For CTEPH (WHO Group 4):
        - Must have a diagnosis of CTEPH confirmed by BOTH of the following:
            - Right heart catheterization (RHC) at least 90 days after the start of full anticoagulation OR 180 days after pulmonary endarterectomy (PEA); must provide a copy of the RHC showing both of the following:
                - Mean pulmonary arterial pressure (mPAP) ≥25 mmHg
                - Pulmonary vascular resistance >3 Wood units
                - NOTE: An exception to the requirement of a RHC can be made if there is clinical evidence of right heart failure and pulmonary hypertension on clinical exam and echocardiogram
            - Ventilation-perfusion scanning (VQ scan) or pulmonary angiography AND echocardiogram showing the member has intermediate or high probability of pulmonary hypertension
        - Must be refractory to surgical treatment (e.g., PEA) or have inoperable CTEPH

If the above criteria are met, riociguat (Adempas) is approved for 3 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or the member continues to benefit from therapy.

• Risdiplam (Evrysdi™) 1633, 1634, 1635, 1636

Initial Authorization Criteria:
- Must be prescribed by a neurologist (specifically a neuromuscular specialist) who specializes in the treatment of spinal muscular atrophy
- Must have chart documentation of a diagnosis of 5q spinal muscular atrophy confirmed by zero copies of the *SMN1* gene OR a biallelic pathogenic variant in the *SMN1* gene (e.g., homozygous deletion) (a copy of the genetic test must be provided)
- Must NOT be used concurrently with other *SMN2* modifying agents [e.g., nusinersen (Spinraza)]

If the above criteria are met, risdiplam (Evrysdi) is approved for 12 months.

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Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon chart documentation of all of the following:

- Must NOT be used concurrently with other *SMN2* modifying agents [e.g., nusinersen (Spinraza)]
- Documentation from the provider that the member remains a candidate for treatment with risdiplam (Evrysdi) based upon the prescriber's assessment while on therapy
- Documentation that the member continues to benefit from risdiplam (Evrysdi)

- **Ritlecitinib (Litfulo®)** [1637, 1638, 1639, 1640, 1641, 1642, 1643]

Initial Authorization Criteria:

- Must be prescribed by a dermatologist
- Must be age 12 years or older
- Must have a diagnosis of severe alopecia areata confirmed by BOTH of the following:
  - At least 50% scalp hair loss or complete loss of eyelashes with scalp hair loss
  - Current episode of alopecia areata lasting at least 6 months
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ONE of the following:
  - Potent topical corticosteroids, intralesional steroids, or oral corticosteroids (e.g., triamcinolone, oral prednisone)
  - Systemic oral immunosuppressant therapy (e.g., methotrexate, cyclosporine)
  - NOTE: An exception to the requirement for a trial of a conventional systemic therapy (e.g., corticosteroid, oral immunosuppressant) can be made if the member has already had a 3 month trial or previous intolerance to an FDA approved therapy for alopecia areata (e.g., ritlecitinib [Litfulo], baricitinib [Olumiant], deuruxolitinib [Leqselvi])
- Must provide documentation to support member does not have hair loss due to androgenetic alopecia (male and female pattern hair loss), chemotherapy-induced hair loss, or other causes of hair loss other than alopecia areata (e.g., trichotillomania, telogen effluvium, and systemic lupus erythematosus)
- Must be using a dose and duration of therapy consistent with the FDA-approved package labeling
- Must not be using an oral targeted anti-inflammatory agent, biologic agent, or potent immunosuppressant (e.g. azathioprine, cyclosporine) in combination with ritlecitinib (Litfulo)

If the above criteria are met, ritlecitinib (Litfulo) is approved for 1 year.

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Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended one year intervals based upon documentation showing ONE of the following:

- For members without complete eyelash or hair regrowth of at least 80% scalp coverage:
  - Must have increased hair growth from baseline
  - Must be using a dose and duration of therapy consistent with the FDA-approved package labeling
- For members with complete eyelash or hair regrowth of at least 80% scalp coverage:
  - Must provide clinical rationale to continue treatment, including why topical treatment could not be used to maintain hair growth
  - Must be using a dose and duration of therapy consistent with the FDA-approved package labeling
- For members being re-treated and had complete hair regrowth with ritlecitinib (Litfulo) during a previous episode of severe alopecia areata
  - Must have documentation of prior treatment response
  - Must have documentation of a current episode of alopecia areata, including the percentage of hair loss or eyelash lossl
  - Must be using a dose and duration of therapy consistent with the FDA-approved package labeling

• Roflumilast (Zoryve™) 1644, 1645, 1646, 1647, 1648

Criteria:
1. For roflumilast cream (Zoryve):

- Must be prescribed by a dermatologist
- Must have one of the following diagnoses:
  - Plaque psoriasis and meet the following criteria:
    - Must be age 6 years or older
    - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to TWO of the following:
      - A topical corticosteroid
        - NOTE: for members with psoriasis on areas other than the face or intertriginous areas, a moderate to high potency topical corticosteroid is required
      - Pimecrolimus cream (Elidel) or tacrolimus ointment (Protopic)
      - Topical calcipotriene or calcitriol

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        - Atopic dermatitis and meet the following criteria:
            - Must be age 2 years or older
            - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to BOTH of the following:
                - A topical corticosteroid
                - NOTE: for members with atopic dermatitis on areas other than the face or intertriginous areas, a moderate to high potency topical corticosteroid is required
                - Pimecrolimus cream (Elidel) or tacrolimus ointment (Protopic)
    - Must not have moderate to severe liver impairment (Child-Pugh B or C)

If the above criteria are met, roflumilast cream (Zoryve) is approved.

2. For roflimulast foam (Zoryve):

- Must be prescribed by a dermatologist
- Must be age 9 years or older
- Must have one of the following diagnoses:
    - Seborrheic dermatitis and meet the following criteria:
        - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to BOTH of the following:
            - A topical corticosteroid
                - NOTE: for members with seborrheic dermatitis on areas other than the face or intertriginous areas, a moderate to high potency topical corticosteroid is required
            - A topical antifungal (e.g., ciclopirox, ketoconazole)
    - Plaque psoriasis and meet the following criteria:
        - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to TWO of the following:
            - A topical corticosteroid
                - NOTE: for members with psoriasis on areas other than the face or intertriginous areas, a moderate to high potency topical corticosteroid is required
            - Pimecrolimus cream (Elidel) or tacrolimus ointment (Protopic)
            - Topical calcipotriene or calcitriol
- Must not have moderate to severe liver impairment (Child-Pugh B or C)

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If the above criteria are met, roflumilast foam (Zoryve) is approved.

• Romiplostim (Nplate™) [1649, 1650, 1651]

Initial Authorization Criteria:

2. For immune thrombocytopenia (ITP)
- Must be prescribed by a hematologist or oncologist
- Must be age 1 year or older
- Must have a diagnosis of immune thrombocytopenia (ITP)
- Must have an adequate trial with an inadequate response or significant side effect/toxicity to ONE of the following:
  - Corticosteroids
  - Intravenous immunoglobulin (IVIG)
- Must have a platelet count <30 x 10^9/L prior to initiation

If the above criteria are met, romiplostim (Nplate) is approved for 6 months.

3. For use in Hematopoietic Syndrome of Acute Radiation Syndrome
- Must have acute exposure to myelosuppressive doses of radiation

If the approve criteria are met, romiplostim (Nplate) is approved for 3 months.

Reauthorization Criteria:

1. For immune thrombocytopenia (ITP)
- All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation showing improvement in platelet count from baseline.

2. For use in Hematopoietic Syndrome of Acute Radiation Syndrome
- Additional authorizations are made on a case-by-case basis, are subject to the above criteria, and require documentation describing previous response to treatment and clinical rationale for re-treatment.

• Romosozumab (Evenity®) [1652, 1653, 1654, 1655, 1656]

Criteria:
- Must have a diagnosis of postmenopausal osteoporosis
- Must be administered by a healthcare provider
- Must meet one of the following scenarios:
  - Bone mineral density (BMD) T-score of ≤ -2.5 at conventional sites including total hip, femoral neck, lumbar spine (post-anterior, not lateral) or radius

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        - History of fragility fracture (other than skull, facial bone, finger, and toes) as an adult
        - Bone mineral density T score of -1.0 to -2.5 at the femoral neck or lumbar spine and a 10-year probability of a hip fracture ≥3% or a 10-year probability of a major osteoporosis-related fracture ≥20% based on the U.S.-adapted World Health Organization (WHO) algorithm (FRAX)
    - Must have an adequate trial of one generic bisphosphonate (oral or intravenous) with an inadequate response or significant side effects/toxicity or have a contraindication to this therapy
        - NOTE: an exception to the requirement for a trial of one bisphosphonate (oral or intravenous) can be made if the patient has a history of fragility fracture OR has very high risk of fracture as defined by ONE of the following:
            - Very low T-score (e.g., < -3.0)
            - High risk for falls or history of injurious falls
            - Very high fracture probability by FRAX (e.g., major osteoporosis fracture >30%, hip fracture >4.5%) or other validated fracture risk algorithm (e.g., Garvan)
    - Must not have a history of myocardial infarction or stroke within one year of starting therapy
    - For patients with established cardiovascular disease, must have an attestation from the prescriber assessing the risks and benefits of therapy and desire to use romosozumab (Evenity)

If the above criteria are met, romosozumab (Evenity) is approved for 1 year. No reauthorization is granted after this period of coverage.

• Ropeginterferon alfa-2b (Besremi®) [1657, 1658]

Initial Authorization Criteria:

- Must be prescribed by a hematologist or an oncologist
- Must be 18 years of age and older
- Must be used for the treatment of polycythemia vera

If the above criteria are met, ropeginterferon alfa-2b (Besremi) is approved for 1 year.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved while on therapy or the member continues to benefit from therapy.

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- **Rotigotine (Neupro®)** [1659, 1660]

Criteria:

1. **For Parkinson's disease**
   - Must have a diagnosis of Parkinson's disease
   - Must have an adequate trial and failure or an inadequate response or intolerance to pramipexole or ropinirole
2. **For Restless Legs Syndrome (RLS)**
   - Must have a diagnosis of moderate to severe RLS
   - Must have an adequate trial and failure of gabapentin (Neurontin), pregabalin (Lyrica), or gabapentin enacarbil (Horizant) defined as one of the following:
     - Insufficient efficacy of gabapentin (Neurontin) at a dose of at least 1800 mg per day, pregabalin (Lyrica) at a dose of at least 300 mg per day, or gabapentin enacarbil (Horizant) at a dose of at least 600 mg per day
     - Intolerance or contraindication to a lower dose of gabapentin (Neurontin), pregabalin (Lyrica), or gabapentin enacarbil (Horizant)
   - Must have an adequate trial and failure or an inadequate response to pramipexole or ropinirole defined as one of the following:
     - Insufficient efficacy of pramipexole at a dose of at least 0.5mg per day or ropinirole at a dose of at least 4mg per day
     - Intolerance or contraindication to a lower dose of pramipexole or ropinirole

If the above criteria are met, rotigotine (Neupro) is approved.

- **Rufinamide (Banzel®)** [1661, 1662, 1663, 1664]

Criteria:

- Must be prescribed by or in consultation with a neurologist
- Must be age 1 year or older
- Must have a diagnosis of Lennox-Gastaut syndrome
- Must have an adequate trial of at least 2 generic antiepileptic medications with an inadequate response or significant side effect/toxicity or have a contraindication to these therapies
- Must be on concomitant therapy with at least one other anticonvulsant medication

If the above criteria are met, rufinamide (Banzel) is approved.

- **Ruxolitinib (Jakafi™)** [1665, 1666, 1667, 1668, 1669, 1670, 1671, 1672, 1673]

Initial Authorization Criteria:

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1. **For myelofibrosis**
   - Must be prescribed by a hematologist or an oncologist
   - Must have a diagnosis of intermediate or high-risk myelofibrosis (includes primary myelofibrosis, post-polycythemia vera myelofibrosis, and post-essential thrombocythemia myelofibrosis)

2. **For polycythemia vera**
   - Must be prescribed by a hematologist or an oncologist
   - Must have a diagnosis of polycythemia vera, currently requiring phlebotomy
   - Must have an inadequate response to or intolerance of hydroxyurea

3. **For acute graft-versus-host disease**
   - Must be prescribed by a hematologist, oncologist, or transplant specialist
   - Must be status-post an allogeneic hematopoietic stem cell transplantation
   - Must have a diagnosis of steroid-refractory acute graft-versus-host disease

4. **For chronic graft-versus-host disease**
   - Must be prescribed by a hematologist, oncologist, or transplant specialist
   - Must be status-post an allogeneic hematopoietic stem cell transplantation
   - Must have a diagnosis of chronic graft-versus-host disease after failure of one or two lines of systemic therapy

If the above criteria are met, ruxolitinib (Jakafi) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or stabilized while on therapy.

- **Ruxolitinib cream (Opzelura®)** [1674, 1675, 1676, 1677, 1678, 1679, 1680, 1681, 1682, 1683]

Initial Authorization Criteria:
1. For atopic dermatitis:
   - Must be prescribed by a dermatologist, an allergist, or an immunologist
   - Must be age 2 years or older
   - Must have a diagnosis of mild to moderate atopic dermatitis
   - Must have a body surface area involvement of ≤ 20%
   - Must have an adequate trial with an inadequate response (despite adherence based on prescriber attestation and/or claims history) or significant side effect/toxicity or have a contraindication to BOTH of the following:
     - Moderate to high potency topical corticosteroid

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        - Pimecrolimus cream (Elidel) or tacrolimus 0.1% ointment (Protopic)
    - Must not be immunocompromised
    - Must not be used in combination with an oral targeted anti-inflammatory agent or biologic agent, or potent immunosuppressive (e.g., azathioprine, cyclosporine)

2. For vitiligo:
    - Must be prescribed by a dermatologist
    - Must be age 12 years or older
    - Must have a diagnosis of nonsegmental vitiligo
    - Must have a body surface area involvement of ≤ 10%
    - Must be using on the head and neck region
    - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to BOTH of the following:
        - Moderate to high potency topical corticosteroid
        - Pimecrolimus cream (Elidel) or tacrolimus ointment (Protopic) for at least 3 months
    - Must not be used in combination with an oral targeted anti-inflammatory agent or biologic agent, or potent immunosuppressive (e.g., azathioprine, cyclosporine)

If the above criteria are met, ruxolitinib cream (Opzelura) is approved for 6 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved based upon the prescriber's assessment while on therapy or the member continues to benefit from therapy.

• Sacrosidase Oral Solution (Sucraid®) [1684, 1685, 1686, 1687]

Initial Authorization Criteria:
    - Must be prescribed by a gastroenterologist
    - Must be age 5 months or older
    - Must have a diagnosis of congenital sucrose-isomaltase deficiency (CSID) confirmed by ONE of the following:
        - Genetic mutation in the SI gene (copy of the genetic test must be provided)
        - Duodenal biopsy showing low sucrose levels and normal amounts of other disaccharides (chart documentation of biopsy result must be provided)

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        - Carbon-13 sucrose breath test below normal limit (chart documentation of laboratory test result with reference range must be provided)
        - Must have chart documentation of ALL of the following:
            - Stool pH <6
            - Increase in breath hydrogen of >10ppm (parts per million) when challenged with sucrose after fasting
            - Negative lactose breath test
    - Must have chart documentation of symptomatic disease (e.g., diarrhea, bloating, abdominal cramping)

If the above criteria are met, sacrosidase (Sucraid) is approved for a 1-month trial.

Reauthorization Criteria:
All prior authorization renewals are reviewed annually to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one-year intervals based upon chart documentation from the prescriber that the member's condition has improved based upon the prescriber's assessment while on therapy.

- **Sapropterin Dihydrochloride (Kuvan®, Javygtor™, Zelvysia™)**[1688, 1689, 1690, 1691, 1692]

Initial Authorization Criteria:
- Must be prescribed by a provider who specializes in the treatment of inherited metabolic disorders
- Must have a diagnosis of hyperphenylalaninemia (HPA) due to diagnosed or suspected phenylalanine hydroxylase (PAH) deficiency (including classic phenylketonuria)
- Must have chart documentation of a history of blood phenylalanine level >360 micromol/L
- Must use in conjunction with a phenylalanine-restricted diet
- Must use a dose and duration of therapy consistent with the FDA-approved package labeling
- Must not use in combination with sepiapterin (Sephience) or pegvaliase (Palynziq)
    - Note: an exception may be made to allow for dose titration of pegvaliase (Palynziq).

If the above criteria are met, the requested product is approved for 3 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon the following:

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- Must have chart documentation of response to therapy evidenced by a decrease in blood phenylalanine level from baseline
  - If member has not had a decrease in blood phenylalanine level, must have documentation of dose increase to 20 mg/kg/day
    - Authorizations for these members may be extended for an additional 1 month trial at the higher dose
  - Note: Authorizations for members who have not had a decrease in blood phenylalanine level at the 20 mg/kg/day dose may not be extended.
- Must use a dose and duration of therapy consistent with the FDA-approved package labeling
- Must not use in combination with sepiapterin (Sephience) or pegvaliase (Palynziq)
  - Note: an exception may be made to allow for dose titration of pegvaliase (Palynziq).

• Satralizumab-mwge (Enspryng®) [1693, 1694, 1695, 1696]

Initial Authorization Criteria:

- Must have a negative tuberculosis skin test [such as Tuberculin PPD (purified protein derivative) test] or Interferon-Gamma Release Assay (IGRA) whole-blood test [such as QuantiFERON®-TB Gold In-Tube test (QFT-GIT) or T-SPOT®.TB test (T-Spot)]
- Must be prescribed by a neurologist
- Must be ≥ 18 years of age
- Must have a diagnosis of antiaquaporin-4 antibody positive NMOSD. Chart documentation of a positive serological test of AQP4 antibodies is required
- Must have an MRI screening to confirm diagnosis and rule out other neurologic disease
- Must have symptomatic disease (e.g., myelitis, optic neuritis, postrema syndrome, diencephalic syndrome, brainstem syndromes, CNS mass lesions)
- Must have history of a prior NMOSD exacerbation treated with corticosteroids or plasma exchange. Documentation is required.
- Must have an adequate trial with an inadequate response, significant side effects/toxicity or have a contraindication to therapy with rituximab.
- Must currently not be using a biologic agent in combination with satralizumab-mwge (Enspryng)
- Must not have active Hepatitis B infection
- Must have no evidence of infection

If the above criteria are met, satralizumab-mwge (Enspryng) is approved for 3 months.

Reauthorization Criteria:

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All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved based upon the prescriber's assessment while on therapy or the member continues to benefit from therapy.

- **Seladelpar (Livdelzi®)** [1697, 1698]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with a gastroenterologist, hepatologist, or liver transplant specialist
- Must be age 18 years or older
- Must have a diagnosis of primary biliary cholangitis (PBC) confirmed by chart documentation of TWO of the following:
  - Elevated alkaline phosphatase (ALP) above the upper limit of normal (ULN) for at least 6 months based on the reference range provided by the laboratory
  - Positive anti-mitochondrial antibodies (AMA) or other PBC-specific auto-antibodies, if AMA is negative
  - Liver biopsy consistent with PBC
- Must have an adequate trial of at least 12 months with ursodiol at a dose of 13-15 mg/kg/day with an inadequate response defined as ALP 1.5-times the ULN or significant side effect/toxicity or have a contraindication to this therapy
- Must be used in combination with ursodiol unless there is a clinical contraindication or intolerance to ursodiol
- Must not be used in combination with obeticholic acid (Ocaliva) or elafibranor (Iqirvo)
- Must not have evidence of decompensated cirrhosis (e.g., ascites, variceal bleeding, hepatic encephalopathy)

If the above criteria are met, seladelpar (Livdelzi) is approved for 1 year.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon chart documentation showing the following:

- The member's condition has improved while on therapy
- Must not have evidence of decompensated cirrhosis (e.g., ascites, variceal bleeding, hepatic encephalopathy)

- **Selexipag tablet (Uptravi®)** [1699, 1700, 1701, 1702]

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- For selexipag intraveneous (Uptravi® IV) formulation requests, please refer to RX.PA.406 Commercial, Exchange Medical Prior Authorization Policies

Initial Authorization Criteria:

- Must be prescribed by a cardiologist or pulmonologist who specializes in pulmonary hypertension
- Must have a diagnosis of PAH (WHO Group 1) confirmed by right heart catheterization (RHC); must provide a copy of the RHC showing ALL of the following:
  - Mean pulmonary arterial pressure (mPAP) ≥25 mmHg
  - Pulmonary capillary wedge pressure (PCWP), left atrial pressure, or left ventricular end-diastolic pressure (LVEDP) ≤15 mmHg
  - Pulmonary vascular resistance >3 Wood units

If the above criteria are met, selexipag (Uptravi) is approved for 3 months.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or the member continues to benefit from therapy.

- **Semaglutide (Wegovy®) injection, tablets** [1703, 1704, 1705, 1706, 1707, 1708, 1709, 1710, 1711, 1712, 1713, 1714, 1715]
  - Requests for weight loss without established cardiovascular disease or for members with weight loss coverage, please refer to 'Weight Loss' criteria below

1. For Cardiovascular Disease [Applies to semaglutide injection and tablets (Wegovy)]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with a cardiologist
- Must be age 45 years or older
- Must have chart documentation showing a diagnosis of overweight or obesity with established cardiovascular disease confirmed by BOTH of the following:
  - Must have an initial BMI of 27 kg/m^2 or greater
  - Must have established cardiovascular disease confirmed by a history of at least ONE of the following:
    - Myocardial infarction (MI)
    - Ischemic or hemorrhagic stroke
    - Symptomatic peripheral arterial disease evidenced by ONE of the following:
      - Intermittent claudication with ankle–brachial index <0.85
      - Peripheral arterial revascularization procedure

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        - Amputation due to atherosclerotic disease
- Must have chart documentation that the member has been evaluated for other comorbidities that increase the risk of cardiovascular disease
    - For members with dyslipidemia:
        - Must be receiving optimized lipid-lowering therapy as defined below:
            - Must be currently taking a maximally tolerated statin
                - If unable to tolerate a statin, must be currently taking either a PCSK9 inhibitor, bempedoic acid (Nexletol®) or bempedoic acid/ezetimibe (Nexlizet®)
        - Must provide clinical rationale for why lipid-lowering therapy is not producing sufficient risk reduction for the member to necessitate adding semaglutide (Wegovy) to the treatment regimen
    - For members with heart failure or chronic kidney disease:
        - Must be currently taking an SGLT2 inhibitor approved for cardiovascular risk reduction
        - Must provide clinical rationale for why SGLT2 inhibitor therapy is not producing sufficient risk reduction for the member to necessitate adding semaglutide (Wegovy) to the treatment regimen
    - For members with type 2 diabetes:
        - Must provide clinical rationale for use of semaglutide (Wegovy) instead of an SGLT2 inhibitor. Clinical rationale must include why an SGLT2 inhibitor is not producing sufficient risk reduction and why semaglutide (Wegovy) would be expected to produce better risk reduction than an SGLT2 inhibitor
        - Must provide clinical rationale for use of semaglutide (Wegovy) instead of semaglutide (Ozempic®). Clinical rationale must include why semaglutide (Ozempic®) is not producing sufficient risk reduction and why semaglutide (Wegovy) would be expected to produce better risk reduction given that they are the same chemical entity
- Must have chart documentation of an outlined reduced-calorie diet plan and increased physical activity plan
- Must provide chart documentation of baseline body weight including date

If the above criteria are met, semaglutide (Wegovy) is approved for 6 months.

<u>Reauthorization Criteria:</u>

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All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon chart documentation showing the following:

- Must continue to follow a reduced-calorie diet and increased physical activity plan
- Must have a recent body weight that supports ≥5% body weight loss from baseline
- Must be adherent (PDC > 80%) to semaglutide (Wegovy) based on claims history
- For members with comorbidities:
  - Must have an attestation from the provider that all other comorbidities continue to be treated with standard of care

2. For metabolic dysfunction-associated steatohepatitis (MASH)/nonalcoholic steatohepatitis (NASH): [Applies to semaglutide injection (Wegovy) only]
Initial Authorization Criteria:

- Must be prescribed by or in consultation with a hepatologist or gastroenterologist
- Must be age 18 years or older
- Must have a diagnosis of noncirrhotic nonalcoholic steatohepatitis (NASH) or metabolic dysfunction-associated steatohepatitis (MASH) with moderate to advanced liver fibrosis (consistent with stages F2 to F3 fibrosis) confirmed by ALL of the following:
  - Must have at least ONE of the following cardiometabolic risk factors:
    - Overweight or obesity
    - Type 2 diabetes or prediabetes
    - Hypertension
    - Hypertriglyceridemia
    - Reduced high-density lipoprotein cholesterol (HDL-C)
  - Must have moderate to advanced liver fibrosis (consistent with stages F2 to F3 fibrosis) confirmed by ONE of the following:
    - Liver biopsy performed within the last 3 years shows fibrosis stage F2 to F3 (chart documentation must be provided)
    - FIB-4 ≥ 1.3 AND one of the following non-invasive tests performed within the last 6 months is consistent with F2 to F3 fibrosis (a copy of the report must be provided):
      - Vibration controlled transient elastography (VCTE; e.g., Fibroscan)
      - Shear wave elastography (SWE)
      - Magnetic resonance elastography (MRE)
      - Enhanced liver fibrosis (ELF) score

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- Must not have evidence of cirrhosis (stage F4 fibrosis) by imaging or liver biopsy or one or more liver-related complications associated with cirrhosis (e.g., variceal bleeding, ascites, hepatic encephalopathy, etc.)
- Must not have significant alcohol use (defined as alcohol consumption of more than 1 drink/day for natal females or more than 2 drinks/day for natal males). Chart documentation must be provided.
- Must have an adequate trial of semaglutide (Ozempic) at the maximum dose of 2 mg/week without significant side effects/toxicities (chart documentation of ability to tolerate this dose must be provided)
- Must provide clinical rationale for use of semaglutide (Wegovy) instead of semaglutide (Ozempic). Clinical rationale must include why semaglutide (Ozempic) would not be expected to produce the same clinical results as semaglutide (Wegovy) given that they are the same chemical entity.
- Must provide documentation that the member is currently receiving standard of care treatment to manage comorbid metabolic conditions (e.g., type 2 diabetes, dyslipidemia, hypertension, etc.)
- Must have an attestation from the provider that the member has received counseling on dietary and lifestyle modifications
- Must be used in conjunction with diet and exercise

If the above criteria are met, semaglutide injection (Wegovy) is approved for 6 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon the following:

- Must have chart documentation the member's condition has improved or stabilized while on therapy
- Must not have evidence of cirrhosis (stage F4 fibrosis) by imaging or liver biopsy or one or more liver-related complications associated with cirrhosis (e.g., variceal bleeding, ascites, hepatic encephalopathy, etc.)
- Must not have significant alcohol use (defined as alcohol consumption of more than 1 drink/day for natal females or more than 2 drinks/day for natal males)
- Must have an attestation from the provider that comorbid metabolic conditions continue to be treated with standard of care
- Must continue to be used in conjunction with diet and exercise
- Must be adherent (PDC > 80%) to semaglutide (Wegovy) based on claims history

- **Sepiapterin (Sephience™)** [1716, 1717, 1718]

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Initial Authorization Criteria:

- Must be prescribed by a provider who specializes in the treatment of inherited metabolic disorders
- Must be age 1 month or older
- Must have a diagnosis of hyperphenylalaninemia (HPA) due to phenylalanine hydroxylase (PAH) deficiency (including classic phenylketonuria) confirmed by chart documentation of genetic test result showing biallelic pathogenic variants in the phenylalanine hydroxylase (PAH) gene (test result is required)
- Must have chart documentation of a history of blood phenylalanine level > 360 micromol/L
- Must use in conjunction with a phenylalanine-restricted diet
- Must have an adequate trial of sapropterin (Kuvan, Javygtor, Zelvysia) at a dose of 20 mg/kg per day with an inadequate response (defined as the inability to achieve a blood phenylalanine level ≤ 360 micromol/L with combination of sapropterin [Kuvan, Javygtor, Zelvysia] and a phenylalanine-restricted diet) or significant side effect/toxicity or have a contraindication to this therapy
- Must use a dose and duration of therapy consistent with the FDA-approved package labeling
- Must not use in combination with sapropterin (Kuvan, Javygtor, Zelvysia) or pegvaliase (Palynziq)
    - Note: an exception may be made to allow for dose titration of pegvaliase (Palynziq).

If the above criteria are met, sepiapterin (Sephience) is approved for 3 months.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon the following:

- Must have chart documentation of response to therapy evidenced by a decrease in blood phenylalanine level from baseline
    - If member has not had a decrease in blood phenylalanine level, must have documentation of dose increase to 60 mg/kg/day
        - Authorizations for these members may be extended for an additional 1 month trial at the higher dose
    - Note: Authorizations for members who have not had a decrease in blood phenylalanine level at the 60 mg/kg/day dose may not be extended
- Must use a dose and duration of therapy consistent with the FDA-approved package labeling
- Must not use in combination with sapropterin (Kuvan, Javygtor, Zelvysia) or pegvaliase (Palynziq)

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- Note: an exception may be made to allow for dose titration of pegvaliase (Palynziq).

- **Setmelanotide (Imcivree™)** [1719, 1720, 1721, 1722, 1723, 1724, 1725, 1726]

Initial Authorization Criteria:

1. For pro-opiomelanocortin (POMC), proprotein convertase subtilisin/kexin type 1 (PCSK1), or leptin receptor (LEPR) deficiency:

- Must be prescribed by an endocrinologist
- Must be age 2 years or older
- Must have a diagnosis of obesity due to pro-opiomelanocortin (POMC), proprotein convertase subtilisin/kexin type 1 (PCSK1), or leptin receptor (LEPR) deficiency, confirmed by chart documentation of ALL of the following:
  - Baseline age, height, weight and body mass index (BMI) including date
  - A diagnosis of obesity, defined as one of the following:
    - If member is ≥ 18 years of age: BMI ≥ 30 kg/m^2
    - If member is 2-17 years of age: weight ≥ 95th percentile for age on growth chart assessment
  - Genetic testing that demonstrates BOTH of the following (chart documentation of laboratory result is required):
    - Variants in one of the following genes: *POMC*, *PCSK1*, or *LEPR*
    - The genetic variant is interpreted as pathogenic, likely pathogenic, or of uncertain significance (VUS)
    - NOTE: Setmelanotide (Imcivree) is not indicated for treatment of the following conditions as it would not be expected to be effective. Coverage will not be provided in the following conditions:
  - Obesity due to suspected POMC, PCSK1, or LEPR deficiency with POMC, PCSK1, or LEPR variants classified as benign or likely benign
  - Other types of obesity not related to POMC, PCSK1 or LEPR deficiency or BBS, including obesity associated with other genetic syndromes and general (polygenic) obesity
- Must have an attestation from the prescriber that setmelanotide (Imcivree) is used in conjunction with healthy lifestyle modifications (e.g., diet, physical activity)

If the above criteria are met, setmelanotide (Imcivree) is approved for 1 year.

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2. For Bardet-Biedl syndrome (BBS):
- Must be prescribed by an endocrinologist
- Must be age 2 years or older
- Must have a diagnosis of obesity due to Bardet-Biedl syndrome (BBS), confirmed by chart documentation of ALL of the following:
    - Baseline age, height, weight and body mass index (BMI) including date
    - A diagnosis of obesity, defined as one of the following:
        - If member is ≥ 18 years of age: BMI ≥ 30 kg/m^2
        - If member is 2-17 years of age: weight ≥ 97th percentile for age on growth chart assessment
    - Genetic testing that confirms the diagnosis of BBS (chart documentation of laboratory result is required)
    - NOTE: Setmelanotide (Imcivree) is not indicated for treatment of the following conditions as it would not be expected to be effective. Coverage will not be provided for other types of obesity not related to POMC, PCSK1 or LEPR deficiency or BBS, including obesity associated with other genetic syndromes and general (polygenic) obesity
- Must have an attestation from the prescriber that setmelanotide (Imcivree) is used in conjunction with healthy lifestyle modifications (e.g., diet, physical activity)

If the above criteria are met, setmelanotide (Imcivree) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation showing the following:
- Must have current age, height, weight and BMI including date to compare to baseline to assess weight loss progress
- Must have response to setmelanotide (Imcivree) therapy as evidenced by one of the following:
    - At least a 5% reduction in baseline body weight
    - At least a 5% reduction in baseline BMI for members with continued growth potential

• Sibeprenlimab-szsi (Voyxact®) [1727, 1728, 1729]

Initial Authorization Criteria:
- Must be prescribed by a nephrologist
- Must be age 18 years or older

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- Must have a diagnosis of primary immunoglobulin A nephropathy (IgAN) confirmed by a kidney biopsy. Documentation of the biopsy results is required.
- Must be at risk of rapid disease progression defined by the following (chart documentation is required):
  - Proteinuria > 0.75g/day OR Urinary Protein-to-Creatinine Ratio (UPCR) ≥1.5g/g despite at least 90 days of optimized supportive care treatment with ALL the following:
    - Maximally tolerated dose of an angiotensin converting enzyme inhibitor (ACEi) or an angiotensin receptor blocker (ARB) unless there is a contraindication to these therapies
      - NOTE: for members who have experienced significant side effects/toxicities to one of these classes, an adequate trial of the other class is required (e.g., ACEi-induced cough requires a trial of an ARB)
    - Other supportive care measures as clinically indicated (e.g., lifestyle modifications, blood pressure management, assessment of cardiovascular risk)
- Must be on a stable dose of maximally-tolerated renin-angiotensin system (RAS) inhibitor therapy (e.g., angiotensin converting enzyme inhibitor or an angiotensin receptor blocker)
- Must have an adequate trial of at least 3 months with an inadequate response (proteinuria > 0.75 g/day OR UPCR ≥ 1.5 g/g), significant side effect/toxicity or have a contraindication to ALL of the following:
  - A sodium-glucose co-transporter 2 (SGLT-2) inhibitor [e.g., empagliflozin (Jardiance)]
  - Sparsentan (Filspari)
  - Atrasentan (Vanrafia)
- Must have an attestation from the prescriber that the member is to remain on optimized supportive care as clinically indicated

- Must not use in combination with the following:
  - Sparsentan (Filspari)
  - Atrasentan (Vanrafia)
  - Iptacopan (Fabhalta)
  - Budesonide (Tarpeyo)

If the above criteria are met, sibeprenlimab-szsi (Voyxact) is approved for 1 year.

<u>Reauthorization Criteria:</u>

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All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon chart documentation showing the following:

- Must have a reduction in proteinuria OR urinary protein-to-creatinine ratio from baseline
- Must be on optimized supportive care as clinically indicated
- Must be on a stable dose of maximally-tolerated renin-angiotensin system (RAS) inhibitor therapy (e.g., angiotensin converting enzyme inhibitor or an angiotensin receptor blocker)
- Must not use in combination with the following:
  - Sparsentan (Filspari)
  - Atrasentan (Vanrafia)
  - Iptacopan (Fabhalta)
  - Budesonide (Tarpeyo)

- **Sildenafil (Revatio® tablet, Revatio® oral suspension)**
  1730,1731, 1732, 1733
  - Requests for sildenafil injection (Revatio® IV), please refer to policy RX.PA.406 Commercial, Exchange Medical Prior Authorization Policies

Initial Authorization Criteria:

- Must be prescribed by a cardiologist or pulmonologist who specializes in pulmonary hypertension
- Must have a diagnosis of PAH (WHO Group 1) with WHO functional class II-IV symptoms confirmed by right heart catheterization (RHC); must provide a copy of the RHC showing ALL of the following:
  - Mean pulmonary arterial pressure (mPAP) ≥25 mmHg
  - Pulmonary capillary wedge pressure (PCWP), left atrial pressure, or left ventricular end-diastolic pressure (LVEDP) ≤15 mmHg
  - Pulmonary vascular resistance >3 Wood units
- For sildenafil (Revatio) oral suspension:
  - Must have documentation of the inability to swallow an oral solid dosage form due to medical condition (e.g., nasogastric tube), or other clinical rationale for why sildenafil (Revatio) tablet cannot be used
- For sildenafil (Liqrev) oral suspension:
  - Must have documentation of the inability to swallow an oral solid dosage form due to medical condition (e.g., nasogastric tube), or other clinical rationale for why sildenafil (Revatio) tablet cannot be used
  - Must provide clinical rationale for use of sildenafil (Liqrev) oral suspension, instead of sildenafil (Revatio) oral suspension. Clinical rationale must include why sildenafil (Revatio) oral suspension would

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not be expected to produce the same clinical results as sildenafil (Liqrev) oral suspension

If the above criteria are met, the requested sildenafil product is approved for 3 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or the member continues to benefit from therapy.

- **Siponimod (Mayzent®)** [1734, 1735]

Initial Authorization Criteria:
- Must be prescribed by a neurologist
- Must be age 18 years or older
- Must have a diagnosis of relapsing form of multiple sclerosis, to include clinically isolated syndrome, relapsing-remitting disease, and active secondary progressive disease
- Must not have CYP2C9*3/*3 genotype
- Must not be on concomitant therapy with a disease modifying drug for multiple sclerosis

If the above criteria are met, siponimod (Mayzent) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or stabilized while on therapy.

- **Sirolimus Oral Solution (Rapamune®)** [1736, 1737, 1738, 1739, 1740, 1741, 1742, 1743, 1744, 1745, 1746, 1747, 1748, 1749, 1750, 1751, 1752, 1753]

Initial Authorization Criteria:
- Must have a diagnosis of one of the following:
  - Prophylaxis of organ rejection
  - Lymphangioleiomyomatosis (LAM)
- For prophylaxis of organ rejection:
  - Must be prescribed by or in consultation with a transplant specialist
- For lymphangioleiomyomatosis (LAM):
  - Must be prescribed by or in consultation with a pulmonologist or oncologist

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- Must provide documentation of the inability to swallow an oral solid dosage form due to age (< 7 years), medical conditions (e.g., nasogastric tube) or other clinical rationale for why sirolimus (Rapamune) tablet cannot be used

If the above criteria are met, sirolimus oral solution (Rapamune) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for the continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved based upon the prescriber's assessment while on therapy or the member continues to benefit from therapy AND that the member continues to meet the initial authorization criteria.

• Sirolimus Topical Gel (Hyftor®) [1754, 1755, 1756, 1757, 1758]

Initial Authorization Criteria:

- Must be prescribed by a provider who specializes in the treatment of tuberous sclerosis complex (e.g., dermatologist, neurologist)
- Must be age 6 years or older
- Must be used for diagnosis of facial angiofibroma associated with tuberous sclerosis
- Must have a definitive diagnosis of tuberous sclerosis confirmed by ONE of the following:
  - Identification of a pathogenic variant in the tuberous sclerosis complex 1 (*TSC1)* or tuberous sclerosis complex 2 (*TSC2*) gene. A copy of the laboratory result confirming the variant is required. OR
  - Chart documentation confirming the presence of either two major features or one major feature with two minor features
    - Major features include: hypomelanotic macules (3 or more); angiofibroma (3 or more) or cephalic plaque; ungual fibromas (2 or more); shagreen patch; multiple retinal hamartomas; multiple cortical tubers and/or radial migration lines; subependymal nodule (2 or more); subependymal giant cell astrocytoma; cardiac rhabdomyoma; lymphangiomyomatosis; angiomyolipomas (2 or more)
    - Minor features include: "confetti" skin lesions; dental enamel pits (3 or more); intraoral fibromas (2 or more); retinal achromic patch; multiple renal cysts; nonrenal hamartomas; sclerotic bone lesions
- Must have 3 or more facial angiofibromas of 2 mm or more in diameter

If the above criteria are met, sirolimus topical gel (Hyftor) is approved for 3 months.

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Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon chart documentation showing the following:
- Must have improvement in angiofibroma size and/or color

• Sodium Oxybate (Xyrem®), Calcium, magnesium, potassium, and sodium oxybate (Xywav™), and sodium oxybate ER (Lumryz™) 1759, 1760, 1761, 1762, 1763, 1764, 1765, 1766, 1767, 1768, 1769, 1770, 1771, 1772, 1773, 1774, 1775, 1776, 1777, 1778, 1779

Initial Authorization Criteria:
Sodium Oxybate (Xyrem) and calcium, magnesium, potassium, and sodium oxybate (Xywav)
1. **For Narcolepsy**
   - Must be prescribed by a board-certified sleep specialist, a pulmonologist, or a neurologist
   - Must have polysomnographic evaluation (i.e., sleep study) with subsequent multiple sleep latency test (MSLT) that shows a mean sleep latency of less than 8 minutes and two or more sleep onset rapid eye movement periods (SOREMPs) including any SOREMP on the polysomnographic evaluation from the preceding night (copy of sleep study must be provided)
   - For calcium, magnesium, potassium, and sodium oxybate (Xywav), must have BOTH of the following:
     - Chart documentation of inability to take sodium oxybate (Xyrem) due to sensitivity to salt intake (history of heart failure, hypertension, or renal impairment).
     - Chart documentation that the member is adhering to a low sodium diet
   - For cataplexy associated with narcolepsy in adult members must have ALL of the following:
     - Must be age 18 years or older
     - Chart documentation of a history of cataplexy
     - An adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to BOTH of the following:
       - ONE of the following:
         - A tricyclic antidepressant
         - Venlafaxine
         - Atomoxetine
         - Fluoxetine
       - Pitolisant (Wakix)
   - For cataplexy associated with narcolepsy in pediatric members must have ALL of the following:

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        - Must be 7 to 17 years of age
        - Chart documentation of a history of cataplexy
        - An adequate trial and failure with an inadequate response or significant side effect/toxicity or have a contraindication ONE of the following:
            - A tricyclic antidepressant
            - Venlafaxine
            - Atomoxetine
            - Fluoxetine
    - For excessive daytime sleepiness associated with narcolepsy in adult members must have ALL of the following:
        - Must be age 18 years or older
        - Chart documentation of a history of excessive daytime sleepiness
        - An adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
            - Modafinil OR armodafinil
            - Solriamfetol (Sunosi™)
            - Pitolisant (Wakix)
    - For excessive daytime sleepiness associated with narcolepsy in pediatric members must have ALL of the following:
        - Must be 7 to 17 years of age
        - Chart documentation of a history of excessive daytime sleepiness
        - An adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to BOTH of the following:
            - A central nervous system (CNS) stimulant (e.g., methylphenidate, dextroamphetamine, amphetamine/dextroamphetamine) OR modafinil
            - Pitolisant (Wakix)
    - Must not be using in combination with sedative hypnotic agents
    - Must not be using in combination with alcohol
    - Must not have succinic semialdehyde dehydrogenase deficiency

2. **For idiopathic hypersomnia:**
    - Must be prescribed by a board-certified sleep specialist, a pulmonologist, or a neurologist
    - Must be 18 years or older
    - For calcium, magnesium, potassium, and sodium oxybate (Xywav), must have BOTH of the following:
        - Chart documentation of inability to take sodium oxybate (Xyrem) due to sensitivity to salt intake (history of heart failure, hypertension, or renal impairment).
        - Chart documentation that the member is adhering to a low sodium diet
    - Must have chart documentation of ALL of the following:

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        - A report of subjective sleepiness for more than 3 months including documentation of an Epworth Sleepiness Scale (ESS) score greater than or equal to 10
        - Absence of cataplexy
        - A workup to rule out other identifiable causes for excessive daytime sleepiness including ALL of the following:
            - Other central disorders of hypersomnolence (e.g., Kleine-Levin syndrome, narcolepsy without cataplexy, hypersomnia due to a medical disorder, hypersomnia due to a medication or substance, hypersomnia associated with a psychiatric disorder, insufficient sleep syndrome)
            - Sleep-related breathing disorders (e.g., obstructive sleep apnea, central sleep apnea syndromes, sleep-related hypoventilation disorders, sleep-related hypoxemia disorder)
            - Circadian rhythm sleep-wake disorders (e.g., non-24-h sleep-wake rhythm disorder, shift work disorder, jet lag disorder)
    - Must have a polysomnographic evaluation (i.e., sleep study) with subsequent multiple sleep latency test (MSLT) that shows ALL of the following:
        - A mean sleep latency of less than 8 min (members with a mean sleep latency greater than or equal to 8 min, must demonstrate 11 or more hours of average daily sleep time with 24-h polysomnography or 1-week actigraphy or sleep logs)
        - Fewer than two sleep onset rapid eye movement periods (SOREMPs) including any SOREMP on the polysomnographic evaluation from the preceding night
        - Evaluation for obstructive respiratory events showing no evidence of obstructive sleep apnea
    - Must provide two weeks of sleep diaries from within the previous month showing an average nocturnal total sleep time of 7 hours or more
    - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
        - A central nervous system (CNS) stimulant (e.g., methylphenidate, dextroamphetamine, amphetamine/dextroamphetamine)
        - Modafinil
    - Must not be using in combination with sedative hypnotic agents
    - Must not be using in combination with alcohol
    - Must not have succinic semialdehyde dehydrogenase deficiency

Sodium oxybate ER (Lumryz)

1. **For Narcolepsy**
    - Must be prescribed by a board-certified sleep specialist, a pulmonologist, or a neurologist
    - Must have polysomnographic evaluation (i.e., sleep study) with subsequent multiple sleep latency test (MSLT) that shows a mean sleep latency of less

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than 8 minutes and two or more sleep onset rapid eye movement periods (SOREMPs) including any SOREMP on the polysomnographic evaluation from the preceding night (copy of sleep study must be provided)

- For cataplexy associated with narcolepsy in adult members must have ALL of the following:
  - Must be age 18 years or older
  - Chart documentation of a history of cataplexy
  - An adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
    - ONE of the following:
      - A tricyclic antidepressant
      - Venlafaxine
      - Atomoxetine
      - Fluoxetine
    - Pitolisant (Wakix)
    - Sodium oxybate (Xyrem) OR calcium, magnesium, potassium, and sodium oxybate (Xywav)
- For cataplexy associated with narcolepsy in pediatric members must have ALL of the following:
  - Must be 7 to 17 years of age
  - Chart documentation of a history of cataplexy
  - An adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to BOTH of the following:
    - ONE of the following:
      - A tricyclic antidepressant
      - Venlafaxine
      - Atomoxetine
      - Fluoxetine
    - Sodium oxybate (Xyrem) OR calcium, magnesium, potassium, and sodium oxybate (Xywav)
- For excessive daytime sleepiness associated with narcolepsy in adult members must have ALL of the following:
  - Must be age 18 years or older
  - Chart documentation of a history of excessive daytime sleepiness
  - An adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
    - Modafinil OR armodafinil
    - Solriamfetol (Sunosi)
    - Pitolisant (Wakix)
    - Sodium oxybate (Xyrem) OR calcium, magnesium, potassium, and sodium oxybate (Xywav)
- For excessive daytime sleepiness associated with narcolepsy in pediatric members must have ALL of the following:

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        - Must be 7 to 17 years of age
        - Chart documentation of a history of excessive daytime sleepiness
        - An adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
            - A central nervous system (CNS) stimulant (e.g., methylphenidate, dextroamphetamine, amphetamine/dextroamphetamine) OR modafinil
            - Pitolisant (Wakix)
            - Sodium oxybate (Xyrem) OR calcium, magnesium, potassium, and sodium oxybate (Xywav)
    - Must not be using in combination with sedative hypnotic agents
    - Must not be using in combination with alcohol
    - Must not have succinic semialdehyde dehydrogenase deficiency

If the above criteria are met, sodium oxybate (Xyrem), calcium, magnesium, potassium, and sodium oxybate (Xywav), or sodium oxybate ER (Lumryz) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved while on therapy.

• Sodium Phenylbutyrate (Buphenyl®, Olpruva®, Pheburane®) and Glycerol Phenylbutyrate (Ravicti®)

1780, 1781, 1782, 1783, 1784, 1785, 1786, 1787, 1788, 1789, 1790, 1791

Initial Authorization Criteria:
- Must be prescribed by a provider who specializes in the treatment of inherited metabolic disorders
- Must have a diagnosis of a urea cycle disorder (e.g., deficiencies of carbamylphosphate synthetase, ornithinine transcarbamylase, or arginosuccinic acid synthetase) unable to be managed by dietary protein restriction or supplementation alone
- Must have a diagnosis confirmed by ONE of the following:
    - A genetic mutation resulting in a urea cycle disorder (a copy of the genetic test must be provided)
    - Hyperammonemia (ammonia level above the upper limit of the normal). Chart documentation of laboratory test result including reference range must be provided.
- **For sodium phenylbutyrate (Pheburane) pellet**

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        - Must have an adequate trial of sodium phenylbutyrate tablet or powder with an inadequate response or significant side effect/toxicity (chart documentation of response must be provided)
            - NOTE: an exception to the requirement for a trial of sodium phenylbutyrate tablets or powder (Buphenyl) can be made if the member is ≤3 years of age
        - Must provide clinical rationale for use of sodium phenylbutyrate (Pheburane) pellet instead of sodium phenylbutyrate tablet or powder (Buphenyl). Clinical rationale must include why phenylbutyrate tablet or powder (Buphenyl) would not be expected to produce the same clinical results as sodium phenylbutyrate pellets (Pheburane)
    - **For sodium phenylbutyrate (Olpruva) oral suspension**:
        - Must have an adequate trial with an inadequate response or significant side effect/toxicity to BOTH of the following (chart documentation of response must be provided):
            - Sodium phenylbutyrate tablet or powder (Buphenyl)
            - Sodium phenylbutyrate pellet (Pheburane)
                - NOTE: an exception to the requirement for a trial of sodium phenylbutyrate (Buphenyl, Pheburane) can be made if the member is ≤3 years of age
        - Must provide clinical rationale for use of sodium phenylbutyrate (Olpruva) oral suspension instead of sodium phenylbutyrate tablet or powder (Buphenyl) or sodium phenylbutyrate pellet (Pheburane). Clinical rationale must include why sodium phenylbutyrate tablet or powder (Buphenyl) or pellet (Pheburane) would not be expected to produce the same clinical results as sodium phenylbutyrate oral suspension (Olpruva)
    - **For glycerol phenylbutyrate (Ravicti):**
        - Must have an adequate trial with an inadequate response or significant side effect/toxicity, or have a contraindication to ALL the following (chart documentation of response must be provided):
            - Sodium phenylbutyrate tablet or powder (Buphenyl)
            - Sodium phenylbutyrate pellet (Pheburane)
                - NOTE: an exception to the requirement for a trial of sodium phenylbutyrate (Buphenyl, Pheburane) can be made if the member is ≤3 years of age
            - Sodium phenylbutyrate oral suspension (Olpruva)
                - NOTE: an exception to the requirement for a trial of sodium phenylbutyrate oral suspension (Olpruva) can be made in pediatric members weighing <20kg or with a body surface area (BSA) of <1.2m^2
        - Must provide clinical rationale for use of glycerol phenylbutyrate (Ravicti) instead of sodium phenylbutyrate tablet or powder

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(Buphenyl), pellet (Pheburane), or oral suspension (Olpruva). Clinical rationale must include why sodium phenylbutyrate tablet or powder (Buphenyl), pellet (Pheburane), or oral suspension (Olpruva) would not be expected to produce the same clinical results as glycerol phenylbutyrate (Ravicti)

If the above criteria are met, the requested product is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine Medical Necessity for continuation of treatment. Authorization may be extended at one-year intervals based upon chart documentation from the provider that the member's condition has improved while on therapy.

- **Solriamfetol (Sunosi™)** 1792, 1793, 1794, 1795, 1796, 1797, 1798, 1799, 1800

Initial Authorization Criteria:

1. **For Narcolepsy:**
   - Must be age 18 years or older
   - Must have polysomnographic evaluation (i.e., sleep study) with subsequent multiple sleep latency test (MSLT) that shows a mean sleep latency of less than 8 minutes and two or more sleep onset rapid eye movement periods (SOREMPs) including any SOREMP on the polysomnographic evaluation from the preceding night (copy of sleep study must be provided)
   - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ONE of the following:
     - Modafinil
     - Armodafinil

2. **For Obstructive Sleep Apnea:**
   - Must be age 18 years or older
   - Must have documentation of diagnosis through sleep study
   - Must have documentation/compliance report from physician that the member is using a CPAP (continuous positive airway pressure) machine on a regular basis, defined by at least 4 hours a night on at least 70% of the nights and symptoms still persist
     - For members who are intolerant to CPAP, must provide chart documentation that troubleshooting strategies were used during the first 90 days of treatment to address barriers (e.g., meeting with clinician or DME provider about mask-related issues, trying at least 2

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different masks, increasing humidity settings, addressing claustrophobia concerns)

- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ONE of the following:
  - Modafinil
  - Armodafinil

If the above criteria are met, solriamfetol (Sunosi) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one year intervals based upon documentation from the provider that the member's condition has improved while on therapy.

- **Somatropin (Serostim®)** 1801, 1802, 1803, 1804, 1805, 1806, 1807, 1808, 1809, 1810, 1811, 1812, 1813, 1814

Initial Authorization Criteria:

- Must be prescribed by or in consultation with a provider who specializes in the treatment of HIV/AIDS
- Must have a diagnosis of HIV-associated wasting or cachexia
- Must have a documented involuntary weight loss of at least 10% from baseline or to a BMI <20 in the absence of a concurrent illness or medical condition other than HIV infection that would explain these findings
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to at least TWO medications for HIV-associated wasting or cachexia (e.g., dronabinol, megestrol)
- Must be on concomitant therapy with antiretroviral medications

If the above criteria are met, somatropin (Serostim) is approved for 12 weeks.

Reauthorization Criteria:
All prior authorization renewals are reviewed on a case-by-case basis to determine the Medical Necessity for continuation of therapy. One additional authorization may be given for therapy continuation for up to 36 additional weeks with documentation from the provider of weight stabilization or weight gain.

- **Sotatercept-csrk (Winrevair®)** 1815, 1816, 1817, 1818,1819, 1820

Initial Authorization Criteria:

- Must be prescribed by a cardiologist or pulmonologist who specializes in pulmonary hypertension

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- Must be age 18 years or older
- Must have a diagnosis of pulmonary arterial hypertension (PAH) WHO Group 1 confirmed by right heart catheterization (RHC); must provide a copy of the RHC showing ALL of the following:
  - Mean pulmonary arterial pressure (mPAP) ≥25 mmHg
  - Pulmonary capillary wedge pressure (PCWP), left atrial pressure, or left ventricular end-diastolic pressure (LVEDP) ≤15 mmHg
  - Pulmonary vascular resistance >3 Wood units
- Must use in combination with at least two conventional PAH therapies from different pharmacologic categories (e.g., phosphodiesterase-5 [PDE-5] inhibitor, endothelin receptor antagonist [ERA], prostacyclin agonist).
  - If the member has a significant side effect/toxicity or contraindication to combination therapy with two conventional PAH therapies, the member must currently be using at least one PAH conventional therapy for ≥ 2 months (chart documentation of previous combination drug trials and response must be provided)
- Must have platelet count of ≥ 50,000/mm^3
- Must be using a dose and duration of therapy consistent with the FDA-approved package labeling

If the above criteria are met, sotatercept-csrk (Winrevair) is approved for 3 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or the member continues to benefit from therapy.

• Sparsentan (Filspari™) [1821, 1822, 1823, 1824]

Initial Authorization Criteria:
- Must be prescribed by a nephrologist
- Must be age 18 years or older
- Must have a diagnosis of primary immunoglobulin A nephropathy (IgAN) confirmed by a kidney biopsy. Documentation of the biopsy results is required.
- Must be at risk of rapid disease progression defined by the following (chart documentation is required):
  - Proteinuria > 0.75 g/day OR Urinary Protein-to-Creatinine Ratio (UPCR) ≥ 1.5 g/g despite at least 90 days of optimized supportive care treatment with ALL of the following
    - Maximally tolerated dose of an angiotensin converting enzyme inhibitor (ACEi) or an angiotensin receptor blocker (ARB) unless there is a contraindication to these therapies

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        - NOTE: for members who have experienced significant side effects/toxicities to one of these classes, an adequate trial of the other class is required (e.g., ACEi-induced cough requires a trial of an ARB)
    - Other supportive care measures as clinically indicated (e.g., lifestyle modifications, blood pressure management, assessment of cardiovascular risk)
- Must have an adequate trial of at least 3 months of a sodium-glucose co-transporter 2 (SGLT-2) inhibitor [e.g., empagliflozin (Jardiance)] with an inadequate response (proteinuria > 0.75 g/day OR UPCR ≥ 1.5 g/g) or significant side effect/toxicity or have a contraindication to this therapy
- Must have an attestation from the prescriber that the member will remain on optimized supportive care as clinically indicated
- Must not use in combination with ANY of the following:
    - Renin-angiotensin-aldosterone antagonists (e.g., angiotensin converting enzyme inhibitors or angiotensin receptor blockers), endothelin receptor antagonists, or aliskiren (Tekturna)
    - A strong CYP 3A4 inducer (e.g., rifampin, carbamazepine, phenytoin)
    - An acid reducing agent (histamine 2 receptor antagonists and proton pump inhibitors)
    - Atrasentan (Vanrafia)
    - Iptacopan (Fabhalta)
    - Budesonide DR (Tarpeyo)
    - Sibeprenlimab-szsi (Voyxact)

If the above criteria are met, sparsentan (Filspari) is approved for 1 year.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon chart documentation showing the following:

- Must have a reduction in proteinuria OR UPCR from baseline
- Must not be using in combination with ANY of the following:
    - Renin-angiotensin-aldosterone antagonists (e.g., angiotensin converting enzyme inhibitors or angiotensin receptor blockers), endothelin receptor antagonists, or aliskiren (Tekturna)
    - A strong CYP 3A4 inducer (e.g., rifampin, carbamazepine, phenytoin)
    - An acid reducing agent (histamine 2 receptor antagonists and proton pump inhibitors)
    - Atrasentan (Vanrafia)
    - Iptacopan (Fabhalta)
    - Budesonide DR (Tarpeyo)

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        - Sibeprenlimab-szsi (Voyxact)

- **Spesolimab-sbzo (Spevigo®) subcutaneous**[1825, 1826, 1827]
    - Requests for spesolimab-sbzo (Spevigo) intravenous injection please refer to policy RX.PA.406 Commercial, Exchange Medical Prior Authorization Policies

Initial Authorization Criteria:

- Must be prescribed by a dermatologist
- Must be age 12 years or older
    - For members 12 to 17 years of age, must weigh at least 40 kg
- Must have a diagnosis of generalized pustular psoriasis (GPP) confirmed by a history of visible pustules on non-acral skin that are not confined within psoriasis plaques. Chart documentation must be provided.
- Must not currently be experiencing a flare of GPP
- Must have a history of at least two moderate-to-severe GPP flares
- Must provide number of GPP flares per year at baseline
- Must have ONE of the following:
    - An adequate trial of at least 3 months of a conventional systemic therapy (e.g., methotrexate, cyclosporine, or acitretin) with an inadequate response or significant side effect/toxicity or have a contraindication to these therapies
    - A history of two moderate-to-severe flares in the past year, with at least one flare associated with systemic symptoms (e.g., fever, elevated CRP, elevated WBC count, asthenia, myalgia)
- Must be using a dose and duration of therapy consistent with the FDA-approved package labeling
    - Members who have received IV spesolimab within the previous 4 weeks may not receive a subcutaneous loading dose
- Must not be using an oral targeted anti-inflammatory agent or biologic agent in combination with spesolimab-sbzo (Spevigo)

If the above criteria are met, spesolimab-sbzo (Spevigo) is approved for 1 year.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon the following:

- Must have a decrease in the number of GPP flares from baseline
    - For members who have not had a decrease in the number of flares from baseline, must provide clinical rationale for why continuation of therapy with spesolimab-sbzo (Spevigo) is medically necessary

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- **Stiripentol (Diacomit®)** [1828, 1829, 1830, 1831, 1832, 1833]

Initial Authorization Criteria:

- Must have a diagnosis of Dravet syndrome
- Must be prescribed by or in consultation with a neurologist
- Must be age 6 months or older
- For members ages 2 or older:
  - Must have had an inadequate response or intolerance to at least 2 generic antiepileptic medications
  - Must have had an inadequate response or intolerance to cannabidiol (Epidiolex)
- Must be used in combination with clobazam (Onfi)

If the above criteria are met, stiripentol (Diacomit) is approved.

- **Sublingual Immunotherapy (SLIT)** [1834, 1835, 1836, 1837, 1838, 1839, 1840, 1841, 1842, 1843, 1844, 1845]
  - Applies to: Oralair®, Grastek®, Ragwitek®, Odactra™

For all SLIT medications:

- Must be prescribed by an allergist or immunologist
- Must have moderate to severe disease
  - Must have a negative effect on quality of life that impacts one of the following: sleep, school, and/or work performance
- Must not have any of the following contraindications:
  - History of any severe systemic allergic reaction
  - History of eosinophilic esophagitis
  - Moderate to severe, unstable, or uncontrolled asthma
  - Currently receiving concomitant immunotherapy
  - Failure of previous subcutaneous immunotherapy due to poor efficacy
- Must submit chart documentation demonstrating a trial of subcutaneous immunotherapy
- Must have the first dose administered in a physician office
- Must submit chart documentation demonstrating severe, persistent, allergic rhinitis symptoms (e.g. runny nose, blocked nose, sneezing, itchy nose, red/itchy eyes, watery eyes) that have not been adequately controlled with both of the following:
  - Inhaled nasal corticosteroid
  - An intranasal antihistamine or an oral antihistamine

In addition, the following criteria must be met for the specific medication being requested:

For Grastek and Oralair:

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- Must be 5 to 65 years of age
- Must have a diagnosis of grass-pollen induced allergic rhinitis with or without conjunctivitis
  - Diagnosis must be confirmed by positive skin test or *in vitro* testing for pollen-specific IgE antibodies for Timothy grass or cross-reactive grass pollens (i.e. Sweet Vernal, Orchard, Perennial Rye, Kentucky Blue Grass)
- Must submit chart documentation demonstrating seasonal symptoms to grass-pollen from the previous pollen season
- The timing of prescribing must meet the following criteria:
  - Grastek: therapy is initiated 12 weeks prior to the expected onset of the grass pollen season
  - Oralair: therapy is initiated 4 months prior to the expected start of grass pollen season

For Ragwitek:

- Must be 5 to 65 years of age
- Must have a diagnosis of short ragweed pollen-induced allergic rhinitis with or without conjunctivitis
  - Diagnosis must be confirmed by positive skin test or *in vitro* testing for pollen specific IgE antibodies for short ragweed pollen
- Must submit chart documentation demonstrating seasonal symptoms to ragweed- pollen from the previous pollen season
- Must have therapy initiated 12 weeks prior to the expected onset of the short ragweed pollen season

For Odactra:

- Must be 5 to 65 years of age
- Must have a diagnosis of house dust mite (HDM)-induced allergic rhinitis, with or without conjunctivitis
  - Diagnosis must be confirmed by positive skin test or in vitro testing for *Dermatophagoides farinae* or *Dermatophagoides pteronyssinus* house dust mites
- Must submit chart documentation of avoidance of known allergens and environmental controls (e.g. the use of central air filtration systems, bed covers, de-humidifiers)

If the above criteria are met, the requested SLIT is approved for 6 months.

<u>Reauthorization Criteria:</u>

All prior authorization renewals are reviewed every 6 months to determine the Medical Necessity for continuation of therapy. Authorizations may be extended for an additional 6 months of therapy based upon chart documentation from the prescriber

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showing that the member has benefited from therapy as evidenced by documentation of the following:

- Compliance with daily administration of the sublingual medication as evidenced by pharmacy claims (defined as PDC > 90%)
- Decrease in symptoms, disease severity, and symptom-relieving medication use
- For Grastek and Oralair:
  - All authorizations are approved for a maximum duration of 3 years.

• Succimer (Chemet) [1846, 1847]

Initial Authorization Criteria:

- Must be prescribed by a toxicologist or other clinician who has experience with chelating agents
- Must have diagnosis of lead poisoning with a blood lead level above 45µg/dL
- Must have chart documentation of identification and removal of the cause of lead exposure

If the above criteria are met, succimer (Chemet) is approved for 1 month.

Reauthorization Criteria:

All prior authorization renewals are reviewed on a case-by-case basis to determine the Medical Necessity for continuation of treatment. Authorizations may be extended on a case-by-case basis based upon the above criteria and clinical rationale from the prescriber for continuation of treatment.

• Tadalafil (Adcirca®, Alyq™, Tadliq®) [1848, 1849, 1850, 1851, 1852, 1853]

Initial Authorization Criteria:

- Must be prescribed by a cardiologist or pulmonologist who specializes in pulmonary hypertension
- Must have a diagnosis of PAH (WHO Group 1) with WHO functional class II-IV symptoms confirmed by right heart catheterization (RHC); must provide a copy of the RHC showing ALL of the following:
  - Mean pulmonary arterial pressure (mPAP) ≥25 mmHg
  - Pulmonary capillary wedge pressure (PCWP), left atrial pressure, or left ventricular end-diastolic pressure (LVEDP) ≤15 mmHg
  - Pulmonary vascular resistance >3 Wood units
- Must have an adequate trial of sildenafil (Revatio) tablet with an inadequate response, significant side effect/toxicity, or contraindication to this therapy
  - NOTE: An exception for the sildenafil (Revatio) trial may be made for members using tadalafil (Adcirca, Alyq) as initial therapy in combination with ambrisentan (Letairis)

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- For tadalafil (Tadliq) oral suspension:
  - Must have an adequate trial of sildenafil (Revatio) with an inadequate response, significant side effect/toxicity, or have a contraindication to this therapy
    - NOTE: An exception for the sildenafil (Revatio) trial may be made for members using tadalafil (Tadliq) as initial therapy in combination with ambrisentan (Letairis)
  - Must have documentation of the inability to swallow an oral solid dosage form due to medical condition (e.g., nasogastric tube), or other clinical rationale for why tadalafil (Adcirca, Alyq) tablet cannot be used

If the above criteria are met, the requested product is approved for 3 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or the member continues to benefit from therapy.

• Tadalafil (Cialis®) 5mg tablet [1854]

Criteria:
- Must have a diagnosis of ONE of the following:
  - Signs and symptoms of BPH OR
  - Erectile dysfunction (ED) with signs and symptoms of BPH
- Must NOT have a diagnosis of ED without signs and symptoms of BPH

If the above criteria are met, tadalafil (Cialis) 5mg tablet is approved.

• Tafamidis meglumine (Vyndaqel®) and Tafamidis (Vyndamax™) [1855, 1856, 1857, 1858, 1859, 1860, 1861, 1862]

Initial Authorization Criteria:
- Must be prescribed by or in consultation with a cardiologist who specializes in the treatment of amyloidosis
- Must be age 18 years or older
- Must have chart documentation of a diagnosis of transthyretin-mediated amyloid cardiomyopathy (ATTR-CM), including ALL of the following:
  - NYHA Functional Class I or II heart failure
  - Evidence of TTR amyloid in the myocardium obtained by ONE of the following:
    - Biopsy and amyloid typing (immunohistochemistry or mass spectrometry) OR

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            - Radionuclide imaging demonstrating cardiac uptake of one of the following radiopharmaceutical agents (a copy of the imaging report must be provided)
                - Technetium-99m pyrophosphate (Tc 99m PYP) OR
                - Technetium-99m hydroxymethylene diphosphonate (HMDP), aka technetium-99m oxidronate
        - Confirmation of the absence of immunoglobulin light-chain amyloidosis by serum studies (immunofixation without free light-chains) or a hematology/oncology consultation
    - Must have an adequate trial of acoramidis (Attruby) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
    - Must not be used in combination with a TTR-lowering agent (patisiran [Onpattro], vutrisiran [Amvuttra], eplontersen [Wainua]), or a TTR-stabilizing agent (acoramidis [Attruby])

If the above criteria are met, tafamidis meglumine (Vyndaqel) or tafamidis (Vyndamax) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for the continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or stabilized based upon the prescriber's assessment while on therapy or the member continues to benefit from therapy.

• Tapinarof cream (Vtama®) [1863, 1864, 1865, 1866]

Criteria:

1. For Plaque Psoriasis
- Must be prescribed by a dermatologist
- Must be age 18 years or older
- Must have a diagnosis of plaque psoriasis
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to TWO of the following:
    - A topical corticosteroid
        - NOTE: for members with psoriasis on areas other than the face or intertriginous areas, a moderate to high potency topical corticosteroid is required
    - Pimecrolimus cream (Elidel) or tacrolimus ointment (Protopic)
    - Topical calcipotriene or calcitriol

2. For Atopic Dermatitis
- Must be prescribed by a dermatologist

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- Must be age 2 years or older
- Must have a diagnosis of atopic dermatitis
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to BOTH of the following:
  - A topical corticosteroid
    - NOTE: for members with atopic dermatitis on areas other than the face or intertriginous areas, a moderate to high potency topical corticosteroid is required
  - Pimecroliums cream (Elidel) or tacrolimus ointment (Protopic)

If the above criteria are met, tapinarof cream (Vtama) is approved.

- **Tasimelteon (Hetlioz®, Hetlioz LQ™)** [1867, 1868, 1869, 1870, 1871, 1872, 1873, 1874, 1875, 1876]

Initial Authorization Criteria:

1. **For Non-24-Hour Sleep-Wake Disorder (Non 24)**
   - Must be 18 years of age or older
   - Must be using tasimelteon capsules (Hetlioz)
   - Must be prescribed by a physician who specializes in sleep medicine.
   - Must a have diagnosis of Non-24-Hour Sleep-Wake Disorder (Non-24) in a totally blind patient. Chart documentation describing how diagnosis was confirmed (e.g., sleep-wake logs, melatonin secretion abnormalities, or progress notes, etc.) is required.
   - Must submit chart documentation of a trial and failure to a prescribed sleep wake schedule.
   - Must submit chart documentation of a trial and failure of melatonin with inadequate response or significant side effects unless there is a documented contraindication.
   - Must submit documentation of a recent urine or serum nicotine screening confirming that member is NOT currently smoking or recently stopped smoking

2. **For nighttime sleep disturbances in Smith-Magenis Syndrome (SMS)**
   - Must be 16 years of age or older for tasimelteon capsules (Hetlioz) OR 3-15 years of age for tasimelteon oral suspension (Hetlioz LQ)
   - Must be prescribed by a physician who specializes in sleep medicine
   - Must submit chart documentation describing the sleep disturbance (e.g. early sleep onset, early awakening, frequent nocturnal awakenings, decreased total sleep time) and showing a negative impact on daily functioning or safety (e.g. nighttime roaming, excessive daytime sleepiness, difficulty concentrating during the day, frequent daily naps)
   - Must submit a copy of the genetic testing that confirms the diagnosis of Smith-Magenis Syndrome

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- Must submit chart documentation of a trial and failure of morning beta-antagonist in combination with nighttime melatonin with inadequate response or significant side effects unless there is a documented contraindication.
- For members 16 years of age and older, must submit documentation of a recent urine or serum nicotine screening confirming that member is NOT currently smoking or recently stopped smoking

If the above criteria are met, tasimelteon (Hetlioz, Hetlioz LQ) is approved for 6 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of treatment. Authorizations may be extended at one-year intervals based upon documentation from the prescriber indicating ALL of the following:

- The member's condition has improved as a result of treatment.
- For members 16 years of age and older, must submit documentation of a recent urine or serum nicotine screening confirming that member is NOT currently smoking or recently stopped smoking

- **Teduglutide (Gattex®)** [1877, 1878, 1879, 1880, 1881, 1882, 1883, 1884, 1885, 1886]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with a gastroenterologist or pediatric gastroenterologist
- Must be age 1 year or older
- Must have a diagnosis of short bowel syndrome defined as follows:
  - Adults:
    - Must have < 200 cm of residual functional small intestine
  - Pediatrics:
    - Must have < 75 cm of residual functional small intestine
- Must provide date of bowel resection
- Must be receiving parenteral or intravenous nutrition support (PN/IV) at least 3 times weekly
  - Must provide baseline PN/IV schedule (frequency and volume)
- Must not have an active intestinal obstruction
- Must not have an active gastrointestinal malignancy
- Must have undergone a colonoscopy prior to treatment (within 6 months), unless contraindicated
  - Pediatric patients are not required to have a colonoscopy/sigmoidoscopy if fecal occult blood test does not show blood in stool

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- Must have had recent (within 6 months) baseline laboratory testing of bilirubin, alkaline phosphatase, lipase, and amylase

If the above criteria are met, teduglutide (Gattex) is approved for 6 months.

Reauthorization Criteria:
All authorization renewals are reviewed on an annual basis to determine Medical Necessity for the continuation of treatment. Authorization may be extended at 1 year intervals based upon:

- Documentation from the prescriber that the member's condition has improved based upon the prescriber's assessment while on therapy, including documentation that member has weaned off or decreased PN/IV requirements
- Documentation from the prescriber that the member does not have an active intestinal obstruction or active malignancy
- Documentation from the prescriber that the patient is undergoing laboratory testing of bilirubin, alkaline phosphatase, lipase, and amylase at least every 6 months during treatment with teduglutide (Gattex)
- For pediatric members:
  - Documentation from the prescriber that the member has had the following during treatment with teduglutide (Gattex), unless contraindicated:
    - Annual fecal occult blood testing
    - A colonoscopy in each of the following instances:
      - After 1 year of treatment
      - At least every 5 years after the first year
      - If new or unexplained gastrointestinal bleeding occurs
- For adult members:
  - Documentation from the prescriber that the member has had a colonoscopywithin the recommended time frame during treatment with teduglutide (Gattex), unless contraindicated:
    - After 1 year of treatment
    - At least every 5 years after the first year

• Tegaserod (Zelnorm™) [1887, 1888]

Initial Authorization Criteria:

- Must be prescribed by a gastroenterologist
- Must be over age 18
- Must be an adult women less than 65 years of age with irritable bowel syndrome with constipation (IBS-C)
- Must NOT have any of the following:
  - A history of myocardial infarction, stroke, transient ischemic attack, or angina
  - A history of ischemic colitis or other forms of intestinal ischemia.

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        - Severe renal impairment (eGFR< 15 mL/min/1.73 m2) or end-stage renal disease
        - Moderate or severe hepatic impairment (Child-Pugh B or C)
        - A history of bowel obstruction, symptomatic gallbladder disease, suspected sphincter of Oddi dysfunction, or abdominal adhesions
    - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
        - Bulk laxative (e.g., psyllium)
        - Osmotic laxative (e.g., lactulose)
        - Hyperosmotic laxative (e.g., polyethylene glycol)
    - Must have an adequate trial (minimum of 3 months) of linaclotide (Linzess), plecanatide (Trulance), and lubiprostone (Amitiza) with an inadequate response or significant side effects/toxicity or a documented contraindication to these therapies.
    - Must have a behavioral health assessment prior to the use of tegerasod (Zelnorm) if the member has a history of depression and/or suicidal thoughts or behaviors, or if the member currently is undergoing treatment for depression and/or suicidal thoughts or behavior. These members must also be followed while on tegerasod (Zelnorm).

If the above criteria are met, tegerasod (Zelnorm) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for the continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved based upon the prescriber's assessment while on therapy or the member continues to benefit from therapy and is not aged 65 or older.

• Telotristat Ethyl (Xermelo™) [1889, 1890, 1891, 1892]

Initial Authorization Criteria:
- Must be prescribed by or in consultation with an oncologist, endocrinologist, or gastroenterologist
- Must be age 18 years or older
- Must have histopathologically documented, well-differentiated metastatic neuroendocrine tumor
- Must have a documented history of carcinoid syndrome
- Must be having an average of ≥ 4 bowel movements per day despite the use of somatostatin analog therapy (e.g., Somatuline® Depot injection, Sandostatin® LAR Depot injection) at a stable dose for at least 3 months
- Must be on concurrent therapy with a somatostatin analog therapy (e.g., Somatuline® Depot injection, Sandostatin® LAR Depot injection)

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If the above criteria are met, telotristat ethyl (Xermelo) is approved for 3 months.

Reauthorization Criteria:
All prior authorization renewals will be reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved based upon the prescriber's assessment while on therapy.

- **Tenapanor (Ibsrela®)** [1893, 1894, 1895, 1896]

Initial Authorization Criteria:
- Must be prescribed by a gastroenterologist
- Must be 18 years of age or older
- Must have a diagnosis of irritable bowel syndrome with constipation (IBS-C)
- Must not have known or suspected mechanical gastrointestinal obstruction
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to TWO of the following:
  - Bulk laxative (e.g., psyllium)
  - Osmotic laxative (e.g., polyethylene glycol, lactulose)
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ALL the following:
  - Linaclotide (Linzess)
  - Plecanatide (Trulance)
  - Lubiprostone (Amitiza)

If the above criteria are met, tenapanor (Ibsrela) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved while on therapy.

- **Tenofovir alafenamide (Vemlidy®)** [1897, 1898, 1899, 1900, 1901]

Initial Authorization Criteria:
- Must be prescribed by or in consultation with an infectious disease physician, a gastroenterologist, a hepatologist, or a transplant physician
- Must have a diagnosis for which the requested drug is FDA-approved or medically-accepted
- Must have an adequate trial of entecavir with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
  - Note: an exception to this trial can be made if the member has documented lamivudine or telbivudine resistance

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- Must have an adequate trial of tenofovir disoproxil fumarate with documented intolerance or contraindication
  - Note: an exception to this trial can be made if the member has documented bone disease (e.g., osteopenia/osteoporosis), renal dysfunction or is post solid organ transplant

If the above criteria are met, tenofovir alafenamide (Vemlidy) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved based upon the prescriber's assessment while on therapy or the member continues to benefit from therapy.

- **Osteoporosis Injectables** 1902, 1903, 1904, 1905, 1906, 1907, 1908, 1909, 1910, 1911, 1912, 1913, 1914, 1915
  - Applies to: Abaloparatide (Tymlos™), Teriparatide 560 mcg/2.24 mL (Forteo®)
  - Applies to: Teriparatide 560 mcg/2.24 mL (Bonsity®), Teriparatide 560 mcg/2.24 mL (NDCs starting with 47781, Manufactured by Alvogen) (Value Choice and Open Choice formularies only)

Criteria:
- **Requests for teriparatide 560 mcg/2.24 mL (Bonsity) and Teriparatide 560 mcg/2.24 mL (NDCs starting with 47781, Manufactured by Alvogen) [Applies to Value Choice and Open Choice Formularies only]**
  - Must meet the indication-specific criteria listed below
  - Must have an adequate trial of generic teriparatide 560 mcg/2.24 mL (Forteo) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
  - Must provide clinical rationale for use of the requested product instead of the prerequisite medication trial. Clinical rationale must include why the prerequisite medication trial would not be expected to produce the same clinical results as the requested product.

The following criteria must be met for all indications:
- Must not have a history of any one of the following conditions:
  - Paget's disease of the bone or unexplained elevations of alkaline phosphatase
  - Open epiphyses
  - Prior external beam or implant radiation therapy involving the skeleton

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1. **For postmenopausal osteoporosis OR for the treatment to increase bone mass in men with primary hypogonadal osteoporosis at high risk for fracture:**
   - Must meet one of the following scenarios:
     - Bone mineral density (BMD) T-score of ≤ -2.5 at conventional sites including total hip, femoral neck, lumbar spine (post-anterior, not lateral) or radius
       - Must have a trial and failure of one generic bisphosphonate (oral or intravenous) with an inadequate response or significant side effects/toxicity or have a contraindication to the use of bisphosphonate therapy
       - NOTE: an exception to the requirement for a trial of one generic bisphosphonate (oral or intravenous) can be made if the patient has a very high risk of fracture as defined by ONE of the following:
         - Very low T-score (e.g., < -3.0)
         - High risk for falls or history of injurious falls
         - Very high fracture probability by FRAX (e.g., major osteoporosis fracture >30%, hip fracture >4.5%) or other validated fracture risk algorithm (e.g., Garvan)
     - History of fragility fracture (other than skull, facial bone, finger, and toes) as an adult
     - BMD T-score of -1.0 to > -2.5 at the femoral neck or lumbar spine and a 10-year probability of a hip fracture > 3% or a 10-year probability of a major osteoporosis-related fracture ≥ 20% based on the U.S.-adapted World Health Organization (WHO) algorithm (FRAX)
       - Must have a trial and failure of one generic bisphosphonate (oral or intravenous) with an inadequate response or significant side effects/toxicity or have a contraindication to the use of bisphosphonate therapy
         - For members who have failed a generic oral bisphosphonate due to BMD loss or intolerance related to the oral formulation, a trial of a generic intravenous bisphosphonate is required
       - NOTE: an exception to the requirement for a trial of a generic bisphosphonate (oral or intravenous) can be made if the patient has a very high risk of fracture as defined by ONE of the following:
         - High risk for falls or history of injurious falls
         - Very high fracture probability by FRAX (e.g., major osteoporosis fracture >30%, hip fracture >4.5%) or other validated fracture risk algorithm (e.g., Garvan)
   - For abaloparatide (Tymlos):

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        - Must have a trial of teriparatide 560 mcg/2.24 mL (Forteo) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy in addition to the above mentioned criteria

2. **For the treatment of osteoporosis associated with sustained systemic glucocorticoid therapy at high risk for fracture [*Criteria does not apply to (abaloparatide) Tymlos]:***
    - Must have a trial and failure of one oral generic and one intravenous generic bisphosphonate with an inadequate response or significant side effects/toxicity or have a contraindication to the use of bisphosphonate therapy

If the above criteria are met, the requested product is approved for up to 2 years. No reauthorization is granted after this period of coverage for abaloparatide (Tymlos).

Reauthorization Criteria:
All prior authorization renewals for teriparatide 560 mcg/2.24 mL (Bonsity), teriparatide (Forteo) or Teriparatide 560 mcg/2.24 mL are reviewed on a case-by-case basis and require chart documentation to determine the Medical Necessity for continuation of therapy including the previous response to treatment and clinical rationale for continuing treatment beyond 2 years including evidence that the member remains at or has returned to having a high risk for fracture. Authorizations may be extended at 6-month intervals.

- **Tesamorelin (Egrifta SV®, Egrifta WR®)** [1916, 1917, 1918, 1919, 1920, 1921]

Initial Authorization Criteria:
- Must be prescribed by or in consultation with an endocrinologist or provider who specializes in the treatment of HIV/AIDS
- Must be age 18 years or older
- Must have a diagnosis of lipodystrophy with excess abdominal fat:
    - Natal male members must have a waist circumference of at least 95cm (37.4in) and a waist to hip ratio of at least 0.94
    - Natal female members must have a waist circumference of at least 94cm (37in) and a waist to hip ratio of at least 0.88
    - Baseline waist circumference and waist to hip ratio must be provided
- Must have an underlying diagnosis of HIV infection
- Must be stable on an antiretroviral regimen for at least 8 weeks prior to beginning tesamorelin (Egrifta SV or Egrifta WR)
- Must not have ANY of the following:
    - Disruption of the hypothalamic-pituitary axis due to hypophysectomy, hypopituitarism, pituitary tumor/surgery, head irradiation, or head trauma

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- Active malignancy
- Current pregnancy

If the above criteria are met, the requested product is approved for 6 months.

Reauthorization Criteria:
All prior authorizations renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation showing a decrease from baseline in visceral adipose tissue as measured by waist circumference and waist to hip ratio.

- **Testosterone** 1922, 1923, 1924, 1925, 1926, 1927, 1928, 1929, 1930, 1931, 1932, 1933, 1934, 1935, 1936, 1937, 1938, 1939, 1940, 1941, 1942, 1943, 1944, 1945, 1946
  - For testosterone pellet (Testopel®) requests, please refer to RX.PA.406 Commercial, Exchange Medical Prior Authorization Policies

Initial Authorization Criteria:

1. **For conditions associated with a deficiency or absence of endogenous testosterone. Applies to all formulary testosterone products (except methyltestosterone [Android®, Methitest™, Testred®] for female members):**
   - Must have one of the following diagnoses:
     - Primary hypogonadism (congenital or acquired) – testicular failure due to cryptorchidism, bilateral torsions, orchitis, vanishing testis syndrome, or orchidectomy
     - Hypogonadotropic hypogonadism (congenital or acquired) – idiopathic gonadotropin or luteinizing hormone-releasing hormone (LHRH) deficiency, or pituitary-hypothalamic injury from tumors, trauma or radiation.
   - Must have repeated morning serum total testosterone level <300ng/dL

   Applies to Open Choice and Value Choice only:
   For brand testosterone products (except methyltestosterone [Methitest] for female members and testosterone pellet [Testopel]):
   - Must meet above criteria for formulary testosterone products
   - Must have documentation of trial and failure or intolerance to two different generic testosterone products

   If the above criteria are met, the requested formulary testosterone product is approved.

2. **For Gender Affirming Care Services:**
   - For Commercial members, must have coverage for gender affirming care services. Members without coverage can be identified through presence of gender affirming care services exclusion rider on benefit.

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- Must be an adolescent or older
- For adolescents:
  - Must be prescribed by or in consultation with a pediatric endocrinologist or a clinician trained in gender affirming health care
  - Must have a persistent diagnosis of gender dysphoria/gender incongruence
  - Must have addressed and reasonably controlled any coexisting psychiatric and medical conditions or social problems that could interfere with treatment
  - Must have an attestation from the prescriber that the member meets ALL of the following:
    - Has been informed of the effects and side effects of treatment
    - Has the capacity to make a fully informed decision and consents to treatment
    - Parents, other caretakers or legal guardians' consent to treatment and are involved in supporting the member throughout the treatment process
      - Exceptions to this can be made on a case-by-case basis and in conjunction with the prescriber when parental consent cannot be obtained
- For adults:
  - Must be prescribed by or in consultation with an endocrinologist or a clinician trained in gender affirming health care
  - Must have a persistent, well-documented diagnosis of gender dysphoria
  - If the member has significant medical or mental health concerns present, they must be reasonably well-controlled
  - Attestation from the prescriber that the member has the capacity to make a fully informed decision and has consented to treatment

For gender affirming care services: testosterone is approved for 1 year.

3. **For methyltestosterone (Methitest) for female members (all formularies):**
   - Must be prescribed by an oncologist
   - Must be used for metastatic, inoperable breast cancer in women 1 to 5 years postmenopausal

If the above criteria are met, methyltestosterone (Methitest) is approved for 6 months.

Reauthorization Criteria:

- For gender affirming care services:
  - Must have coverage for gender affirming care services. Members without coverage can be identified through presence of gender affirming care services exclusion rider on benefit.

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        - For FEHB and PSHB members, please refer to policy RX.011 Request for Non-Covered Medications
        - All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member continues to benefit from therapy.

    - For fluoxymesterone [Androxy] and methyltestosterone [Android, Methitest, Testred] for females:
        - All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of treatment. Authorizations may be extended at one-year intervals based upon documentation from the prescriber that the member's condition has improved as a result of treatment.

- **Tetrabenazine (Xenazine®)** 1947, 1948, 1949, 1950, 1951, 1952, 1953, 1954, 1955, 1956, 1957

Initial Authorization Criteria:

- Must be prescribed by a neurologist who specializes in the treatment of movement disorders
- Must be age 18 years or older
- Must have chart documentation of adiagnosis of chorea associated with Huntington's disease confirmed by a neurologist with expertise in Huntington's disease and ONE of the following:
    - Confirmation of diagnosis by Huntington Disease Mutation Analysis
        - Documentation of laboratory result indicating an expanded CAG repeat (≥ 36) in the huntington gene (HTT) (also known as HD gene) must be submitted
    - A positive family history of Huntington's disease, with an autosomal dominant inheritance pattern
- Must have clinical signs of Huntington's disease. Chart documentation of a clinical work-up must be submitted indicating that the member has one or more of the following clinical signs:
    - Motor (e.g., finger tapping, rigidity)
    - Oculomotor
    - Bulbar (e.g., dysarthria, dysphagia)
    - Affective (e.g., depression)
    - Cognitive
- Must not be actively suicidal or have uncontrolled depression
- Must have an evaluation by a neurologist or behavioral health provider prior to the use of tetrabenazine (Xenazine), if the member has a history of depression and/or suicidal thoughts or behaviors. These members must also

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be followed concurrently with a behavioral health provider while on tetrabenazine (Xenazine). Chart documentation must be provided.
- Must currently not be using a monoamine oxidase inhibitor (MAOI), reserpine, deutetrabenazine (Austedo), valbenazine (Ingrezza) in combination with tetrabenazine (Xenazine)
- Must not have hepatic impairment

For tetrabenzine (Xenazine) requests for doses above 50mg/day:
- Must meet one of the following criteria:
  - Must have an adequate trial of 50 mg per day dosing with an inadequate response. Chart documentation of trial and inadequate response is required.
  - Must be a Cytochrome P450 (CYP) 2D6 intermediate or extensive metabolizer. Documentation of CYP 2D6 genotyping results are required.
- Documentation of slow titration of tetrabenazine dose with close monitoring of side effects
- The maximum dose that can be approved is 100mg daily

If the above criteria are met, tetrabenazine (Xenazine) is approved for 3 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon the following:
- Documentation from the provider that the member's condition has improved based upon the prescriber's assessment while on therapy or the member continues to benefit from therapy.
- For members with a diagnosis of depression and/or suicidal thoughts or behaviors or for members with a current treatment for depression and/or suicidal thoughts or behaviors, chart documentation must be provided to show current evaluation by a neurologist or behavioral health provider.
- For tetrabenzine (Xenazine) requests for doses above 50mg/day, documentation must be provided showing inadequate efficacy of lower doses and slow titration of tetrabenazine (Xenazine) dose with close monitoring of side effects

- **Tezepelumab (Tezspire™) pre-filled pen** [1958, 1959, 1960, 1961, 1962]
  - Requests for tezepelumab (Tezspire) pre-filled syringe please refer to policy RX.PA.406 Commercial, Exchange Medical Prior Authorization Policies

Initial Authorization Criteria:
1. **For severe asthma**

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- Must be prescribed by or in consultation with an allergist, an immunologist, or a pulmonologist
- Must be age 12 years or older
- Must have a diagnosis of severe asthma
- Must have received at least 3 consecutive months of therapy with ONE of the following combinations for the maintenance treatment of asthma:
  - A medium-high dose inhaled corticosteroid AND one additional asthma controller medication (e.g., inhaled long-acting beta-agonist, inhaled long-acting muscarinic antagonist)
  - A maximally tolerated inhaled corticosteroid AND long-acting beta-agonist combination product
- Must have history of one asthma exacerbation in the previous year despite adherence to optimized medication therapy regimen. Asthma exacerbation is defined by ONE of the following:
  - Hospitalization for asthma
  - Emergency room visit for asthma
  - Requirement for 2 or more courses of treatment with systemic (e.g., oral, parenteral) corticosteroids
- Must be used as an add-on maintenance treatment to asthma controller therapy
- Must not be receiving combination therapy with another biologic agent (e.g., Xolair, Cinqair, Fasenra, Nucala, Dupixent) for this diagnosis

2. For Chronic Rhinosinusitis with Nasal Polyps (CRSwNP)

- Must be prescribed by or in consultation with an allergist, immunologist, or an otolaryngologist (ear, nose, and throat [ENT])
- Must be age 12 years or older
- Must have a diagnosis of chronic rhinosinusitis with nasal polyps. Chart documentation confirming the diagnosis is required as outlined below:
  - Must have documentation of one of the following:
    - A baseline endoscopic bilateral nasal polyp score (NPS) of at least 5 out of a maximum score of 8 (with a minimum score of 2 in each nasal cavity)
    - Evidence of polyposis on sinus CT
  - Must have two or more of the following symptoms present for at least 3 months:
    - Nasal obstruction (congestion)
    - Mucopurulent discharge (rhinorrhea or post-nasal drip)
    - Facial pain or pressure
    - Reduction or loss of smell
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ONE of the following:
  - A generic intranasal corticosteroid

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        - Fluticasone propionate (Xhance)
        - NOTE: If the member has a history of sinus surgery, medication trial must be post-surgery.
    - Must be used in combination with an intranasal corticosteroid, unless intolerant or contraindicated
    - Must not be receiving combination therapy with another biologic agent (e.g., Dupixent, Xolair, Fasenra, Cinqair, Nucala)

If the above criteria are met, tezepelumab (Tezspire) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon the following:

1. **For Severe Asthma**
    - Must have documentation from the provider that the member's condition has improved while on therapy (e.g., reduction in exacerbations, hospitalizations, emergency department visits, or requirement for oral corticosteroid therapy)

2. **For CRSwNP**
    - Must have documentation from the provider that the member's condition has improved while on therapy (e.g., reduction in nasal polyp score (NPS), reduction in exacerbations requiring systemic corticosteroids and/or antibiotics)

- **Tiopronin (Thiola®, Thiola® EC, Venxxiva™ DR)** [1963, 1964, 1965, 1966, 1967, 1968, 1969, 1970, 1971, 1972]

Initial Authorization Criteria:
- Must be prescribed by or in consultation with a provider who specializes in the treatment of inherited metabolic disorders or a nephrologist
- Must have a diagnosis of cystinuria confirmed by lab values indicating urinary cystine concentration greater than 250 mg/L or urinary cystine excretion greater than 500 mg per day despite conservative cystinuria management (e.g., increased fluid intake, restricted protein and sodium diet, potassium citrate to alkalize urine.) Chart documentaton is required.
- Must be using tiopronin (Thiola, Thiola EC, Venxxiva DR) in combination with conservative cystinuria management

If the above criteria are met, the requested product is approved for 6 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for the continuation of treatment. Authorizations may be extended

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at one-year intervals based upon documentation from the provider showing the following:
- Clinical improvement demonstrated by ONE of the following:
  - Reduction in urine cystine levels
  - Positive cystine capacity
  - No cystine stone formation
- Must be using tiopronin (Thiola, Thiola EC, Venxxiva DR) in combination with conservative cystinuria management

- **Tirzepatide (Zepbound®)** 1973, 1974, 1975, 1976, 1977, 1978, 1979, 1980, 1981, 1982, 1983, 1984, 1985, 1986, 1987
  - Requests for weight loss without obstructive sleep apnea or for members with weight loss coverage, please refer to 'Weight Loss' criteria below

Initial Authorization Criteria:
- Must be prescribed by or in consultation with a sleep specialist
- Must be age 18 years or older
- Must have chart documentation showing a diagnosis of obesity and moderate to severe obstructive sleep apnea (OSA) confirmed by BOTH of the following:
  - Must have an initial BMI of 30 kg/m^2 or greater
  - Must have polysomnographic evaluation or home sleep apnea testing (e.g., PSG, HSAT, sleep study) that confirms the diagnosis and severity (a copy of the sleep study must be provided showing at least 15 apnea-hypopnea index [AHI] or respiratory event index [REI] events per hour)
- Must have chart documentation of at least ONE of the following clinical symptoms:
  - Excessive daytime sleepiness (e.g., Epworth Sleepiness Scale [ESS] ≥10)
  - Reduced sleep-related QOL (e.g., snoring, nocturnal choking, insomnia, disruption of partner's sleep, morning headaches, nocturia, etc.)
- Must have chart documentation that the member is using a continuous positive airway pressure (CPAP) machine on a regular basis, defined by at least 4 hours a night on at least 70% of the nights for at least 3 months and one of the above symptoms still persist
  - For members who are intolerant to CPAP, must provide chart documentation that troubleshooting strategies were used during the first 90 days of treatment to address barriers (e.g., meeting with clinician or DME provider about mask-related issues, trying at least 2 different masks, increasing humidity settings, addressing claustrophobia concerns)

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        - For members who still experience excessive daytime sleepiness despite compliance with CPAP:
            - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ONE of the following:
                - Armodafinil (Nuvigil®)
                - Modafinil (Provigil®)
    - Must have chart documentation showing ALL of the following:
        - The member has had a trial of lifestyle modification and dietary restriction for at least 3 months and has failed to achieve enough weight loss to improve OSA symptoms
        - The member has an outlined reduced-calorie diet plan and increased physical activity plan
        - The member has been educated on behavioral modifications to prevent worsening of OSA symptoms (e.g., avoiding alcohol or sedating medications before bed or sleeping in a non-supine position)
        - Baseline body weight including date
    - Must not have ANY of the following:
        - Central or mixed sleep apnea
        - Obesity hypoventilation syndrome or daytime hypercapnia
        - A planned procedure for sleep apnea or obesity
        - Significant craniofacial abnormalities that affect breathing
        - Type 1 or 2 diabetes mellitus
            - For members with type 2 diabetes:
                - Must provide clinical rationale for use of tirzepatide (Zepbound) instead of tirzepatide (Mounjaro) that includes why tirzepatide (Mounjaro) is not producing sufficient clinical benefits and why tirzepatide (Zepbound) would be expected to produce better clinical benefit given that they are the same chemical entity
    - Must not use in combination with a GLP-1 receptor agonist (e.g., semaglutide [Ozempic, Wegovy], liraglutide [Victoza, Saxenda], etc.) or a GIP/GLP-1 receptor agonist (e.g., tirzepatide [Mounjaro])

If the above criteria are met, tirzepatide (Zepbound) is approved for 6 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon chart documentation showing the following:

- The member's condition has improved while on therapy as evidenced by symptom improvement (e.g., less daytime sleepiness, fewer sleep arousals, partner-reported snoring episodes or pauses in breathing)

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- Must continue ALL of the following:
  - Behavioral modifications to prevent worsening of OSA symptoms
  - A reduced-calorie diet and increased physical activity plan
  - Continuous positive airway pressure (CPAP) on a regular basis, defined by at least 4 hours a night on at least 70% of the nights
    - For members who are intolerant to CPAP, must provide chart documentation that troubleshooting strategies were used during the first 90 days of treatment to address barriers
- Must have a recent body weight that supports ≥5% body weight loss from baseline
- Must be adherent (PDC > 80%) to tirzepatide (Zepbound) based on claims history
- Must not use in combination with a GLP-1 receptor agonist (e.g., semaglutide [Ozempic, Wegovy], liraglutide [Victoza, Saxenda], etc.) or a GIP/GLP-1 receptor agonist (e.g., tirzepatide [Mounjaro])

• Tolvaptan (Jynarque™) 1988, 1989, 1990, 1991, 1992, 1993

Initial Authorization Criteria:

- Must be prescribed by a nephrologist who is registered with the Jynarque REMS program
- Must be 18 years of age or older
- Must have a diagnosis of ADPKD as evidenced by confirmatory imaging (ultrasound, computed tomography [CT], or magnetic resonance imaging [MRI])
- Must be at risk of rapidly progressing ADPKD as determined by the prescribing provider (e.g., reduced or declining renal function, high or increasing total kidney volume, Mayo imaging classification of 1C, 1D, or 1E)
- Must not have stage 5 chronic kidney disease (CKD) [estimated glomerular filtration rate (eGFR) < 15 mL/min/1.73m^2 or receiving dialysis]

If the above criteria are met, tolvaptan (Jynarque) is approved for 1 year.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one-year intervals based upon documentation from the prescriber that the member's disease has improved or stabilized based upon the prescriber's assessment while on therapy. Additionally, the member must not have stage 5 CKD (estimated GFR < 15 mL/min/1.73m^2 or receiving dialysis).

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- **Tolvaptan (Samsca®)** 1994, 1995, 1996, 1997, 1998, 1999, 2000, 2001, 2002, 2003, 2004, 2005, 2006, 2007, 2008 2009, 2010

Initial Authorization Criteria:

- Must be prescribed by an endocrinologist or a nephrologist
- Must have a diagnosis of hyponatremia due to heart failure or syndrome of inappropriate antidiuresis (SIAD) and the following prior to beginning tolvaptan (Samsca):
  - Serum sodium between 120 and 134 mEq/L that resisted correction with 72 hours of both of the following interventions:
    - Consideration of discontinuation of agents known to cause SIAD when clinically feasible [(e.g., chlorpropamide, selective serotonin reuptake inhibitors (SSRIs), tricyclic antidepressants, clofibrate, carbamazepine, vincristine, nicotine, narcotics, phenothiazines, butyrophenones, ifosfamide, cyclophosphamide, Non-steroidal anti-inflammatory drugs (NSAIDs), MDMA (3,4-methylenedioxy-N-methylamphetamine), desmopressin, oxytocin, vasopressin, carbamazepine, oxcarbazepine]
    - Fluid restriction (< 1000 mL/day)
- For members where SIAD is the underlying cause of hyponatremia, member must have a confirmed diagnosis of SIAD demonstrated by all of the following:
  - Decreased plasma osmolality (< 275 mosm/kg)
  - Increased urinary osmolality (> 100 mosm/kg during hypotonicity)
  - Urinary sodium > 20 mmol/L with normal dietary salt intake
  - Clinical euvolemia
  - Normal thyroid and adrenal function
  - No recent use of diuretics within 24 hours of laboratory testing
- Must be initiated and titrated in a hospital setting with close serum sodium monitoring
- Must have eGFR > 10 mL/min/1.73m^2
- Must not be anuric
- Must be able to sense and respond appropriately to thirst
- Must not be on concomitant therapy with a strong CYP3A inhibitor
- Must not have hypovolemic hyponatremia
- Must not have underlying liver disease, including cirrhosis

If the above criteria are met, tolvaptan (Samsca) is approved for 1 month.

Reauthorization Criteria:

Due to risk of liver injury, tolvaptan (Samsca) should not be administered for more than 30 days.

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- **Topical Facial Dermatoses and Psoriasis Medications** [2011, 2012, 2013, 2014, 2015, 2016, 2017, 2018, 2019,2020, 2021, 2022,]

Criteria:

- For all Commercial and Exchange members the following applies:
  - For members ≤ 35 years of age:
    - Formulary acne medications are covered unrestricted except for the following medications: azelaic acid (Azelex®, Finacea®), dapsone (Aczone®) and tazarotene (Tazorac®, Arazlo™)
  - For members > 35 years of age:
    - Formulary acne medications, except azelaic acid (Azelex, Finacea), dapsone (Aczone) and tazarotene (Tazorac, Arazlo), are covered with a documented diagnosis of at least one of the following:
      - Acne (e.g., acne vulgaris, cystic acne, nodular acne, acne comedones, papular or pustular acne)
      - Folliculitis (e.g., pseudofolliculitis barbae)
      - Ichthyosis (e.g., congenital, lamellar, vulgaris, X-linked)
      - Keratosis follicularis (Darier's Disease, Darier-White disease)
      - Molluscum contagiosum
      - Precancerous or cancerous lesions (e.g., actinic keratosis, basal cell carcinoma, malignant neoplasm, skin cancer)
      - Psoriasis
      - Rosacea
      - Warts (e.g., common warts [verruca vulgaris], plantar warts [verruca plantaris], flat warts [verruca plana], facial warts)

- For Commercial (**Your Choice, Advantage Choice, Value Choice and Open Choice**) members the following criteria apply to **dapsone (Aczone) 5% gel and 7.5% gel pump)**
  - For a diagnosis of acne:
    - Must have an adequate trial and failure of ALL of the following with an inadequate response or significant side effects/toxicity or must have a contraindication to these therapies:
      - Benzoyl peroxide
      - Topical erythromycin or clindamycin
      - Topical tretinoin or adapalene

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        - Must be using as combination therapy with at least one other topical agent for the treatment of acne

- For Commercial (**Your Choice, Advantage Choice, Value Choice and Open Choice**) members the following criteria apply to **azelaic acid (Azelex, Finacea) and tazarotene (Tazorac):**
    - For a diagnosis of acne:
        - Must have an adequate trial and failure of ALL of the following with an inadequate response or significant side effects/toxicity or must have a contraindication to these therapies:
            - Benzoyl peroxide
            - Topical erythromycin or clindamycin
            - Topical tretinoin or adapalene
    - For a diagnosis of rosacea:
        - Must have an adequate trial of at least two generic medications that are used for the treatment of rosacea (e.g. metronidazole, sodium sulfacetamide/sulfur, tretinoin, adapalene, doxycycline)
    - For a diagnosis of psoriasis (applies to tazarotene (Tazorac) only:
        - Must have failed a moderate to high potency topical corticosteroid or have a contraindication/intolerance to topical corticosteroid therapy, such as psoriasis on the face

- For **Value Choice and Open Choice** members the following criteria apply to **dapsone (Aczone) 7.5% gel, tazarotene (Arazlo), minocycline foam (Amzeeq), and clascoterone cream (Winlevi):**
    - For a diagnosis of acne:
        - Must have an adequate trial and failure of ALL of the following with an inadequate response or significant side effects/toxicity or must have a contraindication to these therapies:
            - Benzoyl peroxide
            - Topical erythromycin or clindamycin
            - Topical tretinoin or adapalene

If the above criteria are met, the requested medication is approved.

Diagnoses Not Covered:

- Acne medications are not approved for cosmetic conditions. Examples of cosmetic conditions include:
    - Actinic damage
    - Dermatoheliosis
    - Heliodermatosis
    - Lentigines/freckles (hyperpigmented macules, liver spots)

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- Hypo- or Hyperpigmentation
- Melasma/chloasma
- Milia
- Skin laxity
- Skin roughness
- Solar elastosis
- Sun damage
- Wrinkles

- **Transmucosal Fentanyl Citrate** 2023, 2024, 2025, 2026, 2027, 2028, 2029, 2030, 2031

Initial Authorization Criteria:

1. **For generic fentanyl citrate buccal lozenge:**
   - Must meet criteria for Opioid Analgesics, Short-acting
   - Must be prescribed by an oncologist or pain specialist
   - Must be age 16 years or older
   - Must be using a long-acting opioid analgesic and experiencing breakthrough cancer pain
     - Transmucosal fentanyl citrate is not approved for the treatment of acute or postoperative pain
   - Must be opioid tolerant as defined by requiring around-the-clock medication for one week or longer containing at least one of the following:
     - 60mg/day of oral morphine
     - 25mcg/hour of transdermal fentanyl
     - 30mg/day of oral oxycodone
     - 8mg/day of oral hydromorphone
     - 25mg/day of oral oxymorphone
     - 60mg/day of oral hydrocodone

If the above criteria are met, generic fentanyl citrate buccal lozenge is approved for 1 year.

2. **For single source fentanyl citrate effervescent tablet, fentanyl buccal tablet (Fentora®):**
   - Must meet criteria for Opioid Analgesics, Short-acting
   - Must meet all criteria above for generic fentanyl citrate buccal lozenge
   - Must have an adequate trial of generic fentanyl citrate buccal lozenge with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
   - Must provide clinical rationale for use of the requested drug instead of generic fentanyl citrate buccal lozenge. Clinical rationale must include why generic fentanyl citrate buccal lozenge would not be expected to produce the same

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clinical results as the requested drug given that they are the same chemical entity

If the above criteria are met, the requested product approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one-year intervals based upon chart documentation from the provider that the member's condition has improved orstabilized while on therapy.

- **Treprostinil extended-release tablet (Orenitram®) and treprostinil injection (Remodulin®)** [2032, 2033, 2034, 2035, 2036]

Initial Authorization Criteria:
- Must be prescribed by a cardiologist or pulmonologist who specializes in pulmonary hypertension
- Must have a diagnosis of PAH (WHO Group 1) confirmed by right heart catheterization (RHC); must provide a copy of the RHC showing ALL of the following:
  - Mean pulmonary arterial pressure (mPAP) ≥25 mmHg
  - Pulmonary capillary wedge pressure (PCWP), left atrial pressure, or left ventricular end-diastolic pressure (LVEDP) ≤15 mmHg
  - Pulmonary vascular resistance >3 Wood units
- For treprostinil (Orenitram):
  - Must have WHO functional class II or III symptoms
- For treprostinil (Remodulin):
  - Must have WHO functional class II to IV symptoms

If the above criteria are met, the requested product is approved for 3 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or the member continues to benefit from therapy.

- **Treprostinil inhalation (Tyvaso® inhalation solution, Tyvaso DPI® inhalation powder, Yutrepia® inhalation powder®)** [2037, 2038, 2039, 2040, 2041]

Initial Authorization Criteria:

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- **Requests for treprostinil (Yutrepia®) [Applies to Value Choice and Open Choice Formularies only]**
  - Must meet the indication-specific criteria listed below
  - Must have an adequate trial of treprostinil inhalation powder (Tyvaso DPI) with a significant side effect/toxicity Must provide clinical rationale for use of treprostinil inhalation powder (Yutrepia) instead of treprostinil nebulized inhalation solution or treprostinil inhalation powder (Tyvaso DPI). Clinical rationale must include why treprostinil nebulized inhalation solution or treprostinil powder inhalation (Tyvaso DPI) would not be expected to produce the same clinical results as treprostinil inhalation powder (Yutrepia) given that they are the same chemical entity
- Must be prescribed by a cardiologist or pulmonologist who specializes in pulmonary hypertension
- Must have a diagnosis of ONE of the following:
  - PAH (WHO Group 1) with WHO functional class III symptoms
  - PH-ILD (WHO Group 3)
- Must confirm diagnosis by right heart catheterization (RHC); must provide a copy of the RHC showing ALL of the following:
  - Mean pulmonary arterial pressure (mPAP) ≥25 mmHg
  - Pulmonary capillary wedge pressure (PCWP), left atrial pressure, or left ventricular end-diastolic pressure (LVEDP) ≤15 mmHg
  - Pulmonary vascular resistance >3 Wood units

If the above criteria are met, the requested product is approved for 3 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or the member continues to benefit from therapy.

• Trientine HCl (Syprine®, Clovique™), Trientine tetrahydrochloride (Cuvrior®), and Zinc Acetate (Galzin®) 2042, 2043, 2044, 2045, 2046, 2047, 2048, 2049, 2050, 2051, 2052, 2053, 2054, 2055

Initial Authorization Criteria:
- Must be prescribed by or in consultation with a provider who specializes in the treatment of inherited metabolic disorders or a hepatologist
- Must have a diagnosis of Wilson's Disease confirmed by chart documentation of ONE of the following:

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        - Genetic test results indicating biallelic pathogenic mutation in *ATP7B* gene
        - Hepatic parenchymal copper content of ≥ 250 µg/g dry weight (equivalent to ≥ 4 µmol/g)
        - Presence of Kayser-Fleischer ring(s) in cornea(s) with ONE of the following:
        -
            - 24-hour urinary copper excretion > 40 mcg/24 hours (0.64 micromol/24 hours) in addition to serum ceruloplasmin level < 20 mg/dL (200 mg/L)
            - 24-hour urinary excretion of copper > 100 µg (1.6 µmol/24 hours)
    - For trientine tetrahydrochloride (Cuvrior):
        - Must have an adequate trial with an inadequate response or significant side effect/toxicity to BOTH of the following:
            - Penicillamine capsules (Cuprimine®) or penicillamine tablets (Depen®)
            - Trientine hydrochloride (Syprine, Clovique)
                - Must provide clinical rationale for use of trientine tetrahydrochloride (Cuvrior) instead of trientine hydrochloride (Syprine, Clovique). Clinical rationale must include why trientine hydrochloride (Syprine, Clovique) would not be expected to produce the same clinical results as trietine tetrahydrochloride (Cuviror)
    - For trientine hydrochloride 500mg capsule:
        - Must provide clinical rationale for use of 500mg capsule instead of 250mg capsule. Clinical rationale must include why 250mg capsule would not be expected to produce the same clinical results as 500mg capsule given it contains the same chemical entity.

If the above criteria are met, the requested product is approved for 6 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine Medical Necessity for continuation of treatment. Authorization may be extended at one-year intervals based upon chart documentation from the provider that the member's condition has improved while on therapy.

• Triheptanoin (Dojolvi™) 2056, 2057, 2058, 2059, 2060, 2061

Initial Authorization Criteria:
- Must be prescribed by or in consultation with a provider who specializes in the treatment of inherited metabolic disorders

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- Must have a diagnosis of a long-chain fatty acid oxidation disorder (LC-FAOD) confirmed by BOTH of the following:
    - A newborn screen or blood acylcarnitine profile consistent with a diagnosis of LC-FAOD (chart documentation of laboratory result must be provided)
    - Must have ONE of the following:
        - Genetic test demonstrating one or more known pathogenic mutations in the *CPT1A*, *SLC25A20*, *CPT2*, *ACADVL*, *HADHA*, or *HADHB* genes (a copy of the genetic test must be provided)
        - Decreased enzyme activity per fibroblast or lymphocyte testing (chart documentation of laboratory result must be provided)
- Must provide documentation from the prescriber that member is at risk for severe disease (e.g., metabolic episodes which may include cardiomyopathy, hypoglycemia, rhabdomyolysis, genetic mutations, serum acylcarnitine profile)
- Must provide chart documentation of a dietary assessment completed by a registered dietician that includes metabolic requirements (e.g., daily caloric intake [DCI]) in order to determine an appropriate dose
- Must be prescribed a dose and duration of therapy that is consistent with FDA-approved package labeling, nationally-recognized compendia, or peer-reviewed medical literature

If the above criteria are met, triheptanoin (Dojolvi) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or stabilized while on therapy.

• Trofinetide (Daybue®, Daybue STIX®) 2062, 2063, 2064, 2065, 2066, 2067

Initial Authorization Criteria:
- Must be prescribed by or in consultation with a pediatric neurologist, a neurologist who specializes in neurodevelopmental disabilities, or a medical geneticist
- Must be age 2 years or older
- Must have chart documentation of a diagnosis of Rett syndrome meeting ALL of the following diagnostic criteria:
    - A period of regression followed by recovery or stabilization
    - Partial or complete loss of acquired purposeful hand skills
    - Partial or complete loss of acquired spoken language

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        - Gait abnormalities
        - Stereotypic hand movements (e.g., hand wringing/squeezing, clapping/tapping, mouthing and washing/rubbing automatisms
        - Absence of brain injury secondary to trauma (peri- or postnatally), neurometabolic disease, or severe infection that causes neurological problems
        - Absence of grossly abnormal psychomotor development in first 6 months of life
    - Must have genetic testing showing a pathogenic mutation of the *MECP2* gene (a copy of the genetic testing report must be provided).

If the above criteria are met, trofinetide (Daybue, Daybue STIX) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation showing clinically meaningful improvement in at least one sign or symptom of Rett syndrome from baseline.

- **Ubrogepant (Ubrelvy™)** 2068, 2069, 2070, 2071, 2072, 2073, 2074, 2075, 2076, 2077, 2078, 2079, 2080, 2081, 2082

Criteria:
- Must be 18 years or older
- Must be used for the acute treatment of migraine headaches
    - Must have at least 4 migraines per month
- Must not be taking other acute, migraine-specific therapy (e.g., triptans, ergots, lasmiditan [Reyvow])
- Must have an adequate trial of a generic triptan (e.g., naratriptan, rizatriptan, sumatriptan) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy

If the above criteria are met, ubrogepant (Ubrelvy) is approved.

- **Uridine triacetate (Xuriden™)** 2083, 2084, 2085

Initial Authorization Criteria:
- Must be prescribed by or in consultation with a physician who specializes in the treatment of inherited metabolic disorders
- Must have a diagnosis of hereditary orotic aciduria. Chart documentation of a clinical work up to rule out other diagnoses and clinical rationale for the diagnosis and exclusion of other diagnoses must be provided.

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- Must have an elevated urine orotic acid level based on the reference range provided by the laboratory. Chart documentation of the lab result is required.
- Must have currently symptomatic disease defined as the presence of at least one of the following with initial symptoms starting prior to age 18:
  - Hematologic abnormalities (e.g. megaloblastic anemia, neutropenia, leukopenia)
  - Failure to thrive
  - Developmental delay
- Must provide chart documentation showing specific examples of how quality of life is impacted by the current disease symptoms
- Must have chart documentation of an adequate trial of the dietary supplement uridine with either an inadequate response despite dose titration and compliance with therapy, significant side effects/toxicity, or have a contraindication to this therapy

If the above criteria are met, uridine triacetate (Xuriden) is approved for 2 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one-year intervals based upon:
- Documentation from the prescriber that the member's symptoms have improved based upon the prescriber's assessment while on therapy
- Documentation that the urine orotic acid level is being monitored annually and is lower than the level prior to initiation of treatment

• Valbenazine (Ingrezza™) 2086, 2087, 2088, 2089, 2090, 2091, 2092

Initial Authorization Criteria:
1. For chorea associated with Huntington's disease
- Must be prescribed by a neurologist who specializes in the treatment of movement disorders
- Must be age 18 years or older
- Must have a diagnosis of chorea associated with Huntington's disease confirmed by ONE of the following:
  - Huntington Disease Mutation Analysis showing an expanded CAG repeat (≥ 36) in the huntingtin gene (HTT or HD gene). A copy of the genetic test must be provided.
  - A positive family history of Huntington's disease
- Must provide chart documentation of a clinical work up for Huntington's disease showing ONE of the following symptoms:
  - Motor (e.g., finger tapping, rigidity)

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        - Oculomotor (e.g., slow or unusual eye movements)
        - Bulbar (e.g., dysarthria, dysphagia)
        - Affective (e.g., depression)
        - Cognitive (e.g., lack of impulse control/outbursts, trouble prioritizing or focusing on tasks, slowness of processing thoughts or finding words)
    - Must have an evaluation by a neurologist or behavioral health provider prior to the use of valbenazine (Ingrezza) if the member has a history of depression or suicidal thoughts or behaviors. These members must also be followed concurrently with a behavioral health provider while on valbenazine (Ingrezza). Chart documentation must be provided.

2. For tardive dyskinesia

- Must be prescribed by a neurologist who specializes in the treatment of movement disorders or a psychiatrist
- Must be age 18 years or older
- Must have a confirmed diagnosis of moderate to severe tardive dyskinesia with ALL of the following:
    - Chart documentation of a history of dopamine receptor blocking drug use (e.g., antipsychotic, metoclopramide, prochlorperazine)
    - Chart documentation of involuntary movements for at least 3 months
    - Chart documentation of a baseline Abnormal Involuntary Movement Scale (AIMS) score of ≥ 10, OR chart documentation of tardive dyskinesia that impacts the ability to perform daily functions and essential activities of daily living (e.g., swallowing, writing, walking), which leads to a decrease in quality of life
- Must have been evaluated for dose reduction, withdrawal, or switch of the dopamine receptor blocking agent causing the tardive dyskinesia if currently taking
- Must not have a comorbid involuntary movement disorder (e.g., parkinsonism, akathisia, truncal dystonia)

If the above criteria are met, valbenazine (Ingrezza), is approved for 3 months.

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one year intervals based upon documentation from the provider of the following:

- For chorea associated with Huntington's disease:
    - The member's condition has improved while on therapy or the member continues to benefit from therapy
    - For members with a diagnosis of depression and/or suicidal thoughts or behaviors or for members with a current treatment for depression and/or suicidal thoughts or behaviors, chart documentation must be

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provided to show current evaluation by a neurologist or behavioral health provider.
- For tardive dyskinesia:
  - The member remains a candidate for treatment based upon the prescriber's assessment (e.g., AIMS score, functional assessment) and that the member continues to benefit from therapy
  - An evaluation to taper the dose of or to discontinue therapy with valbenazine (Ingrezza) or clinical rationale for why this would not be appropriate

• Vamorolone (Agamree®) [2093, 2094]

Initial Authorization Criteria:
- Must be prescribed by a neurologist (specifically a neuromuscular specialist) who specializes in the treatment of muscular dystrophy
- Must be age 2 years or older
- Must have a diagnosis of Duchenne muscular dystrophy (DMD) with confirmatory genetic testing demonstrating an out-of-frame mutation [e.g. frameshift (deletion or duplication) mutation or a premature stop codon mutation] of the DMD gene (a copy of the genetic testing must be provided)
- Must have chart documentation of an adequate trial of prednisone AND deflazacort (Emflaza) including ONE of the following:
  - Documentation of significant side effect(s)/toxicity to both prednisone and deflazacort (Emflaza) that impacted quality of life
  - Documentation that target doses of both prednisone and deflazacort (Emflaza) were ineffective
- Must not be using in combination with prednisone or deflazacort (Emflaza)

If the above criteria are met, vamorolone (Agamree) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved or stabilized, or the member continues to benefit from therapy.

• Vericiguat (Verquvo®) [2095, 2096, 2097, 2098]

Initial Authorization Criteria:
- Must be prescribed by a cardiologist
- Must be age 18 years or older
- Must have a diagnosis of chronic heart failure
- Must have a left ventricular ejection fraction (LVEF) < 45%

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- Must have one of the following:
  - Had a hospitalization for heart failure within the past 6 months
  - Received outpatient intravenous diuretics (e.g., furosemide) within the past 3 months
- Must have an attestation from the prescriber that the member is on at least one other medication used for chronic heart failure at maximally tolerated doses (e.g., beta-blockers, ACE inhibitors, ARB, or sacubitril/valsartan)
- Must not have any of the following:
  - Systolic blood pressure less than 100 mmHg
  - Concomitant use with other soluble guanylate cyclase stimulators
  - Currently pregnant

If the above criteria are met, vericiguat (Verquvo) is approved.

- **Vibrant®** [2099, 2100, 2101, 2102]

Initial Authorization Criteria:

- Must be prescribed by a gastroenterologist
- Must be age 18 years or older
- Must have a diagnosis of chronic idiopathic constipation (CIC)
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to TWO of the following:
  - Stool softener (e.g., docusate)
  - Stimulant laxative (e.g., senna, bisacodyl)
  - Bulk laxative (e.g., psyllium)
  - Osmotic laxative (e.g., polyethylene glycol lactulose)
- Must have chart documentation of an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
  - Linaclotide (Linzess)
  - Plecanatide (Trulance)
  - Lubiprostone (Amitiza)
  - Prucalopride (Motegrity)
- Must not have ANY of the following:
  - History of diverticular disease
  - History of intestinal or colonic obstruction, or suspected intestinal obstruction
  - Clinical evidence of current and significant gastroparesis
  - History of significant gastrointestinal disorder (such as inflammatory bowel disease, gastrointestinal malignancy, and/or anal fissures or fistulas)
  - History of Zenker's diverticulum, dysphagia, esophageal stricture, eosinophilic esophagitis, or achalasia

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- Pregnant or nursing

If the above criteria are met, Vibrant is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved while on therapy.

• Vigabatrin (Sabril®, Vigadrone™, Vigpoder™, Vigafyde™) [2103, 2104]

Criteria:

1. **For Refractory Complex Partial Seizures:**
   - Applies to: Vigabatrin (Sabril®, Vigadrone™, Vigpoder™)

   Initial Authorization Criteria:
   - Must be prescribed by a neurologist
   - Must be age 2 years or older
   - Must have a diagnosis of refractory complex partial seizures (i.e., focal impaired awareness seizures)
   - Must have an adequate trial of at least 4 antiepileptic medications with an inadequate response or significant side effect/toxicity or have a contraindication to these therapies. At least one of the regimens must contain phenytoin or carbamazepine.
   - Must be on concomitant therapy with at least one other anticonvulsant medication
   - Must undergo vision testing prior to beginning treatment or have documentation that an examination has been scheduled

If the above criteria are met, the requested product is approved for 3 months. If baseline vision testing has not been completed, the requested product is approved for 1 month while awaiting testing.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation showing the following:
- The member's condition has improved or stabilized based upon the prescriber's assessment while on therapy
- The member underwent baseline vision testing within 1 month of starting therapy (if not completed on initial approval)
- The member is undergoing vision testing at least every 3 months during treatment with Sabril

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2. **For Infantile Spasms:**
   - Applies to: Vigabatrin (Sabril®, Vigadrone™, Vigpoder™, Vigafyde™)

   Initial Authorization Criteria:
   - Must be prescribed by a pediatric neurologist
   - Must be 1 month to 2 years of age
   - Must have a diagnosis of infantile spasms
   - Must undergo vision testing prior to beginning treatment or have documentation that an examination has been scheduled
   - For vigabatrin (Vigafyde) oral solution:
     - Must provide clinical rationale for use of vigabatrin (Vigafyde) oral solution instead of vigabatrin (Sabril, Vigadrone, Vigpoder). Clinical rationale must include why vigabatrin (Sabril, Vigadrone, Vigpoder) would not be expected to produce the same clinical results as vigabatrin (Vigafyde) oral solution given that they are the same chemical entity

If the above criteria are met, the requested product is approved for 1 month.

Reauthorization Criteria:
All prior authorization renewals are reviewed on a 3-month basis to determine Medical Necessity for the continuation of therapy. Authorizations may be extended at 3-month intervals based upon documentation showing the following:
- The member's condition has improved or stabilized based upon the prescriber's assessment while on therapy
- The member underwent baseline vision testing within 1 month of starting therapy (if not completed on initial approval)
- The member is undergoing vision testing at least every 3 months during treatment with vigabatrin
- NOTE: Authorizations may not be extended for infantile spasms beyond the age of 2 years

- **Voclosporin (Lupkynis®)** [2105, 2106, 2107, 2108, 2109, 2110, 2111]

Initial Authorization Criteria:
- Must be prescribed by a rheumatologist or a nephrologist
- Must be age 18 years or older
- Must be autoantibody-positive for systemic lupus erythematosus (SLE), confirmed with documentation of ONE of the following :
  - Positive antinuclear antibodies (ANAs) titre ≥1:80
  - Anti-double stranded DNA (dsDNA) ≥30 IU/mL

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- Must have a diagnosis of class III, IV with or without concomitant class V active lupus nephritis confirmed by kidney biopsy. Documentation of biopsy result with classification must be provided.
- Must use in combination with an immunosuppressive regimen that includes BOTH of the following:
  - Systemic corticosteroids (e.g., prednisone, methylprednisolone)
  - Mycophenolate mofetil (MMF)
- Must have eGFR >45mL/min/1.73m^2
  - For members with eGFR ≤45mL/min/1.73m^2, must start treatment at a reduced dose and have an attestation from the prescriber assessing the risks and benefits of therapy and desire to use voclosporin (Lupkynis)
- Must not be used with baseline blood pressure > 165/105 mmHg or in a hypertensive emergency
- Must have an adequate trial with an inadequate response, significant side effect/toxicity, or have a contraindication to this therapy to BOTH of the following:
  - Belimumab (Benlysta)
  - Obinutuzmab (Gazyva)
- Must not be used in combination with the following:
  - Strong CYP3A4 inhibitors (e.g., ketoconazole, itraconazole, clarithromycin)
  - Cyclophosphamide
  - Belimumab (Benlysta)
  - Obinutuzumab (Gazyva)

If the above criteria are met, voclosporin (Lupkynis) is approved for 6 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider showing:

- Must use in combination with an immunosuppressive maintenance therapy (e.g., MMF and corticosteroids)
- Must show ONE of the following:
  - Stabilization or improvement in renal function demonstrated by laboratory results from baseline (e.g., decreased proteinuria, improvement in urine creatinine ratio, improved eGFR rate). Documentation of laboratory result is required.
  - Reduction in corticosteroid dose
- Must not use in combination with biologic therapies, strong CYP3A4 inhibitors (e.g., ketoconazole, itraconazole, clarithromycin), cyclophosphamide, belimumab (Benlysta), or obinutuzumab (Gazyva)

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- **Vorapaxar (Zontivity™)** [2112, 2113, 2114, 2115]

Initial Authorization Criteria:

- Must be prescribed by a cardiologist
- Must be age 18 years or older
- Must have a history of MI within the past 2 weeks to 12 months or have PAD
- Must provide clinical rationale for the use of vorapaxar (Zontivity) and an assessment of the member's underlying risk of bleeding to show that the benefits of vorapaxar (Zontivity) would be expected to outweigh the increased risk of bleeding
- Must be on concomitant therapy with another antiplatelet agent such as aspirin, clopidogrel (Plavix®)
- Must not have any of the following:
  - History of stroke
  - History of transient ischemic attack
  - History of intracranial hemorrhage
  - Current active pathological bleeding
  - High potential risk of bleeding

If the above criteria are met, vorapaxar (Zontivity) is approved.

- **Vortioxetine (Trintellix™)**[2116, 2117]

Criteria:

- Must be age 18 years or older
- Must have a diagnosis of major depressive disorder (MDD)
- Must have an adequate trial with an inadequate response, duration of at least 4 weeks, or significant side effect/toxicity or have a contraindication to ALL of the following:
  - Three different generic antidepressants from the SSRI or SNRI therapeutic classes AND
  - Combination therapy with one generic antidepressant therapy from the SSRI or SNRI therapeutic class plus an augmenting therapy (e.g., buspirone, TCA, antipsychotic, lithium or liothyronine)

If the above criteria are met, vortioxetine (Trintellix) is approved.

- **Vosoritide (Voxzogo™)** [2118, 2119, 2120, 2121]

Initial Authorization Criteria:

- Must be prescribed by a geneticist, skeletal dysplasia specialist, or endocrinologist
- Must be less than 18 years of age
- Must have a diagnosis of achondroplasia confirmed by a mutation in the *FGFR3* gene (a copy of the genetic test must be provided)

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- Must have documentation of open epiphyses
- Must provide documentation of baseline growth velocity

If the above criteria are met, vosoritide (Voxzogo) is approved for 1 year.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon the following:

- Must have documentation from the provider that the member's condition has improved while on therapy as evidenced by an increase in growth velocity from baseline
- Must have documentation of open epiphyses

- **Weight Loss** 2122, 2123, 2124, 2125, 2126, 2127, 2128, 2129, 2130, 2131, 2132, 2133, 2134, 2135
  - Applies to members with weight loss coverage included in their pharmacy rider. Please confirm coverage before reviewing with the criteria below.
  - Requests for semaglutide (Wegovy) for overweight or obese members with established cardiovascular disease for members without weight loss coverage, please refer to 'Semaglutide (Wegovy)' criteria above
  - Requests for tirzepatide (Zepbound) for obese members using for Obstructive Sleep Apnea for members without weight loss coverage, please refer to 'Tirzepatide (Zepbound)' criteria above

Initial Authorization Criteria:

- For semaglutide tablets (Wegovy):
  - Must be age 18 years or older
- For all other products:
  - Must be age 12 years or older
- For members 12 to 17 years of age, must be obese:
- For members 18 years or older:
  - Must have an attestation from the provider of an initial BMI of:
    - 30 kg/m^2 or greater (obese) OR
    - 27 kg/m^2 or greater (overweight) in the presence of at least one weight-related comorbidity, including, but not limited to hypertension, type 2 diabetes mellitus, obstructive sleep apnea or dyslipidemia
      - Must have an attestation from the provider that confirms the weight-related comorbidity
- Must have an attestation from the provider of BOTH of the following:
  - The member has had a trial of behavioral modification and dietary restriction for at least 3 months and has failed to achieve the desired weight loss

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        - The member has an outlined reduced-calorie diet plan and increased physical activity plan
    - Must have an attestation from the provider of baseline body weight including date

If the above criteria are met, the requested medication is approved for 7 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one-year intervals based upon chart documentation showing the following:
- Must have an attestation from the provider of ongoing engagement in behavioral modification and on a reduced calorie diet
- Must have a recent body weight that supports ≥5% body weight loss from baseline
- For liraglutide (Saxenda):
    - Must have a recent body weight that supports ≥4% body weight loss from baseline

- **Xanomeline and trospium chloride (Cobenfy®)** [2136, 2137, 2138, 2139, 2140]

Initial Authorization Criteria:
- Must have a diagnosis of schizophrenia
- Must be prescribed by or in consultation with a psychiatrist
- Must be age 18 years or older
- Must have an adequate trial of at least 4 weeks with an inadequate response or significant side effect/toxicity or have a contraindication to THREE generic antipsychotics
- Must not have ANY of the following:
    - Urinary retention
    - Moderate or severe hepatic impairment
    - Gastric retention
    - Untreated narrow-angle glaucoma

If the above criteria are met, xanomeline and trospium chloride (Cobenfy) is approved.

- **Zileuton ER (Zyflo CR®) 600 mg tablet** [2141]

Criteria:
- Must have a diagnosis of asthma
- Must be age 12 years or older

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- Must have had an adequate trial and failure of montelukast (Singulair®) AND zafirlukast (Accolate®)
- Must not have active liver disease or persistent hepatic function enzyme elevations ≥3 times the upper limit of normal.

If the above criteria are met, zileuton ER (Zyflo) is approved

• Zilucoplan (Zilbrysq®) [2142, 2143, 2144]

Initial Authorization Criteria:

- Must be prescribed by a neurologist
- Must be age 18 years or older
- Must have a diagnosis of anti-AchR positive generalized myasthenia gravis. Chart documentation of a positive serological test of anti-acetylcholine receptor antibodies must be provided
- Must have history of myasthenia gravis with symptoms that impair the ability to perform daily functions and essential activities of daily living (e.g., difficulty chewing, swallowing, or breathing). Chart documentation must be provided
- Must provide current weight and have a requested dose and frequency that falls within the recommended dosing guidelines from the manufacturer
- Must have an adequate trial of corticosteroid with inadequate response, significant side effects/toxicity or have a contraindication to therapy
- Must have an adequate trial of at least 3 months of ONE of the following immunosuppressants with an inadequate response OR have a significant side effect/toxicity or have a contraindication to TWO of the following:
  - Azathioprine
  - Mycophenolate mofetil
  - Cyclosporine
  - Tacrolimus
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ONE of the following:
  - Two 4-week courses of efgartigimod alfa-fcab (Vyvgart)
    - NOTE: a trial of efgartigimod alfa and hyaluronidase (Vyvgart Hytrulo) is acceptable for the efgartigimod alfa-fcab (Vyvgart) trial
  - One 6 week course of rozanolixizumab-nilo (Rystiggo)
  - Nipocalimab-aahu (Imaavy)
  - Inebilizumab-cdon (Uplizna)
- Must be on concomitant therapy with conventional therapy (e.g., AChE inhibitor [pyridostigmine], steroids, non-steroidal immunosuppressive therapies)
- Must not be on concomitant therapy with ANY of the following:
  - An eculizumab product

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- Ravulizumab-cwvz (Ultomiris)
- Rozanolixizumab-nilo (Rystiggo)
- Efgartigimod alfa-fcab (Vyvgart)
- Efgartigimod alfa and hyaluronidase (Vyvgart Hytrulo)
- Nipocalimab-aahu (Imaavy)
- Inebilizumab-cdon (Uplizna)

If the above criteria are met, the requested medication is approved for 6 months.

Reauthorization Criteria:
All prior authorization renewals are reviewed to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon chart documentation showing the following:

- The member's condition (e.g., daily function, activities of daily living) has improved while on therapy.
- Must provide current weight and have a requested dose and frequency that falls within the recommended dosing guidelines from the manufacturer.

• Zuranolone capsule (Zurzuvae™): [2145, 2146, 2147, 2148, 2149]

Initial Authorization Criteria:

- Must be age 18 years or older
- Must have a diagnosis of postpartum depression (PPD)
  - Must have onset of depressive episode no later than 12 months postpartum

If the above criteria are met, zuranolone (Zurzuvae) is approved for 1 month for 1 course of therapy per pregnancy. Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require clinical rationale for re-treatment.

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Criteria for individual drug or class of drugs for the Your Choice, Value Choice and Open Choice Formularies:

- **Lasmiditan (Reyvow™)** [2150, 2151, 2152, 2153, 2154, 2155, 2156, 2157, 2158, 2159, 2160, 2161, 2162, 2163, 2164]

Criteria:

- Must be prescribed by a neurologist or a board-certified headache specialist
- Must be 18 years or older
- Must have a diagnosis of migraine headache
- Must not be taking other acute, migraine-specific therapy (e.g., triptans, ergots, CGRP antagonists including rimegepant [Nurtec™ ODT™] and ubrogepant [Ubrelvy™])
- Must have an adequate trial and failure or intolerance to ALL of the following:
  - ALL formulations (e.g., tablet, orally disintegrating tablet, nasal spray, injection) of ALL formulary triptan medications with an inadequate response OR must have one or more of the following contraindications to triptan therapy:
    - Ischemic coronary artery disease (CAD) (angina pectoris, history of myocardial infarction, or documented silent ischemia) or coronary artery vasospasm (including Prinzmetal's angina)
    - Wolf-Parkinson-White syndrome or arrythmias associated with other cardiac accessory conduction pathway disorders
    - History of stroke, transient ischemic attack (TIA), or hemiplegic/basilar migraine
    - Peripheral vascular disease (PVD)
    - Ischemic bowel disease
  - Rimegepant (Nurtec ODT)
  - Ubrogepant (Ubrelvy)

If the above criteria are met, lasmiditan (Reyvow) is approved. (Applies to Your Choice, Value Choice and Open Choice formularies only)

Criteria for individual drug or class of drugs for the Value Choice and Open Choice Formularies:

- **Alirocumab (Praluent®)** [2165]

Initial Authorization Criteria

- Must be age 8 years or older
- Must have one of the following diagnoses:
  - Clinical atherosclerotic cardiovascular disease (ASCVD)

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        - Hyperlipidemia (e.g., primary hyperlipidemia, heterozygous or homozygous familial hypercholesterolemia)
    - Must have an adequate trial and failure with an inadequate response or significant side effect/toxicity or have a contraindication to BOTH of the following:
        - A single-entity or combination HMG-CoA reductase inhibitor
            - For members with intolerance to statins, must have an attestation from the prescriber indicating intolerable adverse reactions occurred (e.g., myalgia, myositis)
        - Evolocumab (Repatha®)
            - NOTE: An exception to the requirement for this trial can be made if the member is under 10 years of age

If the above criteria are met, alirocumab (Praluent) is approved. (Applies to Value Choice and Open Choice Formularies only).

• Aripiprazole oral film (Opipza™) [2166]

Criteria:

- Commercial and Exchange members < 12 years of age, please refer to 'Atypical and Typical Antipsychotics – Children Under 12 years of Age' criteria below
- Must have one of the following diagnoses:
    - Schizophrenia
    - Irritability associated with autistic disorder
    - Tourette's disorder
    - Major Depressive Disorder and meet the following criteria:
        - Must have an adequate trial, of at least 4 weeks each, of monotherapy with TWO different antidepressants with an inadequate response or significant side effect/toxicity or have a contraindication to these therapies
        - Must also be taking a Selective Serotonin Reuptake Inhibitor (SSRI) or Selective Serotonin Norepinephrine Reuptake Inhibitor (SNRI) antidepressant as adjunctive treatment
- Must have documentation of the inability to swallow an oral solid dosage form due to age (< 7 years), medical condition (e.g. nasogastric tube), or other clinical rationale for why aripiprazole (Abilify) tablet cannot be used
- For aripiprazole oral film (Opipza), must provide clinical rationale for use of aripiprazole oral film (Opipza) instead of aripiprazole ODT (Abilify Discmelt) or aripiprazole oral solution (Abilify). Clinical rationale must include why aripiprazole oral film (Opipza) would not be expected to produce the same clinical results as aripiprazole ODT (Abilify Discmelt) or aripiprazole oral solution (Abilify) given that they are the same chemical entity

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If the above criteria are met, aripiprazole oral film (Opipza) is approved. (Applies to Value Choice and Open Choice Formularies only).

• Bumetanide nasal spray (Enbumyst™) [2167, 2168]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with a cardiologist, nephrologist, or hepatologist
- Must be age 18 years or older
- Must have a diagnosis of edema associated with congestive heart failure, hepatic disease, or renal disease (including nephrotic syndrome)
- Must be prescribed a dose and duration of therapy that is consistent with FDA-approved package labeling, nationally-recognized compendia, or peer-reviewed medical literature
- Must have an adequate trial of an orally administered loop diuretic (e.g., furosemide, bumetanide, or torsemide) at maximally tolerated doses with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
- Must not have ANY of the following:
  - Anuria
  - Hepatic coma
  - Hypersensitivity to bumetanide or any of its components
- Must have an attestation from the provider including BOTH of the following:
  - Bumetanide (Enbumyst) is not being used as maintenance therapy
  - Treatment with bumetanide (Enbumyst) is to be replaced by an oral diuretic as soon as practical for the member

If the above criteria are met, bumetanide (Enbumyst) is approved for 1 month for 1 course of therapy. (Applies to Value Choice and Open Choice Formularies only).

Reauthorization Criteria:

Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require documentation describing the previous response and clinical rationale for re-treatment

• Dronabinol (Syndros®) [2169]

Criteria:

- Must be age 18 and older
- Must have one of the following diagnoses:
  - Anorexia associated with weight loss with AIDS
  - Nausea and vomiting associated with chemotherapy
    - Must be on concurrent chemotherapy

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        - Must have an adequate trial and failure or an inadequate response or intolerance of BOTH of the following:
            - 5HT$_3$ antagonist (e.g., ondansetron, granisetron) AND
            - One conventional antiemetic treatment (e.g., prochlorperazine, promethazine)
    - Must not have a sensitivity to dronabinol or alcohol
    - Must not have a history of a hypersensitivity reaction to alcohol
    - Must not be receiving or have received disulfiram or metronidazole containing products within the past 14 days
    - Must have documentation of the inability to swallow an oral solid dosage form due to a medical condition (e.g. nasogastric tube), or other clinical rationale for why dronabinol (Marinol) capsule cannot be used

If the above criteria are met, dronabinol (Syndros), is approved. (Applies to Value Choice and Open Choice Formularies only)

• Eltrombopag Choline (Alvaiz™) [2170]

Initial Authorization Criteria:

1. For immune thrombocytopenia (ITP)

- Must be prescribed by a hematologist or oncologist
- Must be age 6 years or older
- Must have a diagnosis of immune thrombocytopenia (ITP)
- Must have an adequate trial with an inadequate response or significant side effect/toxicity to ONE of the following:
    - Corticosteroids
    - Intravenous immunoglobulin (IVIG)
- Must have an adequate trial of generic eltrombopag (Promacta) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
- Must have a platelet count < 30 x 10^9/L prior to initiation

2. For thrombocytopenia in chronic hepatitis C infection

- Must be prescribed by a gastroenterologist, hematologist, hepatologist, or infectious disease specialist
- Must be age 18 years or older
- Must have a diagnosis of chronic hepatitis C
- Must currently be treated with or anticipating hepatitis C treatment with an interferon agent
- Must have an adequate trial of generic eltrombopag (Promacta) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
- Must have a platelet count < 75 x 10^9/L prior to initiation

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3. For severe aplastic anemia with insufficient response to immunosuppressive therapy (refractory)

- Must be prescribed by a hematologist or oncologist
- Must be age 18 years or older
- Must have a diagnosis of severe aplastic anemia
- Must have an adequate trial with an inadequate response or significant side effect/toxicity to immunosuppressive therapy (e.g., antithymocyte globulin, cyclosporine)
- Must have an adequate trial of generic eltrombopag (Promacta) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
- Must have a platelet count < 30 x 10^9/L prior to initiation

If the above criteria are met, eltrombopag choline (Alvaiz) is approved for 6 months. (Applies to Value Choice and Open Choice Formularies only).

Reauthorization Criteria:
All prior authorization renewals are reviewed to determine the Medical Necessity for continuation of therapy. Authorizations may be extended for a duration depending upon the diagnosis and based upon the following:

- For immune thrombocytopenia (ITP): 1 year
  - Must have documentation showing improvement in platelet count from baseline
- For severe aplastic anemia, refractory: 1 year
  - Must have documentation showing improvement in platelet count from baseline
- For thrombocytopenia in chronic hepatitis C infection:
  - Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response and clinical rationale for re-treatment.

• Gepirone (Exxua™) [2171, 2172, 2173, 2174, 2175]

Criteria:

- Must be age 18 years or older
- Must have a diagnosis of major depressive disorder (MDD)
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
  - Two different generic antidepressants from the SSRI or SNRI therapeutic classes

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        - Combination therapy with one generic antidepressant therapy from the SSRI or SNRI therapeutic class plus buspirone
        - Bupropion
    - Must not have ANY of the following:
        - Prolonged QTc interval > 450 msec at baseline
        - Congenital long QT syndrome
        - Concomitant use of strong CYP3A4 inhibitors
        - Severe hepatic impairment
        - Use with an MAOI or within 14 days of stopping treatment with an MAOI

If the above criteria are met, gepirone (Exxua) is approved. (Applies to Value Choice and Open Choice Formularies only)

- **Ingenol Mebutate (Picato®)** [2176, 2177, 2178]

Initial Authorization Criteria:
- Must be prescribed by or in consultation with a dermatologist
- Must be age 18 years or older
- Must have a diagnosis of actinic keratosis on the face, scalp, trunk, or extremities
- Must have an adequate trial and failure of topical fluorouracil or topical imiquimod with an inadequate response, significant side effect/toxicity, or a contraindication to both of these therapies

If the above criteria are met, ingenol mebutate (Picato) is approved for 1 fill. (Applies to Value Choice and Open Choice Formularies only)

Reauthorization Criteria:
All prior authorization renewals are reviewed on a case-by-case basis to determine the Medical Necessity for continuation of treatment. Authorizations may be extended on a case-by-case basis based upon the above criteria and clinical rationale from the prescriber for continuation of treatment at the same site or documentation that therapy is required at an alternative site.

- **Insulin Human Inhalation Powder (Afrezza®)** [2179, 2180, 2181, 2182, 2183, 2184, 2185]

Initial Authorization Criteria:
- Must be prescribed by an endocrinologist
- Must be age 18 years or older
- Must have a diagnosis of diabetes mellitus

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        - For members with type 1 diabetes:
            - Must be used in combination with long-acting insulin
    - Must submit documentation confirming that baseline spirometry (FEV1) testing has been completed
    - Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ONE of the following insulin products:
        - Insulin lispro (Humalog®)
        - Insulin lispro-aabc (Lyumjev®)
        - Insulin glulisine (Apidra®)
    - Must provide clinical rationale for use of insulin human inhalation powder (Afrezza®) instead of a prerequisite insulin product. Clinical rationale must include why insulin human inhalation powder (Afrezza®) would not be expected to produce the same clinical results as a prerequisite insulin product
    - Must not have chronic lung disease, such as asthma or chronic obstructive pulmonary disease (COPD)

If the above criteria are met, insulin human inhalation powder (Afrezza) is approved for 6 months. (Applies to Value Choice and Open Choice Formularies only).

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one-year intervals based upon the following:
- Documentation from the prescriber indicating that the member's condition has improved based upon prescriber's assessment while on therapy
- Documentation confirming the member's pulmonary function is being monitored while on therapy

- **Non-Preferred Inhaled Beta$_2$-Agonist** 2186, 2187, 2188, 2189, 2190, 2191, 2192, 2193
    - Applies to: Proair® Digihaler™, Proair® Respiclick®

Criteria:
- Must be age 4 years or older
- Must be used for treatment or prevention of bronchospasm with reversible obstructive airway disease [e.g., asthma and chronic obstructive pulmonary disease (COPD)] or for prevention of exercise-induced bronchospasm
- Must have a previous trial and failure of or intolerance to Ventolin® HFA or generic albuterol HFA (ProAir® HFA, Proventil® HFA)

If the above criteria are met, the requested non-preferred inhaled beta2-agonist is approved. (Applies to Value Choice and Open Choice Formularies only)

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- **Oxycodone Extended-Release Tablet** [2194, 2195, 2196, 2197, 2198, 2199, 2200]

Initial Authorization Criteria:

- Must meet criteria for Opioid Analgesics, Long-acting
- Additionally, for members age 11 through 17 years:
  - Must be on opioids for five consecutive days with a minimum of 20mg per day of oxycodone or its equivalent for at least two days immediately preceding dosing with oxycodone ER
  - Must have chart documentation of an adequate trial, for at least one month, of morphine sulfate ER (MS Contin®) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
- Additionally, for members age 18 years and older:
  - Must have chart documentation of an adequate trial, for at least one month each, with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following::
    - Fentanyl patch (Duragesic®)
    - Morphine sulfate ER tablet (MS Contin)
    - Oxymorphone ER (Opana® ER)
    - Oxycodone ER capsule (Xtampza® ER)

If the above criteria are met, oxycodone extended-release tablet (generic Oxycontin) is approved for a duration as follows:

- Pediatrics:the anticipated duration of therapy or 1 year (whichever is shorter)
- Adults: up to 1 year

(Applies to Value Choice and Open Choice Formularies only)

Reauthorization Criteria:

All prior authorization renewals are reviewed to determine the Medical Necessity for continuation of therapy. Authorization may be extended on a case-by-case basis for up to 1 year based upon documentation from the provider that the member's condition has improved or stabilized while on therapy. Must also meet reauthorization criteria for Opioid Analgesics, Long-acting.

- **Rilzabrutinib (Wayrilz™)**[2201]

Initial Authorization Criteria:

- Must be prescribed by a hematologist or oncologist
- Must be age 18 years or older

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- Must have a diagnosis of immune thrombocytopenia (ITP)
- Must have an adequate trial with inadequate response or significant side effect/toxicity to ONE of the following:
  - Corticosteroids
  - Intravenous immunoglobulin (IVIG)
- Must have an adequate trial of generic eltrombopag (Promacta) with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
- Must have a platelet count <30 x 10^9/L prior to initiation

If the above criteria are met, rilzabrutinib (Wayrilz) is approved for 6 months. (Applies to Value Choice and Open Choice Formularies only).

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended at one-year intervals based upon documentation showing improvement in platelet count from baseline.

• Sofpironium Bromide Gel (Sofdra®) [2202]

Initial Authorization Criteria:
- Must be prescribed by a dermatologist
- Must be age 9 years or older
- Must have a diagnosis of primary axillary hyperhidrosis
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
  - Aluminum Chloride 20% solution (Drysol)
  - Glycopyrronium cloth (Qbrexza®)
  - OnabotulinumtoxinA (Botox®)

If the above criteria are met, sofpironium bromide gel (Sofdra) is approved for 1 year. (Applies to Value Choice and Open Choice Formularies only).

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon documentation from the provider that the member's condition has improved while on therapy.

• Suzetrigine (Journavx®) [2203, 2204, 2205]

Initial Authorization Criteria:

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- Must be age 18 years or older
- Must have a diagnosis of moderate to severe acute pain anticipated to last less than 1 month
- Must be using a dose and duration of therapy consistent with the FDA-approved package labeling

If the above criteria are met, suzetrigine (Journavx) is approved for 1 course of therapy for 14 days. (Applies to Value Choice and Open Choice Formularies only).

Reauthorization Criteria:
Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response and clinical rationale for re-treatment.

• Tirbanibulin (Klisyri®) [2206, 2207, 2208, 2209, 2210, 2211, 2212, 2213]

Initial Authorization Criteria:

- Must be prescribed by or in consultation with a dermatologist
- Must be age 18 years or older
- Must have a diagnosis of actinic keratosis on the face or scalp
- Must have an adequate trial and failure of topical fluorouracil or topical imiquimod with an inadequate response, significant side effect/toxicity, or a contraindication to both of these therapies

If the above criteria are met, tirbanibulin (Klisyri) is approved for 1 fill. (Applies to Value Choice and Open Choice Formularies only)

Reauthorization Criteria:
All prior authorization renewals are reviewed on a case-by-case basis to determine the Medical Necessity for continuation of treatment. Authorizations may be extended on a case-by-case basis based upon the above criteria and clinical rationale from the prescriber for continuation of treatment at the same site or documentation that therapy is required at an alternative site.

• Viloxazine (Qelbree®) [2214, 2215, 2216, 2217]

Initial Authorization Criteria:

- Must be 6 years of age or older
- Must have a diagnosis of attention deficit hyperactivity disorder (ADHD)
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
  - Atomoxetine (Strattera)

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        - Two different Central Nervous System (CNS) stimulants (e.g., methylphenidate, dexmethylphenidate, amphetamine/dextroamphetamine)
    - Must not have ANY of the following:
        - Concomitant use with a monoamine oxidase inhibitor (MAOI) or use within 14 days after discontinuing an MAOI (e.g., selegiline, rasagiline, isocarboxazid, phenelzine, tranylcypromine, safinamide)
        - Concomitant use with sensitive CYP1A2 substrates or CYP1A2 substrates with a narrow therapeutic range (e.g., alosetron, duloxetine, ramelteon, tasimelteon, tizanidine, theophylline)

If the above criteria are met, viloxazine (Qelbree) is approved. (Applies to Value Choice and Open Choice Formularies only).

- **Zavegepant (Zavzpret®):** 2218, 2219, 2220, 2221, 2222, 2223

Initial Authorization Criteria:

- Must be age 18 years or older
- Must have a diagnosis of acute migraine
- Must not be taking other acute, migraine-specific therapy (e.g., triptans, ergots, CGRP antagonists including rimegepant [Nurtec® ODT] and ubrogepant [Ubrelvy™])
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
    - Two different generic triptans (e.g., naratriptan, rizatriptan, sumatriptan)
    - Rimegepant (Nurtec ODT)
    - Ubrogepant (Ubrelvy)

If the above criteria are met, zavegepant (Zavzpret) is approved. (Applies to Value Choice and Open Choice Formularies only).

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Criteria for individual drug or class of drugs for the Open Choice Formulary:

- **Acetaminophen/Ibuprofen (Combogesic®)** [2224]

Initial Authorization Criteria:

- Must be age 18 years or older
- Must have a diagnosis of mild to moderate acute pain
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to BOTH of the following:
  - Generic acetaminophen tablet or capsule
  - Generic ibuprofen tablet or capsule
- Must provide clinical rationale for use of acetaminophen/ibuprofen (Combogesic) instead of generic acetaminophen and ibuprofen tablets or capsules. Clinical rationale must include why generic acetaminophen and ibuprofen tablets or capsules would not be expected to produce the same clinical results as acetaminophen/ibuprofen (Combogesic) given that they are the same chemical entities.

If the above criteria are met, acetaminophen/ibuprofen (Combogesic) is approved for 1 month. Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response and clinical rationale for re-treatment. (Applies to Open Choice Formulary)

- **Hydrocortisone Solution (Khindivi™)** [2225]

Initial Authorization Criteria:

- Must be 5 to 17 years of age
- Must have a diagnosis of adrenal insufficiency
- Must have an adequate trial of generic hydrocortisone tablet with an inadequate response or significant side effect/toxicity or have a contraindication to this therapy
- Must have an attestation from the prescriber of the inability to swallow an oral solid dosage form due to age (< 7 years), medical condition (e.g., nasogastric tube), or other clinical rationale for why the pre-requisite medication cannot be used
- Must not be used for acute or stress treatment

If the above criteria are met, hydrocortisone solution (Khindivi™) is approved for 1 year. (Applies to Open Choice Formulary only)

Reauthorization Criteria:

All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of therapy. Authorizations may be extended at one-year intervals based upon the following:

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- Documentation from the provider that the member's condition has improved or stabilized while on therapy
- Must have an attestation that the member still has an inability to swallow an oral solid dosage form due to age (< 7 years), medical condition (e.g., nasogastric tube), or other clinical rationale for why the pre-requisite medication cannot be used

• Lidocaine 1.8% topical system (ZTLIDO®) [2226]

Criteria:

- Must have a diagnosis of postherpetic neuralgia
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
  - Gabapentin
  - Lidocaine 5% patch
  - One tricyclic antidepressant (e.g., amitriptyline)
  - Pregabalin

If the above criteria are met, lidocaine 1.8% topical system (ZTLIDO) is approved. (Applies to Open Choice Formulary only)

• Non-Preferred Topical Pain, Anti-Pruritic, and Dermatoses Products [2227, 2228, 2229]

- Applies to: "kit" products indicated for topical pain, topical antifungals, relief of pruritus, scar management, plaque psoriasis and acne

Initial Authorization Criteria:

- Must have an appropriate FDA approved diagnosis
- Must submit chart documentation showing that the member has tried 3 of the following medications (diagnosis appropriate) for at least one month each with an inadequate response or significant side effects/toxicity unless there is a documented contraindication:
  - Topical lidocaine cream, gel, jelly, or ointment
  - Lidocaine/prilocaine cream
  - OTC topical capsaicin or menthol products (e.g. Capzasin, Zostrix, Tiger Balm)
  - Prescription strength topical corticosteroids (e.g. hydrocortisone cream, triamcinolone cream)
  - Prescription strength NSAIDs (e.g. diclofenac gel, ibuprofen, naproxen, diclofenac)
  - Topical calcipotriene, calcitriol or calcitrene
  - Topical antibiotics
  - Topical antifungals

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        - Topical retinoid products
        - Oral gabapentin

If the above criteria are met the requested non-preferred topical pain, anti-pruritic, or dermatoses product is approved for one year. (Applies to Open Choice Formulary only)

Reauthorization Criteria:
All prior authorization renewals are reviewed on an annual basis to determine the Medical Necessity for continuation of treatment. Authorizations may be extended at one-year intervals based upon documentation from the prescriber indicating the member's condition has improved as a result of treatment.

• Prednisone DR (Rayos®) [2230]

Initial Authorization Criteria:

- Must be used for one of the following:
    - As an anti-inflammatory or immunosuppressive agent for certain allergic, dermatologic, gastrointestinal, hematologic, ophthalmologic, nervous system, renal, respiratory, rheumatologic, specific infectious diseases or conditions and organ transplantation
    - Treatment of certain endocrine conditions
    - For the palliation of certain neoplastic conditions
- Must have an adequate trial of ALL of the following with significant side effects/toxicity impacting quality of life for at least 3 months each:
    - Prednisone
    - Methylprednisolone
    - Dexamethasone
    - Hydrocortisone
- Must provide clinical rationale for use of prednisone DR (Rayos) instead of prednisone. Clinical rationale must include why prednisone would not be expected to produce the same clinical results as prednisone DR (Rayos) given that they are the same chemical entity
- Must provide clinical rationale for use of prednisone DR (Rayos) instead of methylprednisolone, dexamethasone, and hydrocortisone. Clinical rationale must include why methylprednisolone, dexamethasone, and hydrocortisone would not be expected to produce the same clinical results as prednisone DR (Rayos)

If the above criteria are met, prednisone DR (Rayos) is approved for up to 1 year. (Applies to Open Choice Formulary only)

Reauthorization Criteria:

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All prior authorization renewals are reviewed on a case-by-case basis and are subject to the above criteria and require chart documentation describing the previous response to treatment and clinical rationale for re-treatment to determine the Medical Necessity for continuation of therapy. Authorization may be extended for up to 1 year of treatment.

• Rolapitant (Varubi®) [2231]

Initial Authorization Criteria:

- Must be prescribed by a hematologist or oncologist
- Must be age 18 years or older
- Must be used to prevent delayed nausea and vomiting associated with emetogenic cancer chemotherapy (e.g., cisplatin, anthracycline, cyclophosphamide)
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to BOTH of the following:
  - Oral generic ondansetron
  - Oral generic granisetron
- Must be using in combination with BOTH of the following:
  - Dexamethasone
  - A 5-HT3 receptor antagonist (e.g., ondansetron, granisetron)

If the above criteria are met, rolapitant (Varubi) is approved for 1 year. Additional authorizations for treatment are made on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response and clinical rationale for re-treatment. (Applies to Open Choice Formulary only)

• Valsartan oral solution [2232]

Initial Authorization Criteria:

- Must be age 6 years or older
- Must have ONE of the following diagnoses:
  - Hypertension
  - Heart failure (NYHA class II-IV)
  - Stable left ventricular failure or left ventricular dysfunction following myocardial infarction
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
  - Preferred ACE inhibitor (i.e., lisinopril) or preferred ACE inhibitor combination product (i.e., lisinopril/ hydrochlorothiazide)
  - Preferred ARB (e.g., losartan) or preferred ARB combination product (e.g., losartan, losartan/hydrochlorothiazide)
  - Enalapril oral solution (Epaned®)

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- Must have an attestation from the prescriber of the inability to swallow an oral solid dosage form due to age (< 7 years), medical condition (e.g., nasogastric tube), or other clinical rationale for why all pre-requisite medications cannot be used

If the above criteria are met, valsartan oral solution is approved. (Applies to Open Choice Formulary only)

Criteria for individual drug or class of drugs for the Advantage Choice and Open Choice Formularies:

- **Non-Preferred Long-Acting Opioids** 2233, 2234, 2235, 2236, 2237, 2238, 2239, 2240
  - Applies to: hydrocodone bitartrate extended release (Zohydro® ER), hydromorphone (Exalgo™), methadone (Dolophine®) (Advantage Choice ONLY)
  - Applies to: all non-preferred long-acting opioids (Open Choice ONLY)

Initial Authorization Criteria:

- Must meet criteria for Opioid Analgesics, Long-acting
- Must have chart documentation of an adequate trial, for at least one month each, with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
  - Fentanyl patch (Duragesic)
  - Morphine sulfate extended-release tablet (MS Contin)
  - Oxymorphone ER (Opana ER)
  - Oxycodone ER (Xtampza ER)

If the above criteria are met, the requested product is approved for a duration as follows:

- Pediatrics: the anticipated duration of therapy or 1 year (whichever is shorter)
- Adults: up to 1 year

Reauthorization Criteria:

All prior authorization renewals are reviewed to determine the Medical Necessity for continuation of therapy. Authorization may be extended on a case-by-case basis for up to 1 year based upon documentation from the provider that the member's condition has improved or stabilized while on therapy. Must also meet reauthorization criteria for Opioid Analgesics, Long-acting.

Criteria for individual drug or class of drugs for the Your Choice and Open Choice Formularies:

- **Infertility** 2241, 2242
  - Applies to members with infertility coverage included in their pharmacy rider.

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Initial Authorization Criteria:

- Must be prescribed by a reproductive or fertility specialist
- Must have a diagnosis of infertility
  - Must have chart documentation of a clinical work-up to confirm diagnosis

If the above criteria are met, the requested medication is approved for 3 months. (Applies to Your Choice and Open Choice Formularies only)

Reauthorization Criteria:
Authorization may be extended on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response to treatment and clinical rationale for re-treatment to determine the Medical Necessity for continuation of therapy. Authorization may be extended for 3 months.

Criteria for individual drug or class of drugs for the Advantage Choice Formulary:

- **Amlodipine-atorvastatin (Caduet®)** [2243]

Criteria:

- Must have had an adequate trial and failure or an inadequate response or significant side effects/toxicity, or a contraindication to of ALL of the following:
  - Amlodipine AND
  - All formulary generically available single-entity HMG-CoA reductase inhibitors

If the above criteria are met, amlodipine-atorvastatin (Caduet) is approved. (Applies to Advantage Choice Formulary only)

- **Anti-Malarial Agents** [2244, 2245, 2246, 2247, 2248, 2249]
  - Applies to: Aralen® (chloroquine), Lariam® (mefloquine), Malarone® (atovaquone-proguanil), primaquine, Qualaquine® (quinine sulfate)

Initial Authorization Criteria:

- Must be prescribed by or in consultation with an infectious disease specialist
- Must be used for the treatment of malaria
- Must NOT be used for prophylaxis of malaria

If the above criteria are met, the requested anti-malarial agent is approved for 1 treatment course based on the agent of choice. (Applies to Advantage Choice Formulary only)

Reauthorization Criteria:

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All prior authorization renewals are reviewed on a case by case basis and are subject to the above criteria to determine the Medical Necessity for continuation of therapy. Authorization may be extended for 1 additional treatment course.

• Besifloxacin (Besivance®) [2250]

Criteria:

- Must have a diagnosis of bacterial conjunctivitis
- Must have an attestation from the prescriber indicating that ALL of the following would not be clinically appropriate treatment options for the member's condition:
  - Ciprofloxacin eye drop
  - Gatifloxacin eye drop
  - Levofloxacin eye drop
  - Moxifloxacin eye drop
  - Ofloxacin eye drop

If the above criteria are met, besifloxacin (Besivance) is approved for 1 month. (Applies to Advantage Choice Formulary only)

Reauthorization Criteria:

All prior authorization renewals are reviewed on a case-by-case basis and are subject to the above criteria, to determine the Medical Necessity for continuation of therapy. Authorization may be extended for 1 month of treatment.

• Clomiphene citrate (Serophene, MiloPhene™) [2251, 2252]

Criteria:

- Must be prescribed by an endocrinologist
- Must be a natal male
- Must have one of the following diagnoses:
  - Primary hypogonadism (congenital or acquired) – testicular failure due to cryptorchidism, bilateral torsions, orchitis, vanishing testis syndrome, or orchidectomy
  - Hypogonadotropic hypogonadism (congenital or acquired) – idiopathic gonadotropin or luteinizing hormone-releasing hormone (LHRH) deficiency, or pituitary-hypothalamic injury from tumors, trauma or radiation
- Must have a repeated morning serum total testosterone level < 300ng/dL
- Must have had an adequate trial and failure with an inadequate response or significant side effects/toxicity, or a contraindication to testosterone therapy

If the above criteria are met, the requested product is approved. (Applies to Advantage Choice Formulary only)

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- **Ergotamine tartrate (Ergomar®)** [2253]

Initial Authorization Criteria:

- Must be 18 years of age or older
- Must have a diagnosis of migraine headaches
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
    - TWO generic triptans (e.g. naratriptan, rizatriptan, sumatriptan)
    - Rimegepant (Nurtec ODT)
    - Ubrogepant (Ubrelvy)
- Must not have ANY of the following:
    - Coronary heart disease
    - Peripheral vascular disease
    - Hypertension
    - Impaired hepatic function
    - Impaired renal function
    - Sepsis
- Must not be on concomitant treatment with a strong CYP3A4 inhibitor (e.g. protease inhibitors, macrolide antibiotics)
- Must not be pregnant

If the above criteria are met, ergotamine tartrate (Ergomar) is approved. (Applies to Advantage Choice Formulary only)

- **Ergotamine-caffeine (Cafergot®)** [2254]

Criteria:

- Must be age 18 years or older
- Must have a diagnosis of migraine headaches
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to ALL of the following:
    - TWO generic triptans (e.g., naratriptan, rizatriptan, sumatriptan)
    - Rimegepant (Nurtec ODT)
    - Ubrogepant (Ubrelvy)
- Must not have ANY of the following:
    - Peripheral vascular disease
    - Coronary heart disease
    - Hypertension
    - Impaired hepatic function
    - Impaired renal function
    - Sepsis
    - Hypersensitivity to ergot alkaloids or caffeine
- Mst not be on concomitant treatment with a strong CYP3A4 inhibitor (e.g., protease inhibitors, macrolide antibiotics)
- Must not be pregnant

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If the above criteria are met, ergotamine-caffeine tablet is approved. (Applies to Advantage Choice Formulary only)

• Ethacrynic acid tablet (Edecrin®) [2255]

Criteria:

- Must have a diagnosis of one of the following:
  - Edema associated with:
    - Congestive heart failure
    - Cirrhosis of the liver
    - Renal disease including nephrotic syndrome
  - Ascites due to malignancy
  - Idiopathic edema
  - Lymphedema
- Must have an adequate trial and failure of ALL of the following loop diuretics:
  - bumetanide
  - furosemide
  - torsemide

If the above criteria are met, ethacrynic acid (Edecrin) is approved. (Applies to Advantage Choice Formulary only)

• Ivermectin 1% cream (Soolantra®) [2256]

Initial Authorization Criteria:

- Must be prescribed by a dermatologist
- Must be age 18 years or older
- Must have a diagnosis of rosacea
- Must have an adequate trial with an inadequate response or significant side effect/toxicity or have a contraindication to THREE of the following:
  - Azelaic Acid (Finacea)
  - Doxycycline
  - Sodium sulfacetamide/sulfur
  - Topical metronidazole
  - Topical tretinoin or adapalene

If the above criteria are met, ivermectin 1% cream (Soolantra) is approved. (Applies to Advantage Choice Formulary only)

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- **Insulin Aspart Protamine and Insulin Aspart vial, Insulin Aspart Protamine and Insulin Aspart Flexpen** [2257]

Criteria:
  - Must have a diagnosis of diabetes mellitus
  - Must have had an adequate trial of Humalog® Mix 75-25 with continued uncontrolled blood sugars despite adherence to an optimized dosing regimen or significant side effects/toxicity or must have a contraindication to Humalog Mix 75-25

If the above criteria are met, Insulin Aspart Protamine and Insulin Aspart is approved. (Applies to Advantage Choice Formulary only)

- **Lemborexant (Dayvigo®)** [2258]

Criteria:
  - Must be 18 years or older
  - Must have a diagnosis of insomnia characterized by difficulty with sleep onset and/or sleep maintenance
  - Must have an adequate trial of THREE FDA-approved or compendia-supported therapies for insomnia (e.g., benzodiazepines, anti-depressants, non-benzodiazepine hypnotics, antihistamines) with an inadequate response or significant side effects/toxicity or must have a contraindication to these therapies
  - Must not have a diagnosis of narcolepsy

If the above criteria are met, lemborexant (Dayvigo) is approved. (Applies to Advantage Choice Formulary only)

- **Mefenamic Acid (Ponstel®)** [2259]

Criteria:
  - Must have one of the following diagnoses:
    - Primary dysmenorrhea
    - Mild to moderate pain
  - For a diagnosis of primary dysmenorrhea:
    - Must have an adequate trial and failure or an inadequate response or significant side effects/toxicity, or a contraindication to of ALL of the following:
      - Hormonal contraceptives
      - THREE other generically available prescription-strength NSAIDs for at least one month
  - For a diagnosis of mild to moderate pain:
    - Must be age 14 years or older

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        - Must be used for a duration of therapy not to exceed 1 week
        - Must have an adequate trial and failure to THREE other generically available prescription-strength NSAIDs for at least one month each with an inadequate response or significant side effects/toxicities unless there is a contraindication to these medications
    - Must not have any of the following:
        - Hypersensitivity to mefenamic acid
        - Asthma, urticaria, or allergic-type reactions after taking aspirin or other NSAIDs
        - Using for the treatment of peri-operative pain in the setting of coronary artery bypass graft (CABG) surgery
        - Acute active ulceration or chronic inflammation of either upper or lower gastrointestinal tract
        - Preexisting renal disease

If the above criteria are met, mefenamic acid (Ponstel) is approved for a duration based upon diagnosis as follows:
- For primary dysmenorrhea: 1 year
- For mild to moderate pain: 1 week

(Applies to Advantage Choice Formulary only)

Reauthorization Criteria:
All prior authorization renewals are reviewed to determine the Medical Necessity for continuation of treatment.
- For primary dysmenorrhea:
    - Authorization may be extended at one-year intervals based upon documentation from the prescriber indicating the member's condition has improved or stabilized based on the prescriber's assessment while on therapy.
- For mild to moderate pain:
    - Authorization may be extended on a case-by-case basis, are subject to the above criteria, and require chart documentation describing the previous response to treatment and clinical rationale for re-treatment to determine the Medical Necessity for continuation of therapy. Authorization may be extended for 1 week of treatment.

• Non-Preferred Generic NSAIDs [2260, 2261, 2262]

- Applies to: fenoprofen calcium 600mg tablet, meclofenamate 50mg capsule, tolmetin 200mg tablet

Criteria:
- Must submit chart documentation showing an adequate trial and failure of THREE other generically-available prescription-strength NSAIDs (e.g.

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ibuprofen, naproxen, meloxicam) for at least one month each with an inadequate response or significant side effects/toxicities unless there is a contraindication to these medications

If the above criteria are met, the non-preferred generic NSAID is approved. (Applies to Advantage Choice Formulary only)

- **Oxymetholone (Anadrol®-50) and oxandrolone (Oxandrin®)** [2263, 2264, 2265, 2266]

Initial Authorization Criteria:

- **For Anadrol-50 (oxymetholone):**
  - Must be prescribed by a hematologist
  - Must have a diagnosis of anemia due to deficient red cell production
    - Must have documentation of a reticulocyte production index < 2 or a corrected/absolute reticulocyte count < 25,000/µL
  - Must provide the specific type of anemia and the rationale for diagnosis of the particular type (such as laboratory tests for serum iron and iron-binding capacity, evaluation of renal and thyroid function, a bone marrow biopsy or aspirate to detect marrow damage or infiltrative disease, and serum ferritin to assess iron stores)
    - Based on type of anemia, must have documentation of an adequate trial of conventional therapy (such as blood transfusion, correction of iron, folic acid, vitamin B12, or pyridoxine deficiency, antibacterial therapy, or corticosteroids) with an inadequate response or significant side effects /toxicity or must have a contraindication to these therapies.
    - Must be using oxymetholone in adjunct to conventional therapy
  - Must not have nephrosis or the nephrotic phase of nephritis
  - Must not have severe hepatic dysfunction
  - For males:
    - Must not have a carcinoma of the prostate or breast
  - For females:
    - Must not have a carcinoma of the breast with hypercalcemia
    - Must have a confirmed negative pregnancy test

- **For Oxandrin (oxandrolone):**
  - Must be prescribed by a specialist within the area for the member's condition that is leading to significant weight loss
  - Must meet one of the following:
    - Must be to promote weight gain after significant weight loss following extensive surgery, chronic infections, or severe

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trauma, AND must provide documentation of 10% unintentional loss of weight over 6 months
- Must be to offset the protein catabolism associated with prolonged administration of corticosteroids AND must provide rationale for necessity to continue chronic use of corticosteroids, including documentation of an adequate trial of all non-steroidal treatment options for the member's condition with an inadequate response or significant side effects /toxicity or must have a contraindication to these therapies
- Must be for bone pain accompanying osteoporosis AND must be currently utilizing at least one treatment for osteoporosis with a recent bone mineral density test showing a T-score < -2.5 AND must provide documentation of an adequate trial of the following: acetaminophen, two prescription-strength non-steroidal anti-inflammatory agents, two opioid medications for acute pain, and exercise or physical therapy

- Must be used as adjunctive therapy
- Must not have nephrosis or the nephrotic phase of nephritis
- Must not have severe hepatic dysfunction
- For males:
  - Must not have a carcinoma of the prostate or breast
- For females:
  - Must not have a carcinoma of the breast with hypercalcemia
  - Must have a confirmed negative pregnancy test

If the above criteria are met, the requested anabolic steroid product is approved for 3 months. (Applies to Advantage Choice Formulary only)

Reauthorization Criteria:
All prior authorization renewals are reviewed on a case-by-case basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended based upon chart documentation from the prescriber that the member's condition has improved/stabilized based upon the prescriber's assessment while on therapy.

• Penciclovir (Denavir®) 1% cream [2267, 2268, 2269, 2270, 2271, 2272]

Criteria:
- Must have a diagnosis of recurrent herpes labialis (cold sores)
- Must be age 12 years or older
- Must submit chart documentation showing adequate trial and failure of ALL of the following medications with an inadequate response or significant side effects/toxicity or must have a contraindication to these therapies:
  - Abreva® cream (available over-the counter)
  - acyclovir 5% ointment

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        - one oral anti-viral therapy (e.g., acyclovir, valacyclovir, famciclovir)

If the above criteria are met, penciclovir (Denavir) is approved for 1 fill. (Applies to Advantage Choice Formulary only)

Reauthorization Criteria:
All prior authorization renewals are reviewed on a case-by-case basis to determine the Medical Necessity for continuation of therapy. Authorization may be extended for an additional fill based upon documentation from the prescriber that the member's condition necessitates additional therapy.

- **Topical Anti-Fungal Agents** 2273, 2274, 2275, 2276, 2277, 2278, 2279, 2280, 2281
    - Applies to: luliconazole 1% cream, Mentax® 1% cream (butenafine), oxiconazole 1% cream, and naftifine 1% cream

Initial Authorization Criteria:
- Must have a diagnosis of one of the following:
    - Tinea pedis
    - Tinea cruris
    - Tinea corporis
    - Tinea (pityriasis) versicolor
- Must have had an adequate trial and failure of THREE other generically-available prescription-strength topical anti-fungal medications (i.e. ketoconazole, econazole, ciclopirox) AND an adequate trial and failure of TWO other generically-available over-the-counter topical anti-fungal medications (i.e. clotrimazole, miconazole, tolnaftate) with an inadequate response or significant side effects/toxicities or must have a contraindication to these medications.

If the above criteria are met, the requested anti-fungal agent is approved for 1 treatment course based on the agent of choice. (Applies to Advantage Choice Formulary only)

Reauthorization Criteria:
All prior authorization renewals are reviewed on a case by case basis, are subject to the above criteria, and require chart documentation to determine the Medical Necessity for continuation of therapy including the previous response to treatment and clinical rationale for retreatment to determine the Medical Necessity for continuation of therapy. Authorization may be extended for 1 additional treatment course based upon documentation from the prescriber that the member continues to benefit from therapy.

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